

Fortis Healthcare Ltd. (FORTIS)

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Call: Aug 2022

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The National Stock Exchange of India Ltd.

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Sub: Transcript of Investors / Analysts' meet under Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015

Dear Madam / Sir,

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed herewith the Transcript of the investors / analysts meet held on August 8 , 2022 to discuss the Company's un-audited financial results for the quarter ended June 30 , 2022 . Further, the said transcript is also available on the website of the Company i.e. <https://www.fortishealthcare.com/investors> .

This is for your kind information and record .

Thanking you,

Yours Faithfully

For Fortis Healthcare Limited

Murlee Manohar Jain

Company Secretary & Compliance Officer

M. No. F9598

Encl: a/a

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"Fortis Healthcare Limited

Q1 FY23 Earnings Conference Call"

August 8 , 2022

MANAGEMENT DR. ASHUTOSH RAGHUVANSHI – MANAGING DIRECTOR
AND CHIEF EXECUTIVE OFFICER , FORTIS
HEALTHCARE LIMITED

MR. VIVEK GOYAL – CHIEF FINANCIAL OFFICER ,
FORTIS HEALTHCARE LIMITED

MR. ANAND K – CHIEF EXECUTIVE OFFICER , SRL
LIMITED

MR. MANGESH SHIRODKAR – CHIEF FINANCIAL
OFFICER , SRL LIMITED

MR. ANURAG KALRA – SENIOR VICE PRESIDENT ,
INVESTOR RELATIONS , FORTIS HEALTHCARE LIMITED

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Moderator : Ladies and gentlemen, good day and welcome to the Q1 FY23 Earnings Conference Call of Fortis Healthcare Limited. As a reminder, all participant lines will be in the listen only mode, and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing "8" then "0" on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Anurag Kalra – Senior Vice President, Investor Relations at Fortis Healthcare Limited. Thank you, and over to you, sir.

Anurag Kalra : Thank you Steven. A very good morning and good afternoon, ladies and gentlemen, and welcome to Fortis Healthcare's quarter 1 FY 23 Earnings Call. I hope all of you have got a chance to go through the Presentation and our Press Release on the earnings that we had circulated on Friday evening.

The call today is being chaired by our Managing Director and CEO – Dr. Ashutosh Raghuvanshi. With him, we have our Chief Financial Officer – Mr. Vivek Goyal. On the SRL side, we have Mr. Anand – the CEO of SRL. And with him is Mangesh Shirodkar – our CFO of the SRL business.

We will start the presentation with some opening comments by Dr. Raghuvanshi, on the earnings gone by post which Anand will take you through certain key highlights of the diagnostics business. And then we shall open the floor for question and answers.

Thank you. Over to Dr. Raghuvanshi.

Ashutosh Raghuvanshi : Thank you, Anura. Very good day, everyone. And thank you for your time to join us on our Q1 financial year '23 Earnings Call . I hope all of you are safe and well. I shall come straight to the performance of the quarter. And then Anand will take you through the highlights of diagnostic business.

We've had a good start to the year. Our consolidated revenues have increased 5.5% versus Q1 of financial year '22 to Rs. 1,488 crore. Within this, our hospital business has done exceedingly well with a robust growth in revenue of 18.5% versus Q1 of financial year '22 and 14.6% versus quarter 4 of financial year '22. The consolidated revenues were impacted by the diagnostic business. And if you recall, we had clearly articulated this in our last earnings call, this business as expected, has seen a decline in gross revenue of about 25% versus the corresponding quarter and 11% versus the trailing quarter, led by a significant fall in COVID test volumes. There was a sizable revenue contribution from COVID, both in Q1 and Q4 of financial year '22, which was not there in Q1 of financial year '23. At the consolidated level, the company's revenue has grown by 5.5%.

On the profitability, our hospitals business EBITDA stands at Rs. 208 crore, an increase of 39% and reflecting margins of 17.4% versus 14.9% in Q1 of financial year '22 and 13.8% in Q4 of Fortis Healthcare Limited

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financial year '22. Adjusting for the losses with respect to the Arcot Road facility in Chennai, EBITDA margins for the quarter stood at 18.3%. The improvement in margins has been led by a healthy performance on all the key hospital operating metrics, which I shall speak about a bit later.

On the diagnostic segment, commensurate with the decline in revenues due to the COVID impact, margins were lower with EBITDA for the quarter at Rs. 64 crore, this reflected margins basis gross revenue at 19.3% for the quarter versus 30.6% in Q1 of financial year '22 and 22.5% in Q4 of financial year '22. Combining both the hospitals and diagnostic business, our consolidated EBITDA for the quarter was Rs. 272 crore, marginally lower as compared to Rs. 283 crore in Q1 of financial year '22. Consolidated EBITDA margin was at 18.3% versus 20.1% in Q1 of financial year '22 and better than 16.5% in Q4 of financial year '22. It is per

tribut to

highlight that the contribution of hospital EBITDA increased to 76% in Q1 financial year '23 versus 53% in Q1 of financial year '22 and 63% in Q4 financial year '22, signifying the strength in hospital business earnings and largely balancing out the decline in earnings from the diagnostic business. At the PAT level, we reported a profit after tax prior to exceptional items of Rs. 134 crore compared to Rs. 124 crore in Q1 of financial year '22. This is an 8% growth versus the corresponding quarter and a robust 55% growth versus Q4 of financial year '22.

Coming to the qualitative aspects of hospital business :

We have made good progress on all fronts with higher footfalls and more demand for elective procedures for surgical revenue contributing to overall revenues reached 61%, which is an all-time high. This compares to 41% in Q1 of financial year '22 and 57% in Q4 of financial year '22. While the overall occupancy in the quarter was similar at 65% versus Q1 of financial year '22, the occupancy mix changes favorably due to the higher non-COVID IPB numbers and this was witnessed across almost all our key specialties. This resulted in a very healthy ARPOB at Rs. 1.96 crore, a 21% increase over the corresponding quarter and even better by 4% for quarter 4 of financial year '22. Our international patient revenue have also seen a good traction for the quarter. International Patient revenues were at Rs. 89 crore, a growth of 126% over the corresponding quarter and 36% over the trailing quarter. International Patient revenue at the end of quarter 1 of financial year '23 contributed 7.5% to the total hospital revenues.

It is also important to highlight that some of our key underperforming facilities such as Sirohi and Jaipur have witnessed higher revenues and better profitability versus both the trailing and corresponding quarter. We have also progressed well on our brownfield expansion plans and by small numbers. We have added a total of 55 beds primarily in Fortis Mulund where we have commissioned an additional floor and have also added some beds in FMRI Gurgaon. In line with our intent to expand our operational bed capacity by approximately 1,500 beds in the next few years, our existing facilities like FMRI, Mohali, Noida and Shalimar Bagh are slated to add in excess of 200 beds each. This I believe will be funded through internal accruals and I have already chalked out plans to move rapidly on this .

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In addition, we have also commissioned the state of art 12 -bedded oncology daycare center providing chemotherapy services in a prominent Delhi location to further expand our reach and presence and complementing our existing oncology service offerings in several of our hospitals in Delhi NCR. Depending on the success of this model, we would replicate this at a scale. Commensurate with our medical progress, we continue to attract high quality clinical talents. We have onboarded clinicians in specialty of urology, nephrology and transplant and rheumatology during this quarter.

Revenues from digital channels such as website, apps and online campaigns have grown 74% over Q1 of financial year '22 and contributed approximately 23% of the overall revenues. All in all, I am pleased with the way our hospital business is tracking and remain hopeful of continuing this momentum. On the diagnostic side, like I had mentioned at the start, business was impacted due to decline in COVID volumes, non-COVID revenues however have grown 29% versus Q1 of financial year '22 and 8% versus quarter 4 of financial year '22. This is on account of increase in the non-COVID test volumes as the pandemic recedes. I would like to highlight that competitive pressure in the diagnostic business remain and hence the operating environment would be challenging in short term. At the same time, our focus on channel expansion, which is our core

refraction center network, our specialized test portfolio and our customer touch points to lab ratio all continue to be further strengthened. I will let Anand speak on that in detail after my comments.

On the balance sheet, we remain quite healthy with a net debt to EBITDA of 0.54x. This is similar to Q4 of financial year '22. Our net debt stands at Rs. 585 crore as on 30th of June 2022. A stronger balance sheet also allows us to evaluate inorganic growth opportunities in line with our cluster strategy approach. In tandem, such opportunities would also help us gain from potential network synergies in our key geographies of Delhi NCR, Maharashtra, Bangalore and Kolkata. As I conclude my comments, I want to reiterate to all of you that we remain committed to becoming a best patient focused organization, one that constantly strives to deliver excellent clinical care and best in class service offering to our patients. This is at the center of everything we do. And I believe this is what will eventually reflect in stronger organization and a progressive improvement in the performance of the company. Thank you. And with that, I would like to hand over to Anand for comments on the diagnostic business.

Anand K. : Thank you, Dr. Raghuvanshi. A very good morning to everyone on the call. Thank you for joining us today. On behalf of SRL Diagnostics, I warmly welcome you all to Q1 FY 2023 results conference call. I hope all of you and your families are safe and in good health. I want to start off by thanking our employees, customers and partners for the trust and loyalty during the

testing time. During the quarter, we reported a revenue of Rs. 332 crore with 96% of our revenue coming from non-COVID testing. Our non-COVID revenue numbers for Q1 FY23, that is excluding COVID and COVID alike tests, stands at Rs. 312 crore against Rs. 242 crore in Q1 FY22, registering a growth of 29%. COVID testing revenues contribution in Q1 FY23 is 4% compared to 26% in Q1 of FY22. Revenue contribution from specialized non-COVID tests have gone to 36% in Q1 of FY23 compared to 20% in Q1 of FY22. Our EBITDA stands at Rs. 64

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crore with a margin of 19.3% for Q1 FY23, compared to a margin of 13.6% in Q1 of FY22. We are confident that non-COVID testing on our specialized categories will aid in growth over the rest of the year. As far as B2C, B2B mix is currently 55/45 in Q1 of FY23 compared to 57/43 in Q1 of FY22. During this quarter, SRL conducted approximately 9.96 million tests, a degrowth of 6% compared to Q1 FY22 and a decline of 7% versus the trailing quarter. This is primarily due to the drop in COVID testing volumes. We serviced 4.3 million patients during this period. Keeping in line with our network expansion strategy, especially in the priority cities, we added 243 new customer touch points in Q1 FY23. We have taken a number of initiatives to improve our customer experience including the launch of our new WhatsApp chatbot and live phlebotomist tracking feature. On the people front, we completed more than 1,400 mandates of training in Q1 of FY23. In the last few years, SRL has progressed to make many tailor-made competency enhancement programs. Training, learning and development continues to be one of our priority areas. In this quarter, we have added 50 plus new tests and technologies to strengthen our testing portfolio. Our R&D team works on care development, technology evaluation and validation studies. Currently, we are undertaking a number of new initiatives, especially in the area of genomics, proteomics and pharmacogenomics that will enable us to be ready for the next big shift in diagnostics. In this year, we will particularly look at co-marketing initiatives, clinical trial studies and contract validation for kit manufacturers and technology providers, co-development of new biomarkers as one of our key areas of growth.

With. We are progressing well on

our project with Microsoft to develop an AI algorithm for the diagnosis of breast pathologies. This would be a breakthrough in digital pathology to usher in new AI-driven tools in histopathology. Our focus on genomics, next generation diagnostics, along with our work in digital pathology and specialized testing category will help us differentiate ourselves and also enable us to be future ready. SRL has been at the forefront of embracing change and quickly adapting to changing customer expectations. Over the years, we will be focused on improving our customer experience, strengthening our test portfolio and going deeper in our priority markets. Thank you for your attention. I would like to now hand over the call to Mr. Anurag Kalra, our Head of Investor Relations.

Anurag Kalra : Thank you, Anand. Ladies and gentlemen, we will now open the floor for question answers. May I please the question the moderator.

Moderator : We will now begin the question-and-answer session. Ladies and gentlemen, we will wait for a moment while question queue assembles. The first question is from the line of Amit Goela from Rare Enterprises. Please go ahead.

Amit Goela : This is a very good performance, and congratulations to all of you. Sir, I've got a couple of questions on the hospital side. Sir, when do you think Arcot will breakeven?

Ashutosh Raghuvanshi : Amit ji, Arcot, the initial progress was a little slow. Normally, we would have expected in 18 months for it to mature. But the first year somehow was impacted greatly by the COVID wave.

However, now that traction is happening, and we expect that it will take another 12 months before it becomes positive.

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Amit Goela : 12 months, like so in the first quarter next year, it should be positive, sir?

Ashutosh Raghuvanshi : That is the hope.

Amit Goela : And sir, in that one particular slide where you've given the hospital margin metrics, almost 30% of your revenue is below 15% EBITDA. And 22% of your revenue is below 10% EBITDA. So, when do you see this part moving up because it can make a big difference to the margins then sir?

Vivek Goyal : Amit ji, if I can take this question, Vivek Goyal. Good morning. Your observation is absolutely correct. So, there is some hospitals where the EBITDA margins are on the lower category and that is dragging overall EBITDA margin below 20%, which is the immediate target for us. And we are working on those hospitals. As Dr. Raghuvanshi covered in the initial part of his address, 2 of those hospitals, FEHI as well as Jaipur have shown very good traction and we are quite hopeful that both these hospitals will very soon come out from this category, means will move

up. And there are a couple of hospitals in Chennai including this Arcot Road, which is a Iso impacting this and Malar also is performing below par. So, we are working on that. And hopefully, this list will narrow down and some of these hospitals will move towards the next category most probably in the next year.

Amit Goela : And sir, you're looking at 20% margin in the medium term, so you're looking at this year or next year.

Vivek Goyal : No, sir. This year it will be difficult, but we are keeping a target for ourselves for next 2 years, we should be reaching there.

Amit Goela : One last question . Did I hear it right, sir, you said you'll be doing a brownfield expansion of 1,500 hospital beds over the next couple of years?

Vivek Goyal : Not couple of years, sir. It will be over a period of for next 4 years because we have already identified the project, lands are available, we have applied for the approval for the building plan and all those stuff. So, this typically takes 2 to 2.5 years' time for construction post approval, and then some stabilization period, equipment ordering and all. So, we are hopeful that in 4 years' time , majority of th

is expansion will kick in.

Moderator : Thank you. The next question is from the line of Shyam Srinivasan from Goldman Sachs. Please go ahead.

Shyam Srinivasan : Just the first one on the hospital business. ARPOB dynamics again remain robust. I'm looking at not necessarily Y-o-Y, but if I do even 3 years 1Q '20, it's like 8% CAGR through the time.

So, just Dr. Ashutosh, if you can help us understand, I think you talked about surgical, non - surgical mix. But what's driving this? Is there an element of price increase that is there? Or is it peer mix rationalization? And what's the prognosis or the outlook for this?

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Ashutosh Raghuvanshi : Shyam, it's more of case mix rather than payer mix or any other pricing intervention. We have taken minimal price increase this year, so that there is not a significant component here or pricing that is very small. The main kick has come from the increase in the procedures and surgical revenue being higher. Now, our estimate is that the ratio will sort of become slightly tempered over some period of time, because some of this is driven by the pent-up demand is our feeling, but we will have to test that hypothesis over the next few quarters. But having said that, we have certain levers still available to us. As I said that we have not taken major pricing revisions. So, we are rationalizing our pricing. We're not necessarily increasing in all the cases, but we are rationalizing to see that it remains competitive, but at the same time is competitive with the rest of the market. And I think by using all those levers, we are fairly certain that the ARPOB trend will continue to improve or at least remain sustainable at the current levels.

Shyam Srinivasan : I'm just understanding, medical inflation, consumer inflation is high. So, what is the hesitancy to increase prices? Is it optically wrong? Maybe it'll also a philosophical question, why can't we raise prices?

Ashutosh Raghuvanshi : So, you're absolutely right. If one looks at it very clinically and compares the inflation and covers inflationary costs every year, I think that would be a natural response. Having said that, I think it is imperative on any business not only healthcare, to build in efficiency over a period of time and reduce the cost and hence keep the prices under control. So, if every industry would just be simply passing on, then I guess there is no end to inflation. So, having said that, we are very conscious of the way healthcare is looked at. And healthcare industry is always under the glare of media and everybody else and there is a certain degree of social responsibility on our industry. We are able to improve our performance by doing several other measures. So, there is no hesitancy. However, there is a sense of responsibility with which we are working. And that's the reason why we are little conservative as our approach. Having said that, we are very clear about one thing that our immediate target, as Vivek said just now is to go to at least 20% margin at consolidated level and in some of the clusters as you would see that we are already above that in almost 60% of our revenue. But that idea is to take at least 80% of the revenue within that range by the next year. So, profitability is extremely important for us, but at the same time we want to be balanced in our approach.

Vivek Goyal : Shyam, just to add what Dr. Raghuvanshi has said, as you know, some of our payees , it is difficult to increase the price, but on cash payers, we are taking measures to increase the price to the extent possible depending upon competition and other things. So, cash payment is not like we are not increasing price at all, but it is in line with competition.

Shyam Srinivasan : Any quantum there on cash, Vivek sir?

Vivek Goyal : Yes, we are at around 4%.

Shyam Srinivasan : 4% increase on cash. Last 2 questions from me,

just on utilization and the pent-up demand point

that Dr. Ashutosh had brought up. Dr. Ashutosh, when you did the call in May, I don't think we

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got the sense that we were going to end the quarter with 65%. At least I didn't infer just looking at whatever I think trends you had talked about April and May, so what has changed? In I think June looks like much higher than 65%, right, must be 67%, 69%, I'm just guessing. And is that sustainable utilizations now possible for us to reach the 70% mark that we have talked about?

Ashutosh Raghuvanshi : Yes. We are seeing better occupancy levels, right. In the month of April and May, the occupancy levels were not that high, but June definitely was higher and that trend is continuing. And I think that going forward that will continue. And also, I would like to remind you that this is on a slightly higher base, though about 60 odd beds have been added which is not a very large number. But even with those 60 odd beds added, the occupancy remains at the level of about 68%, 69% at the moment.

Shyam Srinivasan : Dr. Ashutosh, just sorry to persist. When I add the number of beds in the hospital metrics, it's showing Q-o-Q decline. Am I missing beds there or that's just a subset of the beds.

Ashutosh Raghuvanshi : No, these beds you will see in the coming quarter.

Shyam Srinivasan : The 60 beds you have added, you're saying, okay.

Ashutosh Raghuvanshi : Yes.

Shyam Srinivasan : And my last question, sorry, I'll try and keep it very brief, is on diagnostic services. Anand, again just going to the forward path, how should we look at growth and margins? We've had an earlier margin guidance or a directional sense of 23% to 25%, we're well below that for the quarter. But what are the ways we can build it back? And what's the outlook for non -COVID growth for the remainder of the 9 month? Thank you.

Anand K. : So, as you know that we were having operating leverage because of COVID during last year and we found that during that time the revenues went up. We were able to deliver higher pro fitability margins. So, even though we have given guidance earlier on 22% to 23% kind of margins, this quarter has been lower and we expect that it will keep going up over the quarters because we also need to increase our revenues, which will give us the same operating leverage, as well as since we are moving from a COVID quarter of Q4, since the trailing quarter had some COVID and we are moving forward. So, there are also some costs related to COVID, which will be there in these quarters, so which will trail off over a period of time. So, I think all these factors will really help in growing the profitability. But the growth on the non -COVID looks quite promising for us.

Moderator : The next question is from the line of Sarvesh Gupta from Maximal Capital. Please go ahead.

Sarvesh Gupta : Sir, most of my questions have been answered. Just 2 versions. One is on the court case, if you can give us an update on what's happening. And secondly, are there any incremental thoughts

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with respect to the structuring of SRL stake that we have in terms of either demerger or sale of stake or acquisition of the remaining stake?

Ashutosh Raghuvanshi : Yes, Sarvesh. As far as the legal case is concerned, there was a mention made at the Supreme Court and the judges made a comment that within a couple of weeks, we should hear something.

So, this comment was made about 10 days back. So, we expect very soon a resolution of the case. So, that should be within the month of August is our expectation and that's what we're hearing from our lawyers as well. As far as SRL, I will request Vivek to address that.

Vivek Goyal : So, SRL stake, as I mentioned in the last call also, we will be exploring the different options, probably this may not be the right t

ime because of the volatility in the market. But we are exploring different options and maybe come back with some plan by the end of this year.

Sarvesh Gupta : Are we also considering purchase of the remaining stake as one of the possible options for us?

Vivek Goyal : Right now, there is nothing in pipeline, but that option can also be explored actually.

Moderator : Thank you. The next question is from the line of Nitin Agarwal from DAM Capital. Please go ahead.

Nitin Agarwal: Dr. Raghuvanshi, just going back to the slide on the margin matrix, which is there in the presentation, two things; one is in the bottom layer, which is the five hospitals which are below 10% EBITDA margins, can you just help us understand which are these hospitals which are there below 10%?

Vivek Goyal: So, there are hospitals like Jaipur, Vashi – Mumbai, Malar, Sacred Heart and Arcot Road. These are the hospitals which are in that category.

Nitin Agarwal: In these hospitals, our occupancy is 50%, Arcot Road is understandable, because it's a new hospital, I presume the other hospitals are all reasonably mature hospitals. So, is there a structural problem in these other four hospitals that we have occupancy on an average for the group around 50%?

Vivek Goyal: Each hospital is having its unique problem, some are struggling for the occupancy, as you rightly mentioned, like Vashi hospital in Mumbai, that is struggling on occupancy, similar is the case with Malar, where the occupancy we are not able to build up either because of the infrastructure issue or because of the hospitals for example, in Vashi we could not ramp up post -COVID. In COVID, this hospital was doing quite well. On the other hand, occupancy has gone up to the normal level of 70% -plus level, however, the ARPOB there is quite low, and that may be attributed to a lot of government schemes are there and that is actually dragging the ARPOB lower. And we are working on adding some more stability, therefore, which will improve maybe the profitability margin there. The other hospital like Arcot Road is a new hospital. So, there the occupancy is less.

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Nitin Agarwal: Vivek, if we look at the two clusters which are there, 20% to 25% and 10% to 15%, one observation there was occupancy in both the clusters are similar. The ARPOB is higher in the 10% to 15% cluster, but the margins are a lot lower. So, can you just help us again understand that a little better, what is the dynamic here?

Vivek Goyal: Hospital like Amritsar, Punjab hospital, where we have more of 18% or more there because of the geography and plus Noida also come in that category. Faridabad, we are going for expansion and renovation work is going on, and that is affecting actually the patient flow and things like that. Anandapur is another hospital in that category, which was supposed to be in the upper category, but this quarter was not very good for Anandapur. It has started recovering, I mean, June, July was a good month for them, and we are hopeful that this will also move up, and main reason was for the occupancy growth.

Nitin Agarwal: In which group, FEHI will be now?

Vivek Goyal: FEHI is in the third group.

Dr. Ashutosh Raghuvanshi: FEHI has come up from less than 10%, Nitin, it's gone one up. So, FEHI and C H Road, which were at the bottom rung, have now moved one level up

Nitin Agarwal: Is it sustainable given what we've seen so far?

Vivek Goyal: Yes, we can say that because it is consistently improving in last three quarters if you see, FEHI is consistently improving and last quarter it crossed 10%, and we are quite hopeful that it will continue to move upward only.

Nitin Agarwal: Vivek , you mentioned these costs are prior to the corporate costs. Can you give us a sense on what are annualized corporate cost in the hospital business?

Vivek Goyal: It is around 3% of the overall re

venue. I'm talking hospital business only.

Nitin Agarwal: Is there a scope for optimization around here as we sort of scale up going forward?

Vivek Goyal: So, some measures we have taken post -tariff coming in and Dr. Raghuvanshi joining and the result of which has already come. But there is a big cost sitting in the form of legal costs, which is to deal with the various legacy issues the company's facing. And as these issues will start settling, I am quite hopeful that these corporate cost will also be under control. Plus, entity structure also we want to simplify it post Supreme Court order, because that should also reduce the corporate cost in my view. So, there is a scope, but we are restrained because of Supreme Court to some extent.

Moderator: The next question is from the line of Neha Manpuria from Bank of America. Please go ahead.

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Neha Manpuria: Just extending on the hospital metrics, if I were to look at the hospitals below the 15% margin levels, is it fair to assume that some of this should see an improvement in margin, let's say in the next year or would it take longer for these hospitals to turn around and start contributing meaningfully to margin in a positive way I mean?

Vivek Goyal: I will not be able to give you exact timeline, but all the hospitals are showing very good improvement. And some of the plans which we are having is having a long-term plan, in the sense where we are trying to add some facility, for example, in Jaipur, suppose we want to add some biggest facility there just to get better margins as well as the patient flow. So, that may require some investment for equipment's ordering time, doctor having. So, those type of things sometimes take time. But all hospitals shown move according to my view.

Neha Manpuria: Sir, the reason I'm asking this question is, given our margin guidance of 20%, we're already at 17.5%, there is improvement that you've indicated in the existing hospitals. And if the existing hospitals below 15% move up, isn't the 20% conservative, particularly where mature hospitals for some of our peers are operating at? So, how should we build the bridge between what margins we should be operating at and where we are currently?

Vivek Goyal: No, you're absolutely right. So, there is always aspiration to move upward. But we will like to

move a step-by-step, and our initial step when three years back was to cross 15%, which we have now done, and the next benchmark, we're keeping ourselves for 20%, and then we'll see how much more we can go. And I am 100% sure, looking at the potential in the company and the hospitals and the overall talent pool we are having, we are quite hopeful to move upward, 20%.

Neha Manpuria: My last question is on expansion. The 1,500 beds Brownfield expansion, I think we've mentioned that for some time now. But, given the assets available in the market, isn't entering new markets via, let's say, business development, or inorganic acquisition, part of the plan, we have a decent enough balance sheet to be able to do that, or our focus is on expanding and strengthening the existing hospitals that we have, just wanted to understand capital allocation a little bit more?

Dr. Ashutosh Raghuvanshi: You're absolutely right, with the strong balance sheet, we definitely aspire to look at beyond our existing clusters. However, the philosophy of having a focused cluster if we have to go to a new geography domains, so which means standalone hospitals are going to be evaluated in the given clusters, not in completely new geographies. But, if there was a cluster available, or there was even a couple of hospitals available in a close geographical area, we would certainly look at those. So, definitely, we have sort of activated our evaluation of the inorganic opportunities as well, and then we will be actively seeking those. But the priority to

remain in a clustered

approach as we grow. So, very remote geographies where we have no presence, even if there was a standalone asset, we would rather avoid that.

Neha Manpuria: Sir, from a cluster priority would it be East India over Mumbai, over NCR, just wanted to understand, what would be your priority when you're looking at acquisitions?

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Dr. Ashutosh Raghuvanshi: Order of priority in our given clusters, NCR and Mumbai remain high priority and as well as Bangalore. So, these three are important and then followed by Calcutta and Punjab, so this is the order of priority. But we are open to new geographies if we are getting scale.

Moderator: The next question is from the line of Sabyasachi Mukerji from Centrum PMS. Please go ahead.

Sabyasachi Mukerji: First, I need one clarification. In your Q1 FY'22 presentation, the non-COVID revenue contribution in the diagnostics business was stated as 74%, in Q1 FY'23, it is stated as 55%.

Where is the disconnect here?

Anand K: This is Anand here. So, the COVID revenue that we have talked about in the previous year same quarter was pure RT PCR tests alone. But this time, we have also included the COVID allied test, the CRP, D-dimer, because those steps were having a very significant contribution in Q1 of last year, because that was a second wave of COVID at that time. So, during that time, we have not looked at it that way. So, now, after the calculations, we have identified that it is about 19% of the revenue, so it is 26% plus 19%, so you have 45% is a total COVID contributions for Q1 of FY'22.

Sabyasachi Mukerji: Second thing on the margin improvement trajectory in the diagnostics business, I'm not so clear of how will you kind of go back to the earlier guidance of 22% to 23% given Q1 had 10% YoY growth on the non-COVID test volumes numbers, but still the margins are pretty low. What is the kind of actions you're going to take to kind of drive the margins from here?

Anand K: The margins, primarily because of operating leverage that we got last time, that we will continue to focus on revenue growth. So, I think on one side, we will keep a tight control on the costs, but at the same time, revenue growth is key since we have capacity utilization, so, we have capacities which can be utilized further. So, what we have seen is, we will be able to handle much more than what we are currently doing in our centers. Since the markets are also expanding, and there is some competitive intensity at this point of time, so, we are seeing these kind of changes, but overall I feel that over the next few quarters, we will see this improvement happening.

Sabyasachi Mukerji: What will drive the test volumes, I'm assuming you're going aggressive with the expansion, the test volumes are increasing, so, you will do some competitive pricing?

Anand K: No, actually, we are increasing our network also to see almost to the extent of close to 100 centers per month we are adding to our franchisee network. So, this kind of network expansion phase over a period of time centers start maturing, so, they start delivering more numbers, and since these centers are asset-light, and they deliver directly into an existing lab facility, so, they will contribute more towards margins over a period of time. So, if you see in the last one year, we have added close to about 800 centers. So, that will contribute over the next one or two years to improve the margins.

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Sabyasachi Mukerji: My next question is on the hospital business. If I look at Q2 of last year and even Q3 of last year, we had a revenue run rate of nearly Rs.1,100 crores thereabout, we have clocked Rs.1,200 crores

in Q1 of this fiscal, still the margins are lower. Have the losses increased over this period of time or what is that has changed?

Vivek Goyal: Q2 a

nd Q3 is not exactly comparable because some part of COVID was there. But having said that, we are on the margin improvement trajectory as I have mentioned earlier to the questions also. So, no losses has increased, but, the margin improvement will be gradual, it will not be steep.

Sabyasachi Mukerji: But Q2 and Q3 of last year had COVID revenues, I understand that. The COVID I believe at a lower margin rather than a non-COVID surgeries or operations. Is that correct?

Vivek Goyal: You're right. So, non-surgical business was less, but at the same time, the volume was higher because the beds were occupied because of the COVID patients. And plus, in COVID-related patients the cost was not low, although the gross configuration one can say around 10% lower than the non-COVID business, but volume was compensated more than during that quarter, plus seasonality impact as well. So, first quarter generally remain very low for us, but later quarters generally pick up.

Sabyasachi Mukerji: So, you have been guiding that we'll reach hospital margins of 20% or more in the next couple of years. What is the kind of margins that you expect in this fiscal -- shall we touch 18% on hospital business?

Vivek Goyal: We may be getting 18% depending upon how the circumstances pan out. We are trending toward that level as you can see from our first quarter number.

Sabyasachi Mukerji: This is on reported basis, right, and not on adjusted for the losses of Arcot?

Vivek Goyal: No, it is after Arcot Road.

Moderator: The next question is from the line of Naushad Chaudhary from Aditya Birla AMC. Please go ahead.

Naushad Chaudhary: Some clarifications I have. Firstly, on the legal cost, if you can quantify it, it will be better, how much it is quarterly, if not is it at the similar run rate which it was in last two, three years or has it gone down?

Vivek Goyal: Legal costs had started coming down. It is difficult to quantify quarter-on-quarter because as you know legal costs generally depending upon when the hearing happen and when the things kick in. But as our cases are coming down, like FEHL is more or less settled, and Supreme Court hearing also settled more or less, so our legal costs start seeing a declining trend.

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Naushad Chaudhary: Should we see a meaningful decline or how should we say it in this financial year?

Vivek Goyal: As I said, it's very difficult to comment because a lot of cases are going on and how each case will pan out, it is very difficult to predict, but rationally we are coming down in legal costs.

Naushad Chaudhary: Secondly, just a clarification. I couldn't hear this. You said there are some COVID-led cost, which should come in 2Q. I don't know if I've heard it correctly. What was this we were indicating sir?

Dr. Ashutosh Raghuvanshi: On the diagnostic side, some of the costs which have built up because COVID volumes will come down now. But that will happen gradually over the next two quarters.

Naushad Chaudhary: Lastly, on the hospital margin, sir, you have been sharing the steps which you are taking the mix improvement and turning around some low margin hospitals to good positive EBITDA. But can you give us the specific steps which we are taking to achieve our desired margin because mix is something one which is I am able to understand, otherwise, the low margin hospital and occupancy, these two things I am still unable to understand it, how will it make much dent because at any given point of time, we will have a low margin hospital if I'm correct in understanding your business, can explain this?

Vivek Goyal: As you know, the hospital business margins depend on one is the occupancy as you rightly said, and some of the hospitals in our hospitals network are operating below the average occupancy which other hospitals are achieving. So, one is the improvement in the occupancy by bet

ter

empanelment, by attracting more patients, so, that is one thing which we are trying to achieve. And secondly, the surgical mix also we are planning to improve further where we are adding lot of clinical talent in our hospital network, adding more facility so that we can get the quality revenue, plus the payer mix also we are working and it has started showing results also where the TPA and Care including now the international business is going up which add to the better ARPOB and EBITDA Margin.

Naushad Chaudhary: What should be the hospital occupancy rate,

Vivek Goyal: So, any hospital which is operating below 70% occupancy level, there is scope for improvement, let me put this way.

Naushad Chaudhary: How much can we take, 70%?

Vivek Goyal: In peak, it may go up to 80%, 85% also. Some of the hospitals still operating at 80%, 75% -plus

occupancy.

Naushad Chaudhary: This is on an individual level, right, but on a blended basis if we see the hospital businesses, how should we see it, Can t he entire portfolio go beyond 70% or 70% is the limit?

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Vivek Goyal: Yes, 75% is the level where the hospital start f eeling the shortage of beds.

Moderator: The next question is from the line of Sanjay Shah from KSA Securities. Please go ahead.

Sanjay Shah: Sir, can you highlight upon the change in brand name, is there any update on that side?

Dr. Ashutosh Raghuvanshi: That is subject to the legal case getting settled and whatever direction we get from the hon'ble Supreme Court. So, we have to wait for that.

Sanjay Shah: Sir, can you highlight upon the development on medical tourism side, is there any growth seen on that side?

Dr. Ashutosh Raghuvanshi: So, those numbers are coming to almost normal, as we said that about 7.5% of the revenue came

from internati onal, and with an increase base, so that is almost near to the pre -pandemic levels.

So, we would definitely look at it further.

Moderator: As there are no further questions, I now hand the conference over to the management for their closing comments.

Anura g Kalra : Thank you, Steven. Thank you very much, ladies and gentlemen. If there are any follow -up questions, Gaurav and myself are available to provide any further clarifications you might have.

Thank you once again for joining us on the call and have a go od day.

Moderator: Ladies and gentlemen, on behalf of Fortis Healthcare Limited, that concludes this conference.

We thank you all for joining us and you may now disconnect your lines.

Call: Nov 2022

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November 18, 2022

FHL/SEC/202 2-23

The National Stock Exchange of India Ltd.

Scrip Symbol: FORTIS BSE Limited

Scrip Code:532843

Sub: Transcript of Investors / Analysts' meet under Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015

Dear Madam / Sir,

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed herewith the Transcript of the investors / analysts meet held on November 14, 2022 to discuss the Company's un-audited financial results for the quarter and period ended September 30, 2022. Further, the said transcript is also available on the website of the Company i.e. <https://www.fortishealthcare.com/investors>.

This is for your kind information and record.

Thanking you,

Yours Faithfully

For Fortis Healthcare Limited

Murlee Manohar Jain

Company Secretary & Compliance Officer

M. No. F9598

Encl: a/a

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"Fortis Healthcare Limited

Q2 FY '23 Earnings Conference Call "

November 14, 2022

MANAGEMENT : DR. ASHUTOSH RAGHUVANSHI – MANAGING
DIRECTOR AND CHIEF EXECUTIVE OFFICER – FORTIS
HEALTHCARE LIMITED

MR. VIVEK GOYAL – CHIEF FINANCIAL OFFICER –
FORTIS HEALTHCARE LIMITED
MR. ANAND K –CHIEF EXECUTIVE OFFICER , SRL
LIMITED
MR. MANGESH SHIRODKAR – CHIEF FINANCIAL
OFFICER , SRL LIMITED
MR. ANURAG KALRA – SENIOR VICE PRESIDENT –
INVESTOR RELATIONS – FORTIS HEALTHCARE
LIMITED
MR. GAURAV CHUGH – FORTIS HEALTHCARE
LIMITED

Fortis Healthcare Limited
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Moderator: Ladies and gentlemen, good day and welcome to the Q2 FY '23 Post -results Conference Call of Fortis Healthcare Limited. As a reminder, all participants will be in the listen only mode.

There will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touchtone phone. I now hand the conference over to Mr. Anurag Kalra, Senior Vice President, Investor Relations, at Fortis Healthcare Limited. Thank you. And over to you, Mr. Kalra.

Anurag Kalra: Thank you, Inba. A very good afternoon and good morning, ladies and gentlemen, and thank you for taking the time to be with us on our quarter 2 FY '23 earnings call. The call is being led by our MD and CEO, Dr. Ashutosh Raghuvanshi. With him, we have our Chief Financial Officer, Mr. Vivek Goyal; from SRL, Mr. Anand, who is the CEO there; and Mangesh Shirodkar, the CFO of SRL. Along with me, I also have Gaurav.

I hope all of you have got a chance to go through the results deck and the press release that we had circulated on Friday evening. We will begin with some opening comments by Dr. Raghuvanshi, following which Anand will take you through certain key highlights of the diagnostics business. And then we can open the floor for questions and answers.

I now hand over to Dr. Raghuvanshi.

Ashutosh Raghuvanshi: Thank you, Anurag. Good morning and good afternoon, everyone. Thank you for joining us on our Q2 financial year '23 earnings call. I hope all of you have had a great time with your family and friends during the festival season. I would like to straightaway talk on the performance of company for the quarter. And then Anand will take you through the highlights for the diagnostic business.

We have witnessed a strong set of earnings for the quarter, our consolidated revenues have increased 10% versus Q2 of financial year '22 to INR 1,607 crores. I'm very pleased with the way our hospital business has performed. Our hospital business revenues have grown 18% versus Q2 of financial year '22 and 9% versus Q1 of financial year '23. Our diagnostic business, on the other hand, witnessed a revenue decline versus Q2 of financial year '22, which was primarily on account of the significantly lower COVID contribution. However, it is pertinent to note that compared to the trailing quarter, the business witnessed a 6% revenue growth reflecting early but encouraging signs of pickup post the COVID impact seen previously. Just to recall, the diagnostics business had an approximately 22% revenue contribution from COVID and COVID allied tests in Q2 of financial year '22, which stood at a near 5% in Q2 of financial year '23. If we adjust the COVID revenues on a like -to-like basis, the diagnostic business revenues actually grew 5.3% versus Q2 of financial year '22.

On the profitability metrics, our hospital business EBITDA stands at INR 246 crores, an increase of 30% and reflecting margins of 18.9% versus 17.2% in Q2 of financial year '22 and 17.4% in the Q1 of financial year '23.

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The improvement in margins has been led by a healthy performance of all the key hospital operating metrics, which I shall speak of in a while. On the diagnostics segment, commensurate with the decline in COVID and COVID allied revenues, EBITDA for the quarter stood at INR 73 crores versus INR 103 crores in Q2 of financial year '22. However, EBITDA was higher versus INR 64 crores in the trailing quarter. This reflects margins basis gross revenue at 20.7% for the quarter versus 25.7% in Q2 of financial year '22 and better than margins of 19.3% in Q1 of financial year '23.

Combining both the hospitals and the diagnostics business, our consolidated

EBITDA for the

quarter was INR 319 crores compared to INR 292 crores in Q2 of financial year '22 and INR 272 crores in Q1 of financial year '23. Consolidated EBITDA margin was at 19.8% versus 20% in Q2 of financial year '22 and better than 18.2% in Q1 of financial year '23.

To highlight, the contributions of hospital EBITDA increased to 77% in Q2 of financial year '23 versus 65% in Q2 of financial year '22. This significant -- the momentum in our hospital business earnings, enabling us to maintain our consolidated margins, despite the impacts in diagnostic business as a result of lower COVID volumes.

At the PAT level, we reported a profit after tax, prior to exceptional items of INR 167 crores compared to INR 130 crores in Q2 of financial year '22. This is a 28% growth versus the corresponding quarter and 24% growth versus Q1 of financial year '23. I would like to mention there was an exceptional gain of INR 51.6 crores which pertains to reversal of impairments in one of our associate companies.

Let me also briefly touch on the consolidated H1 financial year '23 financial numbers of the company. For H1, our consolidated revenue stood at INR 3,095 crores, a growth of 8% over the corresponding previous period. EBITDA for the quarter was INR 590 crores versus INR 575 crores for H1 of financial year '22, translating into a margin of 19.1% versus 20% in the corresponding previous period. PAT excluding exceptional items for the period stood at INR 301 crore versus INR 255 crore for the H1 of financial year '22, at a growth of 18%.

On the qualitative aspect of the hospital business, we have made significant progress on all fronts. With the strong demand for elective procedures, our surgical revenue contribution to overall revenue was at 59% compared to 56% in Q2 of financial year '22, led by a better speciality mix and robust traction in our international patient revenue.

Our hospital business witnessed an occupancy of 70% versus 65% in Q2 of financial year '22 and 64% in Q1 of financial year '23, while ARPOB grew 5% to INR 1.97 crores from INR 1.87 crores in Q2 financial year '22. ARPOB was similar to Q1 of financial year '23. If you would recall, we had indicated that our aim is to reach 70% by the end of this fiscal. However, given the strong traction, we were able to achieve this rather quickly. I would tend to be cautiously optimistic in Q3, which is generally a festive season across North India and hence occupancy may see some seasonal impacts in Q3.

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On the international patient business, we continued to witness strong traction. International patient revenue were at INR 109 crores, a growth of 164% over the corresponding quarter and 23% over the trailing quarter. International patient revenue during Q2 of financial year '23 contributed 8.4% to total hospital revenues versus 3.8% in Q2 of financial year '22.

It is also important to highlight that some of our key underperforming facilities have witnessed higher revenues and better profitability versus both the trailing and corresponding quarters. On our brownfield expansion plans, which we have been discussing with you, our expansion plans are well on track and we are adding capacities across all our major facilities like FMRI and Mulund, BG Road, Shalimar Bagh, Noida and Anandapur.

Currently our teams have been working tirelessly on several digital transformation projects such as implementation of electronic medical records, ERP implementation and improving patient experience through website and apps. We are quite excited with the EMR project specifically, as it would further strengthen our core belief of, Care For Good, for all our patients, enabling quicker access to health care records and faster diagnosis and treatment.

During the quarter, we commissioned several medical programs and further strengthened

medical infrastructure at various facilities. These include launch of a state-of-art lung transplant clinic and pulmonary medicine unit at Fortis Jaipur, bone marrow transplant unit at Fortis Mohali and Fortis Faridabad, a new Linac at Fortis Mulund and also robot at Fortis BG Road, a neuro navigation system at Fortis Amritsar and a new Cath lab at Fortis Mohali.

Commensurate with our medical program expansion, we continue to attract high-quality clinical talent. We have onboarded clinicians in the speciality of cardiac, oncology, neurosciences and gastroenterology during this quarter.

On the diagnostics side, though the business was impacted due to decline in COVID volumes, adjusting for the COVID revenues, non-COVID revenues have grown 5.3% versus Q2 of financial year '22 and 5.6% versus quarter 1 of financial year '23. Additionally, if we look at H1 financial year '23, non-COVID revenues grew by 15.5% versus the corresponding previous period.

On SRL, our focus is on strengthening the business' growth drivers by expanding our geographic

footprint and channel network with further additions of stat labs and customer touch points.

Anand will take you through more of the details in a short while.

On the balance sheet, we remained quite healthy with a net debt -to-EBITDA of 0.44x versus 0.74x for Q2 financial year '22. Our net debt stands at INR 565 crores as of September 30, 2020 and our debt -equity mix allows us the flexibility to leverage our balance sheet to further our growth objectives.

With that, I would like to end my comments, reiterating to all of you that we remain steadfastly committed towards our growth path in order to further our operational performance and create longer-term value for all the shareholders. Thank you.

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And with this, I would like to hand over to Anand for his comments on the diagnostics business.

Anand K.: Thank you, Dr. Raghuvanshi. A very good morning to everyone on the call. Thank you for joining us today. On behalf of SRL Diagnostics, I warmly welcome you all for our Q2 FY '23 Results Conference Call. I hope all of you and your families are safe and in good health.

During the quarter, we reported a revenue of INR 351 crores, with 95% of our revenues coming from non-COVID testing. Our EBITDA stands at INR 73 crores with a margin of 21% for Q2 FY '23. We conducted 10 million tests in the quarter and serviced over 4.3 million patients.

While COVID-related testing and volumes have slumped as predicted, we are witnessing an uptick in non-COVID volumes across our network.

SRL's B2C -B2B revenue mix stands at 55 to 45, consistent with our Q1 ratio. Our non-COVID revenue growth stands at 5% in Q2 FY '23 versus Q2 of FY '22 and at 6% versus the trailing quarter Q1 FY '23. Non-COVID revenue grew by 16%, approximately, in H1 FY '23 compared to H1 of FY '22.

In a post-pandemic setting, we are witnessing a heightened wellness consciousness amongst customers and this reflects in our preventive portfolio, which has grown 21% in terms of revenue over Q2 of FY '22. Keeping in line with our network expansion strategy, we have added 207 new customer touch points in Q2 of FY '23. And our customer touch point -per-lab ratio now stands at 18.6 compared to 12.1 in the corresponding Q2 FY 2022.

Our new customer experience initiatives, including WhatsApp chatbots and live phlebotomist tracking feature, have received a good response from our patients we service through our home collection network. On the people front, we completed more than 1,300 man days of training in Q2 FY '23.

In the last few years, SRL has progressed to bring many tailor-made competency enhancement programs. Training, learning and development continues to be one of our priority areas. Over the years, we have built strong learning and development functions at SRL, wh

ere we manage

more than 90% learning interventions internally.

We use technology for learning enforcement as well as talent assessment through learning management systems and assessment tools. We are working relentlessly on building the organization as a great place to work and we have significantly progressed on our overall NPS score and going ahead strong on being the preferred employer in the diagnostic industry.

Our state-of-the-art reference laboratory at SRL Mumbai has been reaccredited by College of American Pathologists USA. This is a testament to our continued focus and commitment to next-generation diagnostic services. We will continue to focus on expanding our network in priority and expansion cities and focus on continuously improving our customer experience across all our network.

Our NPS score for patients remained consistent at 78. On the scientific side, we will continue our concerted efforts in the chronic diseases category; lifestyle diseases; preventive health care

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category; and our specialized portfolio that comprises autoimmune diseases, transplant technologies, infectious diseases and oncology. Our genomics portfolio revenue grew by 27% in Q2 FY '23 compared to Q2 '22. Our sustained focus on genomics and next-generation diagnostics, along with our work in digital technology, will help us differentiate ourselves and also enable us to reach maturity. Thank you very much for your attention. I would like to now hand over the call to Mr. Anurag Kalra, head of our investor relations.

Anurag Kalra : Thank you, Anand. Ladies and gentlemen, we shall now open the floor for question-and-answers. Inba, please moderate that.

Moderator: The first question is from the line of Kunal Randeria from Nuvama.

Kunal Randeria : Just on the hospital business. I still see that 20% of revenue come from CGHS, government, PSUs and ECHS, so I assume the bed contribution will be even higher than this. So now that you have a good 70% type occupancy going forward, do you see this number coming down quite a bit and what we should expect in the coming years.

Vivek Goyal : Yes, I will take this question. Vivek this side. So you are absolutely right. There is opportunity here to change the payer mix in our favor in some of our hospitals where the potency level is quite high, but it will be very selective. It will not be like across the board.

Kunal Randeria : Sure, but your TPA plus, I think, cash is around 70%, 72% -odd, so should we expect it should remain at these levels going forward?

Vivek Kumar Goyal : So the payer will go for TPA care and the international business. And the other business will scheme business will come down.

Kunal Randeria : And one just quick one on diagnostics: The 5% non -COVID year -on-year growth is slightly lower than what some of the players have reported. So just wondering and especially since you've added a lot of touch points in the last few quarters. So just wondering the kind of growth momentum that you expect, especially considering the kind of competition that you are seeing in this industry.

Anand K. So thank you. This is Anand here. Usually the Q2 is the best quarter for the year for diagnostics.

And what we see here is primarily because last year, because of COVID, we see a lot of testing happening, especially around hospitals. So what we calculate as COVID and COVID allied tests is just a few tests related to COVID, but as you know, during the second quarter of last year, there's a lot of hospitalization that happened.

And the testing momentum also increased at that time, so maybe it will not be the right comparison between Q2 and Q2 of this year in terms of non -COVID as well, but at the same time, we internally for us we also have a situation where we had Himachal Pradesh PPP project which we were running la

st year. We've been running it for the last eight years, so we have the contract ended this year, so it was part of Q2, but Q2 of last year, but it's not part of Q2 of this year. So that has also contributed to it. If you adjust for that, our growth will be close to about 9%.

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Moderator: We'll take a next question from the line of Nitin Agarwal from DAM Capital.

Nitin Agarwal : Dr. Raghuvanshi, on the hospital network expansion, we've talked about 300 - to 400 -bed per annum addition over the next two to four years. Now two things, one is, a, this entire expansion is going to be brownfield or are we looking at is there any element of greenfields in this expansion?

Ashutosh Raghuvanshi : This entire expansion is brownfield. It is within the given hospitals, but at some places we have to construct a fresh tower. And in certain places, we already have the civil construction done and only the interior work needs to be done. So it's a mix of both, but essentially this is all in the category of brownfield.

Nitin Agarwal : And this will typically, what, a 50 lakh, 60 lakh per bed sort of expansion cost for further expansion?

Ashutosh Raghuvanshi : Yes. So on a blended basis, the entire thing would come to about 80 lakhs per bed.

Nitin Agarwal : Despite it being like a brownfield sort of expansion.

Ashutosh Raghuvanshi : Yes. But because you see, we will also be enhancing and upgrading equipment and larger diagnostic equipment, then imaging equipment will also be enhanced along with it because, as the capacity of these hospitals will increase, we will need a lot of other medical equipment as well. So compared to kind of a greenfield which would easily be about 1.5 crores, this is definitely much less than that.

Nitin Agarwal : And in terms of the expansion that we're undertaking, is it fair to assume that bulk of it will be coming in what you call Tier 1 town and metros?

Ashutosh Raghuvanshi : Yes, but majority of this is in NCR and in Bangalore and Mumbai and in Kolkata.

Nitin Agarwal : And if I just push that: On the operating facilities which are there that we have right now, beyond this expansion that we will undertake over the next three, four years or adopt, how much more can we add from a brownfield capacities on our existing network? I mean, if you were to hypothetically just stretch it out maximum, how much more can we do on the existing facilities in general?

Ashutosh Raghuvanshi : So other than the planned 1,300 beds which we have given a kind of time lines for, another 700 beds more could be added further.

Nitin Agarwal : So approximately about 2,000 beds on the current network. And thereon, our expansion would be largely has to be on newer assets, whether we buy something out or we do some greenfield.

Ashutosh Raghuvanshi : That's correct.

Nitin Agarwal : And sir, in the metrics that we've discussed, on the hospital -wide improvement, if you can

probably just shed some light on which have been the more notable movements in this quarter.
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There has been some movement in the lower-tier hospitals, lower-profitability hospitals, which have reduced in this quarter.

Vivek Goyal : Yes. So there is improvement, overall improvement, as you have seen, overall improvement, in the profitability of all the hospitals. And the hospital from the lower table has moved to the upper table. Anurag will -- able to explain...

Anurag Kalra : Yes. So Nitin, just to give you some examples. In the highest bracket, we've added BG Road, which was one level lower. We've added Anandapur, which was one level lower. Mohali continues to be in the higher bracket. In the next category, we've added a few more hospitals, like Faridabad and Ludhiana. So those are the ones

that have been doing well, but overall, if you were to just look at it in a synopsis basis when you look at FY '22, the topmost bracket added some about 700-odd beds. Today, the topmost brackets have about 1,170-odd beds. So that gives you a very good indication of how we are moving up level to level.

Nitin Agarwal : And Anurag, of the hospitals which are there in the lower two tiers, any sort of outlook on that? There's another 1,000 beds which are there in that less than 10%, 15% bracket; and specifically 700-odd beds in the less than 10% bracket. How are we looking at these assets?

Vivek Goyal : So if I can answer this question. So there are a couple of hospitals which are already showing very good sign of improvement. We have discussed in every call like, FEHI and Jaipur are in this category, which are definitely showing very good sign of improvement. And we have planned how to improve it further. Maybe some more investment may be required. Maybe, some more speciality, we may be able to add to these hospitals.

And those type of plan are work in progress. And with that, these hospitals we may reach to 15%-plus EBITDA margin category, but that will take some time. And the other hospital in this chain are two in yes, Chennai, which we have one is a sort of new hospital. And the ramp-up is taking some time, but the team is working very hard to improve that ramp-up there. And other is Malar, where the progress is improving but at a slow pace.

Nitin Agarwal: And sir, I mean, it's fair to expect that, of the six hospital which are there in this 10 lower than 15% bracket, all goes to plan. Or we should have probably this number should halve as we go forward in the coming quarters. More than it should be probably 2 or 3 hospitals which probably more than 2 or 3 hospitals going forward ?

Vivek Goyal: Not coming quarter but yes. Maybe, in the medium term, we can sort them with a 1 year to 1.5 years time. We can expect that or at least that we have a plan to move to above 15%.

Moderator: Our next question is from the line of Shyam Srinivasan from Goldman Sachs.

Shyam Srinivasan: Just first, on the guidance for occupancy. Dr. Raghuvanshi, you said that we have touched 70% and we would like to progress toward 75%, so despite some seasonal Q3 may be slower, but just what are the drivers of this, Dr. Raghuvanshi, in terms of is it that you are seeing more and more speciality? Is it international patients? Is it market share gains? If you could help us understand.

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And NCR, clearly at least one of the key reasons for you. There is quite a lot of supply also coming, so how do you kind of embed that into your outlook as well?

Ashutosh Raghuvanshi: Yes. Shyam, as I have said many times before that as far as the additional capacity which is coming across in the region and NCR, that will get very easily absorbed. And that is not of concern because there is a little bit of demand-supply gap in this season. It's of a huge internal. So the main driver for us will be volumes coming because of the addition of clinical talents. As I mentioned, we have added quite a bit of talent in last quarter, and as they start getting stabilized, that would be one of the drivers.

And international definitely have been showing continuous improvement. It has come to a good level now. It is in absolute terms, it is a little better than pre-COVID levels, but since our domestic numbers have grown, so as a percentage of overall revenue, it is still at about 8.5%. So that is likely to go about 10%, 12% as well. So that will be another driver for growth as well.

Shyam Srinivasan: That's helpful, Dr. Raghuvanshi. So the second thing I saw was your speciality mix. Surgical revenues have grown 24%, just calculating it from your pie charts, but something like oncology has grown like 50%. S

o what are we doing here? Maybe there's a base effect. I don't know, but I'm just saying -- which are the speciality mix that you think you will continue to see further

traction?

Moderator: Sir, it looks like the management's line is disconnected. Ladies and gentlemen, we request you to please remain connected while we join them back. Please do not disconnect your lines. We'll call the management right away. Mr. Srinivasan, I think you will have to repeat your question.

[Technical Difficulty]

Shyam Srinivasan: My second question was just on the speciality mix. So if I were to just do simple math on using numbers of the speciality mix, onco revenues have grown 50% roughly. And when I look at overall growth, it's 18%, so I just want to understand how we are achieving this. And it can this kind of sustain the kind of growth we are seeing in some of the higher -growing specialities?

Ashutosh Raghuvanshi: Yes. So onco growth specifically. As we had stated about two years back, that onco is a area where we are focusing. And we had acquired some talent. Plus, we had created some facilities. Like, for example, in BG Road, we did not have oncology earlier. We added there two years back and many other infrastructure things we have done in the oncology segment, so we expect that oncology will continue to show higher growth for at least another one year or so. And that would be sort of then stabilized to a much, yes, more regular rate of growth. Other than that, the other core specialities which we have been focusing on, those also have grown significantly, but oncology definitely will remain a focus area.

Shyam Srinivasan: Yes. Just the third question is on margins. You used to call out the losses at Arcot Road, so can you quantify that? And just to Mr. Vivek's question on slower progress in terms of ramp -up, if you could clarify that also, please, in terms of how long we think a breakeven at Arcot Road could happen?

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Vivek Goyal: Yes. If I can take this question, Shyam. I mean Arcot Road's performance actually has not improved, if we compare with the last quarter. So it is incurring EBITDA negative of around INR 8 crores per quarter, and that is continuing. And that is primarily contributed because we are not able to ramp -up the facility, the occupancy there. And main reason, we have discussed earlier also. There is some construction work going on, some bridges going, just in front of the hospital which is upsetting the patient flow. And that is the main reason. And then I think we are also building the clinical talent there. We are also spending some money on the advertisement and other things. Just to highlight this, this facility is very good facility, state of the art. And all other parameters, it is very good, but I think, just because of the this obstruction, I think the patient flow is affected our team is working. And with regard to your second question by when we expect it to be EBITDA positive -- at least not bleeding, at least, for us, we are expecting in next six -months time, it will be in this range.

Shyam Srinivasan: In 12 months, you're saying, Mr. Vivek?

Vivek Goyal: Six months, Shyam.

Shyam Srinivasan: Six months. Okay. Last question is on SRL, Mr. Anand, just one on you talked about 9% when we, growth for non -COVID if we exclude the government contracts. So if you could also help us understand the split between volume and price for that, sir? Thank you.

Anand K: Thank you, Shyam. So in terms of contribution, if you see, it was about 4% of our revenue in Q2 of FY '22, the HP PPP contract. And largely it was more of a, since it was a government contract, the volumes were quite high. And ARPT was quite low.

Shyam Srinivasan: No, sir. My question was different. I was saying, excluding that, you talked about your growth in non -COVID for Q2

this quarter versus last year was 9%, so I'm looking for the split of that into volume and price?

Anand K: It's about, just a second. Let me look at it. So our ARPT has grown by about 8% on the H1, but it's a sort of a mix of volume and ARPT growth, which a larger contributor on the ARPT side.

Shyam Srinivasan: So Anand, safe to assume some low single -digit volume growth, right? So when can this kind of improve, right? And I don't think it's only for you, right? Everybody else is struggling for volume growth, would that be competition, especially in semi -specialized tests driving this? Or you think patient footfalls into even your B2C have been slower than usual?

Anand K: So in our case, as I told you, it is primarily because of the HP PPP which was very high in volume. So with that, the volume was very high, but the ARPT was really low. So that is why we are seeing the growth here as higher ARPT and lower volume growth. But as if you're see, separately, if you you're seeing, we are seeing good growth on the B2C segment as well.

Shyam Srinivasan: Anand, sorry to persist, but I'm completely removing HP PPP product in both your Q2 last year and Q2 this year, okay? Let's assume, I'm not even talking about that business at all. And you then look at the non, whatever, right, just the business which is non that, and you said it's 9% growth. So that is what I was trying to dissect. I'm not even looking at this project, this

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government project. And there, when I, when you do the math, you suggested about low single-digit volume growth for that business, so, and my question is simple, right? What is, why is that growth slower? And it's not only for you, right? Everybody seems to be facing this issue, so that's where the question is coming from more?

Anand K: So I think it is more to do with the higher revenues during the Q2 of FY '22 driven by COVID. Even though we calculate only COVID and COVID allied tests, only four tests of COVID is considered as COVID allied tests, but as you know, during last Q2, there were a lot of drive into hospitals. And a lot of testing happened on multiple parameters, not just IL-6, D-dimer, CRP, those kind of COVID allied tests. A lot of other tests also happened, so that has boosted the volume during that time in a much bigger way. So I think it's too early for us to really look at any trends on this at this point of time. I think we'll have to allow another one or two quarters to settle on how exactly this growth is turning out to be.

Moderator: Our next question is from the line of Abdulkader Puranwala from Elara Capital.

Abdulkader Puranwala: Sorry if this may be a repeat question, but I'm just going through your hospital margin metric. So if I look at the category of EBITDA margin for your hospital which falls in the 10% to 20% bracket, I mean, and occupancy has surged to around 70%, 72%, but I mean it's still remain in this margin bracket itself, so apart from occupancy, we wanted to know. What will be the other factors which you guys are working upon to improve the margin in these hospital beds?

Vivek Goyal: Yes. So it is a good question. So apart from the occupancy, we are very well working in improving our payer mix, which I have covered in my earlier comments, where we are trying to reduce the scheme business to the extent possible and increase the share of TPA, cash and international business which are high-margin business for us. Secondly, we are also trying to improve the speciality mix also in our favor by adding more speciality by investing in infrastructure, in the form of equipment and things like that. So those are the state ones. And thirdly, we are also working on the margins. So we do come out with some margins and cost-optimizing things where we can maximize the productivity and things like that. So all the

se things will lead to this improvement in the margins.

Moderator: Our next question is from the line of Sanjay Shah from KSA Securities.

Sanjay Shah: Dr. Raghuvanshi, my question is regarding our brand that is our parent's strength. That is IHH worldwide. They are into personalized care ranging from primary to ordinary and even some ancillary offerings. So how in India we compare ourselves with the par treatment to our patients. And is there any scope to add more to our offerings and profiles?

Ashutosh Raghuvanshi: Yes. So Sanjay, we are getting a lot of support, and we are using our, a lot of synergies with IHH, especially regarding the quality. So we participate in that global quality program. Quality benchmarks and metrics are followed on regular monthly basis across the entire IHH network, so the best practices of, on clinical quality and other innovative initiatives is shared across the network, including with Fortis, so that is one big advantage. Another thing which I had briefly mentioned during my statement is about the electronic medical record which we are working Fortis Healthcare Limited

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along with the IHH folks to implement across our Fortis network, which will help us to improve the speed with which we can deliver discharges and medical records to our patients. And also it will help during the treatment and diagnostic post the fact.

Sanjay Shah: So sir, over and above these digital, that EMR what we are talking, do we have any surgical side or any other in-patient care which we can offer and which we are right now not offering in India and have a scope to expand that in India?

Ashutosh Raghuvanshi: So generally we have all these areas being offered in India as well. So there are no specific surgical facilities, but however, in oncology, especially on the CAR T cell therapy and proton therapy, we are coordinating with them so that, some of our patients who may be wanting for proton therapy, we can refer them to Singapore for doing that. So that kind of collaborations are also in place.

Sanjay Shah: Sir, my last question was regarding this EMR, what you mentioned, so is there any regulatory binding and regulatory rules which we need to understand on this EMR? Is that EMR only for our hospitals, our records? Or even we come to share with a patient also?

Ashutosh Raghuvanshi: Yes. So the EMRs, of course, are patients' property. The record, medical record, of an individual patient is something which is patient's property. However, we manage it, so there is the national guidelines which are there. And there are certain requirements for keeping it interoperable so that it can be transported from one provider to the other. So it is absolutely compliant to the guidelines which have been issued, yes.

Moderator: Our next question is from the line of Bharat Sheth from Quest Investments.

Bharat Sheth: Sorry. I joined a little late, so this question, if you just answered, can you repeat, please? Sir, like in diagnostics, we have some government business, so do we have any kind of such obligation in our hospital business where we have to provide them at concessional rate and which, over a period, we may reduce so it can help us to improve the profitability?

Vivek Goyal: No. Except, in FEHI, we have 25 beds dedicated for EWS. I don't know, I don't think there is any of this facility where, no. Apart from FEHI, we don't have any obligation at any of our facilities to take government business, government patient provide free service or concession - grade service.

Bharat Sheth: And second question, sir, on these hub-and-spoke several-tier institution. I mean companies are going in hub and spoke more, say, for Tier 2 and Tier 3 cities. So do we have any kind of such aspiration, or is that clearly a profitable business or make a business case?

Ashutosh Raghuvanshi: Yes. So we

have stayed away from that because there are challenges in Tier 2, Tier 3 cities on the operations side. Typically these markets are very price sensitive and clinical talent doesn't come by so easily over there, so we have not selectively gone for that, but however, if there is in future something which is very synergistic to our larger cluster in a metro city, we will consider that on case-to-case basis, but not on a standalone basis, we are not looking at that.

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Bharat Sheth: And last question, on diagnostics side, sir. How do we really plan to ramp up our facilities as well as, I mean, our geographic diversification?

Anand K: So we are actually adding, as you know, we are probably having the best equitable distribution of our business across all geographies, but we continue to add, expand our network through our focus city expansion plan as well as adding more collection centers. So we have added, even in this quarter, we have added up to 207 customer touch points. So similar, that is a way we are expanding in hub-and-spoke model, where we are adding more reference, adding kind of more regional reference laboratories and then stat labs to complement that as well as collection centers to expand it further.

Bharat Sheth: And sir, recently there is an interview of our parent company where they have suggested that they may change the Fortis name to Parkway because of the kind of, I mean, obligation. I mean this brand belongs to the erstwhile promoter. So will it have any kind of, I mean, what are we doing to promote this new brand or, if at all, that will really can have a kind of a, say, cannibalization or...

Ashutosh Raghuvanshi: Yes. So we had earlier also made it, an announcement that we will consider changing the name to Parkway. So that definitely remains something which we are continuing to evaluate. You are right that brand change always carries a certain amount of risk and expense as well, but there are certain constraints around the brand which is owned by the ex-promoters. But since the area, there are still some clarifications we need to seek from a legal point of view, so this is some time away, but this is definitely our preference, to consider a change in brand, after careful evaluation. But this valuation is to be done further to decide finally as to how to go about it and when to time it. So certainly, the moment those clarities emerge, we will make the proper announcement.

Moderator: We'll take our next question from the line of Dheeresh Pathak from WhiteOak Capital.

Dheeresh Pathak: So I'm not sure if you already answered, but you were explaining the downstream, a potential immediately of 1,300 beds. Have you been searching a patient like Bangalore, Mumbai and Calcutta, but is there a more granular breakup that you have given, like what would be the capex slated in the next few years and which cluster the beds will be?

Vivek Goyal: Yes. So as mentioned earlier, so this bed extension is on our existing hospital only and all are in metro. So major expansion is happening in the NCR region, yes, but followed by Bombay and Calcutta and Bangalore also. Bangalore...

Dheeresh Pathak: Can you give numbers also, sir, if possible? Like how many beds in NCR? How many in Bombay? And how many in Calcutta?

Vivek Goyal: Yes. So 50% of the bed expansion is coming in the NCR region. And Bangalore, we are expanding around 200 beds, and Calcutta around another 200 beds. And Mumbai, we have already completed the construction. We are waiting for the OC. We have partly operationalized the beds. So total expansion of 110 beds. out of that, 45 has already been operationalized. And Kolkata, I've already calls, yes. These are the places where we are doing the major expansion.

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Dheeresh Pathak: And the 50%, the bigger expansion in NCR, what is the time line for commissioning on those beds?

Vivek Goyal: Well, look, these include expanding the capacity in three of our major big hospital in NCR, which include FMRI, Shalimar Bagh and Noida. So there we are expanding bed capacity by almost 170 to 200 beds in each location, plus there is a 60 bed expansion which is underway in Faridabad, yes. So Faridabad expansion is underway, it's may be completed in a year time. However, the other three expansion, we have submitted the drawing to the authorities. For one hospital in Noida, we have already got all the approval and construction work will be started post this NGT inspection has removed.

Dheeresh Pathak: So probably like the bigger one will be in NCR, where probably commissioning is probably like 18 to 24 months away, is that a fair assumption...

Management: Yes, that's correct, absolutely.

Dheeresh Pathak: Sir, second question is in terms of the regulatory from the Supreme court order and other things.

Can you just clarify like what all you're allowed to do? Like name change, as per the interview in the media, understanding, name change, you're allowed. Capital structure and shareholding change in the listed entity, is that allowed? What is allowed and what is not allowed?

Ashutosh Raghuvanshi: Yes. So as we are still evaluating the legal position, and our lawyers are working on that. Some of the areas are very clear that there is no restriction, like brand, as you mentioned, but as far as the other areas are concerned, those, still some clarity has standby. Regarding open offer, of course, it is a matter which IHH needs to clarify with the authorities. And then based on that, they will be making the necessary steps what they need to take. As far as Fortis is concerned, we definitely don't have much effect of this on our operations. The only areas which we are to wait for is the clarity on the RHT transaction we'll, about the court will take a decision.

Dheeresh Pathak: No. What I meant was, let's say for example, if you want to buy an asset, M&A, and issue your shares as currency, will that be allowed, or that also will not be allowed?

Ashutosh Raghuvanshi: So share as currency is, we don't think, is allowed at the moment, but we do have capacity to look at some other smaller acquisitions which we will evaluate.

Dheeresh Pathak: So change in capital structure and shareholding is not allowed. And the last question is on this, I mean the EBITDA margin profile based on number of beds. So there are five hospitals, with 716 beds, which are less than 10% EBITDA. One obviously is the Arcot Road, which are the other assets? Because they will have been operational for a while. So is there a structural issue that these hospitals, can you identify some of these hospitals? And what is the issue with these hospitals that they are less than 10% EBITDA?

Vivek Goyal: Yes. So out of these five, six hospital, Arcot Road and I have explained in detail what is the problem there some sign of improvement, but at the same time, the progress is slightly slow because the fix, they are required some long-term fix in the form of infrastructure improvement and things like that, getting the Fire NOC. All those things are required, which is likely time

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consuming, something is in our hands which we are trying to complete and something we have to get it done through the government agencies.

As regards Jaipur and FEHI's, both are progressing quite well in terms of occupancy as well as in the -- and margins. And in both the hospital, we also feel that there is an opportunity to add further state facilities. So that is something which we are in the, on the exploration stage, and once we complete it, I think that will give a further boost

to the margin expansion of both these hospitals.

The large hospital in this category is the Vashi hospital, a sizable hospital in this category, which is, again, having its own challenge because of a specific area where there will be some hospital being operated, and there are infrastructure related issue also and it is a government hospital and we are operating it. So that hospital is also slightly slow in improving the progress. So I think...

Dheeresh Pathak: So it will have been operational for a while. So you are saying we don't have the full stack of specialities. That is the reason why it has been slower...

Vivek Goyal: Yes. For example, Onco is not there in Jaipur, and it is at a very good location, there is a bed capacity available to expand. So those type of things, we are exploring.

Moderator: Our next question is from the line of Amit Khetan from Laburnum Capital.

Amit Khetan: So your press release mentioned, talked about portfolio rationalization and inorganic expansion going to be drivers of operational performance. Could you elaborate a little bit on both these aspects, especially in light of the supreme court judgment? Are we ready to press ahead on both these levers?

Vivek Goyal: Yes. So we are moving ahead on both these levers. Brownfield expansion, anyway, the capex requirement is quite less. And as you have seen, the cash flow generation from the business is

quite handsome to take care of this brownfield expansion of its own, and as mentioned in Dr. Raghuvanshi's speech also, the leverage ratio is quite comfortable. Our debt level, net debt level, is INR 500 crores-plus level. It is less than half of the one-year EBITDA, so that gives us confidence that we can very well fund it by a little bit leveraging our balance sheet, so we are actively pursuing the acquisition opportunity which makes sense for our overall growth strategy.

Amit Khetan: So should we expect some kind of strategic action over the next six to 12 months?

Vivek Goyal: I mean these deals are very difficult to predict, but yes, our aspiration will be to close that in during this period.

Amit Khetan: And this holds true for both, for portfolio rationalization as well.

Vivek Goyal: Yes.

Moderator: Thank you. Ladies and gentlemen, that was the last question. I now hand the floor back to the management for closing comments.

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Anurag Kalra: Thank you, Inba. Ladies and gentlemen, thank you very much for joining us on the call today.

Gaurav and myself are available, in case you have any further clarification or queries, please feel free to reach out to us or e-mail us. Thank you very much. And have a good day.

Moderator: Thank you, members of the management. On behalf of Fortis Healthcare, that concludes this conference. Thank you for joining us, and you may now disconnect your lines.

Call: Feb 2023

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February 16, 2023

FHL/SEC/202 2-23

The National Stock Exchange of India Ltd.

Scrip Symbol: FORTIS BSE Limited

Scrip Code:532843

Sub: Transcript of Investors / Analysts' meet under Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015

Dear Madam / Sir,

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed herewith the Transcript of the investors / analysts meet held on February 13, 2023 to discuss the Company's un-audited financial results for the quarter ended December 31 , 2022. Further, the said transcript is also available on the website of the Company i.e. <https://www.fortishealthcare.com/investors> .

This is for your kind information and record .

Thanking you,

Yours Faithfully

For Fortis Healthcare Limited

Murlee Manohar Jain

Company Secretary & Compliance Officer

M. No. F9598

Encl: a/a

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"Fortis Healthcare Limited

Q3 FY2023 Post Results Conference Call "

February 13 , 2023

MANAGEMENT :

DR. ASHUTOSH RAGHUVANSHI - MANAGING DIRECTOR & CHIEF
EXECUTIVE OFFICER - FORTIS HEALTHCARE LIMITED
MR. VIVEK KUMAR GOYAL - CHIEF FINANCIAL OFFICER – FORTIS
HEALTHCARE LIMITED
MR. ANAND K - CHIEF EXECUTIVE OFFICER – SRL
MR. MANGESH SHIRODKAR - CHIEF FINANCIAL OFFICER – SRL
MR. ANURAG KALRA – SR VICE PRESIDENT, INVESTOR
RELATIONS, FORTIS HEALTHCARE LIMITED
MR. GAURAV CHUGH – DGM – INVESTOR RELATIONS - FORTIS
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Fortis Healthcare Limited
February 13, 2023

Page 2 of 17 Moderator: Ladies and gentlemen good day and welcome to Q3 FY2023 Post Results Conference Call of Fortis Healthcare Limited. As a reminder, all participant lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during this conference call, please signal an operator by pressing “*” then “0” on your touchtone phone. Please note that this conference is being recorded. I now hand the conference over to Mr. Anurag Kalra, Senior Vice President, Investor Relations at Fortis Healthcare. Thank you and over to you Mr. Kalra!

Anurag Kalra : Thank you Neerav. A very good morning and good afternoon ladies and gentlemen and thank you for taking the time to join us on our Q3 FY2023 earnings call. The call today is being chaired by Dr. Ashutosh Raghuvanshi, our Managing Director and CEO. With him we have Mr. Vivek Goyal, our Chief Financial Officer. Mr. Anand, the CEO of SRL joins us as he is along with Mangesh, who is the Chief Financial Officer of SRL. I have my colleague Gaurav as well with me. I hope all of you got a chance to go through the presentation and the press release that we have sent out on Friday evening. Nonetheless both these documents are also uploaded on the Fortis website for those who have not got a chance to see it. We will start with some opening comments by Dr. Raghuvanshi on the performance of the quarter post which Anand will take you through some highlights of the diagnostics business and then we can open the floor for question and answer. Over to Dr. Raghuvanshi!

Ashutosh Raghuvanshi : Thank you Anurag. Very good morning and good afternoon everyone. Thank you for taking time to join us on our Q3 FY2023 earnings call today. I wish you a

Happy new year and

hope all of you are doing well. I shall come straight to the results of the quarter and my thoughts on the business performance and the way forward. Q3 for us has been reasonably well; however, this is a quarter which was impacted due to seasonal impact of festivals in some of our larger geographies. The business performance has been relatively better vis-à-vis the corresponding previous quarter and expectedly marginally lower than the trailing quarter. Our consolidated revenues were at Rs.1560 Crores a growth of 6% versus the corresponding previous quarter. This compares to Rs.1467 Crores in Q3 of FY2022. Our consolidated EBITDA margins are at 18.5% versus 20% in the corresponding previous quarter and versus 19.8% in the trailing quarter. This is largely due to the decline in the diagnostic business as a result of fall in COVID volumes. Our profit before tax stood at 175 Crores versus Rs.185 Crores in Q3 FY2022 and our profit after tax was Rs.142 Crores similar to the previous corresponding quarter. Our balance sheet remains healthy and given our cash flow generation we have further reduced our net debt by approximate Rs.100 Crores during the quarter. Our net debt as of end of Q3 FY2023 stands at Rs.471 Crores against Rs.565 Crores at the end of Q2 of FY2023. This reflects a net debt to EBITDA of 0.41x compared to 0.53x in Q3 of FY2022.

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Page 3 of 17 Our hospital business has witnessed a healthy performance in the quarter allowing us to offset to a large extent the decline in diagnostic business. The consolidated profitability numbers that I shared with you just now clearly reflect this. We have witnessed an occupancy of 66% in Q3 versus 65% in corresponding quarter and 70% in the trailing quarter. Higher complex surgical volumes in select medical specialties and strong traction in the international business has resulted in an 8.4% increase in ARPOB to Rs.2.02 Crores. All this have enabled the hospital business revenue to reach Rs.1267 Crores in the quarter a growth of 13% versus Q3 of FY2022. The hospital business EBITDA grew 14% to touch Rs.217 Crores reflecting a margin of 17.1% similar to the margin in Q3 of FY2022. If we exclude Vadapalani facility in Chennai which is still in red the overall hospital business margins are at approximately 18%. The international business witnessed a strong recovery in the quarter. Revenue from international business grew 73% versus Q3 of FY 2022 and

Rs.114 Crores it contributed 9% to the hospital revenue versus 6% in the previous corresponding quarter. On the diagnostic side the business is still witnessing muted performance due to the decline in volumes related to COVID and COVID allied tests; however, revenues excluding COVID and COVID allied tests stood at Rs.321 Crores versus Rs.300 Crores for the Q3 FY2022 a growth of 7% for the quarter. Overall the diagnostic business gross revenues stood at Rs.331 Crores versus Rs.388 Crores in Q3 of FY2022. We do believe that the diagnostic business will continue to witness challenges in short term at the same time there are initial signs of stabilization through the base of recovery though the pace of recovery would be gradual as we see the environment currently. We continue our focus on channel expansion, our specialized test portfolio and customer touch points to lab ratio, all of which continue to be further strengthened. Let me briefly touch on the nine month performance of the company. For the nine months ended on December 2022 the company's revenue grew 7.3% to Rs.4655 Crores versus Rs.4340 Crores. In the corresponding previous period EBITDA for the period stood at Rs.878 Crores a margin of 18.9% versus Rs.869 Crores a margin of 20% reported in the corresponding previous year. PBT before exceptional items stood at Rs.567 Crores compared to Rs.547 Crores and PAT was at Rs.

495 Crores versus Rs.703 Crores in the corresponding previous period.

I will briefly highlight a few qualitative comments on both our businesses. On the hospital side investments are on track to increase our bed capacity by 250 to 300 beds per year for the next few years and further bolster our medical and clinical infrastructure. During the quarter the company onboarded eminent clinicians in medical specialties, surgical oncology, pediatric cardiology and orthopedics. In the terms of medical infrastructure the company inaugurated a new digital x-ray service suite at Fortis Vadapalani, introduced a state-of-art surgical navigation neuro navigation system at Fortis Amritsar and launched Fortis Healthcare Limited
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Page 4 of 17 Mako robotic technology for joint replacements at Fortis Bannerghatta Bengaluru. In addition the company commenced its EMR electronic medical record implementation project which we believe will significantly enhance patient care, align quick access to healthcare records and faster diagnosis and treatment. This would be rolled out across the Fortis network in a phased manner as and when it is developed and ready. An other important aspect to highlight is that our digitization initiatives continue to yield encouraging results, revenues from digital channels such as our website app and digital campaigns have witnessed a year-on-year increase in digital revenues of almost 12% versus the corresponding previous quarter and their contribution to overall hospital revenues stood at 23%.

On the diagnostic business I will briefly touch up on few things. SRL's performance was impacted by the decline in COVID volumes; however, non-COVID revenues grew 7% for the quarter and 12% for the nine months of FY2023 versus the corresponding previous periods. As I mentioned previously the business is now showing initial signs of stabilization in the aftermath of the COVID surge and the challenging industry environment. SRL continues to focus on improving its channel mix and adding to its specialized test menu such as those in the area of genomics, in parallel it continues to optimize its network and customer touch points to lab metrics in order to improve utilization. Just to highlight during the quarter SRL added 362 customer touch points reaching over 3300 customer touch points. Anand will take you through further details on this in a bit.

To conclude the quarter gone by has seen the hospital business emerge stronger while the diagnostic business we expect would see better recovery in the next fiscal. We continue to invest in expanding our bed capacity through brownfield expansion across our network and introduction of new medical technologies. It is also important to highlight that while we are in a relatively stronger financial position purvey to focus on large inorganic opportunities to drive our next phase of growth we would need to adequately calibrate our capital requirements and the need for the same. This would help us to further expand our reach and accessibility and enable us to drive future performance and value enhancement for all our stakeholders. Thank you for your time today and with that I will hand over to Anand for the diagnostic business update. Thank you.

Anand K: Thank you Dr. Raghuvanshi. A very good morning to everyone on the call. Thank you for joining us today. On behalf of SRL Diagnostics I warmly welcome you all to our Q3 FY2023 results conference call. During the quarter we reported a revenue of 332 Crores with 97% of our revenues coming from non-COVID testing. Our EBITDA stands at 71 Crores with a margin of 21.5% for Q3 FY2023. We conducted 9.4 million tests and serviced 4 million patients during this quarter. SRL's B2C to B2B revenue mix stands at 55:45 consistent with our Q1 and Q2 ratios. Our non-COVID business revenue registered a

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Page 5 of 17 growth of 7% as compared to Q3 of FY2022 and 12% for the nine months FY2023 period versus the same period last year. Our preventive health portfolio posted a growth of 20% in this quarter versus Q3 FY2022 and 30% in the nine months FY2023 compared to the same last year. Keeping in line with our network expansion strategy we added 362 new customer touch points in Q3 2023 taking our total number of CTPs to 3317. We also added seven new laboratories in strategic markets during Q3 of FY2023. These labs help deliver a quick turnaround time to customers in these markets thereby making us the preferred laboratory choice for their testing needs.

Our customer NPS is constant at 82 for the quarter. Apart from delivering accurate and timely test reports we have also built a convenient customer experience process at our centre for home visits and across our mobile app and website. On the people front we have always prided ourselves on a strong internal learning and development programme. One of our endeavors is to be the preferred employer in the diagnostic industry and we are progressing well on the NPS parameters for the great place to work assessment. SRL has been a pioneer in setting up a robust research and development. Our R&D team undertakes clinical research studies, co-marketing projects, contract validation studies, and collaborative studies for comparing diagnostics. We are also in the process of registering our R&D department for contract evaluation of IVD kits under the CDSCO as well as acquiring membership of the INSACOG. Under clinical research and co-marketing studies we have assessed feasibility of 22 studies and eight studies have been awarded to SRL. We are progressing on contract validations and collaborations for publications. We participated in 13 scientific and academy conferences across the country and this has helped us strengthen brand recall and our relationship with doctors. As an organization we have always been driven by the science of diagnostics and therefore we have built a sustained focus on genomics and next generation diagnostics. Revenue from genomics shows 29% growth and number of tests increased by 22% versus Q2 of FY2023. During the nine months our genomics revenue went up by 43% and number of tests increased by 36% compared to the same period last year. We will continue our concerted efforts in the chronic diseases category, lifestyle diseases, preventive healthcare category and our specialize d portfolio that comprises of autoimmune diseases, transplant immunology, infectious diseases and oncology. Thank you for your attention. I would like to hand over the call now to Mr. Anurag Kalra, Head of Investor Relations.

Anurag Kalra : Thank s Anand. Ladies and gentlemen we will now open the floor for question and answers. May I request the moderator to please begin the session.

Moderator: Thank you very much. We will now begin the question and answer session. The first question is from the line of Amit Khetan from Laburnum Capital. Please go ahead.

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Page 6 of 17 Amit Khetan : Hi good morning and thanks for taking my question. So just wanted clarity on the number of operating beds so if I look at your Q2 presentation we had about 4000 operating beds this has gone down marginally this quarter despite opening 200 bed hospital in greater Noida so if you could just clarify what is going on?

Vivek Goyal : Greater Noida being the O & M so we are not including that in the total overall number.

Amit Khetan : Got it and the income there how is that reported if that is just like a fee income?

Vivek Goyal : It is just like whatever O&M fees we are getting it is getting into the operating income.

Amit Khetan : You do not include this as part of ARPOB and all that

?

Vivek Goyal : Not really.

Amit Khetan : Got it. Thank you.

Moderator : Thank you. The next question is from the line of Kunal Randeria from Nuvama. Please go ahead.

Kunal Randeria : Good morning and thanks for giving the opportunity. Sir on the hospital side on the more profitable hospitals that SRL continue to better their performance so I guess now in future if you want to continue to increase your margin this will depend on how some of the laggers perform so just want to know couple of things initiatives that you have taken and if already you have started to see some impact in the operational performance?

Ashutosh Raghuvanshi : Answer we have mentioned in the earlier calls also so this is an ongoing exercise and we are now seeing very good results in improvement in the operating margins as well as not only in the percentage term but in the absolute term also so that has become possible one is bringing the operating efficiency, improving productivity and controlling costs to the extent

possible by better negotiation, by improving the processes and things like that. As regard to your question on the bigger unit, so our bigger unit like Mohali, FMRI, Sahalimar Bagh, BG Road and Mulund, these are the five key facilities which continue to improve and are operating at a decent occupancy level so we expect to grow there further. SMRI performance has perhaps further improved because of the higher international business flow into the unit. Having said that the other unit like Noida units, the Ludhiana units are also doing okay, the smaller unit are also doing quite okay so I hope I was able to answer your question.

Kunal Randeria : That was helpful but just to follow up on this if I were to look at the hospital margin metrics now there are three hospitals between 10% and 15% now these are at 65% occupancy, the Fortis Healthcare Limited
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Page 7 of 17 ARPOB is also pretty much near the company average, so in revenue terms I do not clearly see a lot of upside here are there scope for cost cuts in these hospitals or you believe that these hospitals remain in this 10 and 15% kind of range?

Vivek Goyal : So 10 to 15% hospitals are fine, nine months so these are three hospitals, two are very small hospital one is mother child hospital in GK and other is CH road in Bengaluru and third is FEHI. So FEHI one as we are saying it is reaching double digit margin and it is operating at that level and we are taking some more steps to bringing it closer to 15% EBITDA margins in the near future so that is where we are. I think there are steps which we are taking from example in CH Road there is some renovation we have done and that has really impacted the performance of this unit which has now been completed. It has not only given us some more operating there but the look and feel of the hospital has also improved so we are hoping that this hospital will very soon be moving towards 15% EBITDA margin category.
Kunal Randeria : Got it. So I have a couple of questions on diagnostics to do so in this quarter there is a healthy growth in realization per patient which you attribute to test mix can you continue at these levels or you see it increasing or just preventive test that you have mentioned that has driven some growth in the business is that contributing some colour on that would be helpful?

Anand K : This preventive health portfolio has grown by about 30% on this nine months but also what you can see is that our ARPT has shown a higher growth because last year we had the HPPP business as part of our Q3 and this year we do not have and as you know that the PPP businesses the ARPT is much lower than the overall ARP T so that has also contributed to extent but otherwise it is the overall test mix that has changed so I think overall I would say about 2 to 3% contribution from ARPT will be there in the coming quarters as well.
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Kunal Randeria : Preventive contribution that could be how much of your diagnostic revenue and how does the realization differ from sickness?

Anand K : So it is about 10% of the total revenue as far as preventive wellness is concerned as part of the total one.

Kunal Randeria : The realization would be able 20 to 30% higher in this preventive segment?

Anand K : ARPA will be higher but because it is being a more of a bundled offering so the realizations per patient will be higher.

Kunal Randeria : I will have a few more questions. I will get back in the queue.

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Page 8 of 17 Moderator : Thank you. The next question from the line of Shyam Srinivasan from Goldman Sachs . Please go ahead.

Shyam Srinivasan : Good morning and thank you for taking my question . I just want to understand how we should look at occupancy trends for say 4Q or what we are trying to project or acquire for FY2024 in terms of utilization improvements and if you could also understand if you could split it into either mix , local patient market share as well as international patient market share if you could help us understand where those utilization improvements could come from?

Ashutosh Raghuvanshi : Occupancy numbers are definitely trending little higher than the previous quarter but not in a very large quantum . Regarding the international business as we said that it has grown which in the previous quarter was about 6 to 7% has grown up to about 9% so that trend is continuing and is likely to continue this quarter as well . Some of the units especially in the northern belt in Punjab as well as little bit towards the NCR also I think I see winter effect so that also as winter goes away generally this quarter we should see upswing there as well .

Shyam Srinivasan : Dr. Ashutosh we earlier had an aspiration to reach 70% like on full year basis let us assume FY2024 do you think we are on track for that?

Ashutosh Raghuvanshi : Most of the larger hospitals are tracking 70 or about but there are many units in the system which are still at a lower occupancy level . One of the larger units which is still at slightly

lower occupancy is Bengaluru where the occupancy levels are about 55 -56% so there we expect some upside because some new clinical talent and new programme and technology has been added recently .

Vivek Goyal: Shyam if you look at the margin matrix also about 2800 beds in the top three EBITDA buckets are already trending as Dr. Raghuvanshi mentioned between 71 and 72% and another about 500 beds are 65 it is just the bottom category beds which are lower which bring down the overall company occupancy.

Shyam Srinivasan : That is helpful . Second point is on clinical talent Dr. Ashutosh you actually started off by saying in your opening remarks that you have managed to attract talent so if you could walk us through what is happening on clinical talent is there a bidding war if I can say how are you in terms of compensation either clinicians' fee or employee fee, are you seeing any kind of an inflation there which is probably something that we need to compete in from an HR or talent perspective ?

Ashutosh Raghuvanshi : Right now the situation seems to be pretty stable not too many moments are happening nor is there any kind of a bidding war but of course in future that can happen . The slightly

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Page 9 of 17 higher payouts on the doctors' fee what you see is related to the surgical revenues being higher so at the moment we are not seeing any great pressure on this front.

Shyam Srinivasan : Dr. Ashutosh single digit attrition if we were to measure some of the numbers ?

Ashutosh Raghuvanshi : Among the senior doctors we see attrition of about less than 6% which is even lower actually in some of the categories .

Shyam Srinivasan : Got it. Thank you and my last question is

on the SRL bit non-COVID. I think we witnessed

about 7% non-COVID revenue growth remind me there is no issue of DDRC or JV right is it a clean number is what I am trying to see at 7% , is it organic growth including say DDRC in the past and currently ?

Anand K : It is like -to-like growth because we consolidated DDRC in April 2021 so the entire FY20 22 has DDRC numbers so it is a 7% growth on quarter -to-quarter basis .

Shyam Srinivasan : If you could help us and disaggregate that 7 into volume and price because I do not think we can infer directly using whatever reported numbers so what you are seeing on volume trends and different listed players have given different numbers in the sense between 7 to 10% volume growth has come so what are we seeing for our SRL non-COVID business ?

Anand K : On the non-COVID business if you see compared to last year's quarter as I was referring earlier as well so we had Himachal Pradesh PPP business which was a high volume low ARPT business so that businesses is not there with us now , so if you remove that our growth is about 11% so out of that if you see the volume is about 5% growth on volumes and accordingly you can see that ARPT growth is there to that extent.

Shyam Srinivasan : So 50% coming from volume, 50% from price looks like or mix change or whatever ?

Anand K : There is no price increase it is a mix change.

Shyam Srinivasan : What is the outlook here from a volume development perspective so FY2024 do we think we go back to a high single digit or a low double digit volume environment for this SRL?

Anand K : So overall if you see we are growing the volume without HPPP non-COVID volumes are growing at about 12% so I think a similar trend will continue here in the range of 10 to 12%.

Shyam Srinivasan : Got it. Thank you so much and all the best .

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Page 10 of 17 Moderator : Thank you . The next question is from the line of Neha Manpuria from Bank of America . Please go ahead .

Neha Manpuria : Thank you for taking my question. Dr. Raghuvanshi in terms of the expansion at Fortis I know we have brownfield expansion plan which are underway but as we look at let us say growth over the next two or three years are there any pockets where we see the need to probably add more capacity in terms of greenfield capacity or M &A based on your presence would you like to see more capacity in some markets and are we pursuing that more aggressively versus the past?

Ashutosh Raghuvanshi : Essentially our focus has been on the brown field expansion because it is a low hanging fruit comes at a lesser capex and has relatively shorter period in which it gets into a profitable zone . Having said that you are right that now the time has come where we have to think of inorganic expansion as well and we certainly are looking for opportunities which will be complementary and synergistic to our existing operations as we have said earlier that in the given geographies clusters where we are present and/or fully formed cluster if it was to be in some other geographies which is attractive so those are the things which we are going to look at . Some of the cities like NCR , Bengaluru and Mumbai are our preferred geographies

where we are looking for possible opportunity .

Neha Manpuria : Based on your assessments valuation for asset available has that come of over the last few years I know at one point of time it was fairly unreasonable or you do not think there has been any change in valuations for assets available ?

Ashutosh Raghuvanshi : The expectations are still quite high in most cases but I think it is for us to evaluate whether it is a project where we can add further value if that is a possibility then of course even in some of the prime location even if the asset was to be of a high value but we think that we can add further value we would consider those but I do not think

things have changed

dramatically in terms of expectations in the hospital space for valuation .

Neha Manpuria : Understood and my second question is in relation to the brownfield expansion that we are doing does this brownfield bed addition allow us to let us say improve ARPOB and I am asking this from a point of view of are you adding this capacity to let us say increase or high ARPOB specialty mix and I should see these beds as being high realization versus the average realization in that particular hospital would that be a fair way to look at our brownfield bed expansion ?

Ashutosh Raghuvanshi : Yes essentially most of these bed what they would lead to do is that the surgical work which is typically a higher ARPOB business is likely to grow so the net effect would be that; however, it is not necessarily that we are building it for that reason alone. The second

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Page 11 of 17 thing which these things are doing is that some of the hospitals which are older they are also simultaneously getting refurbished and the look and feel of the facility is improving, service standards will be improving and as a result of all those factors I believe that the ARPOB will certainly improve.

Neha Manpuria : Understood and my last question is on the payer mix I think if I look at the chart our sort of government business is roughly at about 20% if I go back in history this used to be sub 15% right so is there efforts to sort of increase our cash and TPA mix thereby reducing the payer mix in these institutional government business ?

Ashutosh Raghuvanshi : Yes you are absolutely right . Some of these categories have gone higher and that is because of some newer schemes coming in Rajasthan for instance, etc., which we had to take and in West Bengal, but we are calibrating this and again our aim has always been to keep it around 15% and we want to go to wards that but it may not happen in one or two quarters but definitely over the next two or three quarters we should get it back to about 15 % or lower.

Neha Manpuria : Understood thank you so much for taking my questions Sir.

Moderator : Thank you. The next question is from the line of Dheeresh Pathak from White Oak Capital .

Please go ahead.

Dheeresh Pathak : Yes thank you for the opportunity . Just to the earlier question you just replied that we had to take the government business in Jaipur and West Bengal can you just please elaborate on that?

Ashutosh Raghuvanshi : There are those schemes in the respective states which cover a very large population and because of the kind of local pressures, etc., we had to sort of open that up. The second factor was typically these two hospitals were working at slightly lower occupancy levels as well so we did have a kind of capacity available and on marginal cost basis it made sense to increase that so we had to increase that because of those things.

Dheeresh Pathak : Is the land there based on some concessions with the government can you just explain the asset ownership there ?

Ashutosh Raghuvanshi : In fact both these places there are no such constraints.

Dheeresh Pathak : It is just matching by the government that you had to take it or you wanted to take it , so you are not clear right now ?

Ashutosh Raghuvanshi : It was more than a nudge I would say .

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Page 12 of 17 Dheeresh Pathak : You want you can drop it right later on , there is no mandate from the government to take these schemes right ?

Ashutosh Raghuvanshi : There is a mandate but mandate is being sort of petitioned initially and then we would see how we can work towards that. I think it is an industry wide issue in these two cities at the moment and initially probably there is going to be more demand also when such a scheme comes and that demand sort of goes down as well so we are pretty hopeful

that we should

be able to calibrate it down to our desired level of 10 to 15%.

Dheeresh Pathak : How much lower are the realizations on these schemes versus the cash and private

insurance patients ?

Ashutosh Raghuvanshi : Typically about 30 to 40% lower.

Dheeresh Pathak : Are these two hospitals also in that 10 to 15% EBITDA margin bucket in number of facilities, three facilities which bucket are they part of, these two hospitals ?

Ashutosh Raghuvanshi : Kolkata for this quarter is in that category but because there was some couple of one off payments otherwise Kolkata is above 20 generally and Jaipur is in the other category which is about 10 to 12%.

Dheeresh Pathak : Out of three facilities between 10 - to 15% margin bucket one is Jaipur which are the other two Sir ?

Company Speaker : One is La Femme and the other is CH road and the third one in 10 to 15 is FEHI. The below 10 Jaipur as Dr. Raghuvanshi mentioned is at 10 or below 10.

Dheeresh Pathak : Last time it was 10 to 15 , last time I thought it was a very high occupancy, high catchment area why is last time 10 to 15 %?

Company Speaker : Just in the fringes. It is actually closer to the 15% mark but it is a smaller facility that way.

Dheeresh Pathak : Thank you for taking my question .

Moderator: Thank you. Next followup is from the line of Shyam Srinivasan from Goldman Sachs .

Please go ahead .

Shyam Srinivasan : Thank you for taking the followup . Team is there any update from a legal perspective when is the last hearing at the Delhi High Court and is there some kind of a guided time line on what would happen in the next three or six months ?

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Page 13 of 17 Vivek Kumar Goyal : Actually we are not part of any legal proceedings right now. We are just waiting High Court

to convene a sort of hearing where the decision regarding whether they will be ordering forensic audit or not that decision needs to be taken in that so our lawyer will be present there. There were couple of hearings previously with no conclusion and I think next hearing is on 17th of this month .

Shyam Srinivasan : 17th of Feb ?

Vivek Goyal : Yes 17th of this month.

Shyam Srinivasan : Got it that was helpful. Second question is just on balance sheet the net debt has been reduced so just want to understand the usage of cash that comes through what is our capex for the remainder fourth quarter or even if you want to guide for FY2024 if you could help us understand that please ? Thank you.

Vivek Goyal : The fund from operation were useful and because of that we are able to reduce the debt to the current level. As regard the capex outflow is concerned we are spending at a run rate of 100 to 125 Crores per quarter on the capex and we expect this to go up in next financial year slightly because we are planning to invest in the technology like Dr. Raghuvanshi mentioned in the initial call we want to move in a gradual manner for robotic programme that will require some investment plus this brownfield expansion will also be kicking in so all these things may increase but capital outlay from current run rate of 125 Crores per quarter to maybe 150 Crores per quarter.

Shyam Srinivasan : Got it Sir and last question Dr. Ashutosh started in his opening remarks talking capital allocation and maybe a potentially an inorganic thought process so if you could just clarify that Sir ?

Ashutosh Raghuvanshi : As I was saying earlier to Nehal's question is the possibility of inorganic acquisition is there and we are at this time in a position to consider those so though there is nothing conclusive at the moment but we continue to be open to evaluate those with much more rigor than what earlier we were doing. So in the geographies which are our clusters which are currently operational we are continuing to look if there are any opportunities and of course the question of valuation

comes there so if it at an attractive price point then we would definitely consider them at the moment. Yes we are very open to that now.

Shyam Srinivasan : Last data point. Vadapalani what is the losses it used to be 8 Crores I remember historically ?

Vivek Goyal : It is around same level.

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Page 14 of 17 Shyam Srinivasan : We had like a soft guidance of this breaking even in FY2023 I do not know whether that has moved around ?

Vivek Goyal : It is taking slightly more time looking like so earlier we were expecting to break even in next six months but it may take slightly more time.

Shyam Srinivasan : What is the reason Vivek Sir , Why the delay?

Vivek Goyal : Reason is two fold one is there are some infrastructure work going on in front of the facility

and that is affecting the patient flow and that is the main reason and apart from that I think the clinical talent we were thinking we were able to extract easily we are facing some challenge in getting good clinical talent in that locality.

Shyam Srinivasan : Got it Sir. Thank you.

Moderator : Thank you. The next question is from the line of Tushar Manudhane from Motilal Oswal. Please go ahead.

Tushar Manudhane : Thanks for the opportunity. Sir just on the international patients now as a percentage of sales it is more or less at 9 to 10% versus pre-COVID level so first of all geography wise which geography is these international patients are coming from and how do you see the growth in the terms of volume for this category of patients?

Ashutosh Raghuvanshi : Patients are coming from a kind of widespread geography mainly we get lot of patients from Africa both from east as well as west side but mainly east Africa and some patients from middle east essentially from couple of countries like Yemen as well as from Qatar so those are the larger numbers, other than that there are few specialties like liver transplant, etc., we do get patients from central Asia as well as some times as far off places as Fiji and Philippines as well. As far as the growth and this is concerned I think we have seen a good growth as we said and it should grow further but it is difficult to guess how much more growth will be there on this segment as we continuously see our domestic patients also going up so as a percentage I believe that it is likely to remain in 10 to 11% for some time.

Tushar Manudhane : Got it Sir and just broadly the breakdown of gross block into hospitals and diagnostic business please?

Company Speaker : Diagnostic is not even 10% of the total.

Tushar Manudhane : Roughly we are at 1.2 times asset turn for the hospital business correct?

Company Speaker : Asset turnover we are lower than that.

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Page 15 of 17 Tushar Manudhane : Will be there a scope of improvement in the asset turn when there is improvement in profitability for the hospitals which are yet to reach good matured hospital level or this is what is the asset turn one should think about?

Company Speaker : No, there is improvement possible as Dr. Raghuvanshi mentioned earlier the BG road facility which is the biggest facility operating at around 60% occupancy level and same is the case for another big facility at Mulund in Mumbai so both these facilities are having some capacity sitting there and plus existing capacity also they are operating at 60% so there is opportunity to increase it, apart from these two the Chennai facility both the facilities are operating at suboptimal levels so there also there is a scope.

Tushar Manudhane : Okay Sir that is it. Thank you.

Moderator : Thank you. The next question is from the line of Amit Goela from Rare Enterprises. Please go ahead.

Amit Goela : Good set of numbers. I was just wondering that any kind of M & A or inorganic growth is it preventive from the court proceedings with

high are on now or you are free to do that kind of stuff because now your balance sheet is strong Sir?

Ashutosh Raghuvanshi : No Amitji there is no constraint on that. That is why I said earlier that we are looking at opportunities which come at an attractive valuation and also there is absolutely no constraint otherwise.

Amit Goela : Would you expect the significant improvement in the capacity utilization for this quarter?

Ashutosh Raghuvanshi : So the capacity utilization certainly should be better than the third quarter but it is slightly below our expectation of 75% but certainly it would be better than the previous quarter.

Amit Goela : Okay. Thank you so much. All the very best.

Moderator : Thank you. The next question is from the line of Damayanti Kerai from HSBC. Please go ahead.

Damayanti Kerai : Hi thank you for the opportunity. My question is for Dr. Raghuvanshi so international businesses is focus for most of your peers so for you how do you see this piece moving up in next few years where do you aspire to reach say in two to three years and how do you see competition in this space from the local peers?

Ashutosh Raghuvanshi : You are absolutely this is a competitive space and all the players are focusing on this especially in NCR region and other medical hubs like Chennai, etc., so our main flagship

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Page 16 of 17 facility in Gurugram has a large percentage of international patients coming so as the more capacity we are creating in both Shalimar Bagh as well as in Noida we would have more capacity available to serve these patients, so I expect a steady growth in this segment but that growth should probably take the contribution at an overall basis to about 12 to 13% of the revenues.

Damayanti Kerai : This 12 to 13% you are like hoping to reach in the next two to three years or it can come much earlier ?

Ashutosh Raghuvanshi : Yes it should come within two years.

Damayanti Kerai : Sir how do you see government initiative for pushing this business again I think easing the flow of international patient to India so what is your opinion or view on this Heal in India initiative by government of India ?

Ashutosh Raghuvanshi : Yes so I am quite sort of hopeful that this will help it in a positive way because one of the reasons as you know that it is completely deregulated kind of business and there are multiple agents, etc., who function in this space so once the Heal in India programme is matured then there would be a visibility as to where the patient is coming from and through what channel, etc., so that would make a better playing field and it would to some level control the agents at the current time are sometimes a little unscrupulous to say the least.

Damayanti Kerai : Sir last question for international patient realization against a domestic patient how better it is if we can quantify some numbers say one-and-a-half times two times how do we see realizations here ?

Ashutosh Raghuvanshi : Yes so typically we mark up for the international patients but at the same time the facilitator fee sort of brings it down to a level where it is comparable to the domestic cash patient only so in effect it is not very different from the domestic patient because the mark up essentially goes as facilitator fee.

Damayanti Kerai : Okay Sir that is very helpful. Thank you and wish you all the best.

Moderator : Thank you. Next followup question is from the line of Dheeresh Pathak from White Oak Capital . Please go ahead.

Dheeresh Pathak : Yes thank you just to confirm again in that bucket where the last bucket is less than 10% margin and number of facilities are high so one is Jaipur, Vashi, Arcot Malar, which is the fifth one.

Vivek Goyal : FEHI Bengaluru.

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Page 17 of 17 Dheeresh Pathak : Okay understood . For this SRL margins like

if you were to do Ind AS adjustment if we take

away another 1.5 to 2% of margin right is that a fair understanding ?

Mangesh Shirodkar : The current reported EBITDA is after Ind AS adjustment.

Dheeresh Pathak : I am asking on pre Ind AS 1 16 basis where you have to take the impact of rental in other expense if we do that there will be a lower margin right instead of 20% it will be slightly lower ?

Vivek Goyal : That is right so all our EBITDA are reported as per Ind AS so if we adjust for pre Ind AS the margin will be lower that is right.

Dheeresh Pathak : That impact is roughly 2% of revenue right is that fair understanding because I can work out on total company level I am not able to work out at SRL level ?

Mangesh Shirodkar : I need to check that. It will be in the range of 1 to 2% but exact percentage I will have to check.

Dheeresh Pathak : Okay thank you Sir.

Moderator : Thank you very much. I now hand the conference over to the management for closing comments .

Anurag Kalra : Ladies and gentlemen thank you very much for joining us on the call today . I just want to mention that some of the statements and discussions by the management is also mentioned in the IR presentation and press release may be forward looking in nature and may involve for certain element of risk and uncertainty kindly keep in mind the disclaimer that we mentioned in the presentation and press release. Gaurav and I are here to answer any further questions or clarifications you may have . Please feel free to e-mail us or talk to us. Thank you very much and have a good day.

Moderator : Thank you very much. On behalf of Fortis Healthcare Limited that concludes this conference . Thank you for joining us . You may now disconnect your lines .

Call: May 2023

FORTIS HEALTHCARE LIMITED

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May 26 , 2023

FHL/SEC/202 3-24

The National Stock Exchange of India Ltd.

Scrip Symbol: FORTIS BSE Limited

Scrip Code:532843

Sub: Transcript of Investors / Analysts' meet under Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015

Dear Madam / Sir,

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed herewith the Transcript of the investors / analysts meet held on May 24 , 2023 to discuss the Company's audited financial results for the quarter and financial year ended March 31, 2023 . Further, the said transcript is also available on the website of the Company i.e. <https://www.fortishealthcare.com/drupal-data/2023-05/Earnings%20Call%20Transcript%20for%20the%20quarter%20and%20year%20ended%20March%202023.pdf>

This is for your kind information and record .

Thanking you,

Yours Faithfully

For Fortis Healthcare Limited

Murlee Manohar Jain

Company Secretary & Compliance Officer

M. No. F9598

Encl: a/a

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"Fortis Healthcare Limited

Q4 FY'23 Post-Results Conference Call "

May 24, 2023

MANAGEMENT : DR. ASHUTOSH RAGHUVANSHI – MANAGING DIRECTOR
& CHIEF EXECUTIVE OFFICER , FORTIS HEALTHCARE

LIMITED .

MR. VIVEK GOYAL – CHIEF FINANCIAL OFFICER ,
FORTIS HEALTHCARE LIMITED .

MR. ANAND K. – CHIEF EXECUTIVE OFFICER , SRL.

MR. MANGESH SHIRODKAR – CHIEF FINANCIAL
OFFICER , SRL.

MR. ANURAG KALRA – SENIOR VICE PRESIDENT OF
INVESTOR RELATIONS , FORTIS HEALTHCARE LIMITED .

Fortis Healthcare Limited

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Moderator : Ladies and gentlemen, good day and welcome to the Q4FY'23 and FY'23 Post -Results
Conference Call of Fortis Healthcare Limited.

As a reminder, all participant lines will be in the listen -only mode and there will be an
opportunity for you to ask questions after the presentation concludes. Should you need assistance
during the conference call, please signal an operator by pressing '*' then '0' on your touchtone
phone.

I now hand the conference over to Mr. Anurag Kalra – Senior Vice President , Investor Relations
at Fortis Healthcare Limited. Thank you , and over to you Mr. Kalra .

Anurag Kalra : Very good morning and good afternoon ladies and gentlemen and welcome to Fortis Healthcare's
Q4FY'23 and FY'23 Results call. We have on the call today Dr. Ashutosh Raghuvanshi -
Managing Director and CEO , with him we have Mr. Vivek Goyal - Chief Financial Officer of
Fortis . From SRL we have Mr. Anand - CEO of SRL and with him is Mangesh Shirodkar - Chief
Financial Officer of SRL.

We will start the presentation by some opening comments by Dr. Raghuvanshi followed by some
highlights of the operational diagnostics business that Anand will take you through . And then
we can open the floor for question and answer.

I do hope all of you have got a chance to look at our Investor Presentation and Press Release .

And I just wanted to also state that the discussion today will be subject to the forward -looking
disclosures that we have as we mentioned in both the press release and the presentation. With
that I hand over to Dr. Ashutosh Raghuvanshi .

Ashutosh Raghuvanshi : A very good morning and good afternoon, everyone. Thank you for taking time to join us on our
Q4 Financial Year '23 Earnings Call. I hope all of you are doing well.

Before I delve into Quarter 4 and the full financial year results I am pleased to inform you that
our Board has recommended a maiden dividend of INR 1 per share, 10% of face value subject
to the approval of shareholders. This signifies the transformational journey that the company has
undergone in last few years , and as a result the healthy operational and financial performance
that we have witnessed in our business.

Yearly Performance Update

For the Financial Year 2023 consolidated revenues for the Company stood at INR 6,298 crores
compared to INR 5,718 crores in Financial Year '22 a growth of 10%.

Our hospital business revenue for the year have grown 19.8% to INR 5,107 crores from INR
4,264 crores in Financial Year '22. Our diagnostic business has witnessed a muted performance

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with the significant decline in COVID volumes. Revenue for the diagnostic business recorded a
decline of 16% to reach INR 1,347 crores in Financial Year '23.

A consolidated EBITDA stood at INR 1,163 crores compared to INR 1,096 crores in Financial
Year '22. This translates into a margin of 18.5% in Financial Year '23 versus 19.2% in Financial
Year '22. Our hospital business margins have strengthened from 15.8% in Financial Year '22 to
18.1% in Financial Year '23 clocking an EBITDA of INR 922 crores. However margins in the
diagnostic business were at 19.5% versus 26.5% in Financial Year '22, a drop of almost 700
basis points impacting our overall consolidated margins .

Consolidated profit before tax, before exceptional items was INR 740 crores compared to INR
673 crores in Financial Year '22. Largely contributed by the performance of the hospital
business, consolidated profit after tax before exceptional items stood at INR 559 crores versus
INR 475 crores in Financial Year '22 a growth of 18%.

Quarterly Performance Update

We recorded a consolidated top -line of INR 1,643 crores in Quarter 4, a growth of 19.2%. The
hospital business revenue grew a robust 29.7% to INR 1,351 crores compared to INR 1,041
crores. While the diagnostic

business revenue decline 10.8% to INR 332 crores in Quarter 4 of Financial Year '23.

Our consolidated EBITDA was at INR 285 crores versus INR 227 crores in Quarter 4 of Financial Year '22 translating into a margin of 17.3% versus 16.5% in Financial Year '22. EBITDA for the hospital business stood at INR 230 crores compared to INR 144 crores in Quarter 4 of Financial Year '22 with EBITDA margins at 17% versus 13.8% in Quarter 4 of Financial Year '22. EBITDA for the diagnostic business stood at INR 55 crores compared to INR 84 crores with margins at 16.5% versus 22.5% in Quarter 4 of Financial Year '22, primarily due to the fall in COVID volumes.

Consolidated PBT before exceptional items was INR 173 crores in Quarter 4 of Financial Year '23 an increase of 37% compared to Quarter 4 of Financial Year '22. Our profit after tax before exceptional item increased 47% to INR 128 crores in Quarter 4 of Financial Year '23.

On the balance sheet side of things, we have a healthy net debt to EBITDA of 0.29 versus 0.6 over the corresponding previous period, that is Quarter 4 of Financial Year '22. Our net debt stands at INR 330 crores as of 31st of March 2023 compared to INR 549 crore as on 31st March, 2022. Our finance costs are also lower by 12% both as a result of debt reduction and the lower weight of borrowing.

Highlights of Hospital Business for Financial Year '23.

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Our hospital business had an average occupancy of 67% compared to 63% in Financial Year '22. Our ARPOB witnessed a growth of 11.5% to INR 2.01 crores in Financial Year '23 versus INR 1.8 crore in Financial Year '22. The growth in ARPOB was led by a healthy increase in our revenues from key focus specialties such as Oncology, Cardiac Sciences, Neuro Sciences, Renal Sciences, Gastroenterology and Orthopedics which cumulatively grew 31% in Financial Year '23. The other important driver of ARPOB was the higher contribution for surgical volumes seen in Financial Year '23 which stood at 59% versus 53% in Financial Year '22.

In addition to the above there was strong traction from our international business revenue which grew 98% in Financial Year '23 to INR 425 crores. The revenue contribution of international business stood at 8.3% in Financial Year '23 versus 5% in Financial Year '22.

Our focus on strengthening our clinical programs continue through the year across all our facilities in terms of investing in high end medical infrastructure and onboarding medical talent.

The year witnessed the addition of several eminent revisions across various specialties like Cardiac Sciences, Oncology, Neuro Sciences, Gastroenterology and Orthopedics.

Some of the key medical equipment that we installed during the year included LINAC, PET/CT, Da Vinci Surgical Robots, Cath Labs, Neuronavigation systems and Orthopedic robots. Just to put some numbers in context for this, out of total CAPEX spent of about INR 300 crores in Financial Year '23 approximately INR 200 crore has been incurred for medical equipment.

Amongst the other key strategic growth initiatives that we have spoken off before is our Brownfield bed expansion plans. We have added approximately 140 beds across our key facilities including FMRI, BG Road, Mulund, Mohali, Ludhiana and Nagenahalli in Bangalore. Our plans to add close to 1300 to 1400 bed over the next four to five years are on track, as we have said before our growth would comprise not only of our Brownfield expansion, but also efforts to expand our size and scale through inorganic efforts. This is evidence in our timing of a definitive agreement with the VPS Group for the acquisition of hospital asset of Medeor Hospital in Manesar, Gurugram which could potentially add 350 beds to our network. We further expanded our presence with the launch of a 200 bedded multispecialty hospital in Greater Noida under an O&M arrangement. In addition,

we also commissioned a state-of-the-art Cancer Daycare Centre at Defense Colony in New Delhi.

On our Digital Transformation Initiatives, one of the key highlights in Financial Year '23 was the launch of an Electronic Medical Record system that we believe would eventually transform the way patient's health records and databases are stored and shared and accessed by relevant stakeholders. This has the potential to significantly enhance our patient experience and service parameter.

With respect to cost we have achieved reasonable traction, cost optimization initiatives, primary related to drug and consumables and consumption optimization have also contributed to the hospital business margin improvement. Like I have said before, cost optimization across

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functions and expense line is an ongoing process and we are actually conscious that this can be an important contributor to our profitability going forward.

Some thoughts on our diagnostic business, an important highlight was that while our overall

diagnostic business saw decline, our non-COVID business revenue grew 12% both for the quarter and Financial Year '23 respectively. The diagnostic business environments remain challenging at present, but SRL continues to work towards expanding its customer touch points, have added more than 1,100 customer touch points in Financial Year '23. This takes the total number of customer touch points to approximately 3500 plus as on date. The business conducted 39.1 million tests during Financial Year '23 as against 44.2 million tests in Financial Year '22. Anand will take you through further details on the performance for the year and the quarter. With that I would now like to conclude my comments. Thank you all for your continuing support. And on behalf of my team and I want to assure you that we remain committed to our growth objectives and will ensure that all efforts are made to better our business performance going forward.

I shall now hand it over to Anand for his thoughts on SRL.

Anand K.: A very good morning and good afternoon to everyone on the call, thank you for joining us today. On behalf of SRL Diagnostics, I warmly welcome you all for our Q4FY2023 results conference call.

During the quarter, we reported a revenue of Rs. 332 crores with 97% of our revenues coming from non-COVID testing. Our reported EBITDA stands at Rs. 54.7 crores with a margin of 16.5% for Q4 FY'23. We conducted 9.8 million tests and serviced 4 million patients during the quarter. Non-COVID revenue has posted a growth of 12% over Q4 of '22 and 41% growth over Q4 of '20. Revenue from genomics showed 36% growth in Q4 of '23 versus Q4 of '22, while our preventive testing portfolio has grown by 27% in Q4 of '23 versus Q4 of '22.

For the Financial Year FY'23 SRL reported net revenue of Rs. 1,347 crores. The company's reported EBITDA for the year stood at Rs. 263 crores representing a margin of 19.5%. The business served a total of 16.6 million patients and completed 39.1 million tests during the year. Our preventive portfolio revenues showed a growth of 29% and the genomics revenues showed a growth of 41% in FY'23 compared to FY'22. We have added 1100 plus CTPs and 53 new laboratories during this year. The B2C, B2B revenue mix stands at 54% to 46% in FY'23 compared to 55%, 45% in FY'22. The business continues to have a well-diversified geographical mix with no overdependence on any region allowing it to capitalize on a PAN India network optimally.

Our workforce of over (+7000) employees across our network are the pillars of our strength.

One of the company's core areas of emphasis is learning and development. In recent years, SRL Fortis Healthcare Limited

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has advanced to provide a variety of specialized Competency Enhancement Programs. About 5200 mandates were launched by the c

ompany in learning and development initiatives during

FY'23. We rolled out the Future Ready leadership development program and strength-based leadership development program for SRL identified high potential employees.

We continually expand our tests menu in a way that not only meets the needs of diagnostic options for the present but also keeps us future ready. We are happy to have added over 200 new tests to our test menu in FY'23. We have also launched an online directory of services on the website that helps patients and doctors get the necessary information on our test menu with a simple search.

Digital transformation has been one of our priority areas in FY'23. Our app install numbers have grown by eight lakhs and our website recorded eight million visits in the year. Our customer NPS improved from 74 in FY'22 to 80 in FY'23. Apart from delivering accurate and timely test reports we have also built a convenient customer experience process at our centers for home visits and across our mobile app and website.

SRL operates one of the largest accredited network of laboratories in the country 43 of our labs are accredited by NABL and three of our labs are recognized under the NABL MELT Program. We also have two CAP accredited laboratories in Mumbai and Dubai.

Being prepared for the future is one of our corporate goals. The transition to next generation diagnostics, digitization of diagnostics, along the value chain and the customer demand for omnichannel and on-demand services in our opinion are the three major teams that will -- the diagnostic industry's mid-term future.

By enhancing our expertise in next generation diagnostics, we are staying ahead of the curve.

We are also optimizing our processes through digitization, during sample collection and logistics, Laboratory Automation and Pre-Analytical Phase and in Reporting. The current trend inclines towards consumer choices, consumer empowerment and consumer wellness and we are cognizant of this ever-changing landscape and are making the right strides in this direction.

Finally, I take this opportunity to thank you all for your trust in SRL. I would like to hand over the call now to Mr. Anurag Kalra, Head of Investor Relations.

Moderator: Thank you very much. We will now begin the question-and-answer session. Our first question

is from the line of Kunal Randeria from Nuvama. Please go ahead.

Kunal Randeria : Firstly, on SRL, some of your peers have taken price increases on some of the tests in their portfolio . Have you taken it and if so how much and on what percent of your portfolio?

Anand K.: So, we have still not taken any price increase on our portfolio so far. So, we are evaluating the options and probably look at them during the mid-year.

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Kunal Randeria : I mean if I look at the SRL business as a whole your realization seem to be holding steady. If I were to compare it to pre -COVID years there has been a very sharp margin compression. But at the same time some of your p eers have been able to maintain margins . So, it seems the cost structure might be a bit bloated . So, just if you want to understand what are some of the operational changes that maybe we would like to get to th e SRL business?

Anand K.: No, on our margins what you are asking is the pre -COVID margins are different from what is that I didn't get what you are asking.

Kunal Randeria : So, I mean in FY'19 or even FY'20 you use to clock somewhere around 20%, 21 % kind of margins now in Q4 of this year it's for 16.5% . Your realization are fairly steady so it seems that the issue might be somewhere at the OPEX level. So, just want to understand what SRL has done d ifferently to some of the payers who seem to have la rgely maintain ed some of the margin trends?

Anand K.: So, in FY'19 we were at about 20%, in FY'20 we are at 18% of margins. And in FY'23 our margins are now at about 19%. So, the Q4 has some exceptional items which could have got that drop in that particul ar quarter. But we also have to note that we have grown our infrastructure and we are working on all that effects to make sure that we continue to grow on their margins as well.

Mangesh Shirodkar : We can add here, so Q4, Kunal , the CSR for the entire year of 5 CR, and secondly we had taken, there was one of the SPPP the government contract we had taken a provision as per ECL method of around 6.5 CR in this quarter. These are the two one -offs in this quarter though we are confident of getting payment from HP Government. Our on a conservative basis as per EC L method we have provided for this. The se are the two abnormal in the quarter because of which the percentage, the margins may look lower.

Kunal Randeri a: Would you like to call out that number?

Manges h Shirodkar : On 6.5 CR, as provision for doubtful debts in HP and 5.1 CR in CSR.

Kunal Randeria : And just one more on the hospital side, in the last few quarters your margin seem to have steady at around between 16.5% to 17.5% kind of levels. I think now t he way forward could either be some of the less than 10% kind of hospitals start ramping up. So, just want to understand what should we expect in the next 18 to 24 months in this business?

Vivek Kumar Goyal : So, as you rightly said, our margins are steadily moving forward actually. And we have achieved now 17% margin for this financial year. So, our aim is to quickly move to the 20 % margin. It is now a question of whether we will able to achieve this year or next year.

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Kunal Randeria : I understand your guidance but what I am trying to understand that where will it come from . So, that 300 bps margin expansion so I am not even holding on to FY'24, it could be even '25, but I am just asking where it be come from?

Vivek Kumar Goyal : Yes, so there will be different levers for a margin improvement, one is obviously the improvement in the payer mix and specialty mix. So, payer mix as I was mentioning earlier also, we have government patients of around 20% so we are wor king towards improving that payer mix slowly. It will happen gradually. So, that will give us improvement in the ARPOB.

The second lever is of course the specialty mix, some of the specialties are more focused specialties for us like onco and neuro and car dio of course. And the third lever is of course the cost lever. We are working constantly to reduce the cost structure and optimize the cost structure and that should also help us in improving the margin, so all three lever will add.

And fourthly around th e expansion side as Dr. Raghuvanshi mention in his initial comments, we are expanding our brownfield bed capacity. And this way we will able to achieve the economy of the scale which will help us in improvement in the margin.

And lastly on the portfolio rationalization side, there are certain units we have discussed in the past which we are trying to rationalize our portfolio where we feel there is very little chance of improved by us. And that's why those loss-making unit we are planning to divest and that will further give a boost to the EBITDA margin.

Kunal Randeria : And if I can just follow up on the point that you made on payer mix interestingly because see I mean if I were to see a payer mix today around 18%, 19% would be coming from government CGHS and ECHS. But even four, five years back the mix was fairly similar, so you are confident

that you can reduce your footprint over here going forward, is my understanding correct?

Vivek Kumar Goyal : Yes, so there is some improvement and it needs to be seen unit wise because there are units who are operating for example at 60% occupancy level. So, there is no reason why this would be reducing our government payer there. But there are certain units especially

in the NCR region

where the payer mix, the at occupancy level is at around 80%. And there we feel there is opportunities to rationalize this payer mix to the extent possible. And as I said in the beginning it may not be very prompt.

And lastly on the payer mix, the unit where we are going for expansion and we know that there will be substantial capacity we will be adding there we are going slightly slow on this shifting because quick ramp up of those brownfield expansion program.

Moderator : Thank you. Our next question is from the line of Shyam Srinivasan from Goldman Sachs. Please go ahead.

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Shyam Srinivasan : Just the first one on the growth we have witnessed in the hospital witness and what's the outlook for say Fiscal '24 over '23, about 20% growth we have seen. So, how should we look at Fiscal '24 revenue growth? And if you could disaggregate that into either utilization, ARPOB has grown 11.5% so just want to understand the outlook on your revenue growth for the hospital business for '24?

Vivek Kumar Goyal : So, Shyam on this question, last year means '22 was affected because of COVID and that's why you have seen this 20% type of growth rate. I don't think we will be able to achieve that type of number, but the double-digit growth we are aiming for '24, and we are quite confident we will be able to achieve that.

To your second question we are expecting ARPOB growth of around 6% to 7% in the current year, because ARPOB level has settled at a, we have seen a significant increase in the ARPOB in last couple of years. So, we feel the ARPOB increase may be limited.

The third lever for growth will be the occupancy ramp up so we are at 67% occupancy level so we are targeting at all over hospital level we should be achieving at 70% occupancy level and that should give us the major growth, plus the additional bed capacity we are adding. So, all put together I think we are targeting around 11%, 12% growth rate for the '24.

Shyam Srinivasan : If I were to look at the surgical mix 59% of hospital revenue. Can we see this grow further or you being in some of the key therapies that you have been tracking, I mean there is a nice table that you have given now on what is the individual therapy level growth. So, which are the areas that you can see further expansion in say next year?

Ashutosh Raghuvanshi : So, Shyam as far as the mix is concerned I don't expect a dramatic shift this year. However, there is definitely a phenomena where hospitals such as ours are attracting more quaternary care kind of work so that is why this surgical numbers are growing. As far as specialties are concerned we are seeing quite a significant growth on the Oncology side and in the Oncology surgery component, we are expecting that we would see a similar kind of increase happening this year as well. But overall, as far as the ratios are concerned it would probably not change too dramatically.

Shyam Srinivasan : And if I can quickly squeeze in last one on non-COVID volumes, I know there is a degrowth on the overall headline number for volume growth. But Anand if you could help us understand what's happening on the non-COVID volume, I know value is 12% growth, but just disaggregating that into volume and realization what's happening there.

Anand K. : So, on the non-COVID volumes, Shyam so on like-to-like basis if you look at it we have seen about 15% growth in the non-COVID volume. But because in the last year we had the HP PPP, which was a high contributor to the volume so that contract was not there in this year, in FY'23 so that has led to some shrinkage in the volume because of that. But if you take like-to-like, there is a 15% growth in volume.

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Shyam Srinivasan : And Anand, we are not this contract is not coming right so in '24 we will have complete and

when did this we lose this contract or stopped this contract?

Anand K. : This was in May of FY' 22 so which means that May '23 up to May '22 we had this contract. Only two months of FY'23 had revenues of this contract whereas in FY'24 it will not be there completely.

Moderator : Thank you. Our next question is from the line of Nitin Agarwal from DAM Capital. Please go ahead.

Nitin Agarwal : On the expansion plans for the hospitals, I mean a) if you can probably help us understand more about the Manesar asset that you have acquired, what opportunities you see for that asset and given the fact that it is after a down in Gurgaon I mean what kind of, is it a very useful catchment area versus as for example the FMRI sort of hospital.

And secondly on going forward basis what are the kind of expansion plans do we have in mind apart, you have highlighted some of the bed expansion thoughts but if you can elaborate more about how we are thinking about expansion for the next couple of years?

Ashutosh Raghuvanshi : Yes, so far as Manesar is concerned it is an upcoming area and there is lot of new residential developments as well as infrastructure development which is happening . There is a new highway which is express way which is connecting Delhi to the National Highway that is almost nearing completion. And as a result of that the uptake of the residential apartments in the vicinity in that area being more affordable is happening quite rapidly.

We see that this area will be quite a prime area in the next few years because of the physical infrastructure of the highways and, etc., which is developing and is getting ready. So, that's why it is an important position for us to take within the Gurgaon micro-market . Having these two positions are very important one is present and the other is future.

Then it is a brownfield acquisition so within the next 12 months we should be able to once we have concluded the closure of the deal , within 12 months we should be able to get the hospital going with about 150 beds in the first phase. And then we will take into 350 beds. This is a full service hospital and it serves the lot of hinterland of Rewari etc. as well. So, it is not a small market but it is a large market which is growing very rapidly. So, that is why we are very excited about this.

Similarly the brownfield expansions which we are doing, some of that is almost ready . As we had said that this would take three to five years to do this, about 1300 to 1400 beds, this is very much on track in various stages of operationalizing. So, in Mumbai for instance, the beds about 100 beds are ready and part of that about 45 beds have already been commissioned. As a matter of fact we inaugurated that floor just last week.

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Similarly in NCR in the Noida facility the construction began begun , about few months back and it is absolutely on track. Faridabad the construction has been on for about a year and we expect to complete that within the next six months. So, capacity should be added by the end of this year.

Similarly , we have the finalization, we have the construction which has started in FMRI facility itself and that would add about 180 beds, this would take about two years or two and a half years I would say to complete. Similar is the timeframe for Shalimar Bagh where we are in the process of getting our plans approved and that is almost in the final stages. So, very soon we expect to begin that construction as well.

So, all these are very much on track, similarly the last one I would like to add is about 100 beds which we would commission in Calcutta, are physically ready, just some last mile permissions etc. are due. The moment all that is done those beds will be commissioned as well. And so this would mean about 200 or 250 beds within this year other than Manesar . And Bangalore also we have

some expansion facility where we have approximately about 150 beds which we will be adding over a period of next two years. So, as the occupancy levels over there have been slightly lower that's why we have not commissioned that number of beds there. But we expect that in next two years we should be able to absorb that capacity as well.

Nitin Agarwal : Secondly on the hospitals, if continuing the hospital's part, we have had a pretty reasonable growth in the hospital business revenues through the last three, four quarters. Intuitively one would have thought that our EBITDA margin should have expanded with the increase in revenues through the quarter I mean when I am talking about from Q1 to Q4 of FY'23 , any reasons why despite probably having the best revenues in the year for the quarter, we have had a slightly lower margins for this quarter?

Vivek Kumar Goyal : Yes, so if I can answer this question, there are couple of reasons for this. One is of course the CSR expenses has booked in this quarter which is around Rs. 9 crore for the hospital business.

Two, there are certain expenses which we have booked in this quarter which are a sort of one-off, like the Anandapur facility we are getting the approval and for that we have to pay certain fees. Similarly for the Mohali facility for the adjacent land parcel we have provided for certain penalties which are imposed for non-construction. And there are certain legal costs which have been provided for. So, these are the some one-off, if we exclude those one-off there is a margins improvement actually.

Nitin Agarwal : What will be the aggregate amount of these one-offs if you can just sort of call that out?

Vivek Kumar Goyal : So, it is around, on EBITDA level it is not impacting much, but because certain income also has been added so just to explain this in slightly more detail, certain provisions and advances had

also been written back. So, that's why the revenue has gone up by Rs. 35 crore approximately. And around a similar number has been the one-off expense item has been booked. So, net-net Fortis Healthcare Limited
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there is a no major impact but because the revenue has increased by Rs. 35 crore because of these one-off, the margin has impacted.

Nitin Agarwal : And lastly you talked about closing certain non-viable operations. So, how many such operations which probably potentially we could be considering for closure?

Vivek Kumar Goyal : We always discuss this two, three facilities which are dragging sort of our numbers. Two facilities are in Chennai. So, one actually we have disclosed in our financial, we have classified as a assets held-for-sale which is the Arcot Road facility. And that facility we are exploring to dispose off. And as you might be knowing this is a loss making facility and we are incurring Rs. 3 crore per month EBITDA loss on this entity and that's why we finally decided to hive it off. Another facility in that category, I will not say we have not advanced to that extent but the Marlar facility again we are exploring various options which may include divestment also.

Moderator : Thank you. Our next question is from the line of Amit Khetan from Laburnum Capital. Please go ahead.

Amit Khetan : So, first a follow up on the Medeor acquisitions, so you mentioned you plan to operationalize about 150 beds in the first phase. What is the amount of CAPEX that we need to incur to bring this up to our network standards?

Vivek Kumar Goyal : So, as you might be knowing, Rs. 225 crore we have paid so we will be paying for acquiring this, plus there will be another Rs. 15 crores to Rs. 20 crores GST. Apart from that because this facility is not running for the last year so there will be some expenditure for bringing this facility in the running stage that will require another Rs. 25 crores approximately and balance Rs. 75 crore is for the medical equipment

on its side. So, another Rs. 100 crores we will be spending on these facilities to bring it up to --

Amit Khetan : And secondly on pricing, can you quantify the impact of the price hikes, the scheme price hikes announced by the government on our business. And have we taken any price hike on the non-scheme business starting April?

Vivek Kumar Goyal : So, this is, I will say the sort of regular exercise which operations team do. And the normal price increase we take is in the range of 3% to 4% at one time and it happens two times in a year. And plus, the CGHS price increase impact will also be there. So, I think overall level, we should be able to achieve this time at least 2% price increase on an overall basis.

Amit Khetan : And lastly for this year can you share the CAPEX numbers both maintenance as well as expansion for this for FY'24?

Vivek Kumar Goyal : Yes, so maintenance CAPEX will be in the range of Rs. 250 crores to Rs. 300 crores, because there are certain equipments which are due for replacement. And plus, there is CAPEX as we have said that brownfield expansion we have opened at four sides, so some expansion will come
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from that. So, in my view that cash expenditure will be coming to the extent of Rs. 600 crore to Rs. 700 crore in this financial year.

Amit Khetan : So, Rs. 600 crore to Rs. 700 crore includes the maintenance CAPEX, right.

Vivek Kumar Goyal : Yes, maintenance CAPEX of around Rs. 250 crore to Rs. 300 crores which include IT CAPEX also --

Moderator : Thank you. Our next question is from the line of Nikhil Mathur from HDFC Mutual Funds. Please go ahead.

Nikhil Mathur : My first question is on the ARPOB divergence that we see between your different facilities in the same region as well, Delhi NCR. Now I understand that FMRI obviously has a lot of international patient, the case mix and everything would be amongst the best. But still the divergence versus let's say Shalimar Bagh or FEHI has kind of increased in FY'23. Looking forward what should we expect for the ARPOBs of these assets which are lagging massively versus FMRI, I mean would this divergence increase, would this divergence reduce? So, if you can give some color on what should we expect from some of these assets in Delhi NCR which seems to be pretty lucrative at least on the occupancy side when I see the second chart.

Vivek Kumar Goyal : Yes, so I think when you compare the unit with FMRI you will see the substantial gap. And the reason for FMRI ARPOB increasing as you rightly said is the international patient flow which had increased tremendously. And because there is certain facilitated pieces also included there so that's why gross level the ARPOB level is high. Having said that you might have seen the chart which we have shown in our Investor Presentation, in almost all the facilities the ARPOB has increased and that is a very good sign and this is coming from the better facility mix as well

as some improvement in the payer mix also.

Nikhil Mathur : So, should we expect that in some of these facilities the ARPOB increase can be reasonably high versus FMRI in the coming two, three years? And also, occupancy is quite good, right, I mean FEHI is at 71%, Shalimar is at 75%?

Ashutosh Raghuvanshi : I don't think you should expect that the ARPOB in all these facilities would be at similar level and certainly not at FMRI level, because of the difference in the nature of these hospitals. Like for example in FMRI there is no beds which are given for EWS whereas in Delhi hospitals, that is the case especially in Escorts for example. So, that's why even at the higher occupancy their ARPOBs seems low because 10% of the beds are free. So, some kind of differences like that you would see. We can expect good healthy ARPOB growth in Shalimar Bagh, Noida, Faridabad, etc. but not so much at FEHI. But definitely there will be

a differential between FMRI

and these hospitals because of the addressable communities.

Nikhil Mathur : Circling back to the EBITDA margin expectation from 17% to 20%, if I look at the three or four levers that you have, increasing ARPOB, occupancy and closure of couple of facilities, if these

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three happened only then will you achieve 20% or 20% is achievable even if there are some misses and hits on some of these aspects?

Vivek Kumar Goyal : So, I think 20% EBITDA margin we are reasonably sure and without divestment let me put this way, we should achieve 20% EBITDA margin. And if we are able to achieve the divestment that will further boost the EBITDA margins by 1% to 2%.

Nikhil Mathur : So, is there some exquisite cost control which you are expecting and hence this confidence of 20% EBITDA margin?

Vivek Kumar Goyal : See, one is you know as I said because of the expansion programs. You know the brownfield expansion and the ramp-up of the occupancy level so that is the major lever for the margin improvement followed by adding certain more facilities and the payer mix change. Plus, the cost side we are doing continuous efforts by better negotiations, by optimizing our cost structure by improving the productivity to the extent possible. All those measures are we are taking to improve the margins.

Nikhil Mathur : And one final question I had on the Medeor acquisition, now I think Rs. 320 odd crore is the total CAPEX that is likely to be incurred for 350 beds. Unless you achieve the current EBITDA per bed, which is roughly 33 to 34 lakhs, it will be I think difficult for you to make 20% ROCE in this particular facility because I mean the CAPEX will be roughly around 90 to 95 lakhs per bed by the next year or so. And going by your comments that there are new highways coming up, Rewari is the catchment area in those areas. I am not too sure if this is likely to achieve in one or two years. So, can you give some thoughts on what is the target ROCE that you are looking from this investment and in how much timeframe is that possible to achieve that target ROCE?

Vivek Kumar Goyal : So, you know this is for all practical purpose, this is a sort of new facilities because this hospital is not in operation. So, as Dr. Raghuvanshi mentioned earlier it will take one year time for us to start this hospital and then to reach a particular occupancy level. And that's why the return on capital metric will follow, occupancy ramp up we will be able to achieve. Having said that if we are able to ramp up the capacity first the ROCE will be faster, we are targeting at least 17% to 18% ROCE back by fourth, fifth year we are targeting to achieve at 17% to 18% of ROCE level for this facility. It may improve if we are able to achieve ramp up fast.

Nikhil Mathur : And just one associated question here, will there be some shared cost with FMRI, I mean FMRI is not very close by to this facility but still it's not very far off as well. So, would there be some shared cost between the two units?

Ashutosh Raghuvanshi : No, I think there will be operational synergies but in terms of cost I don't think there would be too much sharing. But definitely there will be a lot of synergies on the operational side and that will help to have a quick ramp up.

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Nikhil Mathur : Do doctors at FMRI have bandwidth to kind of attend to this hospital as well, whenever its up and running?

Ashutosh Raghuvanshi : Yes, of course we had second line of clinicians which is ready to take further responsibilities, but we have seen that just the announcement that we are intending to complete this transaction we have seen lot of interest from many other clinicians as well. So, that is certainly no challenge at all.

Moderator : Thank you. Our next question is from the line of Bino Pathipa

rampil from Elara Capital. Please go ahead.

Bino Pathiparampil : If I look at the 4Q margins, obviously YoY it is looking bad because of the high COVID volume of the prior year, but even if I compare sequentially to Q3 there is some major contraction in EBITDA margin, even if you are just for some sort of seasonality that you used to be there. Could you explain that please?

Anand K : Yes, as explained to an earlier caller as well so this is because of some one-off expenses which have happened in Q4. So, to adjusted that the EBITDA is at 20%, the two adjustments are CSR, CSR for the whole year is accounted in this quarter which is you know Rs. 5.1 crores and there is PFDD for HP PPP which we have taken on a conservative basis. So, which is at Rs. 6.5 crores, and both if you take together the adjusted margin will be 20%.

Moderator : Thank you. Our next question is from the line of Saurabh Kapadia from Sundaram Mutual Funds. Please go ahead.

Saurabh Kapadia : I was asking the Rs. 100 crore CAPEX of Manesar it's for 350 beds.

Vivek Kumar Goyal : No, so 150 is the additional expenditure we have to put in the equipment and for upgrading this facility to bring the facility for the operational level. Apart from that there is around Rs. 240 crore total acquisition cost. So, all put together it will be around Rs. 340 crore total investment for Manesar facility for 350 beds.

Saurabh Kapadia : And secondly if you can provide the revenue and EBITDA or the EBITDA loss for the Arcot Road Hospital for '23?

Vivek Kumar Goyal : So, EBITDA loss was around Rs. 36 crore. So, revenue was Rs. 51 crore.

Saurabh Kapadia : So, growth for next year what we are saying double digit would be excluding the Arcot Road number, right?

Vivek Kumar Goyal : Yes, Arcot is still in our Board, we are in the process of finalizing the deal. So, I think it will take another two months time for us to close this transaction, till that time this loss will continue.

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Moderator : Thank you. Our next question is from the line of Abdulkader Puranwala from ICICI Securities. Please go ahead.

Abdulkader Puranwala : Just one question on your hospital margin so if I look at the segmental breakup and if I compare the whole of FY'23 so the start of the year we had close to four facilities with the EBITDA margin upwards of 25% and where we are ending in FY'23 is just having two facilities. So, just wanted to understand though the revenues across Mohali, Shalimar Bagh and FMRI has grown consistently over the quarters, what would extend this kind of a dip into their EBITDA margins?

Anurag Kalra : So, Abdul when you look at this chart, I think there are couple of highlights that we want to share with you. 1) If you look at the top-three brackets which is the EBITDA margin range from 15%, 20% to going up to 25%, almost the revenue contribution in FY'22 from just these three margin brackets was about 69% in FY'22. And that's actually gone up to 78% in FY'23. Not to mention when you also look at these, the ARPOB numbers as well as the occupancy numbers have also increased significantly.

To your specific question more than 25% margin which were three facilities in FY'22 have gone down to two, that was just a smaller facility of Kalyan which is now fallen below. Kalyan did very well during COVID times now, but it's a small 50 odd bed facility which has now gone below. But other than that, I think all our facilities whether it's FMRI, whether it's Shalimar Bagh, whether it is Mohali they are all in these 20% to 25% and 25% plus brackets. So, when you look at FY'23 Mohali, BG Road, Shalimar Bagh, Mulund, Anandapur, Ludhiana and a couple of more are all in that brackets. So, you have to look at this holistically and how the overall number of facilities are moving towards the higher end of the bracket.

Abdulkader Puranwala : And final one on the diagnostic

side, so I mean going ahead what was the kind of volume growth

we should peg in for the non-COVID business. And on the price increase as well, so a couple of your peers have indicated that they have started to take some price increase, so in our portfolio have we started, what could be the quantum ahead?

Anand K : You are not very clear, Abdul but from what I understand you are asking about any price increase, price increase as I was telling the earlier caller also that we have not done any price increase so far. And we are contemplating an increase probably during the middle of the year. But as of now we have done no price increase. The volume growth on a like-to-like basis will be about 12%. Our ARPT growth is zero so that is actual growth because we have to remove the HP PPP which is high volume business which we had in FY'22 which has only two months of component in FY'23.

Moderator : Thank you. Our next question is from the line of Saion Mukherjee from Nomura Securities. Please go ahead.

Saion Mukherjee : On SRL would you like to give some guidance on growth or margin let's say for the next year or maybe even for the medium term?

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Anand K.: I think as we all know the industry is expected to grow at about 10% CAGR over the next three years. So, I think SRL will be a round one or two basis points higher than that, that would be the expectation.

Saion Mukherjee : If I look at one of the initiative was to sort of increase the touch points and that was quite significant in the last few years, but the growth numbers seems to be muted related to the kind of expansion that we have done. Any particular thing that I am missing or any reason that you can attribute to that the expansion probably hasn't really contributed as much to growth as was expected?

Anand K.: So, the network expansion strategy in terms of adding more network to our labs, we started it around earlier FY'20 and as you know we have been continuously expanding over the last three years and we have added close to about thousand centers every year. But because of the COVID pandemic in between in the last two years we have seen more activity on COVID and we have not been able to fully ramp up these centers. So, usually these centers take about 18 to 24 months to reach a critical volume. So, we are seeing the growth of the centers which are more than three years old, they are reaching some critical numbers. But the newer centers that we added this year will still need some more time to reach those numbers. So, it's going to be growth which will be, we have added capacity so the growth will happen over the period of time.

Saion Mukherjee : Any guidance on margins that you would like to give where we could reach over the next few years on EBITDA margin ?

Anand K.: I think we will not be able to provide any guidance as of now because it's quite a dynamic environment at this point of time.

Saion Mukherjee : Dr. Raghuvanshi, just a few broader level question you talked about looking for acquisitions, so how is the environment and what's the strategy for Fortis in terms of what kind of assets you would look at in terms of geography brownfield like what you have it in Manesar or complete an operational facility. And we are seeing a lot of interest from your peers as well as private equities participating. So, in terms of valuation, how comfortable you are in the market that we could see some large acquisition ? And also, in terms of size, what kind of size you are looking at and up to what size probably you can go ? Can give some color on sort of the M&A landscape and how Fortis wants to participate?

Ashutosh Raghuvanshi : Yes Saion you are absolutely right there is lot of interest in acquiring newer assets so we definitely have competition there. Now having said that as far as our strategy is concerned we are very clear about

out a few things, one of them is our broader geographical strategy, so in any market we want to operate, we would not like to operate in isolation. So, if we are going to a new market, it has to be and a cluster of hospitals and not a single hospital otherwise we want to stick to the clusters where we operate so namely the NCR, Mumbai and its large suburbs and in case of Maharashtra it could be even other cities as well and then Bangalore cluster and our Punjab and Calcutta. These are the areas where we want to remain limited.

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Now the assets we are typically seeking for something which is at least 200 beds. And then we should have a clear visibility amongst ourselves as to what kind of profitability enhancement we can do from the current state so that is very important.

And then of course the valuation has to be reasonable. Now if you look at the Manesar deal which we are in the process of concluding, our per bed cost would come to only about Rs. 95 lakh per bed which today if one was to do a greenfield hospital would typically be somewhere close to about Rs. 2 crores. So, unless and until it comes in at an attractive valuations, it does not make sense. So, yes, there is a competition but I think there would still be assets. And as far as our ability to absorb is concern, we are quite comfortable in our minds taking up something of the tune of about 1200 to 1600 beds. So, any such opportunities it will come to us, and it comes at a proper valuation we would certainly be happy to conclude that.

Saion Mukherjee : One last question pre-COVID and post-COVID in terms of the cost, inflation cost, manpower cost, doctor cost or any other talent cost, have you seen any change in the dynamics in the marketplace?

Ashutosh Raghuvanshi : So, the dynamics has not really changed much. I think the larger change continue to attract more clinical talent and the availability of clinical talent is easing out. The issues with nursing availability is still there, but as far as the specialists are concerned I think that situation is easing to a large extent for the larger organized players. So, I don't see that as a challenge at all.

Saion Mukherjee : And one last I just missed Anurag if you can share the Arcot Road EBITDA loss for the year?

Vivek Kumar Goyal : Rs. 36 crores approximately.

Moderator : Thank you. Due to time constraint that was the last question of our question and answer session.

I now hand the conference over to the management for closing comments.

Anur ag Kalra : Ladies and gentlemen thank you for your time today, I hope we have been able to respond to your questions as best possible. If there are any further clarifications, my colleague Gaurav and I are there to help you. Please do feel free to reach out to us. Thank you once again and have a good day.

Moderator : Thank you. On behalf of Fortis Healthcare that concludes the conference call. Thank you for joining us and you may now disconnect your lines.

Call: Aug 2023

FORTIS HEALTHCARE LIMITED

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August 11 , 2023

FHL/SEC/202 3-24

The National Stock Exchange of India Ltd.

Scrip Symbol: FORTIS BSE Limited

Scrip Code:532843

Sub: Transcript of Investors / Analysts' meet under Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015

Dear Madam / Sir,

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed herewith the Transcript of the investors / analysts meet held on August 7 , 2023 to discuss the Company's un-audited financial results for the quarter ended June 30 , 2023 . Further, the said transcript is also available on the website of the Company i.e. Transcript

This is for your kind information and record .

Thanking you,

Yours Faithfully

For Fortis Healthcare Limited

Murlee Manohar Jain

Company Secretary & Compliance Officer

M. No. F9598

Encl: a/a

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"Fortis Healthcare Limited "

Q1 FY'24 Post Results Conference Call "

August 07 , 2023

MANAGEMENT : DR ASHUTOSH RAGHUVANSHI – MANAGING
DIRECTOR AND CHIEF EXECUTIVE OFFICER – FORTIS
HEALTHCARE LIMITED

MR. VIVEK GOYAL – CHIEF FINANCIAL OFFICER –
FORTIS HEALTHCARE LIMITED

MR. ANURAG KALRA – SENIOR VICE PRESIDENT ,
INVESTOR RELATIONS – FORTIS HEALTHCARE

LIMITED
MR. GAURAV CHUGH – GENERAL MANAGER ,
INVESTOR RELATIONS , FORTIS HEALTHCARE LIMITED

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Moderator: Ladies and gentlemen , good day, and welcome to the Fortis Healthcare Limited Q1 FY '24 Post Results Conference Call of Fortis Healthcare Limited. As a reminder, all participant lines will be in the listen -only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touchtone phone .

I now hand the conference over to Mr. Anurag Kalra , Senior Vice President, Investor Relations at Fortis Healthcare Limited. Thank you, and over to you, sir.

Anurag Kalra : Thank you, Selvin. A very good afternoon, good morning, ladies and gentlemen, and thank you for joining us on the quarter 1 FY '24, Fortis Healthcare call. With me on the call today are our Managing Director and CEO, Dr Ashutosh Raghuvanshi . We have our Chief Financial Officer, Mr. Vivek Goyal. And we have Gaurav, my colleague in Investor Relations.

Before we start off, I hope all of you have got a chance to look at the presentation and the press release. I would also want this opportunity to probably tell you that in addition to our standard disclaimers, we've also given a disclaimer regarding the Board resolution passed by both Fortis and Agilus Diagnostics Limited on initiating the process of the initial public offer. In that respect, I would want to read out the disclaimer for everybody's benefit.

Agilus Diagnostics Limited, a subsidiary of Fortis Healthcare Limited is proposing, subject to the receipt of requisite approvals, market conditions and other considerations, an initial public offer of its equity shares in the near future and is in the process of filing a draft red-herring prospectus with the Securities and Exchange Board of India. The Board of Directors of Fortis and Agilus Diagnostics Limited has approved the process of initiating proposed IPO in its Board meeting held on August 4th, 2023.

In light of the publicity restrictions imposed on Agilus Diagnostics Limited and Fortis, no further information other than that contained in this presentation can be disclosed. The equity shares have not been and will not be registered under the United States Securities Act of 1933, as amended or any other applicable law of the United States and unless so registered, may not be offered or sold within the United States except pursuant to an exemption from, or in a transaction not subject to, the registration requirements of the U.S. Securities Act and applicable state securities laws.

The equity shares may only be offered and sold in the United States only to qualified institutional buyers, as defined in Rule 144A, in private transactions exempt from the registration requirements of the U.S. Securities Act, and outside of the United States in offshore transactions, as defined in and in compliance with Regulation S and the applicable laws of the jurisdiction, where any such offers and sales are made. There will be no public offering in the United States.

It is in light of this disclaimer that we also do not have today the management of Agilus Diagnostics Limited with us. We will start with some opening comments by Dr Ashutosh

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Raghuvanshi post which we can open the floor for question -and-answers. Over to Dr Raghuvanshi.

Ashutosh Raghuvanshi: Thank you, Anurag. Very good morning and good afternoon to everyone, and thank you for your time to join us on our Q1 financial year 24 earnings. I hope all of you are well.

Before I talk about the performance of the company for the quarter, I would like to talk about a few key strategic actions with respect to both our hospitals and diagnostic business. This I must add clearly signifies our intent to move forward and ensure that the company is well placed to further enhance

its growth momentum.

Let me quickly take you through some of these developments. I'll start with the diagnostics business. During the quarter, we have changed the name of our diagnostics business, which is now known as Agilus Diagnostics Limited. I'm particularly pleased to share that the Board of Fortis and Agilus have granted approval for Agilus Diagnostics to initiate an initial public offering process by way of an offer for sale for its equity share subject to receipt of requisite approvals, market conditions and other considerations.

On the hospital business, before I move on to the financials, we are well aware of our portfolio rationalization strategy for quite some time, which would enable us to allocate our resources more efficiently. To this effect in July 2023, we have divested our Arcot Road hospital operation for INR 152 crores. For financial year 23, the Arcot facility had a revenue of INR 51 crores and an EBITDA loss of INR 36 crores.

We have in the quarter also signed definitive agreements to acquire a 350-bedded hospital in Manesar for INR 225 crores, enabling us to increase our footprint in Delhi NCR and accelerate our focus on key geographical clusters. This acquisition complements well with our flagship facility at FMRI in Gurugram, and we hope to close this acquisition shortly. These were some of the key corporate actions.

Let me now take you through the performance of -- for the company -- of the company for the quarter. A year has begun on a steady note, as we witnessed a growth in our consolidated revenue compared to Q1 of financial year 23. Our revenues have increased by 11.4%, reaching INR 1,657 crores in Q1 of financial year 24.

Our consolidated operating EBITDA margins were at 16.5% versus 16.9%, and our reported profit after tax was INR 124 crores versus INR 134 crores.

On the balance sheet side, we remain quite healthy with the net debt-to-EBITDA of 0.35x, as on June 30th, 2023, compared to 0.54x, as on June 30, 2022. Our net debt stands at INR 393 crores as on June 30, 2023.

Coming specifically to the hospital business, we have witnessed a revenue growth of 13.6% to INR 1,354 crores compared to INR 1,192 crores in Q1 of financial year 23. Operating EBITDA stood at INR 206 crores, reflecting a margin of 15.2% and EBITDA growth of 6.8% compared to Q1 of financial year '23.

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Generally, Q1 is a relatively softer quarter. The business performance was also impacted due to a slightly lower occupancy and a comparatively less favourable payor mix, which was a bit skewed towards the government-related business. While occupancy was at 64% versus 65% in the previous period, our ARPOB rose by 12% to INR 60,076 per day, about INR 2.19 crores on an annualized basis. I do expect that we will see an improved set of key performance metrics in the quarters to come.

Our investments in bed expansion and medical equipment are on track with the latter seeing the commissioning of a Da Vinci Robotic system at Fortis Noida in the quarter. We remain on course for a planned addition of close to 1,400 beds in coming 2 years to 3 years. These include expansions in some of our key facilities like Mulund, FMRI, Noida, Shalimar Bagh and a few more.

Some of these facilities that I just mentioned would gain significant scale and size reaching to almost 450 beds to 500 beds. We are also actively enhancing our presence in some key medical specialties like oncology. This specialty has grown 34% on year-on-year basis and now contributes almost 14% to our revenue. Our top 6 specialties contribute over 60% to our hospital revenues.

During the quarter, we have onboarded eminent clinical specialists in the medical field of urology, pulmonology and neurology to further augment our plans to grow these specialties.

International patient revenue continues to see good traction. International patient revenue grew

a healthy 29% contributing 8.5% to the overall hospital business revenue, up from 7.5% in Q1 of financial year 23.

On our diagnostics business, please do appreciate that I would have to be very limited in my comments since we have to adhere to the publicity guidelines in place, having initiated the Agilus Diagnostics IPO process. The business reported gross revenue of INR 343 crores, a growth of 3% versus quarter 1 of financial year 23. However, the non-COVID revenues have grown 9% versus the corresponding previous quarter. Our operating EBITDA margin reached 19.4% compared to 17.4% in Q1 of financial year 23 and 14.9% in Q4 of financial year 23.

With this, I'll conclude my comments. I think it has been a steady start to the fiscal 2024. We have started to progress on some of our growth and portfolio rationalization priority that we have been talking to you about in the past. We will also continue to pursue and evaluate various growth opportunities that fit well with our cluster strategy and provide attractive synergies.

These, I believe, would further augment our growth levers and place us well in the healthcare landscape here. Thank you. And with that, I would like to hand over the call to Anurag.

Anurag Kalra: Thank you, sir. Ladies and gentlemen, we will now open the floor for question-and-answers. Selvin, if I could ask you to moderate, please. Thank you.

Moderator: Thank you very much. We will now begin the question-and-answer session. The first question comes from the line of Nikhil Mathur from HDFC Mutual Funds.

Nikhil Mathur : Sir, my first question is on the revenue by asset that you disclosed in your presentation, the revenue seems to be a kind of a stark difference here in the sense that if I look at on a quarter -on-quarter
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basis, some of the flagship or the larger hospitals have done fairly well, whether it is FMRI, Mohali, BG Road, even Noida and Shalimar Bagh aren't that bad either despite Q1 being seasonally weak, but the smaller hospital seems to have done worse off than the larger ones. So, is there any key reason here, I mean, some competitive pressures or general slowdown, what are the key reasons for the underperformance of the smaller assets?

Vivek Kumar Goyal : Generally, if you have seen the trend in terms of the surgical revenue has gone up, and which is the case for the larger hospitals. So, smaller hospital generally the gynae and other business, which is having a softer quarter over the period. So, that is the main reason plus some of the specific units are having some specific problems like Kalyan in Mumbai, there is a -- because of this rainy season and other things, the accessibility to the hospital was affected. So, those type of regions are specific to some of the units.

Nikhil Mathur : Okay. And in the last quarter call, there was a margin guidance share on the hospital side. I think it was somewhere to trend around 19%, 20% in the coming quarters or so. So, where are you on that particular guidance? Does it still hold or you're revising it onwards?

Vivek Kumar Goyal : Yes. So, we are maintaining that guidance, Nikhil and as Dr. Raghuvanshi said it is a good start in terms of revenue. Profitability is slightly on the lower side, but we expect to pick up the pace in the forthcoming quarters.

Nikhil Mathur : Right. And also, sir, I mean, Dr. Raghuvanshi, now that we are almost 6 quarters, 7 quarters past the peak COVID had happened. What are your thoughts on the footfalls on the hospital side. I mean, there is still a divided opinion whether there was a pent-up angle in the last 3 quarters, 4 quarters or not post-COVID. So, any broad thoughts that you can share whether we are slightly heading into some sort of a slowdown or things are kind of looking, as would have -- you would have thought a couple of quarters back?

Ashutosh Raghuvanshi : No, I don't think there is any slowdown. There is certainly no pent-up demand as well that is whatever was there, was limited to some elective procedures, as in the previous quarters also, we mentioned that there was a surge in like, for example, in joint replacements, et cetera. But overall, most specialties remained in the same ratios, as they are now, or they were before the pandemic.

So, I don't think that is the case nor is the footfall getting affected. The reason why this quarter also has had a little bit of this thing is because of a little bit of unseasonal rains, et cetera. And in our case, specifically, other than the occupancy, we also had an unfavourable mix in terms of the government scheme ratio being slightly higher and that resulted in a slightly lower profitability, though the growth on the revenue side was very healthy.

We expect the trend on the revenue to grow. At the same time, we are very focused on the cost side and make sure that the profitable profile remains same, as what we had guided earlier. And we are very much on that trajectory and some of the corporate actions, which we are taking will also help us to achieve that rather quickly over the next 2 quarters or 3 quarters.

Nikhil Mathur: Sure sir. Thanks a lot. I will get back in the queue.

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Moderator: Thank you. The next question comes from the line of Amit Khetan from Laburnum Capital . Please go ahead.

Amit Khetan: My first question is on occupancy, which appears to be significantly lower than what one might have expected, given how normalized the environment now is. Just wanted to understand beyond the seasonal rains, are there any other one-offs that explain this and how should we think about the long-term steady state occupancies at the group level?

Ashutosh Raghuvanshi : Yes. The occupancy trends, what we see currently in the current month is back towards a healthy number. The reasons why this occupancy was low in the previous quarter, I think one of the major reasons was the seasonal variation, summer vacations had a huge impact sometimes. And I think those were the reasons why the occupancy levels have been low. And as I mentioned in the previous answer that there were also a lot of unseasonal rains in this period, which also disrupted some of the routine work.

Vivek Kumar Goyal : And plus, there is some addition of bed which has happened during the quarter, the ramp up of which will be showing up in the forthcoming quarters.

Amit Khetan : Got it. And you still stick to this guidance of occupancy trending towards 70% over, say, 2 years

or 3 years?

Ashutosh Raghuvanshi : Yes, yes, absolutely.

Amit Khetan : Got it. Secondly, on the diagnostics business, have we considered a demerger instead of an IPO because it will continue to be a subsidiary of Fortis and will attract some kind of a holdco discount?

Anurag Kalra : Amit, this is unfortunately, given the publicity guidelines in force we been advised by legal counsel handling this not to make any comment. So, the apologies, we will not be able to say anything on this one.

Amit Khetan: Fair enough. Thank you.

Moderator: Thank you. The next question comes from the line of Anish Deora from Nomura.

Anish Deora: So, my question is related to ARPOB. So, this quarter has seen actually a good increase of around 5% quarter -on-quarter in ARPOB, and on a year -on-year basis, almost a 12% increase. I think this is supported by increasing like a good surgical mix at 61%, and also improving Onco contribution. So, could you please confirm if the increase in ARPOB is being explained by these reasons and how should we like look at the ARPOB growth number for FY '24, if you could just guide on that?

Vivek Kumar Goyal : Yes. So, there are as you rightly mentioned, there are a couple of reasons. One is this improvement in

the mix towards surgical business, which is mainly in Ortho and Onco. And secondly, there is some price revision that has happened in the government channel also, and we have also taken some price increase, that has also resulted into ARPOB increase. Going forward, we are expecting around 4% to 5% ARPOB increase on a year -to-year basis.

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Anish Deora: Okay. Thanks.

Moderator: Thank you. The next question comes from the line of Saurabh Kapadia from Sundaram Mutual Funds. Please go ahead.

Saurabh Kapadia: Yes. Thanks for the opportunity. Sir, if you can talk about the margin for the individual hospital, the margin metrics you have on your slide, so Q1FY4 compared to Q4 and some of the hospital has come in terms of lower margin or what has happened there?

Anurag Kalra : So, Saurabh, first of all, we just want to provide a clarification. The chart that you've seen in quarter 1 FY '24 is after corporate cost allocation. This is so that we can align with our reported margins also. And versus that, the other chart for FY '23 is before corporate cost allocation. So, allow me to just run you through on a like -for-like basis how that change has happened.

So, when you look at the number of hospitals that are below 10% in quarter -- in FY '23, there was 7 hospitals. Now, there is an addition, there is one more additional hospital there. In the margin range of between 10% to 15%, in FY'23, the whole of FY '23, we had 2 hospitals, that's actually gone up to about 4 hospitals now. In the range of 15% to 20%, we remain static. There were 7 hospitals in FY '23. In quarter 1, FY '24, there remains 7 hospitals.

However, I think the gap is about -- between the 20% to 25% margin range, where the whole of FY'23, we had 6 hospitals and now we have about 3 hospitals and the hospitals that have been missing, the 3 hospitals that has actually come down a tad bit, so that they go into the next category are actually BG Road, Mulund, and Kalyan. So, I think, the crux over here is that we are not very different in quarter 1, FY'24, as we were in FY '23, and we hope to flow those back to show progressively better chart, as we move forward.

Saurabh Kapadia : Okay. Sir, in the below 10%, are there any loss -making unit in this quarter?

Anurag Kalra : So, one of the loss -making unit that is no longer there is what the Arcot Road facility, as Dr. Raghuvanshi in his opening comments mentioned that, that is a facility that was running at about INR36 crores losses on an annualized basis. So, I think, that benefit we will see because we closed this transaction on July 12th, so that benefit we will see in this quarter.

Other than that, yes, there are couple of more facilities, like our facility in Chennai, which is Fortis Malar, and we have a smaller facility Sacred Heart in Bengaluru, that are loss -making. Other than that, there are other facilities in that bracket, but they are not loss -making, they are less than 10%.

Saurabh Kapadia : Okay. And sir, second question on the staff cost, if we compare to Q4 that has seen almost INR 25 odd crores jump in the staff cost, so any one-off there or also in other expenses, is there any one-off in this quarter?

Vivek Kumar Goyal : Yes. You are talking about the employee cost?

Saurabh Kapadia : Employee cost, yes.

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Vivek Kumar Goyal : Yes. So, employee cost in this quarter is slightly on higher side if you compare with the previous quarter because of the annual increment impact is build in this quarter, so which is 7% to 8% build up is there.

Ashutosh Raghuvanshi : And there is some additional clinical talent we acquired, as I mentioned in my opening remarks, that is also something, which has increased the s.

Saurabh Kapadia : Okay. And there are no one -offs in other expense as well, sir?

Vivek K

umar Goyal : Yes. That is not in the employee cost, but there are a couple of one -offs in the current quarter.

One is the building regulations cost, which is coming under general administrative expenses.

For Anandapur, we have now got all the approvals, and we will be very soon operationalizing that 80 -bed, which were long pending , Anandapur in Kolkata, I mean. And then there is slightly higher legal costs in the current quarter, as compared to the previous quarter because of all the legal issues company is grappling it.

Saurabh Kapadia : So, can you quantify the number?

Vivek Kumar Goyal : Yes. Anandapur one is around INR 3.5 crores and the legal cost is -- incremental is around INR 3 crores.

Saurabh Kapadia: Okay. Thank you. That's all from my side. Thank you. All the best.

Moderator: Thank you. The next question comes from the line of Shyam Srinivasan from Goldman Sachs.

Please go ahead.

Shyam Srinivasan: Thank you for taking my question. Just trying to deconstruct the 13.5% growth in the hospitals, looks like most of it has come from just ARPO B increase 12%, right? So, little is there a concern on utilization just going back to one of the earlier participants' question, 64% -- you said July has started better occupancy, so what gives us the comfort, Dr. Ashutosh to say, can we go to 65%, 66% , 70% onwards whatever, right. So, what are some of the things in the pipeline? I also want to know an additional question; I think your slide shows 4,500 operational beds now.

I thought that number was, like, closer to 4,000, which is what you've always historically say. So, what is the addition in terms of beds as well during the quarter I think, Vivek sir, also mentioned something, so?

Anurag Kalra : So, Shyam, just to clarify that, that includes our O&M beds, so we're still around the 4,000, 4,100 number. But if you were to look at our O&M facilities, those being in S. L. Raheja , Vasant Kunj, etc, we are about 4,500 beds. So, that just to give a holistic picture. That chart is just to give a holistic picture on the total number of beds, including O&M.

Ashutosh Raghuvanshi : Yes. And as far as occupancy is concerned, Shyam, we are seeing the trend going towards normalization and we are pretty confident that it will remain at the levels at which we had predicted at about 70% and then go higher on that. So, that is the aim. So, we are pretty confident that this will remain in the right direction. It's going in the right direction. The trend in the current month is indicating that.

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So, I'm not too concerned about this. That definitely this was -- the growth in the revenue was driven by ARPOB, you are absolutely right on that. But I think as Vivek mentioned earlier, one of the growth lines was Oncology, where the contribution margins are little lower. So , in spite of the fact that the revenue grew higher, the margins were not in the same proportion. So, these have been few factors because of which we had a little suppressed ask .

Shyam Srinivasan: Helpful, Dr. Sir. Second question is on the professional charges or the fees to the doctors, the line item there, right, and you're also opening comments around hiring new doctors, so how was the dynamics right now around new hires? When I look at that line item has grown faster than revenue growth, like 14%, so how should we look at new hires?

Is there some kind of inflation that you're seeing? Is there something that -- some of these line items had remained a little suppressed during COVID, we are now starting to see that come back. What are the kind of wage hikes that one is kind of planning? And how do we kind of mitigate this by, if I were to marry the guidance of margins to be 18% plus for the hospital business, how can we achieve higher margins given this wage inflation?

Ashutosh Raghuvanshi :

Yes. So, whatever increases had to be done in the fixed part of remuneration or declaration, that has already been done, and you are right that post COVID in last 6 months, 7 months, we have made quite a few modifications there. However, specific to this quarter, one of the reasons why this element is higher is also because the surgical volume was high, where typically the payouts are larger. So, all these things have led to the current picture. The clinicians, which have come on board new, they obviously will start attracting more work, as we go into the next few quarters and that will sort of even out.

Having said that, we have been mindful always that our overall ratio of the costs on the employee side sometimes have been slightly higher and some bit of rationalization over there is required. So, we will definitely be working in that direction over the next 2 quarters or 3 quarters and by the end of the year, we expect that to come to a better level.

As far as the 18% level is concerned, I think we are well within that direction. This quarter, of course, because of the lower occupancy and the higher ratio of the scheme business, we could not achieve that number. But we are pretty confident that going forward, we will be on this good trajectory to achieve what we had mentioned earlier.

Shyam Srinivasan: Thank you. And, last question, I know you are constrained from giving anything. But on the Agilus, is there any timeline, sir, just from past information that we need to keep in mind? Is it the private equity exit, which is Feb 2024, which should be used as a benchmark around when timelines could materialize? Also, your thoughts on how much will the stake that Fortis will have? Eventually, will it fully divest, or will it still have some ownership, anything, any colour here will be helpful, Dr. Ashutosh?

Ashutosh Raghuvanshi: I cannot give you anything specific in this, Shyam, unfortunately. However, I can tell you that one of the things, which we said, that market conditions is what is going to determine. Of course, the process has its own timelines, and it will flow according to that. So, as we have mentioned and made it known to the public at large is that we are initiating the process. So, there are various
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steps, and we would not like to hazard a guess. But as soon as the steps are achieved and satisfactorily and we get the necessary approvals, etcetera, we will keep moving on it. So, a reasonable timeframe, you can deduct from what is the general rule of the industry.

Shyam Srinivasan: Okay, sir. Thank you and all the best.

Moderator: Thank you. The next question comes from the line of Banshi Desai from JPMorgan. Please go ahead.

Banshi Desai: Yes. Hi, thank you for taking my question. So firstly, on the margin metrics, we see eight hospitals doing less than 10% margins. Sir, if you can quantify how many of these would be potential divestment candidates for us?

Anurag Kalra: Banshi, that is something that we keep on constantly evaluating. Each hospital has to be given an adequate time to have a runway to profitability. If we don't see any progress in that, then those decisions will be taken at that point in time. For example, we saw Arcot Road for quite some time, and I think the performance of the hospital continued to be less than our expectations, and finally we've divested it. So, it will be on a case-to-case basis. It will be difficult to hazard a guess right now.

Banshi Desai: Okay. And when you had quantified in the past that you could see almost 100 basis points to 200 basis points of improvement on account of divestment, you would have assumed a certain number of hospitals to be divested, right? So, I was just trying to understand, would you have any number in mind, as to what can lead to that 100 basis points to 200 basis points addition to our margins?

Anurag Kalra:

Banshi, Arcot EBITDA loss was approx. INR 36 Cr. So, if you take out the top line of INR 50 odd crores and its loss, and you look at the EBITDA, I think that's about an 80-basis points improvement is there.

Banshi Desai: And, secondly, on the expansion, I know you guys have mentioned 1,400 beds over the next 2 years, 3 years, if you can just call out how many beds do we expect in each of these years over the next 2 years, 3 years, and where all would these come?

Vivek Kumar Goyal: Yes. So Banshi, I'll take the question. So, these 1,400 beds are mainly coming in NCR region. So, all of our NCR region, we are expanding, whether it is FMRI, Noida, Shalimar Bagh, Faridabad, all the units we are expanding. Plus, there is an adjacent land parcel in Mohali, that also we'd be expanding. Mulund, we have just completed the expansion program last quarter. And then, BG Road, the building is ready. We are just waiting for the occupancy certificate. Plus the Kolkata, I have just mentioned in the beginning comments. We have got all the approvals. Now we're operationalizing 80 beds in Kolkata.

So, with all these things we will be completing these 1,400 beds. So, it will be -- because most of this is coming in the form of new building and which will take 2 years to 3 years' time to build and then operationalize the bed. So, I am expecting 300 beds to 400 beds every year from now onwards. And then, in the latter part of the year, say third year and fourth year, this number will go up to 500 beds.

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Bansi Desai: Okay. And despite this addition of 300 beds, 400 beds every year, do we still believe that we can touch an occupancy of 70% or on a blended basis that should be lower?

Vivek Kumar Goyal: No, that is our plan. See, we achieved the occupancy of 70% on the extended bed capacity because most of these beds are coming, as we said on the existing operations, where the occupancy level itself is very high, and we are facing the scarcity of beds. So, we are not expecting much challenge in ramping up this capacity. I will now caveat myself from Mulund in Mumbai and BG Road, where occupancy level is slightly on the lower side. So, there we might take slightly longer time to ramp up. But other units, we are quite confident that we'll be able to ramp up faster.

Bansi Desai: Noted. And any thoughts on optimizing our pay or mix further from here on?

Vivek Kumar Goyal: So, there is definitely opportunity here. And as we mentioned in the earlier part of the call, this quarter the profitability is impacted because of higher scheme business we have done. So, there definitely we are manoeuvring ourselves. However, we can increase the sales of good payors like TPA, Cash and International business. And we move our dependence on the scheme business. So, basically, we have various scope, and we are working on that. Definitely, there will be some improvement we should see.

Bansi Desai: And considering all of that, we should assume an ARPOB growth of 4% to 5% is what you are mentioning?

Vivek Kumar Goyal: Yes.

Moderator: The next question comes from the line of Neha Manpuria from Bank of America.

Neha Manpuria: Dr. Raghuvanshi, for the Agilus IPO, would we need any approval from the Delhi High Court or any regulatory -- other than the usual regulatory approval, would there any other approvals be required?

Ashutosh Raghuvanshi: No. Our understanding and the legal advice to us is very clear that we don't require any such approvals.

Neha Manpuria: Okay. So, there shouldn't be any delay because of other than usual regulatory approvals?

Ashutosh Raghuvanshi: Yes. The usual regulatory process is what we are going to follow.

Neha Manpuria: And, sir, on the Manesar acquisition, how should we look at? I think you said the transaction will close in this quarter? And when we made the acquisition, we said this would ramp up in a

phased manner, so you could just give us some sense in terms of how we should look at ramp-up of this and when can we assume normalized contribution from this project?

Ashutosh Raghuvanshi: Yes. So, this hospital, Neha, is not functional at the moment, so, we have to make it functional. For that, we are expecting to take about 9 months post closure. So, I would say, 1 year from now, the hospital would be commissioned. We would start with about 30% of the capacity to total 350 beds. We'll start about 125 beds in the beginning and then we would ramp it up a little faster. Our belief is that this being in our core market, the ramp-up should be slightly faster. So,

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over the next 2 years following that we should commission the entire facility, and the ramp-up should be at a good pace.

Neha Manpuria: And when should we assume breakeven from this facility, sir, given it's in our core market, indicating a much faster ramp-up?

Vivek Kumar Goyal: Yes. So, Neha, as you know, as Dr. Raghuvanshi mentioned, this is absolutely for the new facility...

Neha Manpuria: Correct.

Vivek Kumar Goyal: The hospital is not functional. So, it will take slightly more, longer time in my view, at least 18 months we are waiting for breakeven.

Neha Manpuria: And this is part of the 1,400-bed addition, or this is over and above that?

Vivek Kumar Goyal: No, this is not part of 1,400 bed.

Ashutosh Raghuvanshi: This is over and above.

Neha Manpuria: This is over and above that, right?

Moderator: The next question comes from the line of Sumangal Pugalia from Rare Enterprises.

Sumangal Pugalia: Sir, I had couple of questions. First is on the hospital margin metrics that you shared. So, in financial year FY '23, we had 2 hospitals that were above the 25% threshold and that first quarter, we've gone below 25% for all. So, is there a classification change and specifically when we talk about what happened in these 2 facilities?

Vivek Kumar Goyal: So, as I mentioned earlier, in the current quarter, we have slightly changed the way we were reporting the unit wise EBITDA. Earlier, we were not deducting the corporate expenses from the EBITDA, and that's why the EBITDA margin was showing at higher side. This quarter onwards, we will be showing after deducting the allocated corporate expenses to these units and that's why this difference is there.

Sumangal Pugalia: Okay. So, the operational performance in these 2 facilities we would say, strong versus the other states?

Vivek Kumar Goyal: Yes. So, there is some impact of the operational performance also. I will not say that it is apple-to-apple like last quarter. This quarter we have discussed it's slightly soft for us and it is reflecting in almost all the units because of seasonal impact and on this sector. And I think major reasons for margin drop is the allocation of corporate expenses, of course.

Sumangal Pugalia: Okay. And specifically, the FEHI facility, sequentially, there also -- there is a drop from INR112 crores to INR106 crores, is that just seasonal or there is more -- how has the performance there been?

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Vivek Kumar Goyal: So, FEHI, on the revenue side, they are doing quite okay. This quarter, in profitability side, they are slightly low because of the inflationary impact is there in terms of employee increment and all those stuffs. Capex hit this further from day one. So, they are taking various measures, as Dr. Raghuvanshi mentioned on the manpower rationalization and things like that, which should help in improving the profitability and not just inflationary cost.

Moderator: The next question comes from the line of Abhay Marda from Yashwi Securities.

Abhay Marda: Hello. Am I audible?

Ashutosh Raghuvanshi: Yes.

Abhay Marda: Hello. Yes. So, I just wanted to ask about the current opera-

tional bed situation. Like, what is the number of beds that you're utilizing right now, according to which you are calculating your average bed per operational revenue?

Vivek Kumar Goyal: So current operational beds on a P&L perspective are -- for about 4,100.

Abhay Mar: But at this time, it contrary size, as per the annual reports, like these are showing up, as the number of beds are more than 4,700. So, I'm not able to get the exact figures.

Vivek Kumar Goyal: So, like I'd explained also from a P&L perspective, from a modelling perspective, you guys should purely take the P&L beds. When we add O&M, O&M doesn't contribute significantly.

So, from a network perspective, we would have more than 4,500 beds. From a P&L perspective, it's about 4,100.

Abhay Marda: Okay. So, ARPOB it's been calculated as per 4,100 beds, right?

Ashutosh Raghuvanshi: Absolutely.

Vivek Kumar Goyal: Yes, yes. Absolutely, right.

Moderator: The next question comes from the line of Nikhil Mathur from HDFC Mutual Funds.

Nikhil Mathur: Yes, sir. Thanks a lot for the follow-up. I'm not sure if this has been discussed, but the international growth seems pretty strong in this particular quarter. If I am calculating the numbers right, it's up 30% in this quarter. So, any thoughts you can share on the outlook for the international business? What has happened in this quarter? Was there some base effect at play here or new countries are opening up?

Vivek Kumar Goyal: Yes. So, international business is going up, and this is the trend we are seeing, and we expect a further growth. It is contributing around 9% of our total revenue. We expect it should reach double-digit very soon on an annualized basis. So that is what we are expecting. And, as you know, the borders are opening up, we are seeing more and more international business.

Nikhil Mathur: But, sir, was this always anticipated by the leadership that international will grow at such a such a pace or something post-COVID is emerging as new and that's why a better traction is being seen on the international side?

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Vivek Kumar Goyal: Not really. We were already operating pre-COVID at around 10% to 11% on the international business. And we were confident during COVID period also that post-COVID restrictions go away, we should go back to that 10%, 11% and that is what I'm saying that we should very soon should be seeing that 10%, 11% on annualized basis. In absolute term, we have already surpassed that number. In absolute -- international revenue, we have surpassed that number.

Nikhil Mathur: Sir, I'm not trying to look at on a personal sales basis because the domestic has also gone up, so, there might be some variation in the percentage contribution. But if I purely look in the absolute number for the international business, they are growing pretty heavily, right? I mean, at more than 20% for the last 2 years or so. I mean, pre-COVID also, was there an anticipation that in the coming 4 years, 5 years, India will emerge as a big medical tourism hub and hence the growth will be super normally high, was that also an expectation pre-COVID as well?

Ashutosh Raghuvanshi: Yes. I think, there has been lot of systemic effort from the government side also to project India,

as a destination for COVID. They have come up with the medical tourism portal and there are many other areas of medical value travel on which the government is facilitating now. So, it was anticipated that this will happen. But I think some of these efforts are also leading to increase in the number of patients coming to India for seeking treatment.

Nikhil Mathur: And, sir, one more question. There was an article in the media recently that the top hospitals in the country are facing challenges on retaining the nursing staff. Many nursing staff they get trained for 2 years, 3 year

s and many of them make their way to the Gulf, where the salaries and payments are much higher. So, any challenges are you seeing on this front, or do you think that there are enough initiatives being done to have a good pool of nursing staffs?

Ashutosh Raghuvanshi: The nursing is a challenge for the entire industry and this challenge remains. Initially, the challenge was on the clinicians as well, but that part is being addressed quite a bit by the number of seats, which has been increased in last 10 years. But on the nursing side, lot of effort is still required. I think, we will see this kind of stress for next 3 years to 4 years going forward as well. So, we also face the same challenge, as other hospitals because nurses, who intend to go abroad, they tend to come to the branded hospitals such as ours and others in order to seek experience, because on basis of this experience, they can get positions, which are very remunerative for them. So, this is a challenge of the industry. Together the industry bodies are taking lot of actions in this direction and trying to see how we can bridge the gap. But it is definitely a challenge, Nikhil.

Nikhil Mathur: Could it have I mean, things might get sorted out from a 3 year to 5-year horizon, but in the next 12 months, 15 months, 18 months, could it have an inflationary impact on your margins? I mean, is that budgeted in when you are giving some guidance on the hospital margins?

Ashutosh Raghuvanshi: Yes. This is well factored in.

Moderator: The next question comes from the line of Harsh Bhatia from Bandhan AMC.

Harsh Bhatia: Hope I'm audible?

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Ashutosh Raghuvanshi: Yes.

Harsh Bhatia: Yes. So just 2, 3 quick clarifications. One in terms of the price hike, if you would be able to quantify the price hike for the first quarter because I think for the full year the guidance was 4% to 5% if I'm not wrong? So, the first quarter price hike, including the CGHS effect would be to what extent?

Ashutosh Raghuvanshi: It would be approximately on a blended level about just towards 3%, less than 3%, I think.

Harsh Bhatia: And very quickly in terms of the ARPOB growth trajectory, I think, sir, March quarter, we had guided for the ARPOB to grow at around 6% to 7%, but the first quarter is somewhere around 12%. So, going forward, just from a FY '24 perspective, one should assume a moderation to low - single digit growth rate for the ARPOBs? That would be the right way to think about it?

Vivek Kumar Goyal: Yes. So, because the base has become now high, this quarter, we have seen a significant increase in the ARPOB. That's why I'm giving you a guidance that 4% to 5% is the growth we are expecting for the full year.

Harsh Bhatia: And just in terms of the broader thought process for this 1,400 -bed addition, I think from the previous comment, I could make out that a large part of this has to be in the NCR region. So, in case, if you are thinking about these brownfield plants, how is the bandwidth availability across the spectrum in terms of nursing or surgical staff and the doctors? What is the broader thought process in terms of that bandwidth availability, purely from the NCR perspective?

Ashutosh Raghuvanshi: Yes. So, NCR is a region, where the availability of human resources is not a challenge in general. And, specifically, for us being a brand, we are an attractive place for clinicians, as well as other staffs to come. So, that definitely is not too much of a challenge in NCRs.

Harsh Bhatia: Good. Just to clarify, there has to be some -- a certain level of addition that has to be in sync with the brownfield addition, as well in the existing assets, that's the way to -- right way to think about it?

Ashutosh Raghuvanshi: Yes, certainly. But it would not be in the same ratios, as it would be for a greenfield hospital. So, suppose there is already a running

hospital, where we are increasing the capacity by, say, 100%, the number of staff additions will not be 100%, it will be only up to the tune of about 60% to 70% or even less.

The reason for that is because the scale starts giving the advantage and you won't have to people doing the same function, so because of that, the costs get rationalized. And that was the point, which we were making earlier that post this expansion, most -- many of the hospitals in NCR area and outside as well would be to the tune of about 450 beds to 500 beds each and that would give advantage on the scale, which would definitely get translated into a better utilization and

better of resources and better margins as well.

Moderator: The next question comes from the line of Nitin Agarwal from DAM Capital.

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Nitin Agarwal: Sir, on the Gurgaon, the NCR assets, given all the unrest, which has been happening in the state in recent times, does it have any implications for our business for the quarter?

Ashutosh Raghuvanshi: No, Nitin, not at all. I think, this is an unfortunate event, but it did not really have too much impact on the normal life, day to day life. So, I don't think this has any direct implication for healthcare per se. I think, healthcare is agnostic to the politics and religion.

Nitin Agarwal: And Dr. Raghuvanshi, I think, earlier in the call, there was a comment made around the fact that our staff costs and all little on the higher side and that's an opportunity for us to work on. So, if you can maybe just spend some time on where do we see opportunities for cost reduction? And what could be the extent of cost optimization, rationalization that we can probably see over a period over the next, say, maybe a couple of years?

Vivek Kumar Goyal: So, this is an ongoing exercise, and we do this exercise every 2 years, 3 years to understand, which unit is operating over and above the standard operating metric. And based on that, we'll take some corrective decisions. So, we will be not able to quantify at present, which unit, how much amount and things like that, but definitely we see there is opportunity to reduce or rationalize the structure.

Nitin Agarwal: Sir, I mean, not going in a sort of unit wise detailing of the possible opportunities, but on an overall sense, I mean, just to get a broad sense from whatever that you see, is there a possibility from these cost rationalization measures to take out 1%, 2%, what kind of savings and what the network can accrue over the next, say, year or so?

Vivek Kumar Goyal: Yes. We can easily target 1% at least on the staff cost over a period of 2 years.

Moderator: The next question comes from the line of Kunal Randeria from Nuvama.

Kunal Randeria: Just one question on ARPOB and apologies if it's been discussed. So, your 4% to 5% growth implies somewhere around INR57,000, INR58,000 for the full year. Since you've already done INR60,000, it shows that next few quarters, you will see a very sharp reduction. So, could you share some reasons? Is there a change in case mix that we expect, or some new beds have been commissioned that could come at lower ARPOBs. Any more colour would be helpful?

Anurag Kalra: Sorry, Kunal, can you just repeat? Your voice is not very clear. Couldn't understand the question.

Kunal Randeria: Sure. So, you have said that ARPOB growth could be around 4% to 5%, which implies that it should be around INR57,000, INR58,000 for the full year. Now, since you've already done INR60,000 in Q1, it implies that in the next few quarters could see a sharp reduction. Any colour why that would happen?

Vivek Kumar Goyal: Yes. So, we have to see the ARPOB growth in all aspects. As we mentioned, the current quarter, there was a high increase of the surgical volume. We cannot expect the surgical volume to grow at the same pace, and there is some seasonal impact was

also there. So, looking at all those

things, we are maintaining our ARPOB increase guidance, as we have given you at the beginning of the year, 4% to 5% annually. So, it is to take into consideration the surgical volume will not be growing at the same pace at which it is growing in the current quarter.

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Kunal Randeria: And just one more, slightly high-level question. So, while you have done very well in onco in the last four years, I believe, the contribution would have gone up from 7% to 14 -odd percent now. It has come at the cost of, let's say, a cardiac therapy, which has gone down from 22% to 18% or something like that. So, just trying to understand how the change in your therapy mix has impacted your margins either positively -- I'm assuming, it's positively in the last 3 years to 4 years?

Vivek Kumar Goyal: Yes. Why it's actually is -- something, I could not get the complete...

Kunal Randeria: So, in the last 4 years, your oncology has done very well, but it has come at the cost of cardiac therapies, so I'm just trying to understand the change in the therapy mix to your overall revenue. How would that have impacted the margins, purely from a therapy perspective?

Vivek Kumar Goyal: Yes. So, oncology definitely has a lower margin, and we have to share a substantial portion of the revenue with the surgeon also many times, so that has impacted overall margin if we see after removing the doctor's shares. And oncology business is growing in line with the market. So, we expect that trend will continue.

And what we are also targeting -- the other high-margin facility should also contribute more, so that is the game plan. We don't want to reduce the oncology business. We want to grow oncology

business to the extent possible . But at the same time, the high -margin business should also grow.

So that will keep a good balance between the margin expectations.

Kunal Randeria: But could you sort of just give a bit more colour on the kind of therapies that you classify, as higher margin?

Vivek Kumar Goyal: For example, renal business, their contribution in this quarter has come down, to some extent, which is a high margin unit for us. Similarly, the renal and pulmonary, sir, where the contribution has come down slightly. So, all this facility, we are having some plan on how to improve the specialty mix on these 2 overall.

Kunal Randeria: I have a few more actually. Maybe, I'll take this offline.

Moderator: Thank you. As there are no further questions, I now hand the conference over to the Fortis management for closing comments.

Anurag Kalra: Thank you, Sir. Ladies and gentlemen, thanks for taking the time to be with us on the call today. If there are follow -up questions, me and my colleague, Gaurav, are available to address those either over a phone or email, whatever you guys prefer. Thank you very much, again, for your time today, and have a good day. Thank you.

Moderator: On behalf of Fortis Healthcare, that concludes this conference call. Thank you for joining us, and you may now disconnect your lines.

Call: Nov 2023

FORTIS HEALTHCARE LIMITED

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November 15, 2023

FHL/SEC/202 3-24

The National Stock Exchange of India Ltd.

Scrip Symbol: FORTIS BSE Limited

Scrip Code:532843

Sub: Transcript of Investors / Analysts' meet under Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015

Dear Madam / Sir,

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed herewith the Transcript of the investors / analysts meet held on November 10, 2023 to discuss the Company's un-audited financial results for the quarter and period ended September 30, 2023 . Further, the said transcript is also available on the website of the Company i.e. Transcript

This is for your kind information and record .

Thanking you,

Yours Faithfully

For Fortis Healthcare Limited

Murlee Manohar Jain

Company Secretary & Compliance Officer

M. No. F9598

Encl: a/a

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Fortis Healthcare Limited

Q2 FY'24 Post Results Conference Call

November 10, 2023

MANAGEMENT : DR. ASHUTOSH RAGHUVANSHI – MANAGING
DIRECTOR AND CHIEF EXECUTIVE OFFICER – FORTIS
HEALTHCARE LIMITED

MR. VIVEK GOYAL – CHIEF FINANCIAL OFFICER –
FORTIS HEALTHCARE LIMITED

MR. ANURAG KALRA – SENIOR VICE PRESIDENT ,
INVESTOR RELATIONS – FORTIS HEALTHCARE
LIMITED

MR. AMIT MAHENDRU – DEPUTY GENERAL
MANAGER, INVESTOR RELATIONS – FORTIS

HEALTHCARE LIMITED
MR. AVINASH TRIPATHI – SENIOR MANAGER,
INVESTOR RELATIONS – FORTIS HEALTHCARE
LIMITED

Fortis Healthcare Limited
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Moderator: Ladies and gentlemen, good day, and welcome to the Q2 FY '24 Post Results Conference Call of Fortis Healthcare Limited. As a reminder, all participant lines will be in the listen -only mode . There will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during this conference, please signal an operator by pressing star and then zero on your touch -tone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Anurag Kalra, Senior Vice President, Investor Relations at Fortis Healthcare Limited. Thank you, and over to you, Mr. Kalra.

Anurag Kalra : Thank you, Dorwin . A very good evening, ladies and gentlemen, and thank you for joining us on our Quarter 2 FY '24 call. The call is being chaired by Dr. Ashutosh Raghuvanshi, our Managing Director and CEO. With me, we have Mr. Vivek Goyal, our Chief Financial Officer. And I also have with me my colleagues from Investor Relations and M&A, Amit as well as Avinash.

Before we start the call, I would just like to state that as you're all aware, we are a listed company and our material subsidiary, Agilus Diagnostics, has filed a DRHP for a proposed IPO in September 2023. In light of the publicity restrictions imposed on Agilus and the company due to the proposed IPO, no further information other than that already contained in our investor presentation and press release can be shared. Due to the restrictions, we would also not be in a position to clarify or answer any questions on the diagnostics business performance or on the proposed IPO at this point in time. We do appreciate your understanding on this.

We should begin the call with some opening comments by Dr. Raghuvanshi on the consolidated and the hospital business performance, and then we can open the floor for questions-and-answers. Over to Dr. Raghuvanshi.

Ashutosh Raghuvanshi: Thank you very much, Anurag. A very good evening to everyone. Thank you for joining us at this late hour for our Q2 '24 earnings call. I extend my warm greetings to you for this festive season.

I would like to straightaway talk about the performance of the company in the quarter and the 6 months ended September 30, 2023. We have witnessed a strong set of earnings for the quarter. Our consolidated revenues have increased 10.1% versus Q2 of financial year '23 to INR1,770 crores. Our consolidated operating EBITDA for the quarter was at INR330 crores compared to INR303 crores in Q2 of financial year '23 and INR273 crores in Q1 of financial year '24. Consolidated operating EBITDA margin was at 18.6% versus 18.8% in Q2 of financial year '23 and better than 16.5% in quarter 1 of financial year '24.

At the PAT level, we reported a profit after tax prior to exceptional items of INR180 crores compared to INR167 crores in Q2 of financial year '23. This is approximately 8% growth versus the corresponding quarter and 47% growth versus Q1 of financial year '24. Reported PAT stood at INR184 crores versus INR218 crores in the corresponding previous year. On the balance sheet, we remain quite healthy with a net debt -to-EBITDA ratio of 0.29x versus 0.44x at the end Fortis Healthcare Limited
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of quarter 2 of financial year '23. Our net debt stands at INR393 crores as of September 30, 2023, and our debt equity mix allows us the flexibility to leverage our balance sheet to further our growth objectives.

Let me now also briefly touch on the consolidated H1 financial year '24 financial numbers of the company. For the H1, our consolidated revenue stood at INR3,427 crores, a growth of 10.7% over the corresponding previous period. Operating EBITDA for the period was INR603 crores versus INR554 crores for the H1 in financial year '23, translating into a margin of 17.6% versus 17.9% in the corresponding

previous period. PAT, excluding exceptional items for the period, stood at INR303 crores versus INR301 crores for H1 of financial year '23. It has been relatively flat year -on-year. Reported PAT stood at INR308 crores versus INR353 crores in the corresponding previous year.

I'm very pleased with the way the hospital business has performed. Our hospital business

revenues have grown 12% versus Q2 of financial year '23 and 7.3% versus Q1 of financial year '24. On the profitability metrics, our hospital business operating EBITDA stands at INR268 crores, an increase of 13.1% and reflecting margins of 18.4% versus 18.2% in Q2 of financial year '23 and 15.2% in Q1 of financial year '24.

The improvement in margin is attributed to a healthy performance of all the key hospital operating metrics, which I shall speak of in a while. To highlight the contributions of the hospital operating EBITDA to our total consolidated EBITDA increased to 81% in Q2 of financial year '24 versus 78% in Q2 of financial year '23, which indicates the positive momentum in our hospital business earnings, allowing us to sustain our overall profit margins.

On the international patient business, we continue to witness strong traction. International patient revenue were at INR127 crores, a growth of 15.6% over the corresponding quarter and 10.6% over the trailing quarter. International patient revenue during Q2 of financial year '23 contributed 8.3% of total hospital revenue versus 8% in Q2 of financial year '23 and Q1 of financial year '24.

The revenue contribution from the company's key medical specialty in Oncology, Orthopedics, Renal Sciences, Cardiac Sciences, Neurosciences and Gastroenterology to overall hospital revenues increased to 61.2% in Q2 of financial year '24 from 60.5% in Q2 of financial year '23. Revenue from Gastro Sciences, Oncology and Renal Sciences grew in excess of 20% versus the corresponding previous quarter. All of the above factors contributed to a healthy increase of 11.8% in ARPOB to INR2.21 crores from INR1.97 crores in Q2 of financial year '23. Our occupancy stood at 68.7% versus 69.6% in Q2 of financial year '23 and 63.7% in Q1 of financial year '24.

On our brownfield expansions, which we have been discussing with you, we remain on track to add approximately 250 beds to our network in the current financial year across the facilities such as Mulund, Anandpur in Kolkata and BG Road in Bangalore and Ludhiana with a total planned addition of close to 1,400 beds in the next 3 to 4 years. Further augmenting our bed expansion plans in Delhi NCR and Punjab clusters, we are also evaluating new opportunities, including optimizing the currently available space in our additional beds in Mohali and Shalimar Bagh.

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And our Board has approved the extension of these projects as well. So the beds which are going to come from Mohali are going to be another 400 beds in addition to the beds which we have said earlier.

On the digital transformation front, we continue with our efforts to implement EMR solution that will form a platform to integrate our HIS. The rollout of such a EMR system is currently underway. myFortis app and other applications are also being bolstered, that would enable us to digitize the patient journey and provide a personalized and a better experience for our patients. During the quarter, we commissioned several medical programs and further strengthened our medical facilities. FMRI Gurgaon launched state-of-the-art digital PET CT for advanced imaging and cancer diagnostic. We are getting ready for our MR LINAC, which is going to be the first of its kind in Northern India. We have further augmented our medical infrastructure by commissioning high-value medical equipment, noticeably LINAC, Cath Labs, neuro navigation system and Ortho Robots in our key facilities.

Commensurate with our medical pro

gram expansion, we continue to attract high-quality clinical talent. We have onboarded clinicians in the specialty of Oncology, Renal Sciences, Neurosciences, Cardiology and General Surgery during this quarter.

With that, I would like to conclude my remarks by reiterating to all of you that we remain steadfastly committed towards our growth path in order to further our operational performance and create long-term value for all our stakeholders. Thank you. And now I would like to hand over to Mr. Anurag Kalra for taking the session further.

Anurag Kalra : So thank you, sir. Ladies and gentlemen, we shall now open the floor for question and answers.

Can I request the moderator to begin, please. Dorwin ?

Moderator: The first question is from the line of Syan Mukherjee from Nomura.

Syan Mukherjee : Sir, on hospitals, if I look at this quarter and also for the half year, you recorded 12% -13% growth. But that has not translated into appropriate EBITDA growth. I mean we have seen margin expansion of around 20 basis points year-on-year this quarter. This is after Arcot Road divestment. So I mean, there's a strong ARPOB growth, the specialties that you're focusing on are growing. But it seems that EBITDA growth is not coming through. Is there anything that's sort of hurting EBITDA? And how confident are we to sort of achieve 20% EBITDA for the hospital for the next financial year?

Vivek Goyal: So I can take this question, Syan. Your observation is to some extent correct in terms of EBITDA margin. But in absolute terms, if you see the EBITDA in actual terms has gone up by almost

INR30 crores as compared to the corresponding period for the previous year. And as regard to the Arcot Road divestment, there is not much visibility in the EBITDA margin improvement, there are a couple of reasons for that. One is of course the legal and the professional cost. As you know, company has engaged into this legal battle and there is some amount of work that happened during this period. And because of that, the legal and the professional charges have gone up to some extent. And plus the revenue increase is mainly coming from the high-end
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specialties like onco and ortho, which is typically a low-margin business. So that will lead to not much expansion in the margin but in absolute terms the margin has gone up.

Syan Mukherjee : And sir, are we sticking with our guidance of 20% for next fiscal?

Ashutosh Raghuvanshi: Absolutely. There are several reasons why the margin has been at this level. But our guidance is absolutely intact for next year.

Syan Mukherjee : And I don't know, sir, if you can talk about diagnostics, but the observation is that the growth in margins there are also weak versus, let's say, compared other peers in the industry. Will you be able to comment on any reason why the growth is lower?

Ashutosh Raghuvanshi: No, Syan, I'm sorry. As we said in the beginning of the call, we can make no remarks about the diagnostic business, please.

Moderator: The next question is from the line of Madhav Marda from FIL.

Madhav Marda : I just had two questions. The first one was when you said that we have evaluated new opportunities at Mohali and Shalimar Bagh, does that mean these are additional brownfield expansion opportunities? And would that also mean that the 1,400 beds in the pipeline, which we talked about earlier, does that mean you have a bigger pipeline now available for the company in the next sort of 3 to 4 years as we expand?

Ashutosh Raghuvanshi: Yes, absolutely. As I said earlier, Madhav, these are in addition to the earlier brownfield, which we have already said. And these are both brownfield expansions. Shalimar Bagh is, of course, the new block is going to be constructed. And we are in the stage of getting all the final permissions. All the plans are ready for that. Similarly, in Mohali

also, we're in the process of getting all the permissions. But these would be addition to the 1,400 beds which we have said earlier.

Madhav Marda : So does that make up brownfield pipeline now, that 1,400 beds which we had kind of highlighted, that number is how much now in terms of beds for the company?

Vivek Goyal: It will be around 1,800 beds now.

Madhav Marda : Okay, okay. Wonderful. And just the second question was to Syan's point about the 20% margin guidance. What is the lever that can help us get there in FY '25 like versus the current period?

Vivek Goyal: Yes. So there are a couple of levers, we are continuously working on that. One is, of course, the occupancy ramp-up. As you can see in the current quarter also, occupancy-wise, we have still not reached 70% occupancy level. So we want to achieve that 70% occupancy level on the enhanced bed capacity. And that will lead to increase in the EBITDA margin. And secondly, there is more focus on the cost side, which would lead to some margin improvement in the coming year.

Madhav Marda : Sir, these brownfield beds, which you'll be covering 250 beds this year and somewhere next year. Typically, what is the breakeven? Like I mean given it's a pure brownfield and will be

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adding in facilities with good occupancy, EBITDA breakeven should be much faster. Is that the right understanding?

Vivek Goyal: Yes, absolutely. And that's why we are focusing more on the brownfield expansion. And your assessment is absolutely correct because all these expenses are coming in the facilities where occupancy level is already very high. And that will lead to earlier breakeven time.

Madhav Marda : Got it. And sir, could you just highlight like this additional legal and professional costs, which went up this quarter. How much is that amount?

Vivek Goyal: It is I think in the range of INR6 crores to INR7 crores in the quarter.

Madhav Marda : Incremental cost just on the quarter.

Vivek Goyal: Yes.

Madhav Marda : And this should come down or we should see the same level, or we expect it to come down a bit?

Vivek Goyal: It is very difficult to predict this variance because it will all depend on how quick this court proceeding happens and how much time the court really invests into that.

Moderator : The next question is from the line of Neha Manpuria from Bank of America.

Neha Manpuria : Sir, first on the Delhi High Court, do we have any update there or any time lines when we can look at a closure to that litigation and the forensic audit ?

Ashutosh Raghuvanshi: So the hearings are going on there, but we cannot really predict how much time it will take. There are a couple of dates which have been given during the month of November. I believe that we should have some clarity after these 2 dates, but then possibly the winter vacation s will come. So I expect that it should take at least 3 -4 months before an absolute clarity emerges.

Neha Manpuria : Understood, sir. And the second question is again on the hospital margin, the 20% margin guidance that we're still building. While I understand occupancy and all of the reasons that you've mentioned. But how essentially are we getting to that number, the higher occupancy, because we're also adding brownfield beds? While the breakeven time might be shorter, there is still some amount of ramp -up that will be required. So how exactly are we getting to the 20% margins, especially with the beds that we're adding even if they have are brownfield beds.

Vivek Goyal: Yes. So Neha, as we have mentioned earlier, we are quite confident that next year onwards, we will be adding 20% EBITDA margin on a yearly basis . So this year, there is some improvement in the margin, but the improvement may be higher if we're able to reduce this legal cost. So I think next year, we are quite hopeful because of the reason I've told you, one is the ramp -up of the existing occupancy.

As regard your question on the brownfield expansion and the initial losses , we are not feeling that type of hit in wherever we we could achieve the brownfield expansion. Reason being these

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are our existing hospitals, and they are already facing bed challenge. Most of them are operating at 75% -plus occupancy. So they will start contributing very early as we start these beds.

Ashutosh Raghuvanshi: Yes. And just to add to that is there area couple of hospitals where the occupancy figures are low - our BG Road in Bangalore and Mulund in Mumbai. So we are focusing on enhancing the clinical programs, et cetera, in these 2 places. That is one of the ways how we will be able to increase the occupancy. And then as Vivek said that in some of the other hospitals, already they are working at a good occupancy of 70 -plus or 75 -plus, rather.

Neha Manpuria : Got it, sir. And sir, in terms of capital allocation strategy, incrementally, are we looking at more assets for bolt -on like the Manesar asset that we took over? Do you think this 1,800 is what we are okay with? And do you want to execute on that before looking at more addition?

Ashutosh Raghuvanshi: Yes. Neha, see, as we said that this is our preferred way of growing, but then our aspiration is much beyond this. As we were talking about our balance sheet status and the debt position, we are not leveraged at all. So we have a huge capacity. Last ly, most of the CapEx, which is going to come for the brownfield expansion, 50% of it is from internal accruals and only 50% is on debt.

So this would mean that we would have a further capacity to do smaller projects like the bolt -on, which we did in Manesar. 1,800 beds also, mind you, does not include the Manesar beds. Manesar will have 350 beds over the next 2.5, 3 years, as we will commission them. And we will be starting with 150 beds sometime by the end of this financial year, and then the rest will be commissioned in phases. So it actually makes the brownfield plus Manesar put together close to about 2, 200- 2,300 beds. And then we will continuously look for other opportunities as well. And we believe that we are in a position to consider some larger assets as well.

Neha Manpuria : And this would be in the existing markets that we are in - meaning Delhi, Mumbai, those would be the markets here? Are you also open for tier 2 ? Because historically, we moved out of a lot of Tier 2 markets, which seems to be growing much faster than Tier 1. So are we still focused on our existing markets from a growth strategy point of view?

Ashutosh Raghuvanshi: Yes. We will remain focused on the clusters. Our cluster strategy is what we have , and we will continue with that approach.

Moderator: The next question is from the line of Aneesh Deora from Nomura.

Aneesh Deora : So my question is around ALOS numbers. So if I look at the ALOS numbers, till the last quarter, they were in the range of 3.5, 3.6 days. But in this quarter, I think all the historical numbers have also been revised upwards to 4.2 days and something like that. So could you just tell what has exactly happened? Has there been any small change in the methodology? Or what exactly has happened there?

Vivek Goyal: ALOS numbers .

Ashutosh Raghuvanshi: Yes. I think we will have to get back to you specifically on this.
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Vivek Goyal: So Aneesh, this ALOS number is basically the way we were calculating earlier and the way now we are calculating, we have recently implemented an IT tool – BI tool for our data analysis and MIS purpose, analysis purpose. In that, we have tried to standardize the way we were reporting ALOS. So some units were taking data here, some were not taking data here, those types of adjustments. And because of

that, this number is there. There is no change as such in the business metrics, et cetera. But the way it was calculated, there is a difference. Now the good thing is same way, and going forward, we can track according to this method, the number which is coming now.

Aneesh Deora: I understand. I understand. So essentially, if I look at the ALOS numbers of this quarter versus the quarter of the last year, there has been a reduction of about 5% in the ALOS on the new methodology that you have put out in this quarter. So just wanted to understand what is driving the ALOS reduction.

Ashutosh Raghuvanshi: Yes. One of our oncology work is going up and especially the medical oncology, that has been growing at a very fast pace. Similarly, the cardiology also has done well. So both these typically have lower average length of stay.

Aneesh Deora: Okay. Is there a particular focus on reducing the ALOS further? Any particular focus there?

Ashutosh Raghuvanshi: So the hospitals which work on higher occupancy, we always try to achieve lower and lower ARPOB. And even in the hospitals, it does not make sense for a patient to be in the hospital if they don't need to be. But the nature of the work we do is a lot of quaternary work, high-end neurosurgeries they are doing, et cetera. So orthopedics and other areas, the length of stay is not long. But in case of complex neuro procedures, it is typically much longer. So I think being at a blended basis, we don't expect it to dramatically come down from the current level.

Moderator: The next question is from the line of Dhara Patwa from SMIFS Limited.

Dhara Patwa: Sir, just one question. In the last call, you guided for 70%+ occupancy for FY '24. Sir, are we still maintaining that guidance? And if yes, then it means that Q2 would be much better. H2 would be much better than H1, and we need to have occupancy of 75% in the next 2 quarters to achieve that 70% occupancy.

Vivek Goyal: Yes. So we were talking about the exit occupancy. And this quarter, the occupancy level should be higher, but we should be mindful about (this quarter means that earning quarter, December quarter) – there is a festival season, as we know, and we believe will lead to lower occupancy in certain parts of the month, but our target is 70%.

Moderator: The next question is from the line of Shyam Srinivasan from Goldman Sachs.

Shyam Srinivasan: Dr. Ashutosh, just a question on HR, human resources, doctors. So we have onboarded many clinicians in different medical specialties. I think we have been doing it in the first half as well.

One of your peers earlier today have also mentioned that the guaranteed payouts for their doctors, they're trying to get more people on guaranteed payouts. So is this the time when costs

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are now starting to go up on medical personnel and maybe also nonmedical personnel? And is this one of the risks that could potentially put a question mark around our next year's margin number? What do you think is the biggest risk for achieving, say, a 20% EBITDA margin?

Ashutosh Raghuvanshi: Yes. So currently, the situation is such that there are new beds and the new facilities which are coming up in many parts of the country. So we do expect that there will be some movement of clinicians across various networks, et cetera, which happens from time to time. And as a result of that, definitely some of the guaranteed payments or the minimum guarantees which we offer, that would also go upwards.

But I do not see this as a very high-risk situation, nor is it a very concerning situation, for the simple reason that the availability of talent is also slightly different now compared to what it used to be a few years back. And the larger groups, not only us, but I would say the others as well, find it easier to attract clinical talent. So yes, we would definitely see a little

bit of churn

in coming time, but I don't see this as a risk.

Now if I were to think that what other risks could happen, that could be partly because some delays may happen in commissioning of some of the beds, which are getting ready. So that could be the only risk in my mind which could delay some of the progress which we are expecting in the next year. Other than that, I think all other factors are operational and cyclical in nature, and those can be very easily managed.

Shyam Srinivasan: Got it. So when I look at your professional charges to doctors, would there be a way to kind of gauge what's happening on that particular line item, especially on doctor compensation? Would

that give me the full picture? Or it could also be in employee benefits? So how should we track that wage inflation, especially for doctors?

Ashutosh Raghuvanshi: Yes. So our doctor costs typically is slightly higher. That is partly because our facility sizes are slightly smaller compared to a larger hospital. So we probably don't have that kind of productivity because of that. But I think we should not factor a very large increase in this cost. I think the majority of the cost enhancement which had to happen has already happened during this year.

Shyam Srinivasan : Got it. So that is helpful

Shyam Srinivasan : So I'm just looking at the margin matrix that you put out for the individual hospitals. And I'm just trying to project hypothetically, how this margin matrix will look in fiscal '25. When we look at the 4 cohorts that you have, will they all be like occupancy is the one number in the last column that will all go up to say, I'm assuming everything doesn't go to 70. But would that be the driver? I know I'm repeating this question, other parties may have asked. But I just want to figure out how a mix on these columns is the one that's more important. Or is there an element of payer mix that we are not talking about yet?

Vivek Goyal: So if I can answer that, Shyam. So it is a mixture of both. In some units, it is a question of better utilization of the facility and the manpower because the occupancy is low, we have discussed about Mulund, BG Road in that category.

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And there are other hospitals where we have expansion going on. For example, Faridabad and Noida, where the expansion is going on. Faridabad, we will be completing the expansion by this year on time. So then one, the disturbance because of the expansion will go away and that will lead to the better patient experience and better revenue. And second is more of beds will be coming in. At Faridabad, by the way, we are operating at almost 80% -plus occupancy level. So those types of things will play. It will not be a straight answer face to face. But having said that, most of the hospitals, barring 4-5 which we have discussed, should be moving above 15% EBITDA margin, I believe.

Ashutosh Raghuvanshi: Just to add to your second part of the question as to whether the payer mix is something important. So payer mix-wise, I think we have approximately 20% of scheme patients, which is the CGHS, ECHS, etcetera, we are doing. So we do aim to try and reduce that, but we want to do it in a calibrated manner because it is also important for us to ensure that the occupancy remains high. So that will go in, so some of the units, we will accept a higher proportion of these patients and some of the units, we will not accept in that space.

Moderator: The next question is from the line of Naman Bhansali from Perpetuity Ventures.

Naman Bhansali : First question is on the oncology side. So you had mentioned this in the previous call that oncology is a lower -margin business for you. But I was just wondering on the industry-wide perspective and majorly from the Northern region specifically, there are a lot of different hospital players who are trying to more and

more enter into the oncology therapy. So there are certain peers who are reporting certain numbers which are incrementally giving them north of 35% margins on their oncology business.

And there are certain players who are trying to enter into oncology business incrementally because it's a higher -margin business. And these are not present totally into Delhi NCR, but Noida, etcetera. Just want to understand that why from an industry perspective is oncology a higher -margin or a lower -margin business for you.

Ashutosh Raghuvanshi: No, no. Oncology is not a lower -margin business. But when we look at the oncology business in totality, it is important to understand that different institutions' contribution from medical oncology is slightly higher, then the contribution margin becomes lower. And as a result of that, in the percentage terms, it may not be as high as an institution, which is being more of surgical work or an equal proportion of surgical versus medical oncology.

So the reason why in medical oncology the contribution margins are lower is because the price of drugs is higher and the service charges are also low. So that is precisely the point which we were trying to make. We strongly believe that oncology is a growth area. We have seen a growth of about almost close to 27%, I think, in our oncology. And we have committed a huge CapEx to enhance our oncology services in the entire region.

We also strongly believe that oncology is the business of future. But in absolute terms though, we would have EBITDA being generated. But in terms of profitability, I do not believe that it can go on a very high percentage because of the reasons I mentioned. But one thing which we

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are doing is we are enhancing our surgical oncology load. As that keeps on building, we will see a better margin from this segment.

Naman Bhansali : Got it. Got it. I understand. And as you said, you are trying to increase the surgical mix. So as the overall business, we usually see that Q2 is a lower surgical revenue business for us. So going forward, we maintain our aim to keep the surgical mix above north of 60%? Or would it fluctuate lower than 60%?

Ashutosh Raghuvanshi: Yes. There's some bit of seasonal variations happen. And also sometimes, we don't have a choice as this is a kind of a number in hindsight. We cannot really choose which patients get sick at what time. So because of that, what we typically will see is difficult to predict, but I expect the ratios to remain similar.

Naman Bhansali : Got it, sir. And lastly, we usually mention about rationalizing our portfolio in terms of the hospital which are performing well and which are performing bad . So today, if we have to say what would be the top hospitals which have been underperforming since last couple of years and are on the pecking order on the top for rationalization?

Ashutosh Raghuvanshi: Yes.

Vivek Goyal: So this is a continuous process. And as we are dealing with this metric, so we continuously evaluate. And our first endeavor is to make the hospital more profitable. And after exhausting all our resources if we feel that we are not able to add value, then only we try to rationalize by divesting those assets. Currently, we have done one. As you are aware, Arcot Road, we have divested. We are working on a couple of more. And as things progress, we will be able to tell you more about that.

Naman Bhansali : Okay, got it. And lastly, on the occupancy side. So as we are seeing, we are going to add another 250 beds by the second half of the year. And are we aiming at 70% -plus occupancy, can we see that number in the second half despite the 250 -bed addition? What would be your say on it?

Vivek Goyal: Yes. Overall, the occupancy for the year may not be 70%, which may be slightly lower. But next year, we are hoping of 70% occupancy .

Moderator: The next

question is from the line of Bino Pathiparampil from Elara Capital.

Bino Pathiparampil: So actually, just a request, we are adding 1,800 beds on a base of about 4,000 or 4,500, which is a large number. It would be great if you give a little more color in your press release and investor presentation about where these beds are coming up and a rough time line in terms of each year, each group of beds come up . That will be great and help us understand it more.

Ashutosh Raghuvanshi: Certainly, we can directly share with you. You can connect with Anurag, and our team will be happy to give you further details.

Moderator: The next question is from the line of Madhav Marda from FIL.

Madhav Marda : I just wanted to check on the brownfield expansion when these beds come in. Basically, how much of incremental EBITDA margin on this beds given some of the fixed costs are already in Fortis Healthcare Limited
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place? I don't want the exact number, but if you could give us some qualitative of broad sense, that will be very helpful to understand the kind of operating business [which we are in] .

Vivek Goyal: Yes. Obviously, the EBITDA margin of this brownfield expansion will be higher. As we already mentioned, the fixed cost will be reflected on the larger base. Having said that, there will be some costs which will go directly with the company that will anyway go away. So I am expecting EBITDA margin of at least 35% on this revenue on brownfield expansion.

Madhav Marda : Got it. Because EBITDA for Fortis, on 4,000 beds, 1,800 beds is like a very large number for brownfield, right? I mean, it should drive like from a 3 -5-year view, maybe margins should be quite very supportive, right, because it's a mix of launches that is very large in our existing base.

Vivek Goyal: Yes. Once we achieve a particular ramp -up on this 1,800 beds, definitely, it should give us a higher EBITDA.

Moderator: The next question is from the line of Syan Mukherjee from Nomura.

Syan Mukherjee : Sir, this 12%, 13% top line growth in hospitals, can you split it up between volume growth and realization per patient? And within realization per patient, which is an important driver, whether it is pricing of the procedures or case mix and payer mix, if you can sort of give some granular detail around the growth profile?

Vivek Goyal: Yes. So I will say it is the price increase of around 3%, and 7% to 8% is on account of volume increase and rest is specialties and the mix impact.

Syan Mukherjee : And this 3% price increase, is it like a normal trend, because sort of this is below inflation, right? And I mean, all your cost items would be sort of inflating at a higher rate.

Vivek Goyal: Yes, you're right. So this is basically on the overall revenue. And as you know, we are having government payer, also TPA is phasing down. So we don't have liberty to increase price like that. And plus, there are a major proportion of drugs and consumables, some with logistic -type

controls. So I will say in this may be in line with the industry norm.

Syan Mukherjee : Okay. I mean -- so there is no sort of increase in pricing, right? I mean like one of your peers were mentioning that there is greater demand for, let's say, private rooms or people are willing to spend more. So is that not a lever for you to sort of drive pricing?

Vivek Goyal: No, no, that will go into the ARPOB. That will come into the mix. When we say price means that for the same bed, what price we were charging a year earlier ; for the same procedure, what price we are charging, what price we have increased. Suppose somebody earlier wanted a double bed and now he wants single bed, that will go in the mix.

Syan Mukherjee : Okay. And so this trend is something which you think would sustain? I mean for next year, 7%, 8% volume growth and 3% price increase?

Vivek Goyal: Yes, yes. We are expecting the same.

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Moderator: We have no further questions. I would now like to hand the conference over to the management for closing comments. Over to you, sir.

Anurag Kalra : Yes. Thanks, all. Ladies and gentlemen, thank you very much for joining us on the call at this results. If there are any further queries, clarifications, please feel free to reach out to us, and we'll be able to help you as best possible. Thank you, and good night.

Moderator: Thank you. On behalf of Fortis Healthcare, that concludes this conference. Thank you all for joining us. You may now disconnect your lines.

Call: Feb 2024

FORTIS HEALTHCARE LIMITED

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FHL/SEC/2023 -24 February 10, 2024

The National Stock Exchange of India Ltd.

Scrip Symbol: FORTIS BSE Limited

Scrip Code:532843

Sub: Transcript and Audio Recording of Investors / Analysts' meet under Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015

Dear Madam / Sir,

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed transcript and the link of audio recording of Investors / Analysts' meet held on February 8, 2024 to discuss the Company's un-audited financial results for the quarter and period ended December 31, 2023 and the same is available on the website of the Company i.e. Audio recording Link and recording Link

This is for your kind information and records.

Thanking you,

Yours sincerely

For Fortis Healthcare Limited

Murlee Manohar Jain

Company Secretary & Compliance Officer

M. No. F9598

Encl.: a/a

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"Fortis Healthcare Limited "

Q3 & 9M Ended December 31, 2023 Post Results

Conference Call "

February 08, 2024

MANAGEMENT : DR ASHUTOSH RAGHUVANSHI – MANAGING
DIRECTOR AND CHIEF EXECUTIVE OFFICER – FORTIS
HEALTHCARE LIMITED

MR. VIVEK GOYAL – CHIEF FINANCIAL OFFICER –
FORTIS HEALTHCARE LIMITED

MR. ANURAG KALRA – SENIOR VICE PRESIDENT ,
INVESTOR RELATIONS – FORTIS HEALTHCARE
LIMITED

MR. AMIT MAHE NDR U - INVESTOR RELATIONS –
FORTIS HEALTHCARE LIMITED

MR. AVINASH TRIPATHI - INVESTOR RELATIONS –
FORTIS HEALTHCARE LIMITED

Moderator: Ladies and gentlemen, good day, and welcome to the Q3 FY'24 and 9 months ended December 31, 2023, Post Results Conference Call of Fortis Healthcare Limited. As a reminder, all participant lines will be in the listen-only mode, and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Anurag Kalra, Senior Vice President, Investor Relations at Fortis Healthcare Limited. Thank you, and over to you, Mr. Kalra.

Anurag Kalra : Thank you very much. A very good morning and good afternoon, ladies and gentlemen, and thank you for joining us on our Q3 FY'24 earnings call. The call is being chaired by Dr.

Ashutosh Raghuvanshi, our Managing Director and CEO. With him, we have Mr. Vivek Goyal, our Chief Financial Officer. And I also have my colleagues from Investor Relations and M&A, Amit Mahendru and Avinash Tripathi.

Before we start the call, I would just like to state that, as you're all aware, we are a listed company and our material subsidiary, Agilus Diagnostics has filed a DRHP for the proposed IPO in September 2023.

In light of the publicity restrictions imposed on Agilus and the company, which due to the proposed IPO, we would not be in a position to share any further information other than what is already provided in our investor presentation and press release. Due to these restrictions, we would also not be in a position to clarify or answer any questions on the Diagnostics business performance or on the proposed IPO at this point in time. We do appreciate your understanding on this.

We will begin the call with some opening comments by Dr. Raghuvanshi on both the consolidated and the hospital business performance. And then we can open the floor for question and answers. Over to Dr. Raghuvanshi.

Ashutosh Raghuvanshi: Thank you, Anurag. Good morning and good afternoon, everyone. Thank you for taking time to join us on our Q3 FY'24 earnings call today. I wish you all happy New Year and hope all of you are doing well. I shall come straight to the results of the quarter and my thoughts on the business performance and way forward.

Our business performance in Q3 has been satisfactory considering the seasonal impact of festivals in some of our key geographies. The hospital business has shown a relatively improved performance compared to the corresponding quarter last year and predictably slightly lower than the trailing quarter.

Our consolidated revenues were at INR 1,680 crores, a growth of 8% versus the corresponding previous quarter. This compares to INR 1,560 crores in Q3 of financial year '23. Our consolidated operating EBITDA margin was 16.9% versus 17.7% in the corresponding previous quarter and versus 18.6% in the trailing quarter.

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To highlight the contribution of hospital operating EBITDA increased to 88% in Q3 of financial year '24 versus 76% in Q3 of financial year '23. This indicates a positive momentum in our hospital business earnings, allowing us to sustain our overall profit margin. The strength in our hospital business has also largely offset our performance in Diagnostic business.

At the PAT level, we reported a profit after tax prior to exceptional items of INR 127 crores, compared to INR 131 crores in Q3 of financial year '23. Reported PAT stood at INR 134 crores versus INR 142 crores in the corresponding previous year. Our balance sheet remains healthy with a net debt to EBITDA of 0.45x compared to 0.41x in Q3 of financial year '23. Our net debt at the end of Q3 financial '24 stands at INR 518 crores against INR 471 crores at the end of Q3 of financial

year.

Let me also briefly touch on the consolidated 9-month financial year '24 result numbers of the company. For the 9 months of financial year, our consolidated revenue stood at INR 5,107 crores, a growth of 9.7% over the corresponding previous period. Operating EBITDA for the period was INR 887 crores versus INR 830 crores for the 9 months of financial year '23, translating into a margin of 17.4% versus 17.8% in the corresponding previous period.

PAT, excluding exceptional items for the period stood at INR 429 crores versus INR 432 crores for 9 months of a marginal decline year-on-year, reported PAT stood at INR 442 crores versus INR 495 crores in the corresponding previous year.

I'm pleased to share that our hospital business continues to witness a healthy performance. And

despite the quarter having a seasonality impact, we have shown a better performance on a year-on-year basis. The consolidated profitability numbers that I shared with you just now clearly reflect this.

We have witnessed an occupancy of 64% in Q3 versus 66% in the corresponding quarter, owing to an increase in the operational beds by approx. 100, while the occupied beds remained flat year-on-year. Occupancy levels on a like-to-like basis were at similar levels. ARPO B witnessed a strong growth, growing by 10.6% year-on-year to reach INR 2.23 crores.

The growth was driven by a consistent shift towards higher complexity procedures. This year, on an annualized basis, we will be performing more than 60,000 cardiac procedures, more than 3,500 robotic surgeries and more than 1,100 transplants, which demonstrates the shift we have been talking about. All these factors enable the hospital business revenues to reach revenue of INR 1,389 crores in the quarter, a growth of 10% versus Q3 of financial year '23.

The hospital business operating EBITDA grew 19% to touch INR 251 crores reflecting a margin of 18% versus 15.7% margin in Q3 of financial year '23. The improvement was driven by our consistent focus on improving operational efficiency and optimizing costs against all heads, including general administration services, repair and maintenance, etcetera.

Revenue from international business stood flat at INR 113 crores versus INR 114 crores in the corresponding previous quarter. International patient revenue was flat year-on-year, primarily due to geopolitical reasons in the Middle East and flow of patients from countries such as Iraq and Bangladesh.

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You would recall that we have been speaking about our portfolio rationalization strategy. To that effect, we have divested two of our loss-making facilities in Chennai, first the Arcot, Vadapalani facility in July 23, followed by the Malabar facility, which we concluded last week, leading to an improvement in overall profitability for the company in the future.

We are progressing on our brownfield bed expansion, which are expected to incrementally add almost 50% to our existing bed capacity. And when operationalized, will eventually see some of our key certainties such as Shalimar Bagh, FMRI, Mohali, PG Road, Noida become more than 450 beds each. This is expected to provide a higher degree of operating leverage in such facilities translating into healthy margin expansion.

Our expansion strategy continues to focus on deepening our cluster presence with the launch of new 70-bed unit in Ludhiana. This is the second facility in Ludhiana and the fourth in Punjab. This will take our total bed strength to ~800 in Punjab region. You will also notice that given our focus on bed expansion, we have also, for the first time, provided further granularity on our bed expansion planned in our investor presentation, close to 2,200 beds in the next 4 years will come through.

Our focus on strengthening our clinical programs continued through the quarter across all our facilities in terms of invest

ing in high-end medical infrastructure and on-boarding new medical

talent. The quarter witnessed addition of several M&A clinicians across various specialties like neurosurgery, oncology, cardiology, gastroenterology and GI surgery.

During the quarter, we commissioned several medical programs and further strengthened medical infrastructure, various facilities which includes the UP's first most advanced artificial intelligence powered state-of-art Cath Lab Fortis Hospital, Noida. Neuro ICU and advanced neuro lab at Fortis Hospital, Faridabad and launch of cutting edge surgical robot at Fortis Hospital Anandpur, Kolkata.

Another important aspect to highlight is the on-going success of our digitization initiatives, our strides in digital transformation, particularly with the implementation of EMR advancing positively. Revenue from digital channels such as websites, MyFortisApp and online campaigns grew by 32% year-on-year and their contribution to overall hospital revenues increased to 25.7% versus 21.5% in Q3 of financial year '23.

In this, we have also launched our new patient feedback management system, MyFeedback.

This platform will enable more engaging experience for our patients, as feedback will be collected through WhatsApp and QR codes. The application will also enable collection of feedback and addresses immediate patient concerns through its service request feature.

The revenue contribution from the company's focused medical specialties; oncology, orthopaedics, renal sciences, cardiac sciences, neurosciences, and gastroenterology to overall hospital revenue increased to 61.4% in Q3 from 60.9% in Q3 of financial year '23.

Specifically, revenue from gastrosciences, neurosciences, and oncology grew by 20%, 13%, and 12% respectively versus the corresponding previous quarter.

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In summary, our earning momentum remains healthy, specifically with respect to the hospital business and this I would expect to continue going forward. I say this with a sense of conviction as our future levers of growth are well -defined in our business plans, whether they be towards our brownfield expansion strategy or towards rendering our medical programs and technologies or towards effort to ensure that our patients are delivered a superb patient experience and excellent clinical outcomes .

Thank you and with this I would like to hand over to Mr. Anurag Kalra to take this forward.

Anurag Kalra: Thank you sir. Ladies and gentlemen we will now open the floor for question and answers.

May I request the moderator to begin please.

Moderator: Thank you very much. We will now begin the question and answer session. The first question is from the line of Neha Manpuria from Bank of America. Please go ahead.

Neha Manpuria: Thanks for taking my question, sir. Sir, on the hospital performance, the fact that we managed to keep margins flat , despite the bed addition that we saw and the seasonally weak quarter , does indicate the point that you've mentioned that we've been able to do a lot of cost optimization and improvement in efficiency. One, if you could highlight how much of this is already captured in this 18% margin. I mean, is there more scope for this to contribute to margins?

And second, as we see occupancy improve from the nine -month number of 66%, 67% on the existing beds, what is the trajectory? I mean, should we see margins crossing the 20% in the very near term or could that be a little more longer journey? Just trying to understand the, you know, gradient for the margin expansion that we should see in the hospital business ?

Vivek Goyal: Yes. Neha, I can take this question. As you rightly mentioned about margins, we are moving towards our target and the guidance given earlier of 20% margin by the year -end. So that we are expecting. And the main lever for our margin expansion is on the occupancy side . As you

de . As you

rightly pointed out, occupancy is slightly lower as against our expectation. And we are below 65% now.

And as we're able to move from the occupancy side, I think this margin improvement will be there . And plus, this divestment as Dr. Raghuvanshi mentioned about Malar, which is effective from first of February 2024 , that will also help in improving the margin.

Neha Manpuria: Maybe if I can be a little more specific here. If I have to say the 18% margin improvement, how much of it would come from Malar divestment. And how much of it would then come from, let's say, the other efforts that we are sort of implementing?

Vivek Goyal: Yes. So, I will say around 0.5% - 0.6% of EBITDA margin improvement because of the divestment . The Arcot Road would already factored in this quarter. So I'm not counting that.

So that will be on the Malar divestment side. Rest will be majorly from the occupancy improvement and some improvement in the cost side. And as some of the hospital like Ludhiana have operationalize only in the December. So this quarter, the result of that hospital will be also improved.

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Neha Manpuria: Okay. So the Ludhiana impact will flow through in the March quarter, the new facility effect?

Vivek Goyal: Yes.

Neha Manpuria: Okay. Understood. And my second question on the international patients, that was flat. I know you did mention in your opening comments that this is because of certain geopolitical issue.

How do we now plan to grow this business, given what's going on in the Middle East, and we don't know when that situation improves. Can this number move up from what we are seeing?

Vivek Goyal: Yes. So international business this quarter is slightly less as compared to previous quarter.

However, if you see year -to-date, we are doing quite okay. And in the month of January and February, we have seen that the business has come back to its September quarter number. So we expect this will continue to provide a good lever for increasing the occupancy and profitability.

Neha Manpuria: Understood. And last, I don't know if you want to answer this question. But if I were to do a simple consol minus hospital, the residual diagnostic business seems to have seen significant pressure on margins. Is there any one -off cost there? Any color that we can provide there?

Again, I don't know if you would give any color, but just trying my luck.

Vivek Goyal: Yes, as Anurag has mentioned, we are not supposed to discuss about the financials of the diagnostic, but I can, this much I can say it's some one -off in this. And the performance is impacted because of the one-off.

Moderator: The next question is from the line of Saurabh Kapadia from Sundaram Mutual Funds. Please

go ahead.

Saurabh Kapadia: So first question is on ARPOB so another 19% kind of growth can we sustain? And also, do we have levers further to improve on it given that a few beds additions are coming in.

Vivek Goyal: Yes. So we could see a very good ARP OB increase during this quarter and for the year, also the number is similar. I think it is mainly driven by the high -end cases, the more complex cases companies doing, and as explained by Dr. Raghuvanshi in the initial speech. So we feel that will continue, but the ARP OB increase may not be 10%, 11% going forward, because the base is already very high. So we expect ARP OB increase should be somewhere around 5%.

Saurabh Kapadia: Okay. And this bed addition will lead to some dilution in the payer mix for next year? That would be the assumption?

Vivek Goyal: Not really because the pay or mix we are maintaining at below 20% for the scheme business. And we don't want to pick up back too much because we are having heavy bed addition program. And these pay ors come handy for filling up this bed. So we are not trying to play too much in this, but wherever the opportunity

we are trying to optimize.

Saurabh Kapadia: Okay. And my second question is on the medical talent...

Moderator: Sorry to interrupt you, Mr. Kapadia. May I request you to use your handset, sir, your audio is slightly muffled, sir.

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Saurabh Kapadia: Is it better?

Moderator: So it's a little lower, sir?

Saurabh Kapadia: So my second question is on the costs related to medical talent. So as the entire cost is there in this quarter? Or how much cost increase should be building for the new talent, what we are adding in?

Ashutosh Raghuvanshi: So the cost of medical talent relatively speaking for us has been a little high. And our hypothesis is as scale up the hospitals as we are planning and the occupancy numbers increase in relative terms, it should come down from the current level. I am pretty sure that it will certainly not go above the current level.

Moderator: The next question is from the line of Hardik Doshi from White Whale Partners. Please go ahead.

Hardik Doshi: So looking at your bed expansion plan, next year, you're looking to add quite a lot on called the 2,200 beds, almost 1/3 of them 710 beds. So are you looking to exit this year at 20% kind of EBITDA margin. I mean how are you looking next year this quarter be addition or drop in occupancy next year, we'll see a lot of bed additions. So does that mean that the drop in margin next year?

Vivek Goyal: Yes. So next year, you rightly picked there is a big bed facility will be coming in. But having said that, this bed capacity, one it will coming at a different point of time. Number two, we may not operationalize the full 710 beds, and the bed operationalization will happen as per the occupancy ramp up thing because that will be the most cost -effective way to achieve the bed ramp -up.

Because we are not opening up entire bed and we will be realizing the bed operationalization based on the occupancy, I'm not seeing much impact on the profitability. And most of these bed except the Manesar one, is coming in our existing hospitals where as per our initial estimate, they should start contributing from the day one. Because in the hospital, we have the bed that is coming they are already operating at a very decent occupancy level. So we don't see that type of challenge there on the margin side.

Hardik Doshi: Okay. So then how should we look at margins for FY '25 and what is the target?

Vivek Goyal: We are maintaining our margin guidance but slowly, as you know, this ramp -up will happen with this bed expansion by next 3 - 4 years, we should be aiming towards 25%.

Hardik Doshi: The other thing was on the expansion divestment strategy. So one of the things -- most of our bed additions are brownfield expansions in our existing facilities. And we were trying to get away from, let's say, a smaller size hospitals to enable larger proportion of them to come to tertiary care.

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If you see Ludhiana is a 70 -bed facility. So is that really going to act nearly as the feeder to our larger hospitals in Punjab, that's one? And second thing is we've got these two divestments that we've announced in Chennai. What is the pipeline looking from a divestment perspective?

Vivek Goyal: Yes, so if I can answer the Ludhiana one first, you know Ludhiana 2 make a sort of extension

to our existing facility. As you know we have a very strong presence in the Punjab side and our other hospital Ludhiana One hospital is doing quite okay. And this hospital from the location perspective, Ludhiana two hospital from the location perspective is ideally suited from the clinician as well as from the patient perspective.

So I think it will be extension of Ludhiana one which would see the performance in a combination of these two hospital and with the type of facility we have built here we are quite

hopeful that it will give results going forward at a combined level.

As regard , your other questions on the divestment thing so this is a sort of a continuous type of exercise. So we keep evaluating our portfolio and depending upon our analysis and the result we may look for other hospital also , but right now there is no concrete plan for any of the hospital is not falling in that category for the divestment.

Moderator: Does that answer your question, sir?

Hardik Doshi: Just sorry, to follow up, in the next 12 months, I mean, do you think there will be at least a couple of divestments from you?

Vivek Goyal: Not really. So as I said, there is no active thing we are pursuing right now on the divestment side. And one hospital which is very small, we may look the alternative for that and it is not worth talking also. It is very small hospital.

Moderator: Thank you. Our next question is from the line of Shyam Srinivasan from Goldman Sachs. Please go ahead.

Shyam Srinivasan: Just the first one is on January. I don't know whether you talked about the occupancies, how it's trending. How should we look at quarter 4. And just a related question on this first part is around occupied bed days that they have basically been flat, maybe slightly down. So how should we look at ramping up occupancy for next year? Or is it because of the bed additions, do you think occupancy increase will likely be a challenge?

Vivek Goyal: So Shyam, I can answer this question. Like last quarter, the occupancy is on the lower side because of the two reasons. Our presence in the NCR and Punjab side because of that, this pollution thing and the winter season has affected the occupancy levels. And to some extent, we have extended our bed capacity like Ludhiana two has been added similarly in Mulund we have added the bed capacities.

We are the sizable bed capacity has happened. So that has resulted into the occupancy side. But we are seeing a very healthy trend in January, February. And we are quite hopeful that this occupancy trend will be revived. And we expect this to be towards 70%, maybe in the coming quarter and even for next year.

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Shyam Srinivasan: So what is like January number, just a factual question. Is it -- Q4 is better than Q3?

Vivek Goyal: Yes, it is definitely better than the Q3. I'm not having number in front of me January, but it is better than Q3.

Shyam Srinivasan: Helpful. Just the second question is on your slides 20 and 21. I'm just trying to analyse what has happened both in the margin metric. And when I look at FMRI, the Q-o-Q decline was the highest in the top 10 hospitals, right? It's declined Q-o-Q by about 12% when the company did only a 4% decline. And also it looks like it may have been the biggest one that has moved across the buckets from, I don't know if it was in the top one and it has moved to bucket three. I'm just trying to analyze Q2 versus Q3. So anything that is happening at some of our flagship hospitals, if you could highlight?

Vivek Goyal: Yes. So FMRI rightly picked Shyam and FMRI is, as you know, is having very good international business. And last quarter, international business was affected because of the reasons explained earlier. So that was the reason for FMRI drop, and there was couple of changes, not a couple, one change at the clinician level also. That has impacted the FMRI performance. But the performance has revived in the month of January As I said, the international business has come back to its normal level. And we have stabilized that FMRI hospital to the September quarter number.

Shyam Srinivasan: What was the second point, Vivek you mean institutional business. So you were a little fast. What is the second reason?

Vivek Goyal: So I was saying one is the international business . And second is there was some change in our cardiac specialty. One of our premi

um clinician has moved out. And new will be joining in this

quarter. So that's why that quarter is got affected in that particular business in this quarter.

Shyam Srinivasan: Understood. And if you could also comment on the margin metrics. Anything to keep in mind because we have seen actually -- and is it maybe seasonal, I get it, but the 15 to 20 bucket has seen things slipping into 10% to 15%. So I'm just looking at that. And I'm sure the nine also in

has actually changed, right? Maybe something like FMRI has fallen out. I don't know which are these hospitals.

But if you could comment on, is there something that we need to be worried about looking at Q3 margin metrics or some of it can be explained by seasonality international patients, etcetera?

Vivek Goyal: Yes. So I think one is the FMRI, that is a better impact given this impact. And as I mean, it has bounced at in the current quarter, and we expect that this hospital will be doing much better during the current quarter and a part of that no other hospital which is having that type of impact on these margin metrics.

Shyam Srinivasan: Got it. Thank you. And my last question is just any update on the High Court legal, anything that is there. Are we still incurring cost, retainer fees for some of the lawyers. So if you could just give us an update on what the legal scenario and the implications on cost.

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Vivek Goyal: Yes. So there is not much movement in the legal case. There was a hearing on the potential forensic at the Delhi High Court level. And the hearing was mainly concentrated on the banker side. The bankers, as you know, banker has extended loan facility to ex-promoter entities. Our lawyer was present of course. And we are paying for their fees.

So that for continuing I think the next quarter we should have some hearing and we are quite hopeful that we will be getting some things. That is on the Delhi High Court hearing. Another thing you might have noticed is the list the RHT entity, which is the listed trust. That has been delisted now in Singapore as per the direction of the Singapore Court.

So to that extent, there will be some savings in the cost in the next year because the entity still remain until it is liquidated.

Shyam Srinivasan: Any quantum you want to share Vivek what those cost savings are?

Vivek Goyal: Yes, it will be in the tune of I think, INR 5 crores annual.

Moderator: The next question is from the line of Bansil Desai from JPMorgan.

Bansil Desai: My question is on some of our low-margin hospitals. We have almost 150 beds, which one-fourth of our capacity which are right now yielding less than 10% margin. I know the hospitals here. Each of them have a unique set of challenges. But how is the management thinking about the turnaround here? And should we expect this cohort to have a gradual improvement or this should also contribute meaningfully to our margin improvement goal going forward?

Vivek Goyal: Yes. So we have done some structural changes in this hospital and that have started showing the results but it is still below 10%. So from the negative EBITDA margins in the initial years when 2019-2020, that have started showing around 10% EBITDA margin. But still there is a lot of work still needs to be done on the structure side.

And it will need some time. I don't think so there will be a dramatic improvement in the profitability metrics for this hospital. But I think we are moving in the right direction. And over a period of time, maybe in a couple of years' time, we will see some results of our efforts.

Bansil Desai: And my second question is on the general pricing levels in the industry today. We've seen very strong ARPOP in the industry over the last four, five years. Is insurance regulator worried about these pricing levels? Are you having any kind of dialogue with them? Are they seeing these price

to be high? Or do you think these price increases are largely taken to meet the medical cost inflation and therefore, are justified from that perspective.

So any colour on the pricing dialog that you could be having with the insurance regulator on how they are viewing the pricing today in the industry?

Vivek Goyal: Yes. So I will answer and then Mr. Raghuvanshi may add to that. If you see the price increase is mainly attributable towards the speciality mix change. And the type of procedures we are doing, which is the high-end procedures. So it is not because of increase in the price increase.

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If you see our price increase at overall level from the previous year, it is coming around 1%, 1.5% only.

So at a consolidated level. So of course, it will be coming around 4% - 4.5%, which is linked to the to the inflation side. So I don't see that type of challenge here, the ARPOP increase is mainly attributed to the mix change in the center specialty mix and type of procedures we are in.

Ashutosh Raghuvanshi: Yes. Other than that I would say that the industry body level, these dialogues are continuously on. And I think this debate will be an ongoing issue. And there is no endpoint for this because

as insurance has an upper hand, they would obviously like to control the prices. But there is no specific stress at the moment.

Moderator: Our next question is from the line of Bino Patiparaveer from Elara Capital.

Bino Patiparaveer: Before I get into my question, just a recap on an earlier question on Agilus. I think your voice was a little muffled. Is that there was some one -off or there was no one -off?

Vivek Goyal: Agilus, as I have mentioned, we can't disclose much. So there is a one -off impact in that financial.

Bino Patiparaveer: Coming to my main question. I was just trying to understand a little bit between comparing the two slides that you have given that like expansion plan. So first, I'm looking at FY24, you are adding about 237 beds of which I assume the Ludhiana facility, 70 beds is included in that?

Vivek Goyal: Ludhiana is around 55 beds. Then there is a Mulund facility where we have added 70, 80 beds there. then we have operationalized in Kolkata facility around 30 -40 beds. And rest is I think BG Road in Bangalore, a couple of facilities we have added there.

Bino Patiparaveer: Okay. So Ludhiana is the only new facility per se, right?

Vivek Goyal: And there is Mohali also we have added 35 beds in the industry.

Bino Patiparaveer: But in terms of new facility Ludhiana is the only one, right?

Vivek Goyal: Yes. It is only the Ludhiana. And then we mentioned not new in the true sense because it is having good synergy with existing hospital.

Bino Patiparaveer: Understood. And when I look at FY25, out of the 710, I assume roughly 350 will be the acquired Medeor hospital, right?

Vivek Goyal: Yes.

Bino Patiparaveer: Okay. So may I have a rough understanding of where the rest 350 will be spread out like?

Vivek Goyal: Yes. So I will say out of 700 almost 300 -plus beds is of the Manesar facility. So we have this 350 bed acquisition in Manesar. So our plan is not to open the entire the bed at one go.

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Because of the cost optimizing point of view. So there will be maybe, we will start with 100 bed and then we'll see how the ramp -up goes up.

And if we see increasing we may open another beds during that period. And depending upon the ramp -up, we will try to do that and as we mentioned, it will be in the different time zone. So like Manesar, we will be opening maybe better mid of the year. So that type of impact will also be there. And the other major capacity expansion is coming in the Anandpur facility and the BG road facility.

And one, of course, the Faridabad expansion thing which will be operationalized in t

he next

financial year. So those things, I think we will be operationalizing over a period of time. It will be around 350 beds will be operationalized by the year.

Bino Patiparaveer: Understood. And the last question, the biggest bed increase seems to be happening in the NCR around 1,000 beds over the next two years, 1,000 -plus. Shalimar Bagh and from what I understand would be ground fields. Noida and Manesar, Manesar, are you referring there the same maybe or sorry, Noida and Manesar, are they completely brownfield or new facilities at...

Vivek Goyal: Existing hospital. And there, we are expanding the facility. So it is brown field. Noida is brown field. Manesar is the existing hospital, but it was not fully operational, and we are making it operational after doing some re engineering of it.

Moderator: Our next question is from the line of Nikhil Mathur from HDFC Mutual Fund.

Nikhil Mathur: Yes. So my question is linked to the bed expansion plan. What we see from your margin metrics over the years is that almost 35% - 40% of beds, they tend to be in the less than 15% EBITDA margin range. Now can you give some colour on the bed expansion plan? How much of beds are getting added in this less than 15% EBITDA margin range. I suppose that there are -- I mean, more of these certainties are operating our occupancy of more than 50%. So absorption of fixed costs via a number of -- more number of it is perhaps the only way you can improve your margins structurally in these facilities?

Vivek Goyal: Yes. So let me clarify, the bed expansion is not happening in any of these hospitals, which are less the 15% EBITDA margin. So the bed expansion is happening in the hospital, which are operating at almost 70% to 75% occupancy level. and they are operating at a decent EBITDA margin of 20% plus.

Nikhil Mathur: Sir, but if your occupancy of 10% to 15% EBITDA margin range, facilities is 67%, less than 10% is also not too bad at 54%, so what are the chances of EBITDA margins in these facilities improving structurally? I mean, I fail to understand how that will kind of happen.

Vivek Goyal: Yes. That's why it will not be happening immediately. And it will be a gradual thing. And we have to give two - three years' time for things to materialize. So it will not be immediate. I will not said it will immediately improve EBITDA margin on these hospitals..

Nikhil Mathur: So sir, on a two, three year horizon as well -- I mean, there is inflation that will catch up on various costs. There's been expansion happening across the country. There will be poaching pressure as well on your talent and all unless now you are saying that your ARPO B increases in the less than 10% facilities by substantially maybe 12%, 13% then perhaps your EBITDA margins can improve in these facilities. But even on two to three horizon, how will that kind of happen.

Vivek Goyal: Yes. So as I have mentioned, there are two reasons for increasing the EBITDA margin. Our ability to react prices is limited, as you know. We are operating in a slightly different environment. So we are not expecting we will be able to increase the price at 5% - 6% at a consol level, okay? So that is given assumption. But having said that, the EBITDA margin improvement will happen on two fronts.

One is on the occupancy side. As our occupancy level goes up, we're able to utilize our existing infrastructure and the cost base will be higher. And with that, our EBITDA margin will improve. That is on one side. And similar to that if we are going for brownfield expansion that also lead to the margin improvement because we are adding the bed in the existing infrastructure itself.

So the cost will be shared with the larger base, and that will help in improving the margins.

Nikhil Mathur: Okay. And you mentioned that in the overall above increase, there is very less increa

se in

pricing, it's more of mix. So can you give two other examples of these mix changes, which has kind of contributed to this ARPO B increase. I mean not all there could be many examples of this, but if two -three examples can be given on how this ARPO B is increasing?

Vivek Goyal: Yes. One is immediately in my mind, is the oncology business, you know oncology business it's generally high ARPO B business. And the margin is slightly less. But when we see the gross revenue divided by the number of beds for this onco business, the ARPO B will be higher. Onco business is growing at a decent pace for us and we expect this will continue because of our focus on this specialty. And similarly, the neuro where we are quite strong and we are strengthening our presence in neuro, that is also lead to ARPOB increase. Then we have invested in a lot of robotic surgeries and things like that. And that should lead to the high ARPOB business going forward.

Nikhil Mathur: Any capex number you can share in, let's say, last 12 months, 18 months, which has boosted your capabilities on these onco and neuro cases, any capex number you can think?

Vivek Kumar Goyal: Yes. So I can give you the current year number itself. For the 9 months, we have spent already INR 450 crores in the capex. And I think the similar trend will be for the current quarter. So I think we are investing a lot in our technology. We have invested in the state -of-the-art Gamma Knife in our Flagship hospital.

We have invested in MR -Linac. Two of the Linac we have replaced, the CAT H lab and we have a Da Vinci Robot we have invested. And the robotic surgeries also, Ortho Robot, also we have invested INR 5 crores, INR 6 crores. So we are not hesitating in investment in the technology, and that is yielding the results.

Moderator: Our next question is from Neha Manpuria from Bank of America. Please go ahead.

Neha Manpuria: Just one clarification. The notice that we have got from the Anti -Corruption Bureau for the Mohalla Clinic, is that -- is there some amount of that impact also in the December quarter or that will flow through from this quarter onwards?

Vivek Kumar Goyal: No. That is the major impact actually in that quarter.

Neha Manpuria: In the December quarter, right?

Vivek Kumar Goyal : Yes.

Neha Manpuria: Okay. Understood. And my second question is just a follow -up from the previous participants. If I were to look at less than 10% margin bracket, if you could pick two or three of the largest hospitals that probably dragged the margin profile for the company. And what are the structural measures that you're talking about in each of these. That would probably help us understand it better as to why it will take three, four years to improve performance in these hospitals.

Vivek Kumar Goyal: Yes, I will pick two. One is FEHI and second is Jaipur one. And both are 300 -plus bedded hospital. And both have a good potential, as you all know. So in Jaipur, we have a location advantage. It is located in a premium location. We have good clinician teams, and it is having

a good brand equity in that region. And the thing it is lagging is it is not having the full basket of facilities which we can offer for the hospital of this size.

So we are trying to add radio, onco there, and that will take itself for one, one and a half years to plan that thing because bunker and other thing will be required to be built, and then there will be some construction activity will have to happen. And we hope that with that you know this hospital should be moving above 15% EBITDA margin for sure. So that is on the Jaipur side.

And secondly, on the FEHI side, we are trying to add other specialties. It is mainly a cardiac center. We are trying to add other specialties so that you know the cost base can be paired with the increase in the revenue. And this hospital again

is having a very good brand equity and it will be a decent size hospital. So I think with this attracting clinician talent for a cardiac business and then stabilizing them take some time and that's why I'm saying it will take a minimum two years' time from now to start showing the result for these two hospitals.

Plus we are doing lot of work on the cost side in both these hospitals, whatever we can do on the cost side on the manpower, productivity side, on the other cost initiatives which we are taking. So I think all those will start showing results maybe in a two years' time. Actually already started seeing results if you see the performance is improving for both these hospitals.

Moderator: Our next question is from the line of Harsh from Bandhan AMC. Please go ahead.

Harsh: I just wanted to understand to pick up from the previous question in terms of FEHI and Jaipur, you adding radio, oncology, cardiac division and all the other areas. If you look at the overall Fortis Healthcare Limited

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margin metrics, let's say, nine facilities below 15% margin, could you help us understand that the ability to provide more and more high-end surgeries and clinical work, can this be expanded to, let's say, these other 9, 10 facilities, which are making less than 15% margin? Or this is more of a selective play across, let's say, only two or three facilities, which are making more margin. Because the reason that I'm asking is that the feedback is that, because of insurance penetration, there's a lot more affordability in Tier 1, Tier 2, Tier 3 cities as such. So in that context, if you could help us understand.

Vivek Kumar Goyal: Yes. So if I can answer this way. Out of the 9 hospitals, 2 hospitals are where we are doing the expansion. One is the Faridabad one where the operation is slightly affected because of the ongoing extension it is disturbing the existing operation. And as I mentioned earlier, the extension program will be completed during this next financial year, early part of the next financial year.

And we expect when there will be higher bed capacity and the cost base will be paired with the higher bed. And secondly, the disturbance will not be there. So I'm 100% sure that Faridabad hospital will definitely move up in the ladder. Second is Ludhiana, that is more affected because of this seasonal impact and that is not a worry for us.

So out of seven, two we can take out very easily. Then remaining five; out of five, one we have already divested. Another is a small hospital in Bangalore which is actually a very small hospital and we are looking for various alternatives, maybe including shutting it down also. So the two hospitals we have already discussed.

Another two hospitals are hospitals where we have rental premises. So there is a rent sitting on it and a 6%, 7% rent if we add. Because of that, the EBITDA margin is slightly on the lower side. But we are doing some work and we are hopeful that at least 15% margin, these two hospitals, we will be able to do.

Harsh: So from a clinical -- yes, so from a clinical aspect, Ludhiana, Faridabad, Jaipur and FEHI are the areas where you can possibly provide more in terms of the on-ground clinical options.

Vivek Kumar Goyal: Yes, you're right. These are the four hospitals where that improvement will be slightly on the faster side.

Moderator: Our next question is from the line of Bansi Desai from JPMorgan. Please go ahead.

Bansi Desai: I just needed one clarification. I don't know how much you can provide color on this. But for the private equity investors in Agilus, the timelines of providing exit to them are about February of 2024. Am I correct? And does that stand true or does that hold with the IPO now?

Vivek Kumar Goyal: Yes. So as per existing agreement, the timeline is April '24. And as you all know, we have filed the DRHP we are still waiting for SEBI clearance. So

we are actually we are standing at that point now.

Bansi Desai: Okay. Got it.

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Moderator: As there are no further questions, I now hand the conference over to Mr. Anurag Kalra for closing comments.

Anurag Kalra: Yes. Thank you very much, ladies and gentlemen, for your time. If there are any follow-up questions, please reach out to me or my colleagues. We'll be happy to address them as best possible. Thank you very much again and have a good day.

Moderator: Thank you. On behalf of Fortis Healthcare, that concludes the conference call. Thank you for joining us, and you may now disconnect your lines.

Call: Feb 2024

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FHL/SEC/2023 -24 February 10, 2024

The National Stock Exchange of India Ltd.

Scrip Symbol: FORTIS BSE Limited

Scrip Code:532843

Sub: Transcript and Audio Recording of Investors / Analysts' meet under Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015

Dear Madam / Sir,

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed transcript and the link of audio recording of Investors / Analysts' meet held on February 8, 2024 to discuss the Company's un-audited financial results for the quarter and period ended December 31, 2023 and the same is available on the website of the Company i.e. Audio recording Link and recording Link

This is for your kind information and records.

Thanking you,

Yours sincerely

For Fortis Healthcare Limited

Murlee Manohar Jain

Company Secretary & Compliance Officer

M. No. F9598

Encl.: a/a

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"Fortis Healthcare Limited "

Q3 & 9M Ended December 31, 2023 Post Results

Conference Call "

February 08, 2024

MANAGEMENT : DR ASHUTOSH RAGHUVANSHI – MANAGING
DIRECTOR AND CHIEF EXECUTIVE OFFICER – FORTIS
HEALTHCARE LIMITED

MR. VIVEK GOYAL – CHIEF FINANCIAL OFFICER –
FORTIS HEALTHCARE LIMITED

MR. ANURAG KALRA – SENIOR VICE PRESIDENT ,
INVESTOR RELATIONS – FORTIS HEALTHCARE
LIMITED

MR. AMIT MAHE NDR U - INVESTOR RELATIONS –
FORTIS HEALTHCARE LIMITED

MR. AVINASH TRIPATHI - INVESTOR RELATIONS –
FORTIS HEALTHCARE LIMITED

Moderator: Ladies and gentlemen, good day, and welcome to the Q3 FY'24 and 9 months ended December 31, 2023, Post Results Conference Call of Fortis Healthcare Limited. As a reminder, all participant lines will be in the listen-only mode, and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Anurag Kalra, Senior Vice President, Investor Relations at Fortis Healthcare Limited. Thank you, and over to you, Mr. Kalra.

Anurag Kalra : Thank you very much. A very good morning and good afternoon, ladies and gentlemen, and thank you for joining us on our Q3 FY'24 earnings call. The call is being chaired by Dr.

Ashutosh Raghuvanshi, our Managing Director and CEO. With him, we have Mr. Vivek Goyal, our Chief Financial Officer. And I also have my colleagues from Investor Relations and M&A, Amit Mahendru and Avinash Tripathi.

Before we start the call, I would just like to state that, as you're all aware, we are a listed company and our material subsidiary, Agilus Diagnostics has filed a DRHP for the proposed IPO in September 2023.

In light of the publicity restrictions imposed on Agilus and the company, which due to the proposed IPO, we would not be in a position to share any further information other than what is already provided in our investor presentation and press release. Due to these restrictions, we would also not be in a position to clarify or answer any questions on the Diagnostics business performance or on the proposed IPO at this point in time. We do appreciate your understanding on this.

We will begin the call with some opening comments by Dr. Raghuvanshi on both the consolidated and the hospital business performance. And then we can open the floor for question and answers. Over to Dr. Raghuvanshi.

Ashutosh Raghuvanshi: Thank you, Anurag. Good morning and good afternoon, everyone. Thank you for taking time to join us on our Q3 FY'24 earnings call today. I wish you all happy New Year and hope all of you are doing well. I shall come straight to the results of the quarter and my thoughts on the business performance and way forward.

Our business performance in Q3 has been satisfactory considering the seasonal impact of festivals in some of our key geographies. The hospital business has shown a relatively improved performance compared to the corresponding quarter last year and predictably slightly lower than the trailing quarter.

Our consolidated revenues were at INR 1,680 crores, a growth of 8% versus the corresponding previous quarter. This compares to INR 1,560 crores in Q3 of financial year '23. Our consolidated operating EBITDA margin was 16.9% versus 17.7% in the corresponding previous quarter and versus 18.6% in the trailing quarter.

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To highlight the contribution of hospital operating EBITDA increased to 88% in Q3 of financial year '24 versus 76% in Q3 of financial year '23. This indicates a positive momentum in our hospital business earnings, allowing us to sustain our overall profit margin. The strength in our hospital business has also largely offset our performance in Diagnostic business.

At the PAT level, we reported a profit after tax prior to exceptional items of INR 127 crores, compared to INR 131 crores in Q3 of financial year '23. Reported PAT stood at INR 134 crores versus INR 142 crores in the corresponding previous year. Our balance sheet remains healthy with a net debt to EBITDA of 0.45x compared to 0.41x in Q3 of financial year '23. Our net debt at the end of Q3 financial '24 stands at INR 518 crores against INR 471 crores at the end of Q3 of financial

year.

Let me also briefly touch on the consolidated 9-month financial year '24 result numbers of the company. For the 9 months of financial year, our consolidated revenue stood at INR 5,107 crores, a growth of 9.7% over the corresponding previous period. Operating EBITDA for the period was INR 887 crores versus INR 830 crores for the 9 months of financial year '23, translating into a margin of 17.4% versus 17.8% in the corresponding previous period.

PAT, excluding exceptional items for the period stood at INR 429 crores versus INR 432 crores for 9 months of a marginal decline year-on-year, reported PAT stood at INR 442 crores versus INR 495 crores in the corresponding previous year.

I'm pleased to share that our hospital business continues to witness a healthy performance. And

despite the quarter having a seasonality impact, we have shown a better performance on a year-on-year basis. The consolidated profitability numbers that I shared with you just now clearly reflect this.

We have witnessed an occupancy of 64% in Q3 versus 66% in the corresponding quarter, owing to an increase in the operational beds by approx. 100, while the occupied beds remained flat year-on-year. Occupancy levels on a like-to-like basis were at similar levels. ARPO B witnessed a strong growth, growing by 10.6% year-on-year to reach INR 2.23 crores.

The growth was driven by a consistent shift towards higher complexity procedures. This year, on an annualized basis, we will be performing more than 60,000 cardiac procedures, more than 3,500 robotic surgeries and more than 1,100 transplants, which demonstrates the shift we have been talking about. All these factors enable the hospital business revenues to reach revenue of INR 1,389 crores in the quarter, a growth of 10% versus Q3 of financial year '23.

The hospital business operating EBITDA grew 19% to touch INR 251 crores reflecting a margin of 18% versus 15.7% margin in Q3 of financial year '23. The improvement was driven by our consistent focus on improving operational efficiency and optimizing costs against all heads, including general administration services, repair and maintenance, etcetera.

Revenue from international business stood flat at INR 113 crores versus INR 114 crores in the corresponding previous quarter. International patient revenue was flat year-on-year, primarily due to geopolitical reasons in the Middle East and flow of patients from countries such as Iraq and Bangladesh.

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You would recall that we have been speaking about our portfolio rationalization strategy. To that effect, we have divested two of our loss-making facilities in Chennai, first the Arcot, Vadapalani facility in July 23, followed by the Malabar facility, which we concluded last week, leading to an improvement in overall profitability for the company in the future.

We are progressing on our brownfield bed expansion, which are expected to incrementally add almost 50% to our existing bed capacity. And when operationalized, will eventually see some of our key certainties such as Shalimar Bagh, FMRI, Mohali, PG Road, Noida become more than 450 beds each. This is expected to provide a higher degree of operating leverage in such facilities translating into healthy margin expansion.

Our expansion strategy continues to focus on deepening our cluster presence with the launch of new 70-bed unit in Ludhiana. This is the second facility in Ludhiana and the fourth in Punjab. This will take our total bed strength to ~800 in Punjab region. You will also notice that given our focus on bed expansion, we have also, for the first time, provided further granularity on our bed expansion planned in our investor presentation, close to 2,200 beds in the next 4 years will come through.

Our focus on strengthening our clinical programs continued through the quarter across all our facilities in terms of invest

ing in high-end medical infrastructure and on-boarding new medical

talent. The quarter witnessed addition of several M&A clinicians across various specialties like neurosurgery, oncology, cardiology, gastroenterology and GI surgery.

During the quarter, we commissioned several medical programs and further strengthened medical infrastructure, various facilities which includes the UP's first most advanced artificial intelligence powered state-of-art Cath Lab Fortis Hospital, Noida. Neuro ICU and advanced neuro lab at Fortis Hospital, Faridabad and launch of cutting edge surgical robot at Fortis Hospital Anandpur, Kolkata.

Another important aspect to highlight is the on-going success of our digitization initiatives, our strides in digital transformation, particularly with the implementation of EMR advancing positively. Revenue from digital channels such as websites, MyFortisApp and online campaigns grew by 32% year-on-year and their contribution to overall hospital revenues increased to 25.7% versus 21.5% in Q3 of financial year '23.

In this, we have also launched our new patient feedback management system, MyFeedback.

This platform will enable more engaging experience for our patients, as feedback will be collected through WhatsApp and QR codes. The application will also enable collection of feedback and addresses immediate patient concerns through its service request feature.

The revenue contribution from the company's focused medical specialties; oncology, orthopaedics, renal sciences, cardiac sciences, neurosciences, and gastroenterology to overall hospital revenue increased to 61.4% in Q3 from 60.9% in Q3 of financial year '23.

Specifically, revenue from gastrosciences, neurosciences, and oncology grew by 20%, 13%, and 12% respectively versus the corresponding previous quarter.

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In summary, our earning momentum remains healthy, specifically with respect to the hospital business and this I would expect to continue going forward. I say this with a sense of conviction as our future levers of growth are well -defined in our business plans, whether they be towards our brownfield expansion strategy or towards rendering our medical programs and technologies or towards effort to ensure that our patients are delivered a superb patient experience and excellent clinical outcomes .

Thank you and with this I would like to hand over to Mr. Anurag Kalra to take this forward.

Anurag Kalra: Thank you sir. Ladies and gentlemen we will now open the floor for question and answers.

May I request the moderator to begin please.

Moderator: Thank you very much. We will now begin the question and answer session. The first question is from the line of Neha Manpuria from Bank of America. Please go ahead.

Neha Manpuria: Thanks for taking my question, sir. Sir, on the hospital performance, the fact that we managed to keep margins flat , despite the bed addition that we saw and the seasonally weak quarter , does indicate the point that you've mentioned that we've been able to do a lot of cost optimization and improvement in efficiency. One, if you could highlight how much of this is already captured in this 18% margin. I mean, is there more scope for this to contribute to margins?

And second, as we see occupancy improve from the nine -month number of 66%, 67% on the existing beds, what is the trajectory? I mean, should we see margins crossing the 20% in the very near term or could that be a little more longer journey? Just trying to understand the, you know, gradient for the margin expansion that we should see in the hospital business ?

Vivek Goyal: Yes. Neha, I can take this question. As you rightly mentioned about margins, we are moving towards our target and the guidance given earlier of 20% margin by the year -end. So that we are expecting. And the main lever for our margin expansion is on the occupancy side . As you

de . As you

rightly pointed out, occupancy is slightly lower as against our expectation. And we are below 65% now.

And as we're able to move from the occupancy side, I think this margin improvement will be there . And plus, this divestment as Dr. Raghuvanshi mentioned about Malar, which is effective from first of February 2024 , that will also help in improving the margin.

Neha Manpuria: Maybe if I can be a little more specific here. If I have to say the 18% margin improvement, how much of it would come from Malar divestment. And how much of it would then come from, let's say, the other efforts that we are sort of implementing?

Vivek Goyal: Yes. So, I will say around 0.5% - 0.6% of EBITDA margin improvement because of the divestment . The Arcot Road would already factored in this quarter. So I'm not counting that.

So that will be on the Malar divestment side. Rest will be majorly from the occupancy improvement and some improvement in the cost side. And as some of the hospital like Ludhiana have operationalize only in the December. So this quarter, the result of that hospital will be also improved.

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Neha Manpuria: Okay. So the Ludhiana impact will flow through in the March quarter, the new facility effect?

Vivek Goyal: Yes.

Neha Manpuria: Okay. Understood. And my second question on the international patients, that was flat. I know you did mention in your opening comments that this is because of certain geopolitical issue.

How do we now plan to grow this business, given what's going on in the Middle East, and we don't know when that situation improves. Can this number move up from what we are seeing?

Vivek Goyal: Yes. So international business this quarter is slightly less as compared to previous quarter.

However, if you see year -to-date, we are doing quite okay. And in the month of January and February, we have seen that the business has come back to its September quarter number. So we expect this will continue to provide a good lever for increasing the occupancy and profitability.

Neha Manpuria: Understood. And last, I don't know if you want to answer this question. But if I were to do a simple consol minus hospital, the residual diagnostic business seems to have seen significant pressure on margins. Is there any one -off cost there? Any color that we can provide there?

Again, I don't know if you would give any color, but just trying my luck.

Vivek Goyal: Yes, as Anurag has mentioned, we are not supposed to discuss about the financials of the diagnostic, but I can, this much I can say it's some one -off in this. And the performance is impacted because of the one-off.

Moderator: The next question is from the line of Saurabh Kapadia from Sundaram Mutual Funds. Please

go ahead.

Saurabh Kapadia: So first question is on ARPOB so another 19% kind of growth can we sustain? And also, do we have levers further to improve on it given that a few beds additions are coming in.

Vivek Goyal: Yes. So we could see a very good ARP OB increase during this quarter and for the year, also the number is similar. I think it is mainly driven by the high -end cases, the more complex cases companies doing, and as explained by Dr. Raghuvanshi in the initial speech. So we feel that will continue, but the ARP OB increase may not be 10%, 11% going forward, because the base is already very high. So we expect ARP OB increase should be somewhere around 5%.

Saurabh Kapadia: Okay. And this bed addition will lead to some dilution in the payer mix for next year? That would be the assumption?

Vivek Goyal: Not really because the pay or mix we are maintaining at below 20% for the scheme business. And we don't want to pick up back too much because we are having heavy bed addition program. And these pay ors come handy for filling up this bed. So we are not trying to play too much in this, but wherever the opportunity

we are trying to optimize.

Saurabh Kapadia: Okay. And my second question is on the medical talent...

Moderator: Sorry to interrupt you, Mr. Kapadia. May I request you to use your handset, sir, your audio is slightly muffled, sir.

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Saurabh Kapadia: Is it better?

Moderator: So it's a little lower, sir?

Saurabh Kapadia: So my second question is on the costs related to medical talent. So as the entire cost is there in this quarter? Or how much cost increase should be building for the new talent, what we are adding in?

Ashutosh Raghuvanshi: So the cost of medical talent relatively speaking for us has been a little high. And our hypothesis is as scale up the hospitals as we are planning and the occupancy numbers increase in relative terms, it should come down from the current level. I am pretty sure that it will certainly not go above the current level.

Moderator: The next question is from the line of Hardik Doshi from White Whale Partners. Please go ahead.

Hardik Doshi: So looking at your bed expansion plan, next year, you're looking to add quite a lot on called the 2,200 beds, almost 1/3 of them 710 beds. So are you looking to exit this year at 20% kind of EBITDA margin. I mean how are you looking next year this quarter be addition or drop in occupancy next year, we'll see a lot of bed additions. So does that mean that the drop in margin next year?

Vivek Goyal: Yes. So next year, you rightly picked there is a big bed facility will be coming in. But having said that, this bed capacity, one it will coming at a different point of time. Number two, we may not operationalize the full 710 beds, and the bed operationalization will happen as per the occupancy ramp up thing because that will be the most cost -effective way to achieve the bed ramp -up.

Because we are not opening up entire bed and we will be realizing the bed operationalization based on the occupancy, I'm not seeing much impact on the profitability. And most of these bed except the Manesar one, is coming in our existing hospitals where as per our initial estimate, they should start contributing from the day one. Because in the hospital, we have the bed that is coming they are already operating at a very decent occupancy level. So we don't see that type of challenge there on the margin side.

Hardik Doshi: Okay. So then how should we look at margins for FY '25 and what is the target?

Vivek Goyal: We are maintaining our margin guidance but slowly, as you know, this ramp -up will happen with this bed expansion by next 3 - 4 years, we should be aiming towards 25%.

Hardik Doshi: The other thing was on the expansion divestment strategy. So one of the things -- most of our bed additions are brownfield expansions in our existing facilities. And we were trying to get away from, let's say, a smaller size hospitals to enable larger proportion of them to come to tertiary care.

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If you see Ludhiana is a 70 -bed facility. So is that really going to act nearly as the feeder to our larger hospitals in Punjab, that's one? And second thing is we've got these two divestments that we've announced in Chennai. What is the pipeline looking from a divestment perspective?

Vivek Goyal: Yes, so if I can answer the Ludhiana one first, you know Ludhiana 2 make a sort of extension

to our existing facility. As you know we have a very strong presence in the Punjab side and our other hospital Ludhiana One hospital is doing quite okay. And this hospital from the location perspective, Ludhiana two hospital from the location perspective is ideally suited from the clinician as well as from the patient perspective.

So I think it will be extension of Ludhiana one which would see the performance in a combination of these two hospital and with the type of facility we have built here we are quite

hopeful that it will give results going forward at a combined level.

As regard , your other questions on the divestment thing so this is a sort of a continuous type of exercise. So we keep evaluating our portfolio and depending upon our analysis and the result we may look for other hospital also , but right now there is no concrete plan for any of the hospital is not falling in that category for the divestment.

Moderator: Does that answer your question, sir?

Hardik Doshi: Just sorry, to follow up, in the next 12 months, I mean, do you think there will be at least a couple of divestments from you?

Vivek Goyal: Not really. So as I said, there is no active thing we are pursuing right now on the divestment side. And one hospital which is very small, we may look the alternative for that and it is not worth talking also. It is very small hospital.

Moderator: Thank you. Our next question is from the line of Shyam Srinivasan from Goldman Sachs. Please go ahead.

Shyam Srinivasan: Just the first one is on January. I don't know whether you talked about the occupancies, how it's trending. How should we look at quarter 4. And just a related question on this first part is around occupied bed days that they have basically been flat, maybe slightly down. So how should we look at ramping up occupancy for next year? Or is it because of the bed additions, do you think occupancy increase will likely be a challenge?

Vivek Goyal: So Shyam, I can answer this question. Like last quarter, the occupancy is on the lower side because of the two reasons. Our presence in the NCR and Punjab side because of that, this pollution thing and the winter season has affected the occupancy levels. And to some extent, we have extended our bed capacity like Ludhiana two has been added similarly in Mulund we have added the bed capacities.

We are the sizable bed capacity has happened. So that has resulted into the occupancy side. But we are seeing a very healthy trend in January, February. And we are quite hopeful that this occupancy trend will be revived. And we expect this to be towards 70%, maybe in the coming quarter and even for next year.

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Shyam Srinivasan: So what is like January number, just a factual question. Is it -- Q4 is better than Q3?

Vivek Goyal: Yes, it is definitely better than the Q3. I'm not having number in front of me January, but it is better than Q3.

Shyam Srinivasan: Helpful. Just the second question is on your slides 20 and 21. I'm just trying to analyse what has happened both in the margin metric. And when I look at FMRI, the Q-o-Q decline was the highest in the top 10 hospitals, right? It's declined Q-o-Q by about 12% when the company did only a 4% decline. And also it looks like it may have been the biggest one that has moved across the buckets from, I don't know if it was in the top one and it has moved to bucket three. I'm just trying to analyze Q2 versus Q3. So anything that is happening at some of our flagship hospitals, if you could highlight?

Vivek Goyal: Yes. So FMRI rightly picked Shyam and FMRI is, as you know, is having very good international business. And last quarter, international business was affected because of the reasons explained earlier. So that was the reason for FMRI drop, and there was couple of changes, not a couple, one change at the clinician level also. That has impacted the FMRI performance. But the performance has revived in the month of January As I said, the international business has come back to its normal level. And we have stabilized that FMRI hospital to the September quarter number.

Shyam Srinivasan: What was the second point, Vivek you mean institutional business. So you were a little fast. What is the second reason?

Vivek Goyal: So I was saying one is the international business . And second is there was some change in our cardiac specialty. One of our premi

um clinician has moved out. And new will be joining in this

quarter. So that's why that quarter is got affected in that particular business in this quarter.

Shyam Srinivasan: Understood. And if you could also comment on the margin metrics. Anything to keep in mind because we have seen actually -- and is it maybe seasonal, I get it, but the 15 to 20 bucket has seen things slipping into 10% to 15%. So I'm just looking at that. And I'm sure the nine also in

has actually changed, right? Maybe something like FMRI has fallen out. I don't know which are these hospitals.

But if you could comment on, is there something that we need to be worried about looking at Q3 margin metrics or some of it can be explained by seasonality international patients, etcetera?

Vivek Goyal: Yes. So I think one is the FMRI, that is a better impact given this impact. And as I mean, it has bounced at in the current quarter, and we expect that this hospital will be doing much better during the current quarter and a part of that no other hospital which is having that type of impact on these margin metrics.

Shyam Srinivasan: Got it. Thank you. And my last question is just any update on the High Court legal, anything that is there. Are we still incurring cost, retainer fees for some of the lawyers. So if you could just give us an update on what the legal scenario and the implications on cost.

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Vivek Goyal: Yes. So there is not much movement in the legal case. There was a hearing on the potential forensic at the Delhi High Court level. And the hearing was mainly concentrated on the banker side. The bankers, as you know, banker has extended loan facility to ex -promoter entities. Our lawyer was present of course. And we are paying for their fees.

So that for continuing I think the next quarter we should have some hearing and we are quite hopeful that we will be getting some things. That is on the Delhi High Court hearing. Another thing you might have noticed is the list the R HT entity, which is the listed trust. That has been delisted now in Singapore as per the direction of the Singapore Court.

So to that extent, there will be some savings in the cost in the next year because the entity still remain until it is liquidated .

Shyam Srinivasan: Any quantum you want to share Vivek what those cost savings are?

Vivek Goyal: Yes, it will be in the tune of I think, INR 5 crores annual.

Moderator: The next question is from the line of Bansri Desai from JPMorgan.

Bansri Desai: My question is on some of our low -margin hospitals. We have almost 150 beds, which one - fourth of our capacity which are right now yielding less than 10% margin. I know the hospitals here. Each of them have a unique set of challenges. But how is the management thinking about the turnaround here? And should we expect this cohort to have a gradual improvement or this should also contribute meaningfully to our margin improvement goal going forward?

Vivek Goyal: Yes. So we have done some structural changes in this hospital and that have started showing the results but it is still below 10%. So from the negative EBITDA margins in the initial years when 2019 -2020, that have started showing around 10% EBITDA margin. But still there is a lot of work still needs to be done on the structure side.

And it will need some time. I don't think so there will be a dramatic improvement in the profitability metrics for this hospital. But I think we are moving in the right direction. And over a period of time, maybe in a couple of years' time, we will see some results of our efforts.

Bansri Desai: And my second question is on the general pricing levels in the industry today. We've seen very strong ARPOP in the industry over the last four, five years. Is insurance regulator worried about these pricing levels? Are you having any kind of dialogue with them? Are they seeing these price

s to be high? Or do you think these price increases are largely taken to meet the medical cost inflation and therefore, are justified from that perspective.

So any colour on the pricing dialog that you could be having with the insurance regulator on how they are viewing the pricing today in the industry?

Vivek Goyal: Yes. So I will answer and then Mr. Raghuvanshi may add to that. If you see the price increase is mainly attributable towards the speciality mix change. And the type of procedures we are doing, which is the high -end procedures. So it is not because of increase in the price increase.

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If you see our price increase at overall level from the previous year, it is coming around 1%, 1.5% only.

So at a consolidated level. So of course, it will be coming around 4% - 4.5%, which is linked to the to the inflation side. So I don't see that type of challenge here, the ARPOP increase is mainly attributed to the mix change in the center specialty mix and type of procedures we are in.

Ashutosh Raghuvanshi: Yes. Other than that I would say that the industry body level, these dialogues are continuously on. And I think this debate will be an ongoing issue. And there is no endpoint for this because

as insurance has an upper hand, they would obviously like to control the prices. But there is no specific stress at the moment.

Moderator: Our next question is from the line of Bino Patiparaveer from Elara Capital.

Bino Patiparaveer: Before I get into my question, just a recap on an earlier question on Agilus. I think your voice was a little muffled. Is that there was some one -off or there was no one -off?

Vivek Goyal: Agilus, as I have mentioned, we can't disclose much. So there is a one -off impact in that financial.

Bino Patiparaveer: Coming to my main question. I was just trying to understand a little bit between comparing the two slides that you have given that like expansion plan. So first, I'm looking at FY24, you are adding about 237 beds of which I assume the Ludhiana facility, 70 beds is included in that?

Vivek Goyal: Ludhiana is around 55 beds. Then there is a Mulund facility where we have added 70, 80 beds there. then we have operationalized in Kolkata facility around 30 -40 beds. And rest is I think BG Road in Bangalore, a couple of facilities we have added there.

Bino Patiparaveer: Okay. So Ludhiana is the only new facility per se, right?

Vivek Goyal: And there is Mohali also we have added 35 beds in the industry.

Bino Patiparaveer: But in terms of new facility Ludhiana is the only one, right?

Vivek Goyal: Yes. It is only the Ludhiana. And then we mentioned not new in the true sense because it is having good synergy with existing hospital.

Bino Patiparaveer: Understood. And when I look at FY25, out of the 710, I assume roughly 350 will be the acquired Medeor hospital, right?

Vivek Goyal: Yes.

Bino Patiparaveer: Okay. So may I have a rough understanding of where the rest 350 will be spread out like?

Vivek Goyal: Yes. So I will say out of 700 almost 300 -plus beds is of the Manesar facility. So we have this 350 bed acquisition in Manesar. So our plan is not to open the entire the bed at one go.

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Because of the cost optimizing point of view. So there will be maybe, we will start with 100 bed and then we'll see how the ramp -up goes up.

And if we see increasing we may open another beds during that period. And depending upon the ramp -up, we will try to do that and as we mentioned, it will be in the different time zone.

So like Manesar, we will be opening maybe better mid of the year. So that type of impact will also be there. And the other major capacity expansion is coming in the Anandpur facility and the BG road facility.

And one, of course, the Faridabad expansion thing which will be operationalized in the

the next

financial year. So those things, I think we will be operationalizing over a period of time. It will be around 350 beds will be operationalized by the year.

Bino Patiparaveer: Understood. And the last question, the biggest bed increase seems to be happening in the NCR around 1,000 beds over the next two years, 1,000 -plus. Shalimar Bagh and from what I understand would be ground fields. Noida and Manesar, Manesar, are you referring there the same maybe or sorry, Noida and Manesar, are they completely brownfield or new facilities at...

Vivek Goyal: Existing hospital. And there, we are expanding the facility. So it is brown field. Noida is brown field. Manesar is the existing hospital, but it was not fully operational, and we are making it operational after doing some re engineering of it.

Moderator: Our next question is from the line of Nikhil Mathur from HDFC Mutual Fund.

Nikhil Mathur: Yes. So my question is linked to the bed expansion plan. What we see from your margin metrics over the years is that almost 35% - 40% of beds, they tend to be in the less than 15% EBITDA margin range. Now can you give some colour on the bed expansion plan? How much of beds are getting added in this less than 15% EBITDA margin range. I suppose that there are -- I mean, more of these certainties are operating our occupancy of more than 50%. So absorption of fixed costs via a number of -- more number of it is perhaps the only way you can improve your margins structurally in these facilities?

Vivek Goyal: Yes. So let me clarify, the bed expansion is not happening in any of these hospitals, which are less than 15% EBITDA margin. So the bed expansion is happening in the hospital, which are operating at almost 70% to 75% occupancy level. and they are operating at a decent EBITDA margin of 20% plus.

Nikhil Mathur: Sir, but if your occupancy of 10% to 15% EBITDA margin range, facilities is 67%, less than 10% is also not too bad at 54%, so what are the chances of EBITDA margins in these facilities improving structurally? I mean, I fail to understand how that will kind of happen.

Vivek Goyal: Yes. That's why it will not be happening immediately. And it will be a gradual thing. And we have to give two - three years' time for things to materialize. So it will not be immediate. I will not say it will immediately improve EBITDA margin on these hospitals..

Nikhil Mathur: So sir, on a two, three year horizon as well -- I mean, there is inflation that will catch up on various costs. There's been expansion happening across the country. There will be poaching pressure as well on your talent and all unless now you are saying that your ARPO B increases in the less than 10% facilities by substantially maybe 12%, 13% then perhaps your EBITDA margins can improve in these facilities. But even on two to three horizon, how will that kind of happen.

Vivek Goyal: Yes. So as I have mentioned, there are two reasons for increasing the EBITDA margin. Our ability to react prices is limited, as you know. We are operating in a slightly different environment. So we are not expecting we will be able to increase the price at 5% - 6% at a consol level, okay? So that is given assumption. But having said that, the EBITDA margin improvement will happen on two fronts.

One is on the occupancy side. As our occupancy level goes up, we're able to utilize our existing infrastructure and the cost base will be higher. And with that, our EBITDA margin will improve. That is on one side. And similar to that if we are going for brownfield expansion that also lead to the margin improvement because we are adding the bed in the existing infrastructure itself.

So the cost will be shared with the larger base, and that will help in improving the margins.

Nikhil Mathur: Okay. And you mentioned that in the overall above increase, there is very less increa

se in

pricing, it's more of mix. So can you give two other examples of these mix changes, which has kind of contributed to this ARPO B increase. I mean not all there could be many examples of this, but if two -three examples can be given on how this ARPO B is increasing?

Vivek Goyal: Yes. One is immediately in my mind, is the oncology business, you know oncology business it's generally high ARPO B business. And the margin is slightly less. But when we see the gross revenue divided by the number of beds for this onco business, the ARPO B will be higher. Onco business is growing at a decent pace for us and we expect this will continue because of our focus on this specialty. And similarly, the neuro where we are quite strong and we are strengthening our presence in neuro, that is also lead to ARPOB increase. Then we have invested in a lot of robotic surgeries and things like that. And that should lead to the high ARPOB business going forward.

Nikhil Mathur: Any capex number you can share in, let's say, last 12 months, 18 months, which has boosted your capabilities on these onco and neuro cases, any capex number you can think?

Vivek Kumar Goyal: Yes. So I can give you the current year number itself. For the 9 months, we have spent already INR 450 crores in the capex. And I think the similar trend will be for the current quarter. So I think we are investing a lot in our technology. We have invested in the state -of-the-art Gamma Knife in our Flagship hospital.

We have invested in MR -Linac. Two of the Linac we have replaced, the CAT H lab and we have a Da Vinci Robot we have invested. And the robotic surgeries also, Ortho Robot, also we have invested INR 5 crores, INR 6 crores. So we are not hesitating in investment in the technology, and that is yielding the results.

Moderator: Our next question is from Neha Manpuria from Bank of America. Please go ahead.

Neha Manpuria: Just one clarification. The notice that we have got from the Anti -Corruption Bureau for the Mohalla Clinic, is that -- is there some amount of that impact also in the December quarter or that will flow through from this quarter onwards?

Vivek Kumar Goyal: No. That is the major impact actually in that quarter.

Neha Manpuria: In the December quarter, right?

Vivek Kumar Goyal : Yes.

Neha Manpuria: Okay. Understood. And my second question is just a follow -up from the previous participants. If I were to look at less than 10% margin bracket, if you could pick two or three of the largest hospitals that probably dragged the margin profile for the company. And what are the structural measures that you're talking about in each of these. That would probably help us understand it better as to why it will take three, four years to improve performance in these hospitals.

Vivek Kumar Goyal: Yes, I will pick two. One is FEHI and second is Jaipur one. And both are 300 -plus bedded hospital. And both have a good potential, as you all know. So in Jaipur, we have a location advantage. It is located in a premium location. We have good clinician teams, and it is having

a good brand equity in that region. And the thing it is lagging is it is not having the full basket of facilities which we can offer for the hospital of this size.

So we are trying to add radio, onco there, and that will take itself for one, one and a half years to plan that thing because bunker and other thing will be required to be built, and then there will be some construction activity will have to happen. And we hope that with that you know this hospital should be moving above 15% EBITDA margin for sure. So that is on the Jaipur side.

And secondly, on the FEHI side, we are trying to add other specialties. It is mainly a cardiac center. We are trying to add other specialties so that you know the cost base can be paired with the increase in the revenue. And this hospital again

is having a very good brand equity and it will be a decent size hospital. So I think with this attracting clinician talent for a cardiac business and then stabilizing them take some time and that's why I'm saying it will take a minimum two years' time from now to start showing the result for these two hospitals.

Plus we are doing lot of work on the cost side in both these hospitals, whatever we can do on the cost side on the manpower, productivity side, on the other cost initiatives which we are taking. So I think all those will start showing results maybe in a two years' time. Actually already started seeing results if you see the performance is improving for both these hospitals.

Moderator: Our next question is from the line of Harsh from Bandhan AMC. Please go ahead.

Harsh: I just wanted to understand to pick up from the previous question in terms of FEHI and Jaipur, you adding radio, oncology, cardiac division and all the other areas. If you look at the overall Fortis Healthcare Limited

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margin metrics, let's say, nine facilities below 15% margin, could you help us understand that the ability to provide more and more high-end surgeries and clinical work, can this be expanded to, let's say, these other 9, 10 facilities, which are making less than 15% margin? Or this is more of a selective play across, let's say, only two or three facilities, which are making more margin. Because the reason that I'm asking is that the feedback is that, because of insurance penetration, there's a lot more affordability in Tier 1, Tier 2, Tier 3 cities as such. So in that context, if you could help us understand.

Vivek Kumar Goyal: Yes. So if I can answer this way. Out of the 9 hospitals, 2 hospitals are where we are doing the expansion. One is the Faridabad one where the operation is slightly affected because of the ongoing extension it is disturbing the existing operation. And as I mentioned earlier, the extension program will be completed during this next financial year, early part of the next financial year.

And we expect when there will be higher bed capacity and the cost base will be paired with the higher bed. And secondly, the disturbance will not be there. So I'm 100% sure that Faridabad hospital will definitely move up in the ladder. Second is Ludhiana, that is more affected because of this seasonal impact and that is not a worry for us.

So out of seven, two we can take out very easily. Then remaining five; out of five, one we have already divested. Another is a small hospital in Bangalore which is actually a very small hospital and we are looking for various alternatives, maybe including shutting it down also. So the two hospitals we have already discussed.

Another two hospitals are hospitals where we have rental premises. So there is a rent sitting on it and a 6%, 7% rent if we add. Because of that, the EBITDA margin is slightly on the lower side. But we are doing some work and we are hopeful that at least 15% margin, these two hospitals, we will be able to do.

Harsh: So from a clinical -- yes, so from a clinical aspect, Ludhiana, Faridabad, Jaipur and FEHI are the areas where you can possibly provide more in terms of the on-ground clinical options.

Vivek Kumar Goyal: Yes, you're right. These are the four hospitals where that improvement will be slightly on the faster side.

Moderator: Our next question is from the line of Bansi Desai from JPMorgan. Please go ahead.

Bansi Desai: I just needed one clarification. I don't know how much you can provide color on this. But for the private equity investors in Agilus, the timelines of providing exit to them are about February of 2024. Am I correct? And does that stand true or does that hold with the IPO now?

Vivek Kumar Goyal: Yes. So as per existing agreement, the timeline is April '24. And as you all know, we have filed the DRHP we are still waiting for SEBI clearance. So

we are actually we are standing at that point now.

Bansi Desai: Okay. Got it.

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Moderator: As there are no further questions, I now hand the conference over to Mr. Anurag Kalra for closing comments.

Anurag Kalra: Yes. Thank you very much, ladies and gentlemen, for your time. If there are any follow-up questions, please reach out to me or my colleagues. We'll be happy to address them as best possible. Thank you very much again and have a good day.

Moderator: Thank you. On behalf of Fortis Healthcare, that concludes the conference call. Thank you for joining us, and you may now disconnect your lines.

Call: May 2024

FORTIS HEALTHCARE LIMITED

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National Stock Exchange of India Ltd.

Scrip Symbol: FORTIS BSE Limited

Scrip Code:532843

Sub: Transcript of Investors / Analysts' meet under Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015 .

Dear Sir/Madam,

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed transcript of Investors / Analysts' meet held on May 24, 2024 to discuss the Company's Audited Financial Results for the financial year ended on March 31, 2024 and the same is available on the website of the Company at below hyperlink:

Transcript

This is for your information and record .

Thanking you,
Yours Faithfully
For Fortis Healthcare Limited

Satyendra Chauhan
Company Secretary & Compliance Officer

Encl. a/a

MANAGEMENT DR. ASHUTOSH RAGHUVANSHI -- MANAGING DIRECTOR
AND CHIEF EXECUTIVE OFFICER , FORTIS HEALTHCARE
LIMITED

MR. VIVEK GOYAL -- CHIEF FINANCIAL OFFICER , FORTIS
HEALTHCARE LIMITED

MR. ANAND K. -- CHIEF EXECUTIVE OFFICER , AGILUS
DIAGNOSTICS , FORTIS HEALTHCARE LIMITED

MR. MANGESH SHIRODKAR -- CHIEF FINANCIAL OFFICER ,
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MR. ANURAG KALRA -- SENIOR VICE PRESIDENT ,
INVESTOR RELATIONS , FORTIS HEALTHCARE LIMITED

MR. AMIT MAHENDRU -- DEPUTY GENERAL MANAGER ,
INVESTOR RELATIONS , FORTIS HEALTHCARE LIMITED

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Moderator : Ladies and gentlemen, good day, and welcome to the Q4 FY '24 and Full Year FY '24 Financial Results of Fortis Healthcare Limited.

As a reminder, all participant lines will be in the listen -only mode, and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing "*"and then "0" on your touch -tone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Anurag Kalra, Senior Vice President, Investor Relations at Fortis Healthcare Limited. Thank you, and over to you, Mr. Kalra.

Anurag Kalra : Thank you, Mitchell. A very good morning and good afternoon, ladies and gentlemen. Welcome to Fortis Healthcare's Quarter 4 FY '24 and FY '24 Earnings Call. The call is being chaired by Dr. Ashutosh Raghuvanshi -- our Managing Director and CEO. With him, we have Mr. Vivek Goyal -- our Chief Financial Officer. From Agilus Diagnostics side, we have Mr. Anand K . -- the Chief Executive Officer; and Mr. Mangesh Shirodkar, who is the CFO of Agilus Diagnostics.

We will begin the session with some comments by Dr. Raghuvanshi on the performance for the quarter and the year, followed by which we will request Mr. Anand to give his comments on the Agilus business, and then we can open the floor for question and answers.

I hand over to Dr. Ashutosh Raghuvanshi now.

Ashutosh Raghuvanshi : Thank you, Anurag. Good morning and good afternoon, everyone. Thank you for taking time to join us on our Q4 Financial Year '24 Earnings Call today. I hope all of you are doing well. Before discussing our Q4 and full Financial Year Results , I am delighted to announce that our Board has recommended a dividend of Rs. 1 per share, which is equivalent to 10% of the face value for the second consecutive year, subject to shareholders' approval. This signifies the transformative path the Company has embarked on over recent years, leading to the healthy operational and financial performance. This also underscores the strengthening fundamentals of the Company and its ongoing potential for growth.

Coming to the performance of Company , I shall comment on the year as a whole and then move on to Q4. Our performance in Financial Year '24 has been commendable, demonstrating a marked improvement compared to the preceding years. And noticeably, this performance has been led by a strong performance from our hospital business. For the financial year 2024, consolidated revenues for the Company stood at Rs. 6,893 crores, a growth of 9.5% over Financial Year '23. Within this, our hospital business revenues have grown 11.3% to Rs. 5,686 crores while the diagnostic business revenues have shown a marginal growth of 2% to Rs. 1,372 crores. Our consolidated operating EBITDA increased 15.1% to Rs. 1,268 crores, which

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translates into a margin of 18.4% in Financial Year '24 versus 17.5% in Financial Year '23.

Within this, the hospital business operating EBITDA margins have improved from 16.9% to 18.6% in Financial Year '24, with an EBITDA of Rs. 1,058 crores. The hospital business EBITDA contributed to the total consolidated EBITDA has increased to 83% as against 79% in Financial Year '23, signifying the profitable growth witnessed in the business.

Operating EBITDA margins in the diagnostic business basis gross revenues were at 15.3% versus 17.7% in Financial Year '23. However, after adjusting one-offs related primarily to the rebranding exercise undertaken by Agilus for the brand change and the provisioning related to the business with Directorate General of Health Services New Delhi, the operating margins improved from 17.7% in Financial Year '23 to 19.5% in Financial Year '24.

At the consolidated reported PAT level for the year, we

reported a profit after tax of Rs. 645

crores for the year as compared to Rs. 633 crores in Financial Year '23.

On the quarterly performance, we recorded a consolidated top line of Rs. 1,786 crores in Q4 of Financial Year '24, a growth of 8.7%. The hospital business revenues grew by 10.3% to Rs. 1,490 crores, while the diagnostic business gross revenues witnessed a marginal growth of 2% to Rs. 338 crores.

Our consolidated operating EBITDA for the quarter was Rs. 380 crores, reflecting a margin of 21.3% versus 16.5% in Q4 of Financial Year '23. The operating EBITDA of the hospital business grew 51% to reach at Rs. 333 million with the margin of 22.4% in Q4 of Financial Year '24 versus 16.4% in Q4 of Financial Year '23. The quarter number also includes certain year-end adjustments related to write-backs of excessive provisions, unclaimed balances, expected credit loss and other adjustments, which are accounted for in the quarter, but pertain to the full year.

Operating margins in the diagnostic business were at 14.0% versus 14.9% in Q4 of Financial Year '23. However, after adjusting the one-offs I had mentioned before, the operating margins improved to 15.9% in Q4 from 14.9% in Q4 of Financial Year '23. Our consolidated reported profit after tax for the quarter increased 46.9% to Rs. 203 crores. On the balance sheet side, we have further reduced our debt by Rs. 76 crores to Rs. 264 crores, representing a healthy net debt-to-EBITDA of 0.17x as compared to 0.30x as on 31st March 2023.

Before I start the operational highlights of the business, I would like to touch upon and share with you the good work that we do in our day-to-day lives. We have in Financial Year '24 undertaken more than 60,000 cardiac procedures, approximately 1,100 transplant, close to 28,000 joint replacements and ortho procedures, 7,800 neuro and spine procedures, 11,000 radiation therapy for our patients. This speaks volume of our expertise and the confidence that patients have in us.

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Fortis facilities across the network continue to perform life-saving procedures and successfully undertake complex surgeries in various medical specialties all backed by our dedicated clinicians and state-of-art medical infrastructure. With this continuing care for good motto that is resulting in the performance that you are seeing.

Now some metrics of our hospital business for Financial Year '24, our hospital business had an occupancy of 65% compared to 67% in Financial Year '23. We added 246 beds during the year in our various facilities and hence, our occupied beds have remained largely similar to last year. ARPOB for the hospital business witnessed a growth of 10.8% to Rs. 2.22 crores in the Financial Year '24. This was primarily driven by a substantial uptick in revenue from our key focus specialties, including oncology, cardiac sciences, neurosciences, renal sciences, et cetera. These specialties collectively experienced a 13% year-on-year growth in Financial Year '24 accounting for 62% of the total hospital business revenue, up from 61% in the corresponding previous period. Higher complex surgical volumes in select medical specialties have contributed to the increase in ARPOB. For instance, volume in key procedures such as transplant grew 11%, while in robotic surgeries and radiation therapy, volume growth was in excess of 50%.

In addition to the above, our revenue from medical travel grew 12% in Financial Year '24 to reach Rs. 479 crores. Revenue contribution of international business stood at 8% in Financial Year '24, similar to Financial Year '23. Throughout the year, our commitment to enhancing clinical programs persisted across all facilities marked by investment in advanced medical infrastructure, emphasizing growth in specialties like oncology, neurosciences and cardiac sciences.

We expanded our clinical offerings bolstered

by high-quality talent. Noticeably, the year saw

the addition of reputed clinicians in various specialties, including nephrology, neurology, cardiac sciences, oncology, gastroenterology, general surgery and urology. Our pursuit of superior clinical outcomes and patient experience is augmented by digital initiatives like the

implementation of electronic medical record system across our network, ensuring excellence in care delivery. I am pleased to share that Company has made significant progress in advancing its strategic growth levers including Brownfield bed expansion, portfolio rationalization and investing in the state-of-art medical equipment.

In order to provide advanced treatment options to our patients, we have and continue to upgrade our medical infrastructure, having commissioned LINACs in Mohali and Noida, Cath Labs and MRI at Anandpur, Ortho Robots at FMRI, Shalimar Bagh, Noida and Digital PET CT at FMRI. Further launches of similar such medical equipment across Fortis facilities is expected in the current fiscal year as well.

We are progressing well on our Brownfield bed expansion plans. We have added beds in Financial Year '24 in facilities, such as Mohali, Anandpur, Mulund and BG Road. I am pleased to share that we have also launched a 70 bedded new facility in Ludhiana, making this our second facility in the city and fourth in the state of Punjab, giving a boost to our presence in the Punjab

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cluster. Our extension strategy continues to focus on deepening our cluster presence. Our plans to ramp up the Brownfield beds capacity remains on track and would enable us to potentially close to 6,000 beds over the next few years. When completed, we can also expect to see some of our key facilities such as Shalimar Bagh, FMRI, Mohali and BG Road becoming more than 450 beds each.

As we have mentioned earlier, our growth would comprise not only of our Brownfield expansion effort, but also our effort towards expansion of our size and scale through inorganic forays. We have spoken with you previously on some of our underperforming assets and the need to rationalize our portfolio. Continuing with this, we have successfully divested 2 of our underperforming facilities in Chennai and have exited that market. In parallel, we also acquired a potential 450 bedded facility in Manesar, Gurugram that we expect to commission shortly, which would further strengthen our presence in the NCR cluster.

Another critical aspect worth noting is the continuous success of our digitization efforts. Our progress in digital transformation, notably through the implementation of EMR program is moving forward positively. We have successfully implemented EMR for OP modules across 4 units in the first phase of EMR rollout. Revenue generated from digital channels continue to exhibit healthy growth in Financial Year '24. Revenues from website, mobile apps and digital campaigns increased 27% over the last year and contributed 25.2% to the overall hospital revenue as compared to 22% in Financial Year '23. In order to further improve our patient service experience, we have launched our new patient feedback management system, my feedback, this year. The platform will enable a more engaging experience as the feedback through WhatsApp and QR codes would be collected. The application will enable selection of feedback and address immediate patient concerns through its service request feature.

On the cost side, emphasis in the year gone by has been on further improving our supply chain and procurement efficiencies and also optimizing costs in aspects related substitution and formularies. We have also been focused on optimizing manpower costs and looking at means to increase productivity. While these are ongoing exercise, cost saving and optimization across select expense lines have also had a positive impact on our P&L in the year.

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Some thoughts on our diagnostic business. Our overall diagnostic business showed marginal growth, but our non-COVID business revenue grew 5% for the quarter, and 6% for the Financial Year '24 over corresponding previous period. The diagnostic business performance has been soft, primarily due to the change in the brand to Agilus resulting in comprehensive rebranding exercise and related expenses on such rebranding and marketing. In addition, there were certain provisions related to government business that have also impacted the performance.

Having said that, and while Anand will take you through further details on diagnostic business, I firmly believe that this business has significant potential to scale up both in terms of its revenue and margins. It has a sizable network presence, balance B2B B2C mix and is increasing its focus on the wellness portfolio and specialized testing. The industry has also begun to show signs of

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improvement and lessening competitive pressures. We are undertaking all efforts to ensure that Agilus' performance is progressively better going forward, but I'll leave it to Anand to talk further on this.

Before I end my comments, I will take a moment to thank all our stakeholders, employees, patients and clinicians for their support in the years gone by. We strive for excellence in health care delivery, we strive for good clinical outcomes and diagnosis for all our patients and we

work hard to ensure that all our stakeholders recognize and respect the organization for its patient-centric approach. This is and would remain cornerstone of our organization and organization that each of us feel proud to be part of.

I am hopeful that the coming years would see Company going from stem to stern and add value to all our stakeholders. Thank you. And with this, I will hand over to Anand for his comments on the Agilus.

Anand K. : Thank you, Dr. Raghuvanshi. A very good morning to everyone on the call. Thank you for joining us today. On behalf of Agilus Diagnostics, I warmly welcome you all to our Q4 FY '24 results conference call.

Agilus Diagnostics reported a revenue of Rs. 1,372 crores in FY '24 versus Rs. 1,347 crores in FY '23, representing a 2% increase. Non-COVID revenues increased by about 6% in FY '24 against FY '23. COVID testing contributed to 0.3% of the revenue for this year, down from 4.4% in FY '23. We performed 40 million tests and served 16.4 million patients in the year.

Operating EBITDA for the year is Rs. 209 crores compared to Rs. 239 crores in FY '23. EBITDA margins are 15.3% and 17.7%, respectively. During the year, we incurred onetime expenses of Rs. 58 crores, primarily in relation to Agilus rebranding and the provisioning pertaining to agreements with Directorate General of Health Services, New Delhi. One of the important highlights of the financial year is that we have successfully undertaken a brand transformation exercise and move to a new brand, Agilus. We incurred onetime expenses for rebranding post brand change in May '23. Also, we have made onetime provision in relation to receivables of DGHS Delhi Government. Considering lack of clear timelines on payment of bills, the Company has provided for the entire outstanding.

Our operational EBITDA before onetime expenses of Rs. 58 crores is Rs. 268 crores, representing a margin of 19.5%. Our average realization per test for Q4 '24 is Rs. 352 and realization per patient is Rs. 869. Our average realization for FY '24 is Rs. 342 per test and Rs. 836 per patient. The business continued to have a well-diversified geographical mix with no overdependence on any particular region, allowing you to capitalize on the pan-India network nationally.

Regional FY 2023 to '24 revenue contributions were 33% from North, 21% from West, 29% from South and 14% from East while we had a 3% contribution from international business. Our

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wellness portfolio also went up by 14% in FY '24 compared to previous fiscal and by 18% in Q4 FY '24 compared to Q4 of FY '23.

From a product standpoint, the revenue contributions are 36% from specialized testing, 54% from routine testing and 10% from our wellness portfolio for FY '24. Quarterly contribution from specialized testing is 34%, from routine is 54% and from wellness is 12%. Genomics portfolio also grew by about 27% in FY '24, compared to FY '23. Our B2C:B2B ratio for FY '24 is 53:47 compared to 54:46 in FY '23.

We are focused on building the new brand and delivering high-quality diagnostic care to patients and doctors. We have reinforced our test menu of 3,600 plus tests with 70 new additional tests and test panels this year. Some of the advanced next-generation diagnostic tests that we have added include pharmacogenomics testing, newborn screening by next-generation sequencing, fetal autopsy testing, cancer hotspot gene panel, comprehensive advanced HLA panel, allergy testing by component resolved diagnostics and a few more. We will continue to deepen our network, drive further utilization of existing infrastructure, reinforce our test menu and focus on technology to enhance customer experience.

Thank you all, and I now hand over the call to Anurag for further.

Anurag Kalra : Thank you, Anand. Ladies and gentlemen, we shall now begin the question-and-answer session. Can I request the moderator to begin with, please.

Moderator : Thank you very much, sir. We will now begin the question and answer session. The first question is from the line of Neha Manpuria from Bank of America. Please go ahead.

Neha Manpuria : My first question is on the hospital business. We have gotten to 22% margin in this quarter. I understand there's some one-off. First, if you could quantify what the one-off was? And second, given our exit margins in the quarter, is it fair to assume that we build on this margin in FY '25 because initially we guided to getting to 22% margins probably in '26. So, I just want to understand the trajectory from here.

Vivek Goyal : So, first of all, there is no one-off in the quarter. Only some of the accounting adjustment has happened in the last quarter pertaining to the full year. So, there is no one-off type of expenditure. The normal thing like expected trade losses, provision write-back and things like that. So, those types of things, which is generally done at the year-end. So, the EBITDA margin, excluding those entities also for the quarter is around 21%.

And as regard to your other question on build-on the margins, during this financial year, we could demonstrate around 2% EBITDA margin improvement over the last year. I am expecting

similar type or slightly better than this in the next financial year. So, we are maintaining our guidance which we have given earlier.

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Neha Manpuria : No, what I mean to say is, since we are already at 21% margin for fourth quarter, I understand there is seasonality in the business. So, should I assume we build on that 200 basis points on the 21% or should I assume because we have divested 2 assets through the year. So, hence the question.

Vivek Goyal : Yes, I understand your point. So, as you know, there is a seasonality build in the fourth quarter, generally is a good quarter for everyone. And 2 of the new unit has also been added while we have divested certain nonperforming units. So, looking at all those things, our initial guidance, what we have given, improvement of EBITDA margin by 2% will remain year-to-year.

Neha Manpuria : And how many beds are we adding in FY '25? Will it be 300? And where exactly will these beds be added?

Vivek Goyal : So, one is the Faridabad unit, which we are adding the beds, the expansion is almost completed, 50 beds we will be adding. Shalimar Bagh also,

so, there 50 beds we will be adding, and this takes

us to 100 beds for the current financial year. For Manesar facility, we are expecting to open by second quarter of this financial year, and we will be opening initially 100 beds, and we will see how the ramp-up goes on. Another facility in Kolkata, where we are adding 100 beds, 2 floors, we are operationalizing and all the approvals are in place. And in the first quarter itself, we should be starting those 100 beds. Then rest of the beds are at BG Road, which is in Bangalore where we are expecting some OC to come and that we are expecting by the second quarter end for the current financial year.

Moderator : We will move on to the next question, which is from the line of Shyam Srinivasan from Goldman Sachs. Please go ahead.

Shyam Srinivasan : Just the first one, given the Supreme Court order or whatever the NGO petition around standardized pricing. I just want to know what your take is in terms of how this could likely pan out. And a related question is on the ARPOB. So, we have again seen another 10% kind of ARPOB growth for the quarter, maybe for the full year is also similar. So, how should we look at ARPOB growth looking ahead? Is there any impact? Or are you trying to see whether the standardization will likely have an impact on how you price your services into fiscal '25? And also the split of ARPOB between price and mix, if you could highlight? That's my first question.

Ashutosh Raghuvanshi : Yes. So, the first part related to the PILs and Supreme Court, et cetera, I think the tenor of the affidavit filed by the government is very positive according to us. So, we do not expect too much of an impact of it. On the other hand, what we are hearing is that the CGHS and other rates are going slightly to be revised upwards. So, that would overall, not have too much of an impact on that.

Regarding the question on ARPOB, we definitely had a good growth in the ARPOB and that was primarily led by the increase in the surgical volume by about 8% year-on-year, and that also

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is the case mix, which led to that change. Now going forward, I would request Vivek to add to this.

Vivek Goyal : So, the ARPOB increase split, to answer your last part of your first question. Around 3% is towards price increase and balance is for the case mix and speciality mix, better case mix basically. And going forward, because ARPOB has already set at a higher level, we are not expecting this double-digit growth rate will continue like this. I am expecting more like 5% to 6% ARPOB growth, mainly driven by the specialty and the payor mix improvement.

Shyam Srinivasan : We are not planning any price increase this year, is it? Or we are almost done or so any benchmark with some of our competitors, our pricing is similar?

Vivek Goyal : No, we will be increasing prices, some price increase already factored in the first quarter itself. So, we generally increase our price by 2% to 2.5% depending upon other competition prices and the market research. So, that will continue. So, you can safely assume the price increase of 2.5% and balance is towards case mix and other things. So, total ARPOB increase will be 5.5% to 6%.

Shyam Srinivasan : Understood. Very helpful. Just the second question is on the Agilus. I know we had a DRHP out there. We had to withdraw it for whatever market conditions/whatever the performance of the Company. So, what's the medium-term outlook for either a listing or when should we look at it?

Also, the related question is on the put option liability for the private equity. How are we going to navigate it? October 2024 is the date I remember, but I may be wrong. But in a case where some of them want to kind of exercise their put option, how will Fortis/IHH respond to that?

Vivek Goyal : Yes. I will take that question again. You have already mentioned, we have withdrawn, a

s you

know, the DRHP and there is a put option lies with the private equity investors. So, we are working with the private equity investors to come out with a solution for this particular thing. One option is for the revival of the IPO, where we are working with bankers along with the private equity investors for revival of the IPO. And parallelly, we are working on the various other options, which may be possible in case the revival of IPO is not possible. So, from that angle, I think we have maybe a couple of months more to finalize these things. So, we are working on all the options that are still open, and we are working very closely with the private equity investors.

Shyam Srinivasan : What's the worst case here that we have to take whatever the private equity offers is it? I am just trying to assess what could the worst case be.

Vivek Goyal : Yes. So, Shyam, if suppose IPO does not happen and we have to honor our obligation of this put option liability. And we are disclosing the value anyway, recognizing that as a liability in our books already. So, we will honor that liability and the means of funding we will be decided depending upon your market situation. Looking at the balance sheet side, and the other options we may do entirely through that. We may do a 50% debt, 50% equity or entirely through equity, those things we will be deciding maybe over a period of time. But the funding will not be an

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issue looking at the financial position of the Company right now. If at all what the scenario we have to honor the put option. The funding is the only option for us. And for that, I am saying there are various options available depending upon our fund position, our capital requirements, we will decide on the means of funding.

Shyam Srinivasan : Sorry, sir, the time line is correct, October 2024?

Vivek Goyal : Yes. So, by August, we will be knowing which direction we are going, means the next call for the first quarter. We will be having clear idea, which way we are going and maybe another one month. You are right, absolutely September, October may be the time by when this will be finalized.

Shyam Srinivasan : I am sorry, sir, last question. Equity means capital raise at the holdco at Fortis?

Vivek Goyal : Yes. Because Fortis is having the put option. So, Fortis may look for raising equity.

Moderator : The next question is from the line of Neha from Bank of America. Please go ahead.

Neha Manpuria : Apologies for the last time. On Diagnostics, just wanted to understand how we are looking at revival of the business. We saw a negative volume growth, a number of test growth this quarter. Given the rebranding, should we expect a more slower recovery in that business?

Anand K. : This is Anand here. In fact, for the overall year, we have grown the volumes. Non-COVID volumes have actually grown by 6.4%. And the volumes for the quarter have grown by just 0.6%. We are seeing a recovery on the growth on the volume as well. And also, if you see we have taken a price increase around the end of February. And so we expect that also to kick in. So, we have taken a price increase of about 5% to 7% for the B2C business. So, that also will be kicking in for this financial year. So, we are seeing recovery on the volumes.

Neha Manpuria : How much volume growth should we expect over the, let's say, next 2 years? Should over the next 2 years Agilus be closer to the industry growth? Would that be a fair assumption? Or do you think that takes longer?

Anand K. : So, we will be in line with the industry growth. So, the industry is expected to grow at about 8% to 10%. So, it should be a combination of volume as well as value. So, I think it will be primarily volume and to some extent from the value as well.

Neha Manpuria : This is for FY '25?

Anand K. : For the next 2 to 3 years.

Neha Manpuria : And just one other question on

the entire Fortis business. What is an update on the high court?

What is the next time line that we should look out for or possible hearing on the high court case?

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Ashutosh Raghuvanshi : Yes. So, the hearings have been happening in the case. We expect the next hearing to happen on the 15th of next month. And that would probably be one of the final hearings, after which maybe some arguments will be made, and then we should see some resolution to the case, but we cannot really guess as to how much time that it will take. But it appears now that things are coming towards conclusion.

Moderator : The next question is from the line of Hardik Doshi from White Whale Partners. Please go ahead.

Hardik Doshi : I just wanted to kind of clarify a couple of things regarding margin. So, I think you mentioned that the reported margins were 22.8%, but through the year adjustment at about 21%. So, I guess this happens every fourth quarter. So, what would be the like-to-like comparison for fourth quarter '23? I think you reported 17%, but if we sort of make the adjustments, what would be the

margins then?

Vivek Goyal : Yes, similar around 1% adjustment, you can earmark for the last quarter also. So, last quarter reported margin should be brought down by 1%.

Hardik Doshi : Got it. So, 16% went to 21%.

Vivek Goyal : Yes.

Hardik Doshi : So, is this largely driven by the ARPOB improvement? Or are there any other factors that drove this 500-basis point year -on-year improvement?

Vivek Goyal : Yes. So, there are a couple of things here. One is, of course, the ARPOB improvement, you rightly stated. Second is the volume growth. The occupancy level has gone up for the quarter.

And thirdly, there is an impact of divestment of Malar as well. As we know, Malar was incurring losses and with the divestment which happened on 1st of February. So, 2 months, there were no losses. So, that impact was also not there. So, all this put together, and plus the speciality mix improvement also, which is already factored in the ARPOB. So, these are the 2 - 3 main reasons for the margin improvement.

Hardik Doshi : Got it. Now for FY '24, our margins were around 19.2%. Did I understand the guidance correct that you're expecting a 200-basis point improvement year -on-year in margins. So, FY '25 would be about 21%.

Vivek Goyal : Yes, we will be around that level only because the reported margin should be around 22%. And operating margin is 18.5% for the current financial year. So, that should also go up by 2%.

Hardik Doshi : But you're trying to open 700 beds in this financial year. And usually, when you open or do Brownfield expansion also, there is a near -term dip in occupancy, which obviously weighs on margins. So, how do you expect, like, let's say, 18.5% which is operating EBITDA going to 20.5% for the full year in spite of such a large bed addition.

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Vivek Goyal : Yes. So, most of the bed additions are coming in as a Brownfield, which I've mentioned like Anandpur, this Faridabad, Shalimar Bagh, these are all Brownfield expansion and in the units which are already operating at around 75% plus occupancy level. So, we are not seeing any challenge in filling those beds. Of course, the Manesar one is a new facility because this hospital was not operational. So, that is something which we have estimated initial negative contribution from this unit in the current financial year. Apart from this, I don't see any other unit where we will be having any margin impact. So, that we have already taken care of while giving the guidance.

Hardik Doshi : And then in the medium term, I mean, what are your aspirations from a margin perspective? Where do you think you can go to, let's say, 3 to 5 years around?

Vivek Goyal : Yes. So, we are moving, I will say, on the right track. We have given the guidance the last

quarter, we should be touching 20% margin. We have exceeded that expectation actually. And we expect that we will be around 25% margin in the next 2 to 3 years' time.

Hardik Doshi : Just one last question for me is in terms of divestments, we have exited Chennai now completely. Are there any other divestments where we can look at maybe over the next 12 months?

Vivek Goyal : So, we have done two actually, and there is no immediate plan of any divestment, which is having a meaningful impact on the financials. There is one small unit, which we are looking at, but it is not having any meaningful impact on the financials.

Moderator : The next question is from the line of Sanjay Shah from KSA Securities Private Limited. Please go ahead.

Sanjay Shah : Doctor my question was regarding our CAPEX plan, which we have planned for next few years on the Brownfield side. I need to understand, do we have any scope of growing Brownfield in Faridabad and Anandpur and Shalimar? So, is there limitations or after that, how we plan to go ahead from there? And this CAPEX is going for treasury treatment or secondary treatment facility.

Ashutosh Raghuvanshi : Yes. So, all these hospitals, you mentioned both Faridabad as well as Anandpur are currently working at a high occupancy rate. So, this capacity will easily get absorbed over there. So, Brownfield expansion is being done selectively in the units where the occupancy levels are already significantly higher. And this is probably the most efficient way to have a significant growth. Vivek, would you like to add?

Vivek Goyal : So, these 2 units, there is further capacity for expansion, like in Faridabad, there is space available, and we have planned to further expand and add certain more facilities there. But we will see how the 50 beds ramp -up will happen. As regard to Anandpur, we have acquired a Company with a different land parcel. So, we want to operationalize that also. So, post-expansion of these 100 beds, we will be adding maybe another 50 beds in the next financial year

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in that unit. So, both are having those types of growth perspective available. And Shalimar Bagh, we are building another tower in the adjacent plot and that should give us another 200 -plus beds in Shalimar Bagh. So, this is already included in our overall plan of expansion of 2,200 beds which we have shared earlier.

Sanjay Shah : 2,200 beds totally. So, what CAPEX will be required for the Brownfield total 2,000 beds addition?

Vivek Goyal : Yes. So, some of the CAPEX has already been incurred in that. So, additional CAPEX will be somewhere around Rs. 1,200 crores to Rs. 1,300 crores, which include equipment also, everything put together.

Sanjay Shah : There focus will be more of a tertiary one or it will be a mix of secondary, primary and tertiary.

Vivek Goyal : It is tertiary only. So, all our hospitals are multi -facility hospital and especially this where the Brownfield expansion is coming. So, we will strengthen our position there, and we may add certain more facilities.

Sanjay Shah : And sir, how we did last year on international patient demand. And what was the occupancy on that side? And how do you see that future going ahead?

Vivek Goyal : So, international business is contributing more than 8% to our total revenue. We expect this percentage to slightly go up going forward. And this is a good profitable segment for us and a focus area for us for further growing this segment.

Sanjay Shah : So, my last question was regarding any update on rebranding our Fortis to Parkway. Any highlight on that? Any progress on that side?

Ashutosh Raghuvanshi : So, Sanjay, we are not immediately considering anything. Our preference is to retain the brand. And since it's kind of a court -related matter and legal issue. So, we are waiting for that to get resolved. Once that is resolved,

and we have clarity on the brand, then only we will take a decision on actually making a change if necessary.

Moderator : We'll take the next question from the line of Bino P from Elara. Please go ahead.

Bino Pathiparampil : Doctor Raghuvanshi in response to an earlier question, you had mentioned about CGHS rates being revised up. Could you please put that in context for us, please? Because I think a few months back or a year back, we had a revision and as I understand, once i n 5 years that it typically happens. So, are you recurring some out of term increase that is likely.

Ashutosh Raghuvanshi : Yes. So, in 2014 was when the last rate revision happened. Last year, some of the diagnostic rates were revised upwards. However, the rest of the packages, which forms the bulk of the CGHS work was not revised at that time. But what we believe we have been made to understand Fortis Healthcare Limited

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by the Health Secretary is that the work is in progress, and that is likely to be revised sometime in near future.

Bino Pathiparampil : And does it happen in tandem for the entire industry? Or is it group by group or hospital by hospital?

Ashutosh Raghuvanshi : No, no, it happens for the entire industry.

Bino Pathiparampil : And since it is after 10 years, are you expecting a very big jump and maybe 20%, 30%, 40%?

Ashutosh Raghuvanshi : It's difficult to predict with the government. I think I would leave, and wait and watch the space.

Bino Pathiparampil : Sir, earlier, you had a target of 6,000 beds by FY '28. Is that the valid target as of date?

Vivek Goyal : Yes. So, we are moving at pace to achieve that target. We are on target.

Bino Pathiparampil : And sir last question regarding the Agilus private equity exit. Even if you have to pay that money to the PE and buy out their portion, still it would be only a cash flow in, right? There won't be any P&L provision that's required, right? Is my understand ing correct?

Vivek Goyal : Yes. So, there will not be any P&L impact and cash flow, I have already explained that we have enough resources right now to honor that liability if at all, it will come up.

Moderator : The next question is from the line of Amit Ashok Thawani from Clear Blue Capital Advisors LLP. Please go ahead.

Amit Ashok Thawani : Congratulations on a good set of numbers. My first question is on the last call, we had mentioned that we are running at about 70% capacity occupancy. And I think we had the call somewhere mid-February, if I am not mistaken. And we had mentioned that we'll exit the quarter at that rate, but our occupancy has come lower than that. I just wanted to understand what happened out there?

Vivek Goyal : Yes. So, you're absolutely right. So, our target level was 70% only, but at 3 of our hospitals, the occupancy level was not at the desired level. Two of them in Bombay, Mulund is the big one. And another BG Road also is operating at around 60% only. And Jaipur also there is capacity shortfall. So, we are working on these units. And because of these 3 units only, our overall occupancy level, we could not achieve 70%. So, we are working on the plan. Good news is for the first quarter of the curre nt financial year, Mulund and BG Road, they have started seeing very good results.

Amit Ashok Thawani : So, we are actually now almost end of May. So, what kind of occupancy have we seen in April and May?

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Vivek Goyal : So, right now, we are at around 70%.

Amit Ashok Thawani : For the overall Company ?

Vivek Goyal : Yes, overall Company .

Amit Ashok Thawani : So, this quarter, we should report 70%.

Vivek Goyal : Let us wait for another 1.5 months.

Moderator : The next question is from the line of Nitin Agarwal from DAM Capital. Please go ahead.

Nitin Agarwal : Sir, on the hospital metrics margin that we provide, we have got still about 8 hospita

Is, which

are below 15% margins. When you look through the next 2 - 3 years, how many hospitals do you see actually moving out from this margin range with the initiatives that you have in mind?

Vivek Goyal : Yes. So, luckily, the hospitals which are not on good margin metrics are improving, okay? And so we feel that out of 8, Faridabad, definitely will soon move out because of this expansion because the economy of scale this year hospital will be getting. And Jaipur is also moving in the right direction. So, I will say 2 to 3 hospitals we can easily target that they will be moving up.

Other hospitals are either very small or there are some issues like Sacred Heart is very small hospital. So, CH Road, it is a rental mode I. So, that's why the margin is below 15%. So, Ludhiana is the new unit. So, we have to give some time for this unit to improve.

Nitin Agarwal : Fair to say that these hospitals right now that you mentioned about 20% of our revenues a good chunk of this 20% of the revenues, will see meaningful margin expansion over the next 2, 3 years or rather a couple of years with some of these initiatives pla ying out? About 20% of our revenue will have a meaningful margin delta.

Vivek Goyal : Yes. That will also continue to achieving the overall EBITDA margin, which as we guided earlier.

Nitin Agarwal : Right, sir. And sir, lastly, on the top end of the EBITDA, where we have got 8 hospitals in 20%, 25%. So, is 25% the upper range for our sort of best -performing hospitals? Or are there hospitals which have gone much beyond that also?

Vivek Goyal : Yes. So, I think it is not the range. I think there is the possibilities there that we can achieve even higher margin on these hospitals and these hospitals are also expanding. So, we will be getting economy of scales and the manpower productivity, which Dr. Raghuvanshi had mentioned earlier. So, all those things would play. And in my view, there is a scope to improve in at least some of the hospital EBITDA margin higher than 25%.

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Nitin Agarwal : And the last one, sir, what is our share of CGHS revenue? I probably missed that number for the year?

Vivek Goyal : Sir, overall government revenue is around 20%. It has slightly gone up in the financial year if we compare with the previous year, and that is 1 lever which we will be excluding in the current financial year to improve.

Nitin Agarwal : And sir, how much of that would be CGHS.

Vivek Goyal : CGHS is 4%.

Nitin Agarwal : Of the overall revenues?

Vivek Goyal : Yes.

Nitin Agarwal : And sir, when the CGHS rate changes happen, does it have impact on the other part of your government revenue also?

Ashutosh Raghuvanshi : Yes, yes. A lot of rates like ECHS, et cetera, are linked to the CGHS rates. CGHS, they are sort of like the mother scheme and most of the rates are equal to that.

Nitin Agarwal : So, then it's fair to assume that a fair chunk of our regional proportion of the 20% will probably get better rates as and when the CGHS rates gets revised?

Ashutosh Raghuvanshi : Yes. But overall, then still, we would aim to bring this percentage from 20%, we'll aim to bring this slightly lower though we expect that the realizations here might improve in the future. But at the same time, our objective is to keep this around 15%, which we may not be able to achieve immediately. But over the next 1.5 years or so, we should try to bring this down to below 15%.

Moderator : The next question is from the line of Nancy Yadav from Allegro Capital. Please go ahead.

Nancy Yadav : I wanted to understand the IndAS impact on our financials for both the Quarter and the financial year. And it would be great if you could provide the numbers for the hospital and diagnostic business separately.

Vivek Goyal : Which IndAS , you mean lease rent or leasing?

Nancy Yadav : Yes. 116.

Vivek Goyal : Maybe Anurag may give you separate

ly, send the number separately. Right now, it is not readily available to us.

Anurag Kalra : Nancy, we'll connect and we can provide those numbers to you.

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Participant : Sir, second question is regarding the margin improvement again in the hospital. I was looking at the operating EBITDA numbers. On Y -o-Y, we have gone up 16.4% to 22.3% for the quarter. If you could provide some color even accounting for divestment, how much of this is coming from the divestment of facilities and margin improvement? And how much is coming from, let's say, the surgical mix improvement?

Vivek Goyal : Yes. So, I think around 1%, you can assume from the divestments and the other initiatives which we have taken, and the rest is all from the improvement in the occupancy level as well as on the improvement in the speciality mix.

Participant : And sir, if I may know that from when the divestments have been excluded from the financials, is it for the whole quarter or from February?

Vivek Goyal : From February only, from the date when we have divested. I was telling on an annualized basis.

Participant : So, if I compare with, let's say, Q3 previous quarter, 82%, 22%. So, 1% is coming from the divestment and 3% is coming from the occupancy and surgical, is that the right understanding?

Vivek Goyal : Yes. annualized basis, it will be around 5% and 6% because Malar divestment only occurred in February. though Arcot Road has happened in July. So, it is around 5% and 6% in the year, the margin improvement may be attributed towards this divestment. And plus, some cost measures which we have taken on in the financial year. And plus, the facility mix lead to this margin improvement.

Participant : So, I could not understand. If you can please repeat.

Vivek Goyal : Yes, what I said when you're comparing last year with the current year, 5% and 6% may be attributable towards the portfolio rationalizing initiative. And balance is towards the cost optimization, mainly in the manpower cost and some of the procurement related improvement which we have done, formally and procurement related initiative, which the Company has done. And rest is all attributed towards a better case mix and better specialty mix.

Moderator : We'll take the next question from the line of Saion Mukherjee from Nomura Securities. Please go ahead.

Saion Mukherjee : Sir, 1 question on the way you think about ARPOB going forward, which is slowing down from 10% to 5%. Now if you have 2%, 3%, let's say, price growth, is it that the case mix is kind of the change or the favorable change in the case mix is slowing down? And because a lot of the growth and expansion is also likely to come in facilities which have relatively higher ARPOB.

So, I am just wondering why the growth expectations on ARPOB is on a lower side.

Vivek Goyal : Yes, Saion. So, I think the reason for predicting 5% to 6% ARPOB increase compared to double - digit ARPOB increase we are seeing in the last 2 years, the base has already reached a particular

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level. And plus, if you see our specialty mix also, there is a good growth we could have demonstrated in the onco business, which is our high ARPOB business. Having said that, we still feel there will be some further improvement in the ARPOB and 2% to 3% may be attributed towards the price increase. Balance is from the case mix and the specialty mix.

And you rightly mentioned in some of our facilities, we have added good equipment, especially in our Gurgaon facility, we have the Gamma Knife and the MR -Linac, the benefit of that which we'll be getting and that has been factored into this ARPOB increase.

Saion Mukherjee : And when you think slightly medium term, like next 2, 3 years, you think 5%, 6% is what should sustain?

Vivek Goyal : Yes. I will say around 4% to 5% in the medium term.

Saion Mu

herjee : And just 1 question on pricing because this year also, you talked about 2%, 3% kind of a price increase. I think that's similar commentary we hear from most companies. I am just wondering because in the health care system, we have seen like pharma companies or even diagnostic companies take price hikes, which are even higher. Given the nature of the business, where you're sort of providing high -end services. I am just wondering why your price growth is even below the general inflation.

Ashutosh Raghuvanshi : Yes. And that's a million -dollar question actually. You see the problem is that the health care industry remains so much under the glare of media and public interest groups that the industry has to calibrate the price changes very, very carefully. It is true that the inflation rate doesn't

cover the price hikes, which are taken generally. However, that is being covered by building in efficiencies and usual case mix change, et cetera. I wouldn't say that the pace of the tertiary quaternary care coming to the organized sector, I would say, not only us, but the other groups as well, that will continue to happen. As more and more business starts getting organized, the smaller nursing homes and smaller setups will find it difficult with the penetration of insurance to do that kind of administrative activity. But prices will remain under a little bit of pressure simply because of the sentiment around it.

Saion Mukherjee : And just 1 last question, the guidance of improvement in margin, 200 basis points that you have provided, does that factor in the rate revision for CGHS or if that happens, that's over and above any benefit we get there?

Ashutosh Raghuvanshi : No. See that rate revision of CGHS may happen. What of time it may happen, we don't know. So, we haven't really factored that in our calculation.

Moderator : We'll take the next question from the line of Dhawal Khut from Jefferies. Please go ahead.

Dhawal Khut : I just wanted to know what was the branding expense in the quarter? And do we expect some branding expenses to continue in next 2, 3 quarters?

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Anand K. : So, I think you are asking about the branding expense for Agilus, right?

Dhawal Khut : Yes.

Anand K. : We have done about Rs. 31 crores of branding expense last year, and this will continue over this year as well. We will have additional spending on branding this year as well.

Dhawal Khut : What was the expense for the quarter 4.

Anand K. : Rs. 6 crores.

Dhawal Khut : And secondly, my understanding was that Supreme Court had ordered status quo in terms of the promoter shareholding in Fortis. So, for our put option liability, it was answered that you could look at a mix of debt plus equity. So, can that litigation become a hindrance to our ability to raise funds?

Ashutosh Raghuvanshi : Yes. So, Supreme Court stay, the thing that petition was finally dismissed in Supreme Court. So, that doesn't exist anymore. So, we have consulted our regional advisors as well as we have had an informal discussion with some bankers and SEBI as well. And we are pretty confident that there should not be any challenge in that direction.

Moderator : The next question is from the line of Madhav Marda from FIL. Please go ahead.

Madhav Marda : I have 2 questions. The first one was, given that we have a good, interesting Brownfield pipeline which will unfold for us over the next 3, 4 years, could you give us some sense in terms of how much could be the IP volume growth or the occupied beds growth that we could see if you could build in some reasonable occupancy assumptions over the next 3, 4 years. If you could give us some sense in terms of the overall top line guidance, given that, let's say, ARPOB grows at 5%, 6%, how much would the volume of the patient dips add to that?

Vivek Goyal : Yes, we expect around 7% to 8% from the volume side, and it will

be mainly driven by better

occupancy growth as well as this Brownfield expansion kicking in.

Madhav Marda : So, it could be like low to mid-teens kind of revenue growth, basically, right, with volume and ARPOB coming through together for us broadly.

Vivek Goyal : Yes.

Madhav Marda : And my second question was that fingers crossed that the legal case gets resolved in the next few months or quarters, whenever that happens, would that mean that I think we have been incurring a decent amount of legal cost towards this case for some time now. So, could you just help us understand that if this case gets resolved, how much legal cost savings could we have for the Company?

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Vivek Goyal : Yes. So, as this case is finalized, we expect the legal cost should come down, but it will all depend how quickly this would be taken care of. Right now, we are incurring almost Rs. 30 crores to Rs. 50 crores in the legal cost, which is relating to these legacy type of issues. As we are able to resolve all these issues, this cost will definitely go down.

Madhav Marda : Sir, Rs. 30 crores, Rs. 50 crores, just this particular legal case. So, it goes away, that should almost completely go away. That's how we should think?

Vivek Goyal : No, I am saying total legal costs relating to all the legacies, so there is a brand litigation. There is this forensic audit related litigation. Then there is RSC also we are trying to wind up where we are incurring some costs. All this put together I have for this number.

Moderator : The next question is from the line of Atul Minoja from UnitedHealthcare. Please go ahead.

Participant : Hello, Dr. Raghuvanshi. First of all, congratulations for the wonderful quarter. My question is

with respect to inorganic growth, are you looking to acquire some valuable assets in existing clusters? That's my question.

Ashutosh Raghuvanshi : Yes. Yes. Thank you so much. Definitely, we have been looking for assets within the given clusters, 5 defined clusters where we are present. And there are some possible opportunities, we would look for that.

Participant : And any immediate pipeline visibility on that front?

Ashutosh Raghuvanshi : No, nothing we can comment on at the moment.

Moderator : The next question is from the line of Prashant Nair from AMBIT Capital. Please go ahead.

Prashant Nair : Just a couple of questions. The first question is on your current network. I mean, would you still expect some more rationalization of the network or hospitals either being sold or discontinued? Or are you largely done with that?

Ashutosh Raghuvanshi : No, we will definitely consider an option which is value accretive to our Company . But some of the kind of ideas we have around that is that it should be within the given cluster or there should be some attractive reason why we would go to a new geography, then that asset should be of that nature. So, that is why we are not close to going out of our cluster. However, the focus would be to be in the existing clusters so that we can take the advantage of synergies, brand power, cost, et cetera. So, we will continue to do that.

Prashant Nair : And from the current hospitals that you have, are there still any which are not doing as well as you want them to do and where you could look at options similar to what you did with the Chennai hospitals? Or do you think that this set of hospitals you will continue with and work on to improve them.

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Ashutosh Raghuvanshi : Yes. So, there are about 4, 5 hospitals which fall in that bucket. Two of them are strategic in nature, and we believe that we can turn them around. So, there the focus is to do that. But there are certain smaller setups, which we may consider, as Vivek said earlier, they are not very significant in size, but there are a couple of small ones, which we would ce

rtainly be looking to rationalize them in the future.

Prashant Nair : And last question, this is related to the put option on Agilus. So, I mean, you mentioned you have adequate capacity to pay if that put option gets exercised. Would that have any bearing on your bed addition plans, the 2,000 -odd beds that you have mentioned? Or do you think you'll have enough funding to execute on that as well.

Vivek Goyal : No, our Brownfield expansion will not be affected at all if we have to acquire this even through debt and the answer is no, it will not be any impact on our Brownfield expansion if we acquire this.

Moderator : The next question is from the line of Amit Ashok Thawani from Clear Blue Capital Advisors LLP. Please go ahead.

Amit Ashok Thawani : Follow -up question. I am not sure if I missed the answer to this question. I wanted to know what our average realization per test and our average realization per patient has jumped this quarter. Can you explain what has happened exactly in the quarter?

Anand K. : So, see, our average revenue per patient has grown by about 2.7%, and our average realization per test has jumped by about 4.2% in this quarter. So, this is primarily due to 2 reasons. We had a price increase around middle of February, so which has a higher impact on our March numbers, as well as in the last 3 years, same quarter, we had high contribution from our Delhi government project, which has been reduced in this quarter.

Amit Ashok Thawani : But is it fair to say there is some kind of pricing power that is returning to this industry as a whole?

Anand K. : We have taken a price increase very recently. And as we know that most of the diagnostic companies have taken a price increase during this financial year, in the previous financial year. And the online players have all now settled, and they are also taking price increases. So, I think overall, there is no big pressure on prices.

Moderator : Ladies and gentlemen, that was the last question for today. I would now hand the conference over to Mr. Anurag Kalra for closing comments. Over to you, sir.

Anurag Kalra : Thank you, Michelle. Ladies and gentlemen, thank you very much for taking the time to be with us on the call today. I hope we have been able to answer your questions as much as possible. In case of any follow -up questions and data, please feel free to reach out to Amit, my colleague or myself, and we'll be happy to help you. Thank you once again, and have a good day.

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Moderator : Thank you, members of the management. Ladies and gentlemen, on behalf of Fortis Healthcare Limited, that concludes this conference call. We thank you for joining us, and you may now

disconnect your lines. Thank you.

Call: Aug 2024

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August 12 , 202 4

FHL/SEC/202 4-25

The National Stock Exchange of India Ltd.

Scrip Symbol: FORTIS BSE Limited

Scrip Code:532843

Sub: Transcript of Investors / Analysts' meet under Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015.

Dear Madam / Sir,

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed transcript of Investors / Analysts' meet held on August 7, 2024 to discuss the Company's Un -Audited Financial Results for the quarter and period ended on June 30, 2024 and same is available on the Website of the Company at below hyperlink:

Transcript

The date and time of occurrence of event is August 7, 2024 at 1 200 hours.

This is for your kind information and records.

Thanking you,

Yours Faithfully

For Fortis Healthcare Limited

Satyendra Chauhan

Company Secretary & Compliance Officer

ICSI Membership: A14783

Encl.: a/a

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"Fortis Healthcare Limited Q1 FY25 Earnings
Conference Call"

August 07, 2024

MANAGEMENT DR. ASHUTOSH RAGHUVANSHI -- MANAGING DIRECTOR
AND CHIEF EXECUTIVE OFFICER , FORTIS HEALTHCARE
LIMITED

MR. VIVEK GOYAL -- CHIEF FINANCIAL OFFICER , FORTIS
HEALTHCARE LIMITED

MR. ANAND K. -- CHIEF EXECUTIVE OFFICER , AGILUS
DIAGNOSTICS

MR. MANGESH SHIRODKAR -- CHIEF FINANCIAL OFFICER ,
AGILUS DIAGNOSTICS

MR. ANURAG KALRA -- SENIOR VICE PRESIDENT ,
INVESTOR RELATIONS , FORTIS HEALTHCARE LIMITED

MR. AMIT MAHE NDRU -- INVESTOR RELATIONS , FORTIS
HEALTHCARE LIMITED

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Moderator: Ladies and gentlemen, good day, and welcome to Q1 FY '25 Post -results Conference Call of Fortis Healthcare Limited.

As a reminder, all participant lines will be in the listen -only mode, and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing “*” and then “0” on your touch -tone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Anurag Kalra – Senior Vice President, Investor Relations at Fortis Healthcare Limited. Thank you, and over to you, sir.

Anurag Kalra : Thank you very much. A very good morning and good evening, ladies and gentlemen, and welcome to Fortis Healthcare's Quarter 1 FY '25 Earnings Call.

The call is being chaired by our MD and CEO – Dr. Ashutosh Raghuvanshi. With him, we have our Chief Financial Officer – Mr. Vivek Goyal. From the Agilus side, we have Mr. Anand – the CEO; and Mr. Mangesh Shirodkar – the Chief Financial Officer.

We will start with some Opening Comments by Dr. Raghuvanshi on the Results and the Business Performance , followed by which Anand will take you through some of the Key Highlights of the Diagnostics Business , and then we can open the floor for question and answers.

Over to Dr. Raghuvanshi.

Ashutosh Raghuvanshi : Thank you, Anurag. Good morning, and good afternoon, everyone. I hope everyone is doing well, and you have taken time to join us on our Q1 Financial Year '25 Earnings Call today.

I will dive right into the quarterly results and share my thoughts on the Business Performance and our path forward.

Our performance in Q1 Financial Year '25 has been impressive, showing a significant improvement over the previous year. In fact, this achievement has been driven by strong performance of our hospital business . We reported a consolidated top line figure of Rs. 1,859 crores, a growth of 12.2% over Q1 of Financial Year '24. Noticeably, our hospital business revenues have grown 14.4% to Rs. 1,549 crores, while the Diagnostics business revenues stayed on par with previous year at Rs. 343 crores.

Our consolidated operating EBITDA saw an uptick of 25.5%, reaching Rs. 343 crores, yielding a margin of 18.4% versus 16.5% in Q1 of Financial Year '24. The Hospital business operating EBITDA was Rs. 287 crores, resulting in improvement in margins by a strong 330 basis points from 15.2% in Q1 of Financial Year '24 to 18.5% in Q1 of Financial Year '25.

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The Hospital business EBITDA now accounts for 84% of total consolidated EBITDA, up from 75% in Q1 of Financial Year '24, indicating strong profitable growth in this segment. In 8 of our facilities, we have recorded operating EBITDA margin above 20% during the 1st Quarter . These 8 facilities contributed 69% of the hospital revenue compared to 62% in Financial Year '24.

Operating EBITDA margin in the Diagnostics business basis gross revenues were at 16.1% vis -a-vis 19.4% in Q1 of Financial Year '24. The operating margins were 18.7% after factoring the impact of one -offs related primarily to the rebranding exercise undertaken by Agilus for the brand change and the provisions pertaining to certain government business. On a like -for-like basis, the adjusted operating margins were 20.8% in Q1 of Financial Year '24 and 15.9% in Q4 of Financial Year '24.

Our consolidated reported profit after tax for the quarter increased 40.4% to Rs. 174 crores.

Coming to the balance sheet side, we remain quite healthy with a net debt-to-EBITDA of 0.22x as on June 30, 2024, as against 0.35x on June 30, 2023 (basis Q1 annualized EBITDA). Our net debt stands at Rs. 308 crores as on June 30, 2024.

Generally, Q1 is a relatively softer quarter, however, our hospital occupancy improved to 67%

compared to 64% in Q1 of Financial Year '24. This translated into occupied beds increasing by 4.6% to 2,715 compared to 2,595 in Q1 of Financial Year '24.

Our Hospital Business saw a 9.7% increase in ARPOB, reaching Rs. 2.41 crores per annum. This growth was largely fueled by revenue gains in our key specialty areas: oncology, Neuro Sciences, Cardiac Sciences, Orthopedics, Gastroenterology, and Renal Sciences. Collectively, these specialties achieved a 15.7% year-on-year growth and contributed 63% of the total hospital revenues, which is on the similar lines as Q1 of Financial Year '24. Specifically, growth in Neuro Sciences at 23% and Oncology at 22% was quite encouraging. This is also reflected in the number of procedures performed in Neuro Sciences that reported a growth of 23% compared to the corresponding previous period, while the number of robotic surgeries performed increased by a strong 59% compared to Q1 of Financial Year '24.

Our revenues from medical travel grew 11% compared to Quarter 1 of Financial Year '24 to reach Rs. 127 crores. Revenue contribution of international business stood at approximately 8% in Q1 of Financial Year '25, similar to Q1 of Financial Year '24.

Most of our key facilities performed well during the quarter with revenues from Mulund, Anandpur, BG Road, Shalimar Bagh, and Amritsar, each growing in excess of 20% compared to the corresponding previous period. During the quarter, we further strengthened our medical talent with onboarding of specialists in the area of Cardiac Sciences, Neurology, Orthopedics, Obstetrics and Gynecology, and Ophthalmology.

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I am excited to share that our commitment towards augmenting our medical programs has made notable strides in advancing our strategic growth initiatives, marked by brownfield bed expansion and continuous investment in the state-of-the-art medical equipment. Recently, Fortis Nagarbhavi, Bengaluru, that was previously a 50-bedded facility, expanded to 80-bed multi-specialty tertiary care facility in a significant upgrade that included introduction of 24/7 emergency and trauma care, ICU and Critical Care, Heart & Vascular Centre and Fortis Cancer Institute. I am pleased to announce that FMRI Gurgaon introduced South Asia's first Gamma Knife Esprit radiosurgery equipment for neurosurgical treatment that employs non-surgical computer-guided precision to target brain tumors, both malignant and benign.

We are progressing well on our brownfield expansion planned for this year, and we will be adding capacities across our key facilities, including Faridabad, Anandpur, Shalimar Bagh and Noida. In addition, we expect the 350-bed Manesar facility that we acquired in Financial Year '24 to be operational in the ongoing quarter.

I would like to touch upon our continued efforts and the resulting success of our digitization efforts as well. We have successfully implemented Electronic Medical Records for OP modules across 6 of our units now while the application development for IP module is underway.

Revenues from digital channels continue to demonstrate robust growth. Revenues from website, mobile applications, and digital campaigns witnessed a 52% year-over-year growth and 17% quarter-on-quarter growth in Q1 of Financial Year '25. Revenues from digital channels contributed almost 30% to overall hospital revenues versus 23% in Q1 of Financial Year '24.

On our diagnostics side, the business reported gross revenues of Rs. 343.5 crores versus Rs. 342.7 crores in Q1 of Financial Year '24, and Rs. 338.4 crores in Q4 of Financial Year '24. Our operating EBITDA margins (basis gross revenues) were at 16.1% compared to 19.4% in Q1 of Financial Year '24. Operating EBITDA margins (basis gross revenues) in Q1 Financial Year '25 were better than the trailing quarter margins of 14%, primarily driven by cost optimization

on

initiatives including, amongst others, improved network efficiency and reduction in manpower cost.

Continuing with our network expansion strategy, primarily the addition of new customer touch points (CTPs), total CTPs as on 30th June '24 stood at 4,055. The preventive portfolio revenue in Agilus' overall revenues grew 13% in Q1 Financial Year '25 and contributed 12% to the operating revenues versus 10% in the Q1 of Financial Year '24.

The Diagnostics business performance is still adjusting to the impact of Agilus rebranding exercise, which involved extensive rebranding efforts and associated marketing costs.

Additionally, certain provisions related to government business have also affected its performance.

That said, I am confident in Agilus' potential to scale up both in terms of its revenue and margins

based on its considerable network presence, a balanced B2C, B2B mix, and the increased focus
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on preventive care and specialized testing. To also add, I do believe that the Agilus brand is being well accepted and gaining recognition. This would place the company in a better position to further scale our performance.

With this, I will conclude my comments. I think it's been a healthy start for the fiscal year 2025. We are making significant progress on the growth at all possible fronts. We will also continue to explore and assess various growth opportunities that align with our cluster strategy and offer promising synergies. I believe these initiatives will further enhance our growth potential and strengthen our position in the health care sector.

Thank you, and I will hand over to Mr. Anand for his comments on the diagnostics business now.

Anand K. : Thank you, Dr. Raghuvanshi. A very good morning and good afternoon to everyone on the call. Thank you for joining us today. On behalf of Agilus Diagnostics, I warmly welcome you all to our Q1 FY '25 Results Conference Call .

Agilus Diagnostics reported a revenue of Rs. 343.5 crores in Q1 of FY '25 compared to Rs. 342.7 crores in Q1 of FY '24. Q4 of FY '24 revenue stood at Rs. 338.4 crores. During this quarter, Agilus conducted 9.92 million tests compared to 9.95 million tests in Q1 of FY '24, and 9.61 million tests in Q4 of FY '24. Operating EBITDA for the quarter stands at Rs. 55 crores compared to Rs. 66 crores in Q1 of FY '24 (16.1% versus 19.4% EBITDA margin) , and Rs. 47 crores in Q4 of FY '24. Operating EBITDA before one -off expenses is Rs. 64 crores compared to Rs. 71 crores in Q1 of FY '24 (18.7% versus 20.8% EBITDA margin) , and Rs. 54 crores in Q4 of FY '24 (15.9% margin). During the quarter, we incurred onetime expenses of around Rs. 9 crores, primarily in relation to Agilus' rebranding and the provisioning pertaining to certain government contracts.

Our average realization per test for Q1 of FY '25 is Rs. 340 and per patient revenue is Rs. 845. The business continues to benefit from a well -diversified geographical mix with no overdependence on any particular region, allowing us to capitalize on our pan-India network optimally.

Regional revenue contributions are 32% for North, 21% from West, 30% from South and 14% from East, and 3% from international markets. Our B2C:B2B ratio is currently at 54:46. Our wellness portfolio increased by 13% in Q1 of FY '25 compared to Q1 of FY '24, and 3% compared to Q4 of FY '24. Our genomics portfolio also showed strong growth in Q1 of FY '25 with 12% growth compared to the previous year and 13% growth compared to the previous quarter. From a product standpoint, revenue contributions of 34% from specialized testing, 54% from routine tests, and 12% from our wellness portfolio. During this quarter, we expanded our network by adding over 185 new centers and 3 hospital labs.

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Post a successful brand transition in the previous year, we are s

ustaining our brand awareness

campaigns through digital platforms and various other ATL models. We aim to enhance our range within the medical community through a range of initiatives, including continuing medical education programs, sponsored events, in-clinic visual merchandise and scientific literature.

Continuing our partnership with the Indian Olympic Association, Agilus Diagnostics is the official lab partner for the ongoing Paris Olympics 2024. On the clinical side, we continue to invest in new tests and technologies with a special focus on genomics. We are witnessing consistent healthy growth in this segment, and one of our goals is to continually build on this portfolio. As an organization, we are consolidating to boost efficiency, strategically focusing on key cities to establish leadership, and enhancing the customer experience to build loyalty to the new brand.

Thank you, and over to you, Anurag.

Anurag Kalra : Thank you, Anand. Ladies and gentlemen, we shall now open the floor for questions and answers. May I request the moderator to begin, please?

Moderator : Thank you. We will now begin the question -and-answer session. The first question is from the line of Hardik Doshi from White Whale Partners. Please go ahead.

Hardik Doshi : If I see your presentation, our overall occupancy on a sequential basis has improved from 66% to 67%. Our ARPOB also on a sequential basis has improved quite healthily. But even then, when I look at the operating EBITDA margin, we are down from 22.4% to 18.5%. I understand there were some one -offs in Q4, but even adjusted for that, I think Q4 margins were 21%. It's still a pretty meaningful decline. So, any kind of one -offs in Q1 or any investments that we

made? What was the reason for this dip in margins?

Vivek Goyal : Yes. If I can take this question. Vivek this side. Yes, you are absolutely right, there are certain one-offs in the quarter 4. And if you adjust that one-off, it is around 21% EBITDA margin. So, there are a couple of reasons for drop in the EBITDA margin. One is, of course, there is certain one-off in the current quarter also. So, there is around 0.5% EBITDA improvement, I am talking Hospital business alone, which 0.5% cost has been absorbed in this 18.5%, which is nonoperational in nature. One of the costs is we have to increase the price capital for converting some of our debt in one of our subsidiary companies. Then there is certain GST-related liability on some corporate guarantee earlier company has given. And there is a higher provision for bad and doubtful debt because of the lower collection in this 1st Quarter. Typically, 1st Quarter, the collection is lower and as a result the provision for bad and doubtful debt goes up. So, this takes care of 0.6% of the EBITDA margin thing. So, the balance 2% EBITDA margin drop is attributed mainly to the specialty mix. In this quarter, as Dr. Raghuvanshi mentioned earlier, we did a higher share of ortho and onco, and the surgical mix has also gone up, where the gross contribution margin is lower and the doctor share is generally on the higher side. So, these are the main reason for the EBITDA drop.

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Hardik Doshi : So, then how does this kind of change your outlook for the full year margin that you have guided to?

Vivek Goyal : So, 1st Quarter, we have targeted around 19% EBITDA margin, and we are around that level. So, we are well on track for our EBITDA margin improvement guidance, which we have given earlier.

Hardik Doshi : The second question, if I look at your hospital margin metrics for full year FY '24, I think there were 5 hospitals that were less than 10% margin, which account 28% of revenues, but now that is 7 hospitals now accounting for 17.5%. So, which were these 2 kind of facilities where margins might have kind of fallen? And again, is this transitory? Or I mean, are there any issues that

you're facing here?

Vivek Goyal : So, one is, of course, the FEHI Hospital, where this quarter the EBITDA margin is lower than the previous quarter, and that has fallen to the lower category. And another hospital which is in this category actually is CH Road, which is a smaller hospital in Bangalore. And there, again, because these are mainly heart-related institutes. And the 1st Quarter's generally include the increment-related cost and this hospital unable to pass on those costs in terms of price increase. And that is the reason the EBITDA margin is lower in these 2 hospitals. But we are taking other initiatives through which we will be able to bounce both these hospitals back. Other hospitals have performed relatively okay.

Hardik Doshi : And then just moving on to Agilus. I think we had gotten an extension from our private equity players, where maybe I think August was the deadline to kind of come up with some resolution or they have the put option. Any update in terms of the IPO plans or sale or anything to provide an update there?

Vivek Goyal : Yes. So, you are right, there was a deadline type of date of 13th August. So, we have already started the process of going ahead as per agreement, because IPO process is not looking like we will be able to start immediately. And in that backdrop, we have appointed 1 valuer for doing the put option valuation as per the agreement, and we got the valuation now. And I think we are at the final stage of getting maybe notice from the private equity investors for exercising the put. So, we are at that juncture at present.

Hardik Doshi : And so then if you have gotten the input from the valuer, I mean, what would be the cash that we'd have to shell out to buy out the stake?

Vivek Goyal : Yes. So, valuation of 31% stake of private equity is coming around Rs. 1,780 crores as per this independent valuer. So, we have committed the facilities available to fund this type of accretion. So, from the funding perspective, it will not be a challenge. From the leverage perspective also, we are still in a very comfortable position. Our debt/EBITDA will be around 1.5x post taking this additional debt for this put option. And we will continue to do our CAPEX as per our original plan, and we will continue to look for new accretion opportunities as per our original plan. So, Fortis Healthcare Limited
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this will not hamper our growth prospects in the Hospital business. This put option, we feel this may be the right approach right now to acquire the stake and then work on Agilus to improve the profitability of Agilus and look at the other alternative for Agilus maybe at a future point of time.

Moderator : The next question is from the line of Neha Manpuria from Bank of America. Please go ahead.

Neha Manpuria : Sir, what would be the timeline for completion of the acquisition of the PVP stake in Agilus? Would this be done this year? Or this could require some approvals , etc. ?

Vivek Goyal : Yes. So, Neha, the timeline, as I mentioned, is by 13th August, we are expecting put option notice from the PE investor. Post serving of the notice, there is a 65 days' time to purchase condition . So, during this period, we will be seeking shareholder approval, because we may be raising debt in the form of FPI, which is listed debt. So, there will be requirement to take the shareholder approval for that. And plus, some sort of SPA we have to enter with the private equity investor, share purchase agreement basically. So, for that also, we will be requiring shareholders' approval. We are checking with our lawyer. There may be requirement of CCI approval also for acquiring this stake. So, that we will be taking. So, all those formalities need to be completed within this 60 -day period.

Neha Manpuria : And the remaining stake in Agilus, what about that?

Vivek Goyal : So, there is no immediate plan for that r

emaining stake. The other shareholder will remain like minority shareholder there. We will hold around 88% plus shareholding in Agilus post this acquisition.

Neha Manpuria : And just continuing on Agilus. I know we have been commenting on growth trends improving, but we aren't really seeing anything or at least on the volume side in terms of growth coming back. What gives us the confidence of growth? When should we start seeing that in numbers? And what are our internal guidelines when we think about revenue growth of this business? Historically, we have indicated in line with the industry, but when do we eventually get there?

Anand K. : So, Neha, this is Anand here. So, after the brand transition, we have seen some degrowth, so to say, in some segments, especially those segments that are directly related to the customer revenues in the B2C segment. So, those segments, we are now catching up after we have started spending on the brand, and we have started doing the process of rebranding from the middle of previous year. So, from that time, we have seen whatever degrowth we had in those segments, they are all coming back to normal. So, we are expecting that through this year, we will be able to consolidate and get back on track in terms of our growth expectations.

Neha Manpuria : Sorry, sir, I missed the last part. You said consolidate this year and?

Anand K. : And we expect to be back on track in terms of growth expectations.

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Neha Manpuria : So, probably get to industry growth next year?

Anand K. : Yes.

Neha Manpuria : And last on the high court litigation. I think we had the hearing last month. Any incremental update post the hearing?

Ashutosh Raghuvanshi : Yes. No, the hearing, I think, has been completed as far as the bank, Fortis , etc. , are concerned. We believe that the next hearing is due where the Religare would be heard. That, in our belief, is the last party to be heard. So, I would say that it appears that at least the hearings are likely to conclude very soon.

Moderator : The next question is from the line of Amey from JM Financial. Please go ahead.

Amey Chalke : So, I have first question on the Agilus. So, there has been underperformance in this segment for the last few quarters. So, is this underperformance across the region? Or do you see any specific region where the decline has been substantial, which is dragging down the overall performance?

Anand K. : See, we did the brand change around end of 1st Quarter last year. So, the maximum impact will be seen in this particular quarter. But as we saw that the trailing quarters, the Q2, Q3 and Q4 of last year also were impacted by the brand change, and we were seeing some drop in the business.

And as I told earlier, the drop has been primarily on the B2C side of the business, which is directly impacted by the brand change. And those segments, we are now getting them back on track since we have started spending more on the brand and so creating more brand awareness through various activities. So, that's how we are bringing it back on track.

Amey Chalke : When we see the B2C business, I believe it will be still a prescription generation for the diagnostic tests will be still through doctors, right? So, it is basically the doctors who might not be aware about the new brand name basically. So, the activity would be basically our sales force going to the doctors and making them aware, right, about the new brand. Is it how it will work? Or what else would we need to understand on this?

Anand K. : So, in our B2C business, about 60% is driven by doctors, 40% is driven by self -prescription. So, the patients come by themselves. So, the prescription part is not difficult, because all the doctors are now aware of this change and that is not an issue. The issue is more about patients who are walking into our centers. So, as you know, mostly these diagnos

tic spends are out -of-pocket,

and even though the doctors direct them, mostly the patients have their own choice as well in terms of where they go. So, that's where we have been impacted to some extent in that business.

So, that we are bringing it back online.

Amey Chalke : So, second question I have in the hospitals, where we are seeing a good sequential growth for FMRI and Mohali. If you can highlight what are the drivers for growth for these 2 hospitals and outlook for these 2 hospitals for the full year.

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Vivek Goyal : So, there was growth in these hospitals. Mohali, we have added around 30 beds last year. So, those beds, we were able to fill, and there is some growth in this ortho business also there. So, their occupancy level has gone up basically with the enhanced bed capacity. And in FMRI actually, we are full in terms of occupancy. We are operating at 75% plus occupancy in FMRI. And we have added certain new clinical talent, especially in onco and cardi ac side. So, that has led to filling up the beds faster. So, this hospital is now operating at 75% plus occupancy level, and we are trying to expedite our expansion program to the extent possible, so that we may accommodate more patients there. So, these are the 2 main reasons for higher profitability in this.

Amey Chalke : So, Mohali might see continuous increase basically. But FMRI, till the expansion, the increase would be driven by largely the case mix and the price rise basically.

Ashutosh Raghuvanshi : Yes, there are 2 phases of expansion in FMRI, which is one is the brownfield expansion, which we have been talking about for quite some time. So, there, the physical construction is actually happening and the civil part should be over by the end of this year. And then in the next 6 months, we should commission those beds. So, obviously, that will give the major flip. But in the interim, within the existing structure, we are adding few beds. So, we will be getting about 21 beds in the interim, which we should hopefully get by the month of November. So, that would be another kicker as well.

Amey Chalke : And the last question I have is in general. Basically, when the hospital chain is as large as Fortis with more than 25 hospitals in the portfolio, there will be some or other assets where the patient dissatisfaction can increase because of several issues like the pricing or things not going well.

So, how does the management at the top would be keeping aware of these things? And what steps one can take to correct the recourse in such facilities? And in terms of our experience, have we experienced such things? And what steps we have taken and how has been the outcome?

Ashutosh Raghuvanshi : Yes. Like any other consumer industry which is spread across multiple locations, service industry, we would definitely be having the situations where there would be either complaints in the service or there could be issues regarding the billing, etc., as you mentioned. So, we have a very robust system in place. There are 2 things which we monitor, which is the NPS, which we monitor on a regular basis, which is given by the patients once they are discharged from the hospital. That gives us a general pulse of what is happening around the network. But we also have an internal system of monitoring events wherever there is any grievance in a hospital, etc. And there are many times when the patients directly connect with the corporate patient experience team and sometimes directly with our features, including me. So, that's how it gets addressed. So, there is a mechanism at the unit level, and then there is a mechanism at the corporate level to deal with these kind of issues and problems so that we can address them with some speed.

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Amey Chalke : And how did we benchmark set in terms of every operationa

l aspect of the hospital, let's say, cleaning or, let's say, outcome of some cases, etc. Are there any benchmarks set so that the service is at par across the hospitals, basically?

Ashutosh Raghuvanshi : Yes. So, as far as clinical governance is concerned, we have a very robust mechanism. Every hospital is NABH accredited. And because of that, there are certain parameters of outcomes which have to be reported. So, we collect those. And our central medical services and operations group has a separate quality division which looks at those. We also have additionally started reporting some of the outcomes in major cardiac procedures, etc., which would be available on our website. But we are in the process of sort of making this system even more robust, so that we can do an open reporting, to which patients have access as well in terms of outcome. As far as the infrastructure-related monitoring is concerned of, say, the cleanliness, security level, and all other aspects, we have a separate internal mechanism by which we monitor. So, we have a central team which does an audit every quarter of each and every unit.

Moderator : The next question is from the line of Bino Pathiparampil from Elara Capital. Please go ahead.

Bino Pathiparampil : I had a doubt on the Agilus EBITDA margin numbers. The reported EBITDA is Rs. 55 crores. And in your footnotes, you have mentioned that you have taken a Rs. 19 crore write-off related to Mohalla Clinic. So, if I add that, it becomes Rs. 74 crores EBITDA, but still you say that

excluding one-offs, the EBITDA was only Rs. 64 crores. What am I missing here?

Anand K. : No. So, Bino, operating EBITDA for the Diagnostics business is Rs. 64 crores after accounting for that Rs. 8.8 crores. So, if you add that back, then it becomes an operating EBITDA of about 18.7%, which is there in Slide 16 of our presentation. This Rs. 8.8 crores is primarily rebranding expenses and some other additional provisions for write-off of the Mohalla Clinic business.

Bino Pathiparampil : Sorry, sorry. I think you didn't get my question. In the footnotes to your financial statements, it is said that in this quarter, Rs. 19 crores of write-off was taken related to Mohalla Clinic. Am I correct there?

Vivek Goyal : No. The Rs. 19 crores is the total provision till date of Mohalla Clinic since the beginning. For this quarter, we had taken around Rs. 2.5 crores of provision for the Mohalla Clinic.

Bino Pathiparampil : I think the wording is a little confusing there.

Moderator : The next question is the line of Nancy Yadav from Allegro Capital Advisors. Please go ahead.

Nancy Yadav : Could you please shed some light on the IndAS -116 impact on our numbers for this quarter?

Vivek Goyal : There is hardly any IndAS impact on the Hospital business. Mangesh is just pulling out the numbers for the diagnostics business.

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Nancy Yadav : Sorry, sir, I didn't get you.

Vivek Goyal : So, what I am saying, in Hospital business, there is hardly any impact of the IndAS -116. Diagnostics business, we have ascertained these arrangements. For that, Mangesh is just pulling the numbers. Just wait for a minute. Or we can send you separately if it is okay with you.

Anand K. : So, Nancy, we can share offline those numbers with you related to the Diagnostics business.

Moderator : The next question is from the line of Saurabh Kapadia from Sundaram Mutual Funds. Please go ahead.

Saurabh Kapadia : If I look at the payer mix, there has been increase in the scheme patients, both CGHS and ECHS. So, how should we look at the mix of the scheme patients going ahead with no beds getting added over the next 6 to 9 months?

Vivek Goyal : Yes. So, there is a little bit increase in the payer mix in some of our hospitals, and as a result, overall business also, there is an increase in the scheme business. Although we have targeted, let's

say, a slight reduction in the scheme business. So, we are working on these particular metrics. And hopefully, we will be able to correct it in the coming quarters, and that should lead to some improvement in the profitability margins.

Saurabh Kapadia : But new beds addition will not lead to higher scheme patients?

Vivek Goyal : Yes. So, new bed addition is coming in Faridabad and Manesar also. So, there, of course, there will be some impact, but wherever possible, like in NCR, most of our hospitals, we are operating at a decent occupancy levels. So, there we may try to correct these metrics a little bit better.

Saurabh Kapadia : And do you see any impact on the international patients because of the Bangladesh situation?

Vivek Goyal : So, there is some impact because of the recent development in Bangladesh as well as Israel and other countries as fight is going on. But I think the impact may not be material looking at our patient flow from these countries.

Moderator : The next question is from the line of Abdulkader Puranwala from ICICI Securities. Please go ahead.

Abdulkader Puranwala : Sir, my first question is with regards to the Manesar operating cost. So, the 18% margin what you have on the Hospital business, was there an impact of the clinical talent cost and other overheads of the Manesar facility? Or you are currently capitalizing that cost?

Vivek Goyal : So, Manesar has yet to be operationalized. So, till operationalization, whatever expenditure we are incurring, whether it is the administrative cost, administrative staff, plus some clinical talent

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we have added, so that cost is right now getting capitalized. But as Dr. Raghuvanshi said, we are in the final stage of opening that hospital. Post that it will be coming as a loss to the business.

Abdulkader Puranwala : And with that operational cost, you're guiding for close to 19% margin for the full year?

Vivek Goyal : Not really. You are talking about consolidated margin of Hospital business?

Abdulkader Puranwala : Yes, sir.

Vivek Goyal : Yes. So, consolidated margin for the full Hospital business, we have guided 20% EBITDA margin, and we are well on track on that.

Abdulkader Puranwala : Sir, my second question was pertaining to the promotion or the marketing on the rebranding side of Agilus. Sir, I mean, how should we see this cost in this particular fiscal, because I think compared to last year, it's gone up a bit, but for the full year, any color on how the margins on Agilus would look like?

Anand K. : So, we have planned to spend around Rs. 50 crores this year for the rebranding expenses. So,

that will be the one -off cost for this year. So, apart from that, we will be doing our regular other marketing and regular activities as well.

Moderator : The next question is from the line of Adrit Chaturvedi from Nomura India. Please go ahead.

Adrit Chaturvedi : I would just like to understand the revenue mix on Agilus. So, you have (inaudible) roughly flat. So, I believe that tests per patient are sort of rising. So, less patients are coming, you're just selling more tests to them. And when I view that in the revenue mix, where wellness testing, that's essentially bundled, that's gone up 2% share. And I believe that, that's translating to a lesser realization per test, because you're charging lesser for the individual test in the bundle than you would if you would sell them separately. So, since there is sort of a drive to increase wellness share, shall we come to expect that as you grow this product mix, realizations per test would sort of not increase? Or would you look to offset that by more specialized testing?

Anand K. : No. Actually, see what is happening is, currently, when we want to promote directly to patients, so we opt for wellness as a source of promoting directly to the patients. When we do that, we also offer additional discount

ts. So, during these times, we do some campaigns. In the last quarter as well as this quarter, we are doing some campaigns around discounted packages. So, naturally, that's having an impact. But those discounts will not be available always. So, that will not happen overall as we move forward on your average realization per test.

Adrit Chaturvedi : So, like the targeted increase will not have an impact. And so the specialized bit, so we have not been able to increase a lot of the share there. However, that is a priority. So, could we come to expect that as wellness share increases and you reduce the discounts there and any ancillary

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products and specialized keeps increasing, could there be like a guidance on how realizations per test would move? Because you're suggesting that they would significantly improve.

Anand K. : Yes. If you see, even on the specialized side, we are focused on next -generation diagnostics. In genomics portfolio, we are seeing a higher growth rate. So, that is also we're focusing on. That will also help us build, as we tie up with more hospitals, more specialists, as we work with oncologists and all the new -age specialties, our contribution from this part of the revenue will also move up.

Adrit Chaturvedi : Sir, very quickly on that. So, I believe most of the specialized is going to be driven by the pickup business, the B2B business going forward?

Anand K. : Specialized tests are usually driven by hospitals and other segments that outsource tests to us. So, it will be a pickup business, yes.

Adrit Chaturvedi : And just on the Hospital business. So, you have seen even a sequential increase in occupied beds and occupancy, both. And generally, Q4 is very strong for occupancies. And it was also earlier like referenced in the call that the institutional share has sort of increased. So, a lot of the footfall has actually come on the institutional bed side. So, I am just wondering that this kind of a strategy to drive up occupancies and then to drive up these OBDs, you sort of increased the institutional bed share. How is that going to affect EBITDA per bed? Because realizations are going to be lower and margins are going to be lower. So, is that a decent strategy when you have 2 options where you can increase occupancy and sort of have a lower EBITDA per bed, or you can just keep occupancies lower and have a higher EBITDA per bed. Is there like a strategic direction there?

Vivek Goyal : So, if I can answer this question differently. First of all, the payer mix has not deteriorated that much. If you see, it is almost like flat, little bit increase in the scheme business by 0.5%. And that is true when you are having expansion drive, like brownfield expansion, as you know, we are having a lot of brownfield expansion. And on the marginal profitability point of view, we know this incremental occupancy, even if it is coming from the government payer, it is giving us decent EBITDA margin, because our fixed cost is fixed. So, fixed cost is a portion on the higher base. Having said that, to reduce these government payers, that's impossible. As I mentioned, it is hospital -specific strategy. Wherever we are having higher occupancy and we are facing challenge of filling the beds, we try to optimize there. But where we have a lower occupancy, there, our endeavor is to fill the bed first and then look for this share and things. I hope I was able to answer your question.

Adrit Chaturvedi : Yes, that's what I was trying to understand that in a lower occupancy cluster of your hospitals, EBITDA per bed is not really a priority. It's more of like growth and revenue. And in your higher hospitals, you're sort of driving EBITDA per bed.

Moderator : The next question is from the line of Prashant Nair from AMBIT Capital. Please go ahead.

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Prashant

t Nair: So, my first question is on the Manesar project. I mean, how many beds would you be operationalizing at the start? And can you just outline how this could evolve over the next, say, 2 to 3 years? By when do you see maximum beds being operationalized here?

Vivek Goyal : So, Manesar, the facilities have a total capacity of around 350 beds. So, we will be initially operationalizing around 100 beds. And once we reach a particular occupancy level of 100 beds, say, 60%, 65% occupancy level, then we will open up the balance beds. So, it will be gradual. And in our estimate, if things go as per our plan, we will be able to operationalize all 350 beds in, say, 18 months' period of time from now.

Prashant Nair: All right. And second question, again, related to Manesar. So, from a pricing perspective, or from an ARPOB perspective, I mean, how can we benchmark this with, say, your FMRI? How much lower could it be versus that hospital when it starts? And then over a period of time, will it narrow, or will it remain a slightly lower ARPOB hospital compared to FMRI?

Vivek Goyal : Yes, so FMRI is our flagship and premium hospital, where we are not having any scheme business. A lot of international patients are there. And the type of work we are doing there in terms of the clinical work we are doing, that is a high -end work. So, with that, I think the ARPOB of FMRI is the highest in our network actually. So, we are not expecting that type of ARPOB.

The ARPOB will be maybe 25%, 30% lower than FMRI.

Prashant Nair: Okay. And when it starts, at the time of its commissioning? Or is it what it will grow to over a period of time, the 25%, 30% lower than FMRI?

Vivek Goyal : Yes, this is at the start, we are saying, and we will see how things pan out, and then accordingly, we will take a call on this.

Moderator : The next question is from the line of Aashita Jain from Nuvama Institutional Equities. Please go ahead.

Aashita Jain : My first question is on the diagnostics business. Sir, have we taken any price hikes in the diagnostics business in the last 6 to 8 months? And what's the impact of it, if we have taken any?

Anand K. : So, in fact, we have taken a small price increase around the end of Q4 of last year. That's only on the B2C business, on the walk -in business, and also only in some of the segments that we have taken. So, overall, the impact is not very significant, as we also do some discounting on that business at this point of time for brand promotion. As such, if you see overall, compared to Q4, we have an improved performance in Q1.

Aashita Jain : And my second question is on the hospital margin metrics. So, now we are selling hospitals which are below 10% margin mark. Which are these hospitals, and do we have plans to divest any of these? Also, you talked about some other initiatives to turn around these facilities. So, could you please elaborate on the same?

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Vivek Goyal : Yes. So, these metrics we are sharing for quite some time. So, there are usual candidates here.

One is FEHI, Jaipur, and Vashi. These are the 3 big hospitals which are part of this bucket. And we are making a lot of efforts to improve this. FEHI, I have made a comment earlier, because of the annual increment thing, they have not been able to absorb, and we are working on the cost side of FEHI, and also improving the revenue mix there. Because right now it is majorly a cardiac center. So, we are trying to add other facility. We are adding the clinical talent and other things there. We are working on the infrastructure improvement also there. We are spending a lot of money on the CAPEX on this hospital. So, hopefully, the things will improve in FEHI with all these efforts. This quarter, it has come down from 10% EBITDA margin able to achieve last quarter. Jaipur, there are other issues which we are grappling with. And as a result, our revenue

has also fallen, and EBITDA margin, it has actually gone into negative territory. Vashi is grappling with competition around that area and the state of the facility is not that great. And we are making a lot of effort, but Vashi, as we said in the earlier call also, we are not able to crack anything there right now, and we are working to improve margins through increasing the occupancy by whatever means we can increase, through the marketing, sales, etc., and things like that.

We have added one more hospital here in Ludhiana too which is a new hospital, we have added 6 months back. So, it is early days for this hospital. Although I must admit this hospital has not performed the way we wanted to perform in the 6 months, but we are working on this.

Moderator : The next question is from the line of Atul, an individual investor. Please go ahead.

Atul : My question is what has been the occupancy level so far since August? July has already passed and we are like 7th day of August, so if you can answer that/

Vivek Goyal : No, we will not be able to give you specific numbers, because it is price sensitive. But directionally, it is better than the last quarter.

Atul : And any active inorganic opportunities which are in pipeline?

Vivek Goyal : So, there are a lot of opportunities we are exploring right now, but these things have not reached a stage which we can disclose. So, we are working on that and as and when we reach a particular stage, we will definitely come back to the market.

Atul : And since all the hearings in the courts are done, are you anticipating the legal costs to come down in the next quarter?

Vivek Goyal : Not really, because just the hearing is completed. I don't know what will be the reaction of Daiichi, whether they will appeal in the higher court and things like that. And we have not come out from the woods completely. We have yet to receive the order. So, all those things will determine the legal cost. So, my estimate is legal cost will continue to be high in this year. And
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this has already been budgeted and accounted for when I am giving various guidance on the EBITDA margin front.

Moderator : Ladies and gentlemen, that was the last question for today. We have reached the end of our question -and-answer session. I would now like to hand the conference over to Mr. Anurag Kalra for closing comments.

Anurag Kalra : Ladies and gentlemen, thank you very much for taking the time to join us today. In case there are further clarifications or questions, my colleague, Amit, and I are available to address those either telephonically or by e-mail. Thank you very much once again and have a good day.

Moderator : Thank you. On behalf of Fortis Healthcare, that concludes this conference. Thank you for joining us. You may now disconnect your lines.

Call: Nov 2024

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November 13 , 2024

FHL/SEC/2024-25

The National Stock Exchange of India Ltd.

Scrip Symbol: FORTIS BSE Limited

Scrip Code:532843

Sub: Transcript of Investors / Analysts' meet under Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015.

Dear Madam / Sir,

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed transcript of Investors / Analysts' meet held on November 8 , 2024 to discuss the Company's Un-Audited Financial Results for the quarter and half-year ended on September 30, 2024 and same is available on the Website of the Company at below hyperlink :

Earnings Call Transcript

The date and time of occurrence of event is November 8, 2024 at 1830 hours.

This is for your kind information and records.

Thanking you,

Yours Faithfully

For Fortis Healthcare Limited

Satyendra Chauhan

Company Secretary & Compliance Officer

ICSI Membership: A14783

Encl.: a/a

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"Fortis Healthcare Limited"

Q2 FY '25 Post Results Conference Call"

November 08, 2024

MANAGEMENT : DR ASHUTOSH RAGHUVANSHI – MANAGING
DIRECTOR AND CHIEF EXECUTIVE OFFICER – FORTIS
HEALTHCARE LIMITED

MR. VIVEK GOYAL – CHIEF FINANCIAL OFFICER –
FORTIS HEALTHCARE LIMITED

MR. ANURAG KALRA – SENIOR VICE PRESIDENT ,
INVESTOR RELATIONS – FORTIS HEALTHCARE
LIMITED

MR. ANAND KUPPUSWAMY – CHIEF EXECUTIVE
OFFICER – AGILUS DIAGNOSTICS

MR. AKSHAY TIWARI – CHIEF FINANCIAL OFFICER –
AGILUS DIAGNOSTICS

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Moderator: Ladies and gentlemen, good day, and welcome to the Q2 FY '25 Post Results Conference Call of Fortis Healthcare Limited. As a reminder, all participant lines will be in the listen -only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during this conference call, please signal an operator by pressing star then zero on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Anurag Kalra, Senior Vice President, Investor Relations at Fortis Healthcare Limited. Thank you, and over to you, sir.

Anurag Kalra: Thank you, Nirav. A very good evening, ladies and gentlemen, and thank you for taking the time to join us on our quarter Q2 FY '25 Earnings Call. We realize it's a bit late, so we do really appreciate you joining us for the call. The call is being chaired by Dr. Ashutosh Raghuvanshi, our CEO and Managing Director. With him, we have Mr. Vivek Goyal, our Chief Financial Officer. Mr. Anand, the CEO of Agilus Diagnostics, joins us remotely. And along with him, we have Mr. Akshay Tiwari, the CFO of Agilus.

We will begin with some opening comments by Dr. Raghuvanshi on the business performance of the quarter gone by, post which Anand will take you through his views on the business for the quarter, and then we can open the floor for question and answers. Over to Dr. Raghuvanshi.

Ashutosh Raghuvanshi: Thank you, Anurag. Good evening, everyone, and thank you for taking time to join us for our Q2 financial year '25 Earnings Call today. I extend my warm greetings for festival season, and I hope all of you are doing well.

Before diving right into the numbers and sharing my thoughts on the business performance, I would like to highlight that our performance in Q2 financial year '25 and H1 financial year '25 continues to witness a healthy improvement over the corresponding previous year, led largely by our hospital business.

Coming to the quarterly results, we reported a consolidated top line of INR1,988 crores, a growth of 12.3% over the Q2 of financial year '24. Our hospital business revenues have increased by 13.9% to INR1,655 crores, while the diagnostic business revenue stood at INR372 crores versus INR360 crores in Q2 of financial year '24.

Our consolidated operating EBITDA increased 31.9% to INR435 crores, delivering a margin of 21.9% versus 18.6% in Q2 of financial year '24. The hospital business reported an operating EBITDA of INR355 crores, driving a robust 300 basis point improvement in margins from 18.4% in Q2 of financial year '24 to 21.4% in Q2 of financial year '25.

The hospital business EBITDA now accounts for 82% of the total consolidated EBITDA. In 11 of our facilities, we have reported operating EBITDA about 20% during the second quarter.

These 11 facilities together contributed 75% to the hospital revenue. In comparison, in financial year '24, we had only 8 units, which were having an operating EBITDA margin above 20%, and contributed at that time, 62% to the hospital revenues.

Operating EBITDA margin in the diagnostic business basis gross revenue have improved to 21.5% from 17.2% in Q2 of financial year '24. Excluding one-off items like rebranding exercise

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by Agilus first brand transition, reversal of provisions related to certain government businesses and the contingent consideration payment for an earlier lab acquisition, the operating margin stood at 24%.

On a like-for-like basis, adjusted operating margins were 22.7% in Q2 of financial year '24. Our consolidated profit after tax before exceptional items for the quarter increased 40.3% to INR253 crores. For H1 of financial year '25, our consolidated revenues stood at INR3,847.3 crores, up 12.3% versus

versus H1 of financial year '24. The operating margin for H1 financial year '25 increased to 20.2% against 17.6% in the corresponding previous period.

H1 financial year '25 hospital business revenue increased by 14.2% to INR3,204 crores.

Operating margin for the hospital business improved by 310 basis points to 20% for the period versus 16.9% in the corresponding previous period.

On the balance sheet side, we remain comfortable and healthy with a net debt-to-EBITDA of 0.16x as on September 30, 2024, as against 0.29x on September 30, 2023. Our net debt stands at INR281 crores as of September 30, 2024.

Continuing from Q1, our hospital occupancy further improved to 72% compared to 69% in Q2 of financial year '24. This translates into occupied bed increase by about 5.8% to 2,939 beds compared to 2,779 beds in the Q2 of financial year '24. Our hospital business saw a 7.6% increase in ARPOB, reaching INR2.37 crores per annum.

This growth was largely driven by revenue gains in our key specialty areas such as oncology, neurosciences, cardiac sciences, orthopedics, gastroenterology and renal sciences. Collectively, these specialties achieved a 13.6% year -on-year growth and accounted for 61% of total hospital revenue, consistent with the share in Q2 of financial year '24.

To highlight , the oncology specialty registered a growth of 19%. Neurosciences reported a growth of 17%. This is also reflected in the number of procedures performed in neurosciences with a growth of 21% compared to Q2. Encouragingly, the number s of robotic surgery performed increased by a strong 57% compared to the corresponding previous period.

Our revenue from medical travel grew 6% compared to Q2 of financial year '24 to reach INR134 crores. The revenue contribution of international business stood at approximately 8% in Q2 of financial year '25 on similar lines with Q2 of '24.

Most of our key facilities delivered strong performance this quarter with revenue for Shalimar Bagh, Mohali and Mulund registering a growth close to or in excess of 20 % compared to the corresponding previous year. During the quarter, we further strengthened our medical talent with the onboarding of specialists in the area of cardiac sciences, renal sciences, gastroenterology, internal medicine and obstetrics and gynecology. I'm pleased to share that our dedication to enhancing our medical programs has made significant progress in driving our strategic growth initiatives.

Marked by brownfield bed expansion and continued investment in advanced medical equipment, Fortis Ma nesar, a 350 -bedded hospital facility commenced its operations in September, offering

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an entire spectrum of clinical services, including all the key specialties and latest state -of-the-art medical equipment.

I'm also pleased to announce that FMRI launched the first MR Linac of North India to treat tumors with unparalleled precision. We are making good progress on our brownfield expansion plans for the year with capacity additions across key facilities, including Faridabad, Noida, BG Road, FMRI, Shalimar Ba gh and Anandpur coming during this financial year.

I would also like to highlight the ongoing success of our digitalization initiatives. We have successfully implemented electronic medical records for outpatient modules across 11 units now. At the same time, revenues from digital channels via website, mobile application and digital campaigns have witnessed a 30% year -on-year growth in Q2 of financial year '25. Digital revenue contributed 29.3% to overall hospital revenue versus 25.6% in Q2.

On the diagn ostic business front, we are moving ahead to consolidate our stake in Agilus by acquiring 31.52% stake from the PE investors. Our operating EBITDA improved to 21.5% compared to 17.2% in Q2 of financial year '24. This was primarily driven by cost optimizati on initiatives

, including, among others, improved network efficiency and optimizing manpower costs.

As part of our ongoing network expansion strategy, the total number of CTPs reached 4,085 as of September 30, 2024. The preventive portfolio revenues in Ag ilus overall revenue grew 20% in Q2 of financial year '25 and contributed 12% to the operating revenues versus 10% in Q2 of '24.

While expenses related to rebranding would con tinue to be incurred for the remaining part of the current fiscal year, Agilus ' steady recovery and improvement in margin reassures my faith in its ability to scale its revenue and profitability.

Driven by its strong network presence, a well -balanced B2C and B2B mix and an increasing focus on preventive care and specialized testing, I believe the Agilus brand is gaining acceptance and recognition, which will position the company to further enhance its performance going forward.

With this, I'll conclude my comments. Just to reiterate, we are making considerable progress on investments in multiple business growth drivers, be those in bed expansion, state -of-the-art medical equipment, digitization and clinical talent. We also continue to assess various inorganic growth opportunities that align with our cluster strategy and offer promisin g synergy. I believe all those initiatives will further enhance our growth potential and strengthen our position in health care sector. Thank you.

And with this, I would like to hand over to Anand for his comments now.

Anand Kuppuswamy: Thank you very muc h Dr. Raghuvanshi. Good evening, everyone. Thank you for joining us today. On behalf of Agilus Diagnostics, I welcome you to our Q2 FY '25 Results Conference

Call. Agilus Diagnostics reported a revenue of INR372.5 crores in Q2 FY '25, marking a 3.4% increase from INR360.3 crores in Q2 of FY '24.
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This follows last quarter's revenue of INR343.5 crores, representing an 8.4% growth compared to the trailing quarter. Operating EBITDA reached INR80 crores in Q2 FY '25, up from INR62 crores in Q2 FY '24 with the margin improving to 21.5% from 17.2%. In comparison, Q1 FY '25 EBITDA stood at INR55.4 crores, yielding a margin of 16.1%, adjusted for one-off expenses, operating EBITDA in Q2 of FY '25 was INR89.3 crores with a 24% margin versus INR81.9 crores or 22.7% in Q2 of FY '24.

For the half year period, Agilus posted a revenue of INR716 crores, reflecting a 1.9% growth from INR702.9 crores in H1 of FY '24. Operating EBITDA for H1 of FY '25 was INR135.4 crores, with an 18.9% margin, up from 18.3% in the previous year. Adjusted EBITDA before one-off expenses was INR149.6 crores, with a margin of 21.0%.

Agilus Diagnostics conducted a total of 11.1 million tests in Q2 of FY '25, up from 10.6 million in Q2 of FY '24 and 9.9 million in Q1 of FY '25. In H1 FY '25, the company processed 21 million tests compared to 20.5 million in H1 of FY '24. Agilus expanded its network significantly, adding over 150 customer touch points in Q2 of FY '25 and 330-plus touch points in H1 of FY '25.

The B2C to B2B revenue mix stood at 54% to 46% in both Q2 FY '25 and H1 of FY '25, slightly higher than the 53% to 47% in the same period last year. From a product standpoint, revenue contributions are 33% from specialized testing, 55% from routine testing and about 12% from our wellness portfolio in Q2 of FY '25. Our wellness portfolio stood at 20% growth in Q2 of FY '25 versus Q2 of FY '24 and 17% in H1 of FY '25 versus H1 of FY '24. Regional revenue contributions are 31% from the North, 21% from the West, 32% from the South, 13% from the East and about 3% from the international markets.

On the brand side, we have introduced Anil Kapoor as our new brand ambassador through a campaign centered on the theme keep testing yourself. We expect this brand-building initiative to strengthen our brand presence, particularly

by supporting growth in our B2C and wellness segments. We continue to focus on improving our efficiencies through various cost management initiatives. Thank you so much.

Moderator: Thank you so much. We will now begin with the question-and-answer session. First question is from the line of Amey Chalke from JM Financial Services.

Amey Chalke: Congrats to the management for the good numbers. The first question I have on the margins, especially hospital margins, we have been -- like this is second quarter, we have clocked more than 20% margins. Margins are improving consistently sequentially. This is even before our brownfield expansion initiatives are yet to kick in. So what are the key levers here? If you can call out any specific hospitals which have helped deliver improvement in margins over the last 6 months?

Vivek Goyal: Yes. If I can take this question. As you rightly said, we are moving nicely on our margin expansion journey. And for the quarter, we achieved 21% plus margin. And for the half year also, the margin is above 20%, which is quite encouraging.

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The main lever for margin expansion is the occupancy. If you see the occupancy level has gone to 72%, which is -- which has led to the higher margin. In terms of the hospitals, almost all our premium facilities have witnessed the good occupancy and plus there is a volume growth. As a result, there is a volume growth. If you see this quarter, the main contribution for revenue growth is coming from the volume and the occupancy. So I will say that is the main contributor to the margin apart from some of the specialties like Pulmo and other high-margin spaces, they have also contributed a lot.

Amey Chalke: Sure. Is it possible to tell which two hospitals have moved to the 20% to 25% range during this quarter?

Vivek Goyal: Yes. So one is, of course, Mulund, which is -- has done well. Mulund, there is a decent increase in the occupancy level and so is the margin expansion. Another unit which has moved to this level is Nagarabhavi, which is in Bangalore. There last quarter, there was certain additional marketing and sales expenses because of which the margin was low. But this time, this unit is really doing well in terms of revenue, occupancy, EBITDA margin front. And Kalyan also moved up in the margin metric.

Amey Chalke: Mulund, we have come back to the original occupancy level what we used to deliver? And is it a seasonal impact you think? Or do you think it is a structural which would continue going ahead?

Vivek Goyal: Mulund, we were struggling actually, we have mentioned in the earlier call also, we are struggling around 60% type of occupancy level. So the unit has moved to about 65% occupancy level, and that has resulted into the EBITDA margin improvement. And because this improvement is gradual, it is not sudden. And we feel this is sustainable. And the unit will continue to improve its occupancy and EBITDA margins going forward.

Amey Chalke: Sure. And the second question I have is on the Diagnostics segment. That also has delivered a stellar margin this quarter around 24%. Is it possible to call out the reason for the same?

Vivek Goyal: Yes. Anand, will you like to take this question?

Anand Kuppaswamy: Yes. Thanks. So regarding our margins, as stated by Dr. Raghuvanshi as well, so we are focused on improving our network efficiencies in terms of -- both in terms of improving our direct costs as well as improving our network efficiency by combining certain lab operations and creating some efficiencies there as well as some amount of optimization in terms of other costs as well. So this has led to the margin improvement.

Amey Chalke: So should we assume these margins to sustain in coming quarters? Or we should expect some normalization?

Anand Kuppaswamy: This particular increase in this particular quarter

is also driven by an increase in revenue. As you

know, that the second quarter is usually the best quarter for diagnostics. So there has been an increase in revenue as well. So I would say that the optimization levels will continue, but the

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revenue levels will also determine the kind of profitability that we will deliver. And we are seeing steady increase in revenue levels as well.

Moderator: Next question is from the line of Neha Manpuria from Bank of America.

Neha Manpuria: You mentioned that you're adding quite a few brownfield capacity in the second half. Could we quantify the number of beds that will get added in the second half?

Vivek Goyal: Yes. So one is Manesar facility we have operationalized. So the benefit of that should be coming in this quarter. And in terms of revenue, although it will be contributing slightly on the EBITDA side -- negative on the EBITDA side. Plus we are expecting at the latter part of the quarter this Faridabad facility to be commissioned.

Neha Manpuria: Okay. Only Faridabad will come in the second half.

Vivek Goyal: And then we are also having some additional bed at our premium facility in FMRI, where we are rejigging certain operation. And through that, we will be adding another 20, 25 beds. So that should also be commissioned.

Noida, we are expecting -- I don't know whether it will happen in this quarter, third quarter or it will move to fourth quarter. But Noida is also progressing quite well. So in all our expansion, we are progressing quite well. But major benefit will be coming, I will say, in the fourth quarter, this expansion capacity.

Neha Manpuria: And for Manesar, sir, how many beds have we commissioned so far? And what's the plan in terms of breakeven in the Manesar facility?

Vivek Goyal: Yes. So we have initially started with 50 beds. And as we progress in occupancy metrics, we will open more beds. So we have assumed, we expect around 15 months' time for the breakeven of this facility in terms of EBITDA. But the initial response is quite good, and we are quite hopeful that we may achieve even better, even earlier this breakeven.

Neha Manpuria: And this 15-month breakeven is assuming bulk of that capacity is commissioned?

Vivek Goyal: Sorry, bulk of?

Neha Manpuria: 15 months is assuming bulk of the beds is operational by then?

Vivek Goyal: Yes. So as I said, we are seeing very encouraging signs in terms of occupancy and the response we are getting in that area. So we expect that occupancy ramp-up will be quite fast, and that will result in earlier achievement of the breakeven.

Neha Manpuria: Okay. And on the diagnostic business, what should we think about the margins, let's say, 12 to 18 months from now or probably in fiscal '27. In your view, once growth starts to normalize for this business, ideally, what is the sustainable margin that the diagnostic business can get to? And second, by when do we complete the stake purchase from the private equity?

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Vivek Goyal: Yes. I will take that question on private equity, and then I will ask Anand to comment on the

margin. The private equity side, we have progressed quite well. We have got the CCI approval, and we have also got in -principle approval from the stock exchange for the institutions. We are in the final stage of finalizing the agreement with the key investors. Hopefully, we should be concluding all this thing by this month end. And then we will conclude the deal by this month end and maybe early week of December. Anand, over to you on the margin side.

Anand Kuppaswamy: Yes. Thank you. So on the margin front, so what we feel is margins are basically dependent on 2 factors here. One is the operating leverage that we get from revenue and another is cost optimization and network efficiencies

that come through over a period of time. So what we are seeing -- what we feel is that by in another 15 to 18 months, we should be able to be in the range of about 25%, 26% kind of margins. In the upcoming quarters, we'll be able to see steady progress on these numbers.

Neha Manpuria: You said 25% to 26%. Did I hear that correctly?

Anand Kuppaswamy: Yes. I'm talking about FY '27 or '28 those kind of times.

Moderator: Next question is from the line of Shyam Srinivasan from Goldman Sachs.

Shyam Srinivasan: Just the first one on the balance sheet post this stake sale -- stake increase rather in Agilus. I just want to understand how does this start looking? Today, it is at 0.16 EBITDA, net debt to EBITDA. But I just want to understand with the INR1,500 crores -odd NCD, any rough numbers we should be working with? And also, is the valuation for the 31.5% is the put option liability, is that the best benchmark to look at how the stake increase is going to happen? If you could highlight how the balance sheet could likely change?

Vivek Goyal: Yes. Sure, Shyam. So Shyam, balance sheet will remain to be very healthy even after post acquisition stake through debt of INR1,500 crores because we are seeing a very steady increase in the EBITDA. And with this run rate, by year-end, our debt to EBITDA will be below 1.5, which is quite healthy from that angle. And it may go, it may further come down because of, as I mentioned, the EBITDA, run rate of the EBITDA is quite high. And with this brownfield expansion, the EBITDA should even go further.

Shyam Srinivasan: Got it. Vivek, I just want to take a step back and look at this deployment of capital, right? Is that the best use of our capital to buy the stake in a subsidiary? I know there is a put option liability and there is a timing. But when we were to evaluate all the different ways to deploy capital, including brownfield or maybe Greenfield, I just want to understand, is there any ROCE kind of analysis that you did before we made that? Or is this a compulsion of the business and then you had to kind of acquire it?

Vivek Goyal: Yes, Shyam, I will say, I will try to answer this by two ways, okay? One is you rightly mentioned this is a liability on us and we have to honour this liability. So there is no two ways about it. So we have to honour it. Having said that, looking at the performance level of Agilus on which the valuation has been done and future potential of this unit, we feel it is the best use of the capital. And plus also, I would like to ensure everybody here that by this capital deployment, we are not compromising at all with our hospital business operation.

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So we are moving forward on our brownfield expansion journey as we have committed earlier. And also, we are looking actively on some acquisition target also. So from that angle, we are not compromising or restricting ourselves in the hospital business at all because of this expansion. Company is having a decent balance sheet.

We have other options also if we want to raise capital. So there is no restriction as such on companies we are putting for our growth and expansion. And this acquisition, I think acquisition of stake in long term will create a lot of value for the shareholders.

Shyam Srinivasan: Just any implied EBIT to EBITDA multiple for the stake increase for the Agilus?

Vivek Goyal: Yes. So the implied valuation of this, based on the valuation report, which has finally been agreed by all the parties is around 17x to 18x EBITDA multiple 1 year forward. And we are seeing better than our expected EBITDA numbers in Agilus. And as Anand mentioned earlier, he is quite hopeful of improving it even further. So that's why I'm quite hopeful that this investment will be very fruitful for all -- for the investors.

Shyam Srinivasan: My last question is just on Agilus oper

ations, right? Despite the margin improvement, I think growth still remains a challenge, 0% or flat growth in patients. Most of the growth has come from this test increase. And this is clearly trailing some of the peers, right? So what are the plans to kind of improve top line growth at Agilus? And is that the key lever for getting to 25%, 26% margins? Or it will be still -- we will still lag peers in terms of top line growth?

Anand Kuppaswamy: Thanks, Shyam. So in fact, if you actually see our volume growth has been quite consistent compared to last year. If you see that last year, we had a large component of PPP business, which

was there, which was contributing very high on volume, but on value, it was very low. So such business is now very low in volume at this point of time. So that's why you're seeing sort of a flattish volumes there. But when you look at sort of a core business growth kind of a thing, so you will see that the volumes are also growing as well as the price increase that we have done in February of last year. So that has an impact of about 1.5% on the growth. So the balance is the volume growth.

So -- and we see this combination of volume and value growth will be the primary drivers for the overall growth going forward as well because what we are seeing is that when we changed the brand in June of last year, June '23, so from there, the kind of impact which we saw on our B2C businesses, those impact is now much reduced, and we are seeing a good recovery on all fronts.

And with the upcoming campaigns that we are doing with our new brand ambassador, Anil Kapoor and all that. So we are also expecting that this to be further improved compared to the current levels.

Moderator: Next question is from the line of Bino from Elara Capital.

Bino: Just following up on the question on Agilus. Is the valuation -- absolute valuation number public? Can you share that?

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Vivek Goyal: Yes, it is in public domain. We have gone for the shareholder approval also for that. I think the number is INR1,778 crores enterprise value.

Anand Kuppuswamy: Equity value.

Vivek Goyal: Yes, for 31 % stake.

Bino: Okay. This is the price of the 31% stake that you are buying?

Vivek Goyal: Yes.

Bino: Okay. And when you execute this transaction, would it have any sort of P&L impact, any profit loss, anything passes through the P&L?

Vivek Goyal: Yes, the impact will not particularly be because of acquisition of this transaction, but because we are funding it through interest, so that interest charge will be coming in the number.

Bino: Okay. Interest charge will come, but there is no particular P&L in terms of any onetime profit or loss, right?

Vivek Goyal: No, nothing like that. Nothing on that side.

Bino: Okay. Understood. And finally, one question on hospitals. Earlier, you had given a guidance of 200 basis point margin expansion in the hospital business for the full year. That remains as it is because you are mentioning to Manesar contributing a little negatively to the EBITDA, etcetera. Even after all that, you will maintain that 200 basis point margin expansion for the year?

Vivek Goyal: Yes, yes. So we are maintaining our margin guidelines. And when we had given that guidelines, we have factored in these losses of Manesar, which is obvious because it is a sort of Greenfield project. So initially, some losses are expected to happen.

Moderator: Next question is from the line of Nitin Agarwal from DAM Capital Advisors.

Nitin Agarwal: So just going back to Agilus question again. I mean, when we look forward '26, '27 onwards, can we expect the business to or do you expect the business to go back to double-digit growth trajectory?

Anand Kuppuswamy: Yes, definitely, that will happen. So we are seeing that trend happening. So I think after this year, you will see t

hat the basis will change because of whatever changes we have done compared to last year, the brand transition, all those things happening now. So those will have a better impact in the coming years.

Nitin Agarwal: So essentially, when you look at Agilus going forward, essentially, it's a 10% -- it's a double-digit growth business with a 25%, 26% EBITDA margin.

Anand Kuppuswamy: I think in line with the industry, we'll be able to move forward in terms of growth numbers. The margins, as you know, our structure is slightly different from other companies. So accordingly,
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our margins would be different. But at the same time, they will be definitely much more improved compared to where we are today.

Nitin Agarwal: And secondly, on the hospital part, and FMRI we're looking to, you mentioned about 180 beds that we're adding, 220 beds rather a new tower. How many beds will be commissioned in the first phase or it's going to get commissioned, the 220 beds all together?

Vivek Goyal: No, generally, for the expansion of this size, beds will be available from, say, April '25 onwards.

But we will open the bed as per our occupancy to contain the cost factor. Initially our plan is to open 100 beds. And as we achieve a particular occupancy level, then we will open up the balance beds. Maybe in 6 months to 1 year time.

Ashutosh Raghuvanshi: 20 beds, Nitin, which we will open earlier, which will be somewhere around January of this coming year.

Nitin Agarwal: You said 20 beds.

Ashutosh Raghuvanshi: 20.

Nitin Agarwal: Okay. And sir, what is the current base of the FMRI? How many operational beds do we have in FMRI right now?

Vivek Goyal: 310 beds, currently, we have.

Nitin Agarwal: Okay. And sir, when you are evaluating these brownfield projects in your assessment, what is the kind of time you're seeing for these brownfield capacities to come back to the levels of the existing bed profitability? How much time do you think it's going to take for these brownfield expansions?

Vivek Goyal: Yes, it is generally from day 1, it start contributing because it is on the running hospital, and we plan the bed opening accordingly. So we are expecting almost immediate return type of things in the brownfield.

Nitin Agarwal: You don't see any drag on EBITDA on the brownfield expansion at all?

Vivek Goyal: Not really.

Nitin Agarwal: The last one, already in the presentation, you've highlighted the beds that you will add in F Y '25 on the brownfield side. For F Y '26, which are the major capacities which will come on board apart from FMRI?

Vivek Goyal: So Noida will be fully commissioned. FMRI will be fully commissioned. There will be some bed addition in Anandpur as well, Anandpur is in Kolkata and then BG Road in Bangalore.

Nitin Agarwal: All in all, another maybe 350, 400 beds will come through next year incrementally in F Y '26?

Vivek Goyal: Around that number, we are expecting, yes.

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Moderator: Next question is from the line of Banshi Desai from JP Morgan Chase.

Banshi Desai: So my question is on hospitals. So if I look at the top -performing hospitals for us, I think consistently, these hospitals have been doing occupancy of 70% plus, around 70%, 72% in that range. Do we have scope to kind of improve our occupancies here for these the best performing hospitals for us?

Vivek Goyal: Yes. So occupancy except 2 hospitals, BG Road and Mulund, I'm talking premium hospital, which are the high -performing hospital. The occupancy is about 70% in almost all the hospitals. And that's why we are going for this expansion thing.

However, once the new brownfield capacity kick in, as I mentioned from, say, March, April onwards, then we may see some drop in the occupancy level on the enhanced bed capacity. But overall, I

think we will be able to maintain this type of occupancy level in this premium facility.

Banshi Desai: Okay. So the reason I ask is that I do see 2, 3 hospitals of ours, which are in the bracket of 15%, 20% EBITDA margins, even achieving 77% kind of occupancy levels in the first half of this year. So I was just trying to understand that probably what is stopping us from achieving those kind of numbers even for our top performing ones?

Vivek Goyal: Yes. So each hospital has a different dynamic. So, for example, Faridabad, for example, it is around 18%, EBITDA level. And occupancy level of this hospital is quite high. It is around 80% plus occupancy level, it is consistently operating. Because there are certain personnel costs relating to the labor union and those type of costs.

With this expansion program, which will start kicking in from this quarter onwards, we are expecting this hospital definitely should move into 20% plus EBITDA margin radar. And similarly, for Amritsar, for example, it is hovering near 20%. It is slightly low. That's why it is coming in the 15%, 20% bracket.

But this unit, again, is a good unit from the EBITDA margin perspective. Ludhiana is the only one unit, I will say, 15%, 20% bracket, which will in the immediate vicinity will remain in 15% to 20% region. We are not expecting in a year time to move to 20% EBITDA margin.

Banshi Desai: So with expanded capacity, do we expect our contribution from institutional patient revenue also go up? It's already at 20%. So how do you think about that?

Vivek Goyal: Yes. So the unit where we are expanding capacity like Noida, for example, where we will be adding 160, 170 beds. Our immediate priority will be to fill those beds. So for that, we may opt for some scheme business.

But unit like FMRI, where we are adding beds and it is having almost 0 guidance business, we will continue to do that. And our strength on international front patient and the strong TP and care business should be able to absorb this type of bed expansion. But I think in our expansion

thing, this Anandpur and Noida are the 2 units where I expect there will be a little bit increase in the scheme business.
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Bansi Desai: Okay. And last one, I also see a couple of hospitals moving in the less than 10% EBITDA margin bracket. And we now have 7 hospitals out there. And obviously, this all excludes the loss-making ones which we divested. So any thoughts on how do we see improvement in these underperforming hospitals from here on?

Vivek Goyal: Yes. So there are very small dip in 2 of our hospitals, one is FEHI and one is in CH Road. That's why the 2 hospitals which were earlier in the range of 10% to 15% that has come below 10% thing. So we are not very much worried because it is just a mathematical calculation. It has just come down by 0.5 percentage point, and that's why it is categorized in this thing.

So FEHI specifically, we are quite happy with the performance in terms of the revenue growth, occupancy, ARPOB and other things. And we are moving very nicely in terms of making it multi-specialty type of hospital rather than cardiac-specific hospital. And that is yielding the results, and we are consistently now on double digits, so which is quite encouraging.

I think another hospital, big hospital in this category is Jaipur, which we already discussed about. And first quarter was really bad for Jaipur. I have explained the reason in the earlier call because of that organ transplant related issue. And now I think it is moving back to its normal level. And with this, we are quite hopeful that Jaipur will reach to double-digit plus EBITDA margin maybe in the next 6 to 8 months' time.

Other units are quite small. Ludhiana, just started 8 months back, so it is giving negative EBITDA. Another hospital in this category is a very small hospital Sacred He

art in Bangalore,

which actually we are planning to divest and get rid of this hospital because it is very small hospital. It is not having very big impact.

The remaining hospitals are below 10% because they have a big charge on the rental side, like CH Road, La Femme. These are the hospitals which has 6% to 7% rental sitting into this EBITDA. And after absorbing that rental cost, the EBITDA margin.

Moderator: Next question is from the line of Atul, an individual investor.

Atul: So my question is on the legal side. Any updates on the High Court side on the case?

Ashutosh Raghuvanshi: Yes. So in the High Court in matter of forensic, the hearings are happening. We expect the next hearing to be there sometime end of this month. And that hopefully should be the last hearing, but then the vacation period will begin. So we are not expecting much movement for another 1 or 2 months.

Whereas in the matter of brand, the court has ordered for an auction of the Fortis brand. So for that on 21st, the court officials will decide on the mechanism and the terms, etcetera. Post that, we will see how we can participate and secure this for us.

Atul: Another question which I have is on the legal cost side, like since these hearings will be done maybe 1 month or 2 months, right, are we expecting legal costs to come down, which means improving our EBITDA?

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Ashutosh Raghuvanshi: Yes. So this year, Atul, we don't expect the legal cost to be low. In fact, this year, the legal costs have been slightly higher than the last year because more number of hearings, etcetera has happened in the high court compared to last year. So the cost has been slightly higher this year, but we expect that to go down from next year.

Atul: And my last question is, I'm not sure whom to ask, but it is more on the side of -- I'm not looking for numbers, but on the occupancy side in the current quarter, how the things are shaping up, if we can get some indications because we are already like 1 month and 8 days over.

Vivek Goyal: Yes. So we are expecting -- and this always will be the case, this particular quarter, current quarter is affected because of the seasonal impact. As you know, there is a festival season. So there will be some dip in the occupancy. But overall, we are not expecting a very drastic change in the occupancy and the profitability matter.

Atul: My question is more on the side of the culmination of the point, which is going for now like a couple of years, maybe 3 years, 4 years, right? So from that front, like seasonality will always be there. But in terms of the overall numbers, you see like improvement finally reflecting on the margins and everything?

Ashutosh Raghuvanshi: Yes. I think the gains we have achieved so far, we will keep on going on that track and the trend is healthy.

Atul: Momentum is there in the right direction, right? Is that the right way of saying it?

Ashutosh Raghuvanshi: That's correct.

Moderator: Next question is from the line of Abdulkader Puranwala from ICICI Securities.

Abdulkader Puranwala: Sir, my first question is with reference to your Mulund hospital. So if I look at the quarterly revenue numbers what you have shared, there has barely been any movement in the quarterly sales, while the margins have gone up significantly. So if you could throw some color here as to what has happened in Mulund?

Vivek Goyal: Yes. So in Mulund, as I said, one is the occupancy level has gone up. One is occupancy level has gone up. Second is the facility in which the revenue has gone up, which is high-margin facility. As a result, there is an overall improvement in the EBITDA margin in the Mulund.

Abdulkader Puranwala: Got it. And sir, did I hear it correct that you would be divesting one more hospital at Bangalore?

Ashutosh Raghuvanshi: We are evaluating that, smaller facility

, we are evaluating that.

Abdulkader Puranwala: So will that be the one at Rajajinagar?

Ashutosh Raghuvanshi: No. This is on Richmond Road, which is called the Sacred Heart.

Moderator: Next question is from the line of Mayank from Axis Asset Management.

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Mayank: I have two questions on the diagnostics side. First is for the touch points added last year, is there any regional bias per se? And secondly, what is the plan for addition of more customer touch points for the remaining of FY '25 and FY '26?

Anand Kuppuswamy: So we are normally adding close to about 900 touch points every year before the brand change. After the brand change, our touch point addition has slowed down to about 600 -odd. So this year also, we expect to add close to about 600 to 700 touch points. And we are seeing that improving every quarter. So this quarter, we have added close to about 150 touch points.

Mayank: So this run rate is likely to continue into FY '26 also?

Anand Kuppuswamy: Yes, yes. We continue to add touch points because we have a network across the country. So it's not -- as you are asking also that whether it is any specific to any particular region. So there is an increased focus on some of the focus regions like we have some of the territories, some of the regions like our Kolkata region, our Mumbai region, Bangalore region, Delhi, NCR and Chandigarh region. So these regions, we have a higher focus on improving our network, whereas across the country also, we continue to add new touch points.

Mayank: Okay. And on a blended basis, if I have to talk about pricing, pricing broadly -- I mean, different regions have different pricing, but broadly, the pricing is kind of maintained? Or do you see any change in the pricing as we'll in terms of price hikes or lowering prices to get in line with the competition in the specific regions?

Anand Kuppuswamy: You're talking about pricing region-wise.

Mayank: Yes. So on a blended basis, I mean, region-wise it will be difficult to give. I understand. But on a blended basis, do you see any price change happening on a blended basis in diagnostics?

Anand Kuppuswamy: No, it's fairly constant. So we have differential pricing across different segments, across different regions. And it's kind of very similar to how the rest of the industry players move.

Mayank: Second, on the diagnostics part only, so you alluded to the fact that the margin is coming back, it's a function of the efficiency improvement, productivity and the overall top line level. So fair enough to say in case whenever you hit INR360 crores plus kind of number, these level of margins are sustainable. There is nothing like which is a one-off if this level of revenue kind of is achieved. Is that right?

Anand Kuppuswamy: Correct.

Mayank: Just one last question on the hospital side. But given the fact that we have a fairly good pipeline of capacity coming in the next 2 to 3 years' time frame. And given the fact that we will get -- we'll have some debt because of the stake purchase in the diagnostic business, would the company still be evaluating any additional asset either through M&A or additional land building in the next 1 to 2 years' time frame, which can basically be an addition to the already existing capacity addition? Or will it be avoiding it because of the leverage or -- because of the leverage primarily?

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Ashutosh Raghuvanshi: No, no, absolutely, we will be evaluating opportunities, inorganic opportunities at all time. We are very mindful as to what kind of value we are acquiring for the shareholders. So we are looking at individual assets to a large extent, but they have to fit into the geographical strategy of ours, our cluster strategy of ours. So based on that, we are continuously evaluating pr

objects,

and we will certainly be doing certain inorganic acquisitions.

Moderator: Next question is from the line of Nitin Agarwal from DAM Capital Advisors.

Nitin Agarwal: Just quick one. Sir, what kind of capex spend should we assume for this year and next year?

Vivek Goyal: Yes. So we are expecting around INR800 crores to INR900 crores approximate capex, which includes maintenance as well as growth capex for both the years, each year.

Moderator: Ladies and gentlemen, we'll take that as the last question. I'll now hand the conference over to the management for closing comments.

Anurag Kalra: Thank you, Nirav. Ladies and gentlemen, thank you very much for joining us on the call today.

Really appreciate your time. If there are any follow-up queries, please feel free to e-mail us. Me and my colleague, Amit are there available to speak with you and try and resolve your queries as much as possible. Thank you, and have a good evening.

Moderator: Thank you very much. On behalf of Fortis Healthcare, that concludes this conference. Thank you for joining us, and you may now disconnect your lines. Thank you.

Call: Feb 2025

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February 14, 2025

FHL/SEC/2024-25

The National Stock Exchange of India Ltd.

Scrip Symbol: FORTIS BSE Limited

Scrip Code:532843

Sub: Transcript of Investors / Analysts' meet under Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015

Dear Madam/Sir,

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed transcript of Investors / Analysts' meet held on February 10, 2025 to discuss the Company's Un-Audited Financial Results for the quarter and period ended on December 31, 2024 and same is available on the Website of the Company at below hyperlink: -

Earnings Call Transcript - December 2024

The date and time of occurrence of event is February 10, 2025 at 2.30 p.m.

This is for your information and records.

Thanking you,

Yours Sincerely,

For Fortis Healthcare Limited

Satyendra Chauhan

Company Secretary & Compliance Officer

M. No. – A14783

Encl: as above

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"Fortis Healthcare Limited "

Q3 FY '25 Earnings Conference Call "

February 10, 2025

MANAGEMENT : DR ASHUTOSH RAGHUVANSHI – MANAGING
DIRECTOR AND CHIEF EXECUTIVE OFFICER – FORTIS

HEALTHCARE LIMITED
MR. VIVEK GOYAL – CHIEF FINANCIAL OFFICER –
FORTIS HEALTHCARE LIMITED
MR. ANURAG KALRA – SENIOR VICE PRESIDENT ,
INVESTOR RELATIONS – FORTIS HEALTHCARE
LIMITED
MR. ANAND KUPPUSWAMY – CHIEF EXECUTIVE
OFFICER – AGILUS DIAGNOSTICS LIMITED
MR. AKSHAY TIWARI – CHIEF FINANCIAL OFFICER –
AGILUS DIAGNOSTICS LIMITED

Fortis Healthcare Limited
February 10, 2025

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Moderator: Ladies and gentlemen, good day, and welcome to the Fortis Healthcare Limited Q3 FY '25 Earnings Conference Call. As a reminder, all participant lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touch-tone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Anurag Kalra, Senior Vice President, Investor Relations at Fortis Healthcare Limited. Thank you, and over to you, sir.

Anurag Kalra: Thank you very much. A very good afternoon and good evening, ladies and gentlemen, and thank you for taking the time to join us on our quarter 3 FY '25 earnings call. The call is being chaired by our MD and CEO, Dr. Ashutosh Raghuvanshi, with him, we have our Chief Financial Officer, Mr. Vivek Goyal. From Agilus Diagnostics side, we have Mr. Anand, the CEO of Agilus Diagnostics, as well as Mr. Akshay Tiwari, the CFO.

We will start with some opening comments by Dr. Raghuvanshi, post which Anand will take you through certain key highlights of the Diagnostics business, and then we can open the floor for question and answers. Over to Dr. Raghuvanshi.

Ashutosh Raghuvanshi: Thank you, Anurag. Good evening, everyone, and thank you for taking the time to join us on our Q3 financial year '25 earnings call today. Before diving right into the numbers and sharing my thoughts on the business performance, I would like to highlight that our performance in Q3 financial year '25 and the 9 months financial year '25 continues to witness a healthy improvement over the corresponding previous year, led largely by our Hospital business. Our performance in Q3 has been quite satisfactory, showing a significant improvement over the previous year. This achievement has been driven by strong performance of our Hospital business. We reported a consolidated top line figure of INR 1,928 crores, a growth of 14.8% over Q3 of financial year '24. Noticeably, our Hospital business revenues have grown 16.8% to INR 1,623 crores, while Q3 financial year '25. Diagnostic business growth revenues were at INR 342.3 crores versus INR 331 crores in Q3 of financial year '24.

Our consolidated operating EBITDA increased 32% to INR 375 crores, delivering a margin of 19.4% versus 16.9% in Q3 of financial year '24. The Hospital business reported an operating EBITDA of INR 325 crores, driving a 200-basis point improvement in margin from 18% in Q3 financial year '24 to 20% in Q3 of financial year '25. The Hospital business revenue now accounts for 84% of our consolidated revenue and operating EBITDA accounts for 87% of our EBITDA.

In 11 of our facilities, we have reported operating EBITDA above 20% during the third quarter. These 11 facilities together contribute 74% to the hospital revenue during the third quarter and 9 months of financial year '25. In comparison to financial year '24, we had 8 of our facilities operating in EBITDA margin of about 20%, contributing 62% to the hospital revenue. Our consolidated reported profit after tax before exceptional item for the quarter increased 82.2% to INR 231 crores.

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For 9 months financial year '25, our consolidated revenue stood at INR 5,776 crores, up 13.1% versus financial year '24. The operating margin for 9 months for financial year '25 increased to 20% against 17.4% in the corresponding previous period. Nine months of financial year '25, Hospital business revenue increased by 15% to INR 4,827 crores. Operating margin for the Hospital business improved by 270 basis points to 20.0% for the year versus the previous year of 17.3%.

Coming to the balance sheet side, we remain comfortable

e and healthy with a net debt -to-

EBITDA of 0.41x as on December 31, 2024, as against 0.45x on December 31, 2023. Our net debt stands at INR 644 crores as on December 31, 2024. In December 2024, we successfully raised INR 1,550 crores through the issuance of nonconvertible debentures, leveraging these funds along with internal accruals, we consolidated our stake in Agilus by acquiring 31.52% stake from our private equity investors.

As a result, our company now holds a commanding 89.2% equity stake in Agilus as on date. Hospital occupancy improved to 67% compared to 64% in Q3 of financial year '24. This translated into occupied beds increased by 6.2% to 2,790 beds compared to 2,627 beds in Q3 of financial year '24. Our Hospital business saw a 9.9% increase in ARPOB, reaching INR 2.45 crores per annum.

This growth was largely driven by revenue gains in our key specialties such as Oncology, Neurosciences, Cardiac Sciences, Gastroenterology, Orthopedics and Renal Sciences. Collectively, these specialties achieved a 17% year-on-year growth and contributed 62% to the overall Hospital business revenue compared to 61% in Q3 of financial year '24.

To highlight, the Oncology specialty registered a growth of 30% and neurosciences reported a growth of 18% year-on-year. Growth in oncology was led by an increase in revenue from Hematology and Bone Marrow Transplant by 44% compared to the same period last year. To highlight, key surgical procedure volumes performed across neurosciences and robotic surgeries witnessed a strong growth of 23% and 77%, respectively, compared to the corresponding previous period.

Our revenues from medical travel grew 17% compared to Q3 of financial year '24 to reach INR 132 crores. Revenue contribution of international business stood at approximately 8%, which is similar to Q3 of financial year '24. The company's key facilities such as Shalimar Bagh, FMRI, Faridabad, Ludhiana, F EHI, Mulund and NagarBhavi, registered a revenue growth in excess of 20% compared to the corresponding previous year.

Continuing with the portfolio rationalization strategy, we divested business operations of Richmond Road Hospital in Bangalore in December of 2024. This divestment supports our focus on improving overall profitability and margins. This is the third facility divested by the company after the divestment of Malar facility and Vadapalani facility in Chennai.

Revenues from digital channels where website, mobile applications and digital campaigns witnessed a 36% year-on-year growth in Q3 of financial year '25. Digital revenues contributed to approximately 29.9% to overall hospital revenues versus 25.7% in Q3 of '24.

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On the Diagnostic business front, operating EBITDA margin stood at 14.4% versus 10% in Q3 of financial year '24. Excluding one-offs, the operating EBITDA margin stood at 21.3% versus 18.3% in Q3 FY24. As part of our ongoing network expansion strategy, the total number of new customer touch points reached 4,126 as of December 31.

The preventive portfolio revenues in Agilus overall revenue grew 17% in Q3 and contributed 10% to the operating revenues versus 9% in Q3 of financial year '24. The Diagnostic business performance is still adjusting to the impact of Agilus rebranding exercise, which involved extensive rebranding efforts and associated marketing costs. We expect the branding expense to taper off towards the end of this financial year.

That said, I'm confident in Agilus' potential to scale up both in terms of its revenue and margins based on its considerable network presence, a balanced B2C and B2B mix and the increased focus on preventive care, as well as specialized testing. To also add, I do believe that the Agilus brand is being well accepted and is gaining recognition. This would place the company in a better position to further scale our performance.

With this, I will conclude my comments. We are making significant progress on the growth at all possible fronts, leveraging our robust balance sheet, we will continue to explore and access various growth opportunities that align with our cluster strategy and offer promising synergies. I believe these initiatives will further enhance our growth potential and strengthen our position in health care sector.

Thank you, and I will now hand over to Mr. Anand for his comments now.

Anand Kuppaswamy: Thank you, Dr. Raghuvanshi. Good afternoon, everyone, and thank you all for joining us today.

On behalf of Agilus Diagnostics, I welcome you to our Q3 FY '25 results conference call. Agilus Diagnostics reported a revenue of INR 342.3 crores in Q3 FY '25, marking a 3.5% increase from INR 330.7 crores in Q3 of FY '24. Revenues for Q2 of FY '25 were at INR 372.5 crores.

Operating EBITDA stands at INR 49 crores in Q3 FY '25, up from INR 33 crores in Q3 FY '24 with margins at 14.4% and 10%, respectively.

Operating EBITDA for Q2 FY '25 stood at INR 80 crores, a 21.5% margin. Adjusted for one-off expenses, operating EBITDA in this quarter, that is Q3 FY '25 is INR 73 crores with a 21.3% margin versus INR 60 crores at 18.3% margin in Q3 FY '24 and INR 89 crores at 24% margin

in Q2 of FY '25. For the 9-month period, revenue stands at INR 1,058 crores compared to INR 1,033.6 crores in the 9-month of FY '24, a growth of 2.4%.

Operating EBITDA stands at INR 185 crores during the period versus INR 162 crores in the corresponding period in the last financial year. Adjusted EBITDA before one-off expenses stands at INR 222 crores in 9 months of FY '25 versus INR 214 crores in 9-month of FY '24, which is a margin of 21.1% versus 20.7% EBITDA margin, respectively.

In the quarter 3, we conducted a total number of 10.29 million tests compared to 9.85 million tests in the corresponding quarter last year. In the 9-month period, we have conducted a total of 31.31 million tests. Agilus expanded its network significantly, adding over 160 customer touch

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points in Q3 of FY '25 and 500-plus touch points in the 9 months of FY '25. The B2C to B2B revenue mix stood at 51% to 49% in Q3 of FY '25.

From a product standpoint, revenue contributions of 33% from specialized testing, 57% from routine testing and 10% from our wellness portfolio. Our wellness portfolio revenue showed a 17% growth in Q3 FY '25 versus Q3 FY '24 and 17% in the 9 months period of FY '25 compared to the 9 months period of FY '24. Regional revenue contributions are 30% from North, 21% from West, 32% from South, 13% from East and 4% from international markets in Q3 of FY '25.

Over the past 9 months, we have expanded our test menu with cutting-edge advancements in onco diagnostics and genomics. We recently introduced precision assays for myeloid malignancies with an industry-leading 3-day turnaround time. Notably, we are the first lab in India to launch the FDA-approved Claudin 18.2 test for gastric cancer.

We have also launched other specialized tests like the lymphoma NGS panel, lymphoid leukemia panel and gut microbiome test in this quarter. Our strong focus on preventive wellness and genomics continues to drive innovation, while our efforts to strengthen the Agilus brand are progressing well. We remain committed to continuously enhancing the customer experience.

Thank you, and over to you, Anurag.

Anurag Kalra: Thank you, Anand. Ladies and gentlemen, that was it on the highlights of both the Hospital and Diagnostics business. I'm going to request the moderator to now open the line for question and answers, please. Thank you.

Moderator: The first question is from the line of Neha Manpuria from Bank of America.

Neha Manpuria: First, on the Manesar greenfield expansion, could you give us some color on what the losses were in this quarter? And how s

ould we think about ramp-up of the facility? Any guidance on when the facility can breakeven?

Vivek Goyal: Neha, Vivek this side. So Manesar facility, we have started almost at the beginning of this quarter. So, it is currently contributing around INR 5 crores per month revenue. And I think breakeven point will be somewhere around INR 9 crores per month.

Neha Manpuria: And we'll get there by when in your view?

Vivek Goyal: Neha, we are targeting this by first quarter next year.

Neha Manpuria: And this would be, I'm assuming when you're saying INR 9 crores per month, this would be on a much larger bed count, right? Because currently, I think you commercialized only about 60, 70 beds.

Vivek Goyal: You are right. So currently, the hospital is functional. We have opened only 53 beds right now. Maybe another 50 beds, we will be opening by March.

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Neha Manpuria: Okay. Okay. My second question is on the Richmond Road facility. I understand this is very small. So is it fair to assume that there isn't too much cost benefit or margin savings that we will see from the asset being divested like we saw the last 2 times?

Vivek Goyal: Yes, it is not that much because as you rightly said, it is a very small facility. It has been incurring losses for quite some time, which we tried to revive, could not do that. Now, as per the last year financial, it has incurred around INR 8 crore EBITDA level loss last year. So that much benefit we will be getting in the forthcoming years.

Neha Manpuria: Understood. And my last question is on Agilus. I understand you mentioned that the cost related to rebranding will probably taper off by the end of this fiscal year. But if I were to think about the growth momentum and our target of helping this business grow in line with the industry, do you think that is possible in fiscal '26? Or would it take us much longer for us to normalize growth in this business? And what according to you, Anand, is required for growth to start

improving in Agilus?

Anand Kuppuswamy: Neha, this is Anand here. So, I think we will be able to come back to the growth momentum in terms of industry by the second or third quarter of next year itself. Because as of now, this year, we have seen quarter -by-quarter, we are seeing growth in terms of how our core business grows because if you look at the things like public -private partnership and other things, we will see that our core revenue has grown quite well.

And it is growing. What I'm suggesting is, it's growing. Every quarter, we are seeing improvement in the growth. And we are hopeful that by the second quarter of next year, we would be able to reach industry levels of growth.

Neha Manpuria: That would be closer to 10%? Or should I take more like...

Anand Kuppuswamy: Correct.

Moderator: The next question is from the line of Shyam Srinivasan from Goldman Sachs.

Shyam Srinivasan: Just the first one on the ARPOB dynamics. We have for the 9 months, I think, grown 10%. Even for the quarter, we have grown close to 10%. So just want to understand what are some of the drivers of it? I think Dr. Raghuvanshi talked about some of the specialties. But just want to understand, is it just mix? Or is there some pricing element that is also there on a Y-o-Y basis that is helping us keep the ARPOB buoyancy where it is?

If I were to look at some of your peers, I don't see like a double -digit, if I can make it double -digit kind of growth for others. So are we starting off a different base? Or is there something micro to Fortis that's leading to this buoyancy?

Ashutosh Raghuvanshi: So, Shyam, we have said in our commentary for last many quarters that we expect that the ARPOB growth should taper off a little bit, and it should probably be somewhere close to 6%, 7%. But it has been better than our expectation. And the reasons for that for us is

primarily case

mix. Out of this 9%, just maybe about 1.5% or so will be because of the price revisions. We Fortis Healthcare Limited

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have taken very minor price increases earlier in the year. And other than that, there is essentially the case mix, which has been happening.

As I was pointing out that some of the specialties like bone marrow transplants, et cetera, the growth has been as high as above 40%. And those are typically high ticket items. So that has led to this kind of growth. But I think we would expect that this kind of trend will persist for some more time, and we should see similar ARPOB going into the next couple of quarters as well.

Vivek Goyal: Yes. Just to add, Shyam here, apart from what Dr. Raghuvanshi said about specialty things, there is a lot of robotic work the company has done where the ticket size and ARPOB is slightly on higher side. Plus day care work has also gone up for us.

Shyam Srinivasan: Understood. That's helpful. Second question is just on the hospital margin metrics. In the 5 hospitals in the 9 -month data that are below 10%, you have -- I'm excluding Ludhiana 2 and Manesar. What are the other 3? And is this a possibility that we could look at divestments, some of these other 3 as well?

Vivek Goyal: So other 3, Shyam here is FEHI, Jaipur and Vashi. FEHI is a bottom line case. There was some exceptional expense of the previous year has been booked there. That's why it is coming below 10% type of margin. And Vashi and Jaipur, we have discussed in the past also, they are having their own problem. Jaipur particularly has not able to come up. Vashi has come up to some extent, but Jaipur is still struggling.

Ashutosh Raghuvanshi: Yes. We are making some changes in these hospitals in terms of leadership, et cetera. And we expect that, that should yield some good results. We have seen a positive momentum in FEHI in the last quarter, which continues during the running quarter as well, where we are seeing the revenues improving. And as a result of that, a lot of that will flow to the EBITDA also. So certainly, we would be going above 10% here and slowly inching towards the 15% level.

Jaipur, we have different plans. And certainly, we believe that, that hospital and the city has a potential, and we can revive it, but we will have to make some structural changes in terms of both manpower and the services offered.

Shyam Srinivasan: Got it. And my last question is just on the competitive intensity. A lot of beds being added by yourself or many of your peers. If you could kind of give us a little bit of how the industry landscape is looking like, especially on hiring and retaining talent? Are you seeing some kind of a war for clinicians, medical talent perhaps? And if you could share some of the attrition numbers, that will be helpful.

Ashutosh Raghuvanshi: Yes. Shyam, I think the talent acquisition and talent retention is an ongoing. I would say it is not a war, but a battle, which keeps happening here and there. But I think overall, there is no reason for concern. One of the things in our case, the majority of the hospitals which are coming online are brownfield in nature. So we do not need that much of additional clinical talent, except for junior doctors and nursing staff and the support staff. So that kind of pressure is not there at the

moment. But you are right that this is an ongoing thing.
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But what we have also noticed is that, in the last 10, 12 years, the number of specialists has started increasing because of all the reforms which have happened in the medical education for the last 15 years. And as a result of that, now you're seeing better supply. And the larger chains are not finding it so difficult. But I think as you go towards peripheral centers and smaller towns and cities, this problem becomes acute. But in places like Delhi, Bombay, et cetera, it's not so

severe. There is some degree of competition, but not anything to be worried about.
Moderator: The next question is from the line of Bino Pathiparampil from Elara Capital.

Bino Pathiparampil: This is Bino from Elara. Yes, a couple of questions from my side. Dr. Raghuvanshi, we are seeing a lot of your competitors being very aggressive in expansion. They are buying a lot of land for new setups, greenfield setups in many of the big cities, plus they are aggressive on the M&A side as well. But we don't hear what is being so aggressive in expansion in the space. Is it that you don't intend to be so aggressive? Or are these sort of deals you also look at and you decide not to do it?

Ashutosh Raghuvanshi: No. The answer is very clear that we are here for good quality assets, which fit into our regional cluster strategy, as well as it is a value acquisition. So, we are going on that path, and we certainly are very active in the market evaluating some of these opportunities, and we would certainly be looking at creating some of these things pretty soon. We do have some things in the pipeline, which we are evaluating, and we would come back at the right time to inform you about them.

Bino Pathiparampil: Understood. Is that more on the M&A side or you are looking at land acquisition in large cities as well?

Ashutosh Raghuvanshi: We are looking at land as well because there are certain areas where we are seeing a lot of development and we feel that there is a future potential, whereas no assets would be available in those geographies. So in such areas, we are scouting for land as well.

Bino Pathiparampil: Got it. Second question from Diagnostics. These one-off expenses of rebranding, which are continuing now for quite a few quarters. How long do we expect this to continue?

Anand Kuppaswamy: We had planned to spend about INR 50 crores on rebranding this year. So I think over the last quarter and maybe a little bit flow through into next quarter may happen. So it will be in Q4 is what the major balance will happen in the Q4 respect.

Bino Pathiparampil: And next year, it won't be there or...

Anand Kuppaswamy: We have as such not planned anything as of now. So we will -- as of now, there's no one-off expenses planned. It will be part of the regular budget. But in case if we are having any carryover from this, we'll be doing it in the first quarter of next year.

Bino Pathiparampil: Understood. Okay. Just a couple of bookkeeping questions. This quarter, the consolidated tax rate was very low at 9% or so. Why was it low? And does it change the annual consolidated tax rate?

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Vivek Goyal: Yes. So we have created deferred tax asset on certain unabsorbed losses accumulated for one of our subsidiary. Actually, if you might be knowing 3 years back, we have stopped recognizing deferred tax asset till we see the visibility of profitability. Now it is clearly visible. So this quarter, we have booked a deferred tax asset of around INR 27 crores. So that is the only anomaly this quarter.

Bino Pathiparampil: Okay. So next quarter, the tax rate should go back to around 25%?

Vivek Goyal: Yes, you're right.

Bino Pathiparampil: Okay. And going forward, next year also likely around 25%?

Vivek Goyal: Yes, yes.

Moderator: The next question is from the line of Atul, an Individual Investor.

Atul: Yes. My question is on the Fortis brand itself, if we are able to acquire that brand?

Ashutosh Raghuvanshi: Yes. So that brand was auctioned quite recently through a court process. So that process is on. We did -- we were declared a successful bidder. However, the court needs to confirm that sale, and that process is currently on. There are some objections by the original brand holder. So we are expecting that this matter will get resolved in the forthcoming

couple of months or so.

Atul: Okay. And update on the legal side of the open offer, which is stalled?

Ashutosh Raghuvanshi: So on the legal side, the high court hearings have been sort of almost concluded. And I think

there would be a couple of hearings more. Post that, we should expect some kind of a decision from the high court about the forensic audit. Other than that, there are no major cases per se. Atul: Okay. So are we expecting the legal cost to kind of taper down from the quarter starting next year, right?

Ashutosh Raghuvanshi: So I think the early part of next year also, there will be some significantly higher legal costs. But post that, we should see these costs coming down.

Vivek Goyal: It will also depend upon the outcome of the court cases. So if the outcome is favourable, that will come down. And it all depends on how many hearings are happening.

Atul: Okay. So in terms of like your long-term vision of being a 10,000 operating bed organization, right, what will be the run rate of acquisition of these other opportunities which are there in the market, like, for example, in years like you might be targeting 2 or 3 acquisitions? So any road map on that front?

Ashutosh Raghuvanshi: As I said earlier, we are always actively evaluating the opportunities. We cannot give a kind of a particular time frame in which we will achieve this. But definitely, there are some individual assets which are coming at good value. So we will evaluate those. There may be some other opportunities which might come our way. So since this is more of an opportunistic kind of an approach, it will be difficult to spell out the exact yearly kind of number.

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But we are pretty confident that overall, over the period of next 5 years, we should be able to get to our target.

Moderator: The next question is from the line of Tausif from BNP Paribas.

Tausif: Congrats on good set of numbers. Sir, we have seen 17% growth this quarter international patient. Can you throw some color? And also, if you can share with us what is the contribution from patient from Bangladesh for Fortis?

Ashutosh Raghuvanshi: Yes. So Bangladesh numbers are not very significant for us. We do get some patients from Bangladesh in Calcutta. But overall, our number from Bangladesh is small. However, this growth is primarily from Central Asia and as well as from the Middle East and partly from Africa. But if you notice that overall revenue contribution from last year to this year hasn't grown. So, though, in absolute terms, this business has grown, but as a proportion to the overall business, it has remained almost at the same level.

Tausif: What's the current status between India and Bangladesh medical tourism? Has it been completely stopped?

Ashutosh Raghuvanshi: No, no. It doesn't get completely stopped, but because of the difficulties in travel, et cetera, the numbers have significantly come down in other places also. That's what we have heard from the industry.

Moderator: Does that answers your question, Mr. Tausif?

Tausif: Yes, that answers my question.

Moderator: The next question is from the line of Sneha Jain from SKS Capital.

Sneha Jain: Sorry if I missed it, what would be your margin guidance for businesses basically Hospital and Diagnostics?

Vivek Goyal: Yes. So we are maintaining our margin guidelines where we have said for the current financial year, we should be for Hospital business around 20.5% and for the Diagnostic business around 21%, 22%.

Sneha Jain: Okay. And sir, do you have any capex guidance?

Vivek Goyal: So, capex for all the brownfield projects that we are moving forward and there is a capex requirement for normal maintenance also. So, total capex need for a year is around INR 900 crore, which includes expansion-related need of around INR 600 crore and INR 300 crore

is basically maintenance capex.

Moderator: The next question is from the line of Nitin Agarwal from DAM Capital.

Nitin Agarwal: Congrats on a pretty good set of numbers. Sir, on the hospitals, we've outlined in the presentation of about 400 beds getting added on a brownfield basis in FY'26. Now, sir, if we take a slightly

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long view, will we have a similar sort of opportunities to add around 400 beds per year even for the next couple of years? Or this is -- how should we think about the period beyond FY'26?

Vivek Goyal: Yes. So we are estimating around this number, 350 to 400 type of number year-on-year. It all depends how quickly we can operationalize our brownfield expansion.

Nitin Agarwal: But, sir, there are enough opportunities for you to add incrementally another 1,000-odd beds on

a brownfield basis even after F Y'26?

Vivek Goyal: Yes. So around 350 beds.

Nitin Agarwal: Okay, sir. And sir, secondly, on the debt number that you've given out for December, does it take into account the full payout for PE buyout?

Vivek Goyal: Not really. Up to December, we have only acquired IFC stake. Other 2 private equity stake we acquired in the month of January. So after acquiring private equity stake, our gross debt is around INR 2,300 crores at consol level and net debt of around INR 2,000 crores.

Nitin Agarwal: That's our peak debt right now.

Vivek Goyal: That is the debt at pre sent, yes.

Moderator: The next question is from the line of Ankush Mahajan from Axis Securities.

Ankush Mahajan: Congrats for the good set of numbers. Sir, just try to understand what is the revised guidance for the hospital margins?

Vivek Goyal: Yes. I just mentioned 20.5% we should be closing for the full year.

Moderator: The next question is from the line of Shyam Srinivasan from Goldman Sachs.

Shyam Srinivasan: Just on overall growth guidance, top line, like I think Dr. Raghuvanshi said at the start, we're doing a little better in terms of at least ARPOB. So how should we look at, say, medium -term growth for us, like '25, '26, '27? I'm just asking a range. And can we still pencil in like 8% to 10% ARPOB in there and the rest coming from, say, volume growth, both bed expansion as well as same store?

Vivek Goyal: I will say ARPOB growth will be in the range of 5% to 6% going forward. And I'm first talking of the Hospital business, and our revenue growth should be somewhere around 14% year -on-year. So balance is coming from the bed expansion and utilizing the existing capacity better.

Shyam Srinivasan: Understood. So 14% to 15% kind of medium -term growth is what you're guiding to, right, Vivek?

Vivek Goyal: Yes.

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Shyam Srinivasan: Understood. Okay. And if I were to take your -- the question further on the margin guidance, 20.5% for this year, do you think there is scope for further margin expansion in the upcoming years?

Vivek Goyal: Yes, we are targeting margin expansion. And hopefully, we'll be seeing margin improvement year-on-year. So our ultimate target is to reach to 25% sooner than later.

Shyam Srinivasan: So, Vivek, we have kind of seen a plateau of like between 19% and 20% for a few quarters now. So what can do the step change, right? You have also divested, let's assume, the low -hanging fruit. So what can materially take us from, say, 20.5% or 21%, let's assume, to 25%? What are the key things that need to work? Or what are the key things you're pushing at?

Vivek Goyal: So I think one is, this brownfield bed expansion and availability of the bed because as I mentioned earlier also, these capacities are coming in the units which are already operating at 75%, 80% occupancy. So we see less challenge in filling those beds. That is number one. Number two, the existing hospital - some of our existing hospitals are still

operating at a low occupancy level.

These are big hospital, very good location, very good clinical talent we have in those and good equipment. So I think when these hospitals will also grow in terms of occupancy, that should add to the EBITDA margin. And so, these are the 2 basic levers which I am banking upon on the EBITDA margin improvement, which will provide us overall efficiency in the cost and other things.

Shyam Srinivasan: Got it. And my last question is on the top line growth prospects for diagnostic services. So I think, Anand, you mentioned that we can come back to growth in a couple of quarters and maybe target like a double -digit growth, like 9%, 10%, let's assume. So how can we again -- if you can kind of split that into -- are you contemplating any pricing action? I don't know whether Agilus has taken any price increases, most of our peers have taken over the last 2 to 3 years.

And in terms of volume growth, we have lagged. So what are some of the efforts that we will likely see on the volume growth post the rebranding exercise that can push up our volume growth higher?

Anand Kuppaswamy: Thanks, Shyam. So on the growth side, we are seeing consistent changes and improvements on a quarter-to-quarter basis. Every quarter, we are seeing that compared to the previous quarter.

So, as I told earlier, so the growth, what we are expecting somewhere around second quarter of next year is about 8% to 10% kind of overall growth, out of which it will be largely driven by volume and to some extent, by value, which will be due to the intensity of tests in the sense that the mix of tests that will have an influence on that.

As of now, price increase, we have taken a price increase in February of '24. We have taken a price increase. So I think we are not contemplating any price increase as of now. So you can

imagine that the large part of this growth is going to be volume growth.
Moderator: The next question is from the line of Madhav from Fidelity.
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Madhav: My question was on the Hospital business. Manesar, given this is the first full quarter of operations, is it fair to assume that we would have booked our peak opex losses in this facility this quarter and this should keep coming down? And if you could quantify the opex loss in Q3 at Manesar?

Vivek Goyal: Yes. So Manesar, we have operationalized major operation in this quarter itself. And as a result, there is an operating loss we have booked INR 12 crores to INR 13 crores during this quarter.

Madhav: Got it. And you said that this facility should breakeven at EBITDA level Q1 of next year. So that's June quarter, basically, we expect to breakeven at this facility?

Vivek Goyal: Yes, June, September quarter, it should be breakeven.

Madhav: Got it. That's encouraging. And I think we just wanted to clarify that this is the only greenfield expansion in the next 2, 3 year pipeline, right? The remaining -- all the beds which we are adding are all brownfield in nature for Fortis, at least what has been announced so far?

Vivek Goyal: Yes, absolutely. This is the only greenfield in nature.

Moderator: As there are no further questions from the participants, I now hand the conference over to the management for closing comments.

Anurag Kalra: Thank you, ladies and gentlemen, for joining us on our quarter 3 FY '25 earnings call. In case there are any follow-up queries, Amit and myself are available to speak to you over phone or over e-mail. Please do feel free to reach out to us. Thank you very much, and have a good day.

Moderator: Thank you. On behalf of Fortis Healthcare Limited, that concludes this conference. Thank you for joining us, and you may now disconnect your lines.

Call: Feb 2025

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February 14, 2025

FHL/SEC/202 4-25

The National Stock Exchange of India Ltd.

Scrip Symbol: FORTIS BSE Limited

Scrip Code:532843

Sub: Transcript of Investors / Analysts' meet under Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015

Dear Madam/Sir,

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed transcript of Investors / Analysts' meet held on February 10, 2025 to discuss the Company's Un -Audited Financial Results for the quarter and period ended on December 31 , 2024 and same is available on the Website of the Company at below hyperlink: -

<https://www.fortishealthcare.com/investor/investor%20presentations%20&%20transcripts/earnings%20call%20transcript%20for%20the%20quarter%20ended%20december%202024>

The date and time of occurrence of event is February 10, 2025 at 2.30 p.m.

This is for your information and records.

Thanking you,

Yours Sincerely ,

For Fortis Healthcare Limited

Satyendra Chauhan

Company Secretary & Compliance Officer

M. No. – A14783

Encl: as above

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"Fortis Healthcare Limited "

Q3 FY '25 Earnings Conference Call "

February 10, 2025

MANAGEMENT : DR ASHUTOSH RAGHUVANSHI – MANAGING
DIRECTOR AND CHIEF EXECUTIVE OFFICER – FORTIS
HEALTHCARE LIMITED
MR. VIVEK GOYAL – CHIEF FINANCIAL OFFICER –
FORTIS HEALTHCARE LIMITED
MR. ANURAG KALRA – SENIOR VICE PRESIDENT ,
INVESTOR RELATIONS – FORTIS HEALTHCARE
LIMITED
MR. ANAND KUPPUSWAMY – CHIEF EXECUTIVE
OFFICER – AGILUS DIAGNOSTICS LIMITED
MR. AKSHAY TIWARI – CHIEF FINANCIAL OFFICER –
AGILUS DIAGNOSTICS LIMITED

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Moderator: Ladies and gentlemen, good day, and welcome to the Fortis Healthcare Limited Q3 FY '25 Earnings Conference Call. As a reminder, all participant lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touch-tone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Anurag Kalra, Senior Vice President, Investor Relations at Fortis Healthcare Limited. Thank you, and over to you, sir.

Anurag Kalra: Thank you very much. A very good afternoon and good evening, ladies and gentlemen, and thank you for taking the time to join us on our quarter 3 FY '25 earnings call. The call is being chaired by our MD and CEO, Dr. Ashutosh Raghuvanshi, with him, we have our Chief Financial Officer, Mr. Vivek Goyal. From Agilus Diagnostics side, we have Mr. Anand, the CEO of Agilus Diagnostics, as well as Mr. Akshay Tiwari, the CFO.

We will start with some opening comments by Dr. Raghuvanshi, post which Anand will take you through certain key highlights of the Diagnostics business, and then we can open the floor for question and answers. Over to Dr. Raghuvanshi.

Ashutosh Raghuvanshi: Thank you, Anurag. Good evening, everyone, and thank you for taking the time to join us on our Q3 financial year '25 earnings call today. Before diving right into the numbers and sharing my thoughts on the business performance, I would like to highlight that our performance in Q3 financial year '25 and the 9 months financial year '25 continues to witness a healthy improvement over the corresponding previous year, led largely by our Hospital business. Our performance in Q3 has been quite satisfactory, showing a significant improvement over the previous year. This achievement has been driven by strong performance of our Hospital business. We reported a consolidated top line figure of INR 1,928 crores, a growth of 14.8% over Q3 of financial year '24. Noticeably, our Hospital business revenues have grown 16.8% to INR 1,623 crores, while Q3 financial year '25. Diagnostic business growth revenues were at INR 342.3 crores versus INR 331 crores in Q3 of financial year '24.

Our consolidated operating EBITDA increased 32% to INR 375 crores, delivering a margin of 19.4% versus 16.9% in Q3 of financial year '24. The Hospital business reported an operating EBITDA of INR 325 crores, driving a 200-basis point improvement in margin from 18% in Q3 financial year '24 to 20% in Q3 of financial year '25. The Hospital business revenue now accounts for 84% of our consolidated revenue and operating EBITDA accounts for 87% of our EBITDA.

In 11 of our facilities, we have reported operating EBITDA above 20% during the third quarter. These 11 facilities together contribute 74% to the hospital revenue during the third quarter and 9 months of financial year '25. In comparison to financial year '24, we had 8 of our facilities operating in EBITDA margin of about 20%, contributing 62% to the hospital revenue. Our consolidated reported profit after tax before exceptional item for the quarter increased 82.2% to INR 231 crores.

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For 9 months financial year '25, our consolidated revenue stood at INR 5,776 crores, up 13.1% versus financial year '24. The operating margin for 9 months for financial year '25 increased to 20% against 17.4% in the corresponding previous period. Nine months of financial year '25, Hospital business revenue increased by 15% to INR 4,827 crores. Operating margin for the Hospital business improved by 270 basis points to 20.0% for the year versus the previous year

of 17.3%.

Coming to the balance sheet side, we remain comfortable

and healthy with a net debt-to-

EBITDA of 0.41x as on December 31, 2024, as against 0.45x on December 31, 2023. Our net debt stands at INR 644 crores as on December 31, 2024. In December 2024, we successfully raised INR 1,550 crores through the issuance of nonconvertible debentures, leveraging these funds along with internal accruals, we consolidated our stake in Agilus by acquiring 31.52% stake from our private equity investors.

As a result, our company now holds a commanding 89.2% equity stake in Agilus as on date. Hospital occupancy improved to 67% compared to 64% in Q3 of financial year '24. This translated into occupied beds increased by 6.2% to 2,790 beds compared to 2,627 beds in Q3 of financial year '24. Our Hospital business saw a 9.9% increase in ARPOB, reaching INR 2.45 crores per annum.

This growth was largely driven by revenue gains in our key specialties such as Oncology, Neurosciences, Cardiac Sciences, Gastroenterology, Orthopedics and Renal Sciences. Collectively, these specialties achieved a 17% year-on-year growth and contributed 62% to the overall Hospital business revenue compared to 61% in Q3 of financial year '24.

To highlight, the Oncology specialty registered a growth of 30% and neurosciences reported a growth of 18% year-on-year. Growth in oncology was led by an increase in revenue from Hematology and Bone Marrow Transplant by 44% compared to the same period last year. To highlight, key surgical procedure volumes performed across neurosciences and robotic surgeries witnessed a strong growth of 23% and 77%, respectively, compared to the corresponding previous period.

Our revenues from medical travel grew 17% compared to Q3 of financial year '24 to reach INR 132 crores. Revenue contribution of international business stood at approximately 8%, which is similar to Q3 of financial year '24. The company's key facilities such as Shalimar Bagh, FMRI, Faridabad, Ludhiana, FEHI, Mulund and Nagarbhavi, registered a revenue growth in excess of 20% compared to the corresponding previous year.

Continuing with the portfolio rationalization strategy, we divested business operations of Richmond Road Hospital in Bangalore in December of 2024. This divestment supports our focus on improving overall profitability and margins. This is the third facility divested by the company after the divestment of Malar facility and Vadapalani facility in Chennai.

Revenues from digital channels where website, mobile applications and digital campaigns witnessed a 36% year-on-year growth in Q3 of financial year '25. Digital revenues contributed to approximately 29.9% to overall hospital revenues versus 25.7% in Q3 of '24.

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On the Diagnostic business front, operating EBITDA margin stood at 14.4% versus 10% in Q3 of financial year '24. Excluding one-offs, the operating EBITDA margin stood at 21.3% versus 18.3% in Q3 FY24. As part of our ongoing network expansion strategy, the total number of new customer touch points reached 4,126 as of December 31.

The preventive portfolio revenues in Agilus overall revenue grew 17% in Q3 and contributed 10% to the operating revenues versus 9% in Q3 of financial year '24. The Diagnostic business performance is still adjusting to the impact of Agilus rebranding exercise, which involved extensive rebranding efforts and associated marketing costs. We expect the branding expense to taper off towards the end of this financial year.

That said, I'm confident in Agilus' potential to scale up both in terms of its revenue and margins based on its considerable network presence, a balanced B2C and B2B mix and the increased focus on preventive care, as well as specialized testing. To also add, I do believe that the Agilus brand is being well accepted and is gaining recognition. This would place the company in a better position to further scale our performance.

With this, I will conclude my comments. We are making significant progress on the growth at all possible fronts, leveraging our robust balance sheet, we will continue to explore and access various growth opportunities that align with our cluster strategy and offer promising synergies. I believe these initiatives will further enhance our growth potential and strengthen our position in the health care sector.

Thank you, and I will now hand over to Mr. Anand for his comments now.

Anand Kuppuswamy: Thank you, Dr. Raghuvanshi. Good afternoon, everyone, and thank you all for joining us today.

On behalf of Agilus Diagnostics, I welcome you to our Q3 FY '25 results conference call. Agilus Diagnostics reported a revenue of INR 342.3 crores in Q3 FY '25, marking a 3.5% increase from INR 330.7 crores in Q3 of FY '24. Revenues for Q2 of FY '25 were at INR 372.5 crores.

Operating EBITDA stands at INR 49 crores in Q3 FY '25, up from INR 33 crores in Q3 FY '24

with margins at 14.4% and 10%, respectively.

Operating EBITDA for Q2 FY '25 stood at INR 80 crores, a 21.5% margin. Adjusted for one-off expenses, operating EBITDA in this quarter, that is Q3 FY '25 is INR 73 crores with a 21.3% margin versus INR 60 crores at 18.3% margin in Q3 FY '24 and INR 89 crores at 24% margin in Q2 of FY '25. For the 9-month period, revenue stands at INR 1,058 crores compared to INR 1,033.6 crores in the 9-month of FY '24, a growth of 2.4%.

Operating EBITDA stands at INR 185 crores during the period versus INR 162 crores in the corresponding period in the last financial year. Adjusted EBITDA before one-off expenses stands at INR 222 crores in 9 months of FY '25 versus INR 214 crores in 9-month of FY '24, which is a margin of 21.1% versus 20.7% EBITDA margin, respectively.

In the quarter 3, we conducted a total number of 10.29 million tests compared to 9.85 million tests in the corresponding quarter last year. In the 9-month period, we have conducted a total of 31.31 million tests. Agilus expanded its network significantly, adding over 160 customer touch points.

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points in Q3 of FY '25 and 500-plus touch points in the 9 months of FY '25. The B2C to B2B revenue mix stood at 51% to 49% in Q3 of FY '25.

From a product standpoint, revenue contributions of 33% from specialized testing, 57% from routine testing and 10% from our wellness portfolio. Our wellness portfolio revenue showed a 17% growth in Q3 FY '25 versus Q3 FY '24 and 17% in the 9 months period of FY '25 compared to the 9 months period of FY '24. Regional revenue contributions are 30% from North, 21% from West, 32% from South, 13% from East and 4% from international markets in Q3 of FY '25.

Over the past 9 months, we have expanded our test menu with cutting-edge advancements in onco diagnostics and genomics. We recently introduced precision assays for myeloid malignancies with an industry-leading 3-day turnaround time. Notably, we are the first lab in India to launch the FDA-approved Claudin 18.2 test for gastric cancer.

We have also launched other specialized tests like the lymphoma NGS panel, lymphoid leukemia panel and gut microbiome test in this quarter. Our strong focus on preventive wellness and genomics continues to drive innovation, while our efforts to strengthen the Agilus brand are progressing well. We remain committed to continuously enhancing the customer experience.

Thank you, and over to you, Anurag.

Anurag Kalra: Thank you, Anand. Ladies and gentlemen, that was it on the highlights of both the Hospital and Diagnostics business. I'm going to request the moderator to now open the line for question and answers, please. Thank you.

Moderator: The first question is from the line of Neha Manpuria from Bank of America.

Neha Manpuria: First, on the Manesar greenfield expansion, could you give us some color on what the losses were in this quarter? And how

should we think about ramp-up of the facility? Any guidance on when the facility can breakeven?

Vivek Goyal: Neha, Vivek this side. So Manesar facility, we have started almost at the beginning of this quarter. So, it is currently contributing around INR 5 crores per month revenue. And I think breakeven point will be somewhere around INR 9 crores per month.

Neha Manpuria: And we'll get there by when in your view?

Vivek Goyal: Neha, we are targeting this by first quarter next year.

Neha Manpuria: And this would be, I'm assuming when you're saying INR 9 crores per month, this would be on a much larger bed count, right? Because currently, I think you commercialized only about 60, 70 beds.

Vivek Goyal: You are right. So currently, the hospital is functional. We have opened only 53 beds right now. Maybe another 50 beds, we will be opening by March.

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Neha Manpuria: Okay. Okay. My second question is on the Richmond Road facility. I understand this is very small. So is it fair to assume that there isn't too much cost benefit or margin savings that we will see from the asset being divested like we saw the last 2 times?

Vivek Goyal: Yes, it is not that much because as you rightly said, it is a very small facility. It has been incurring losses for quite some time, which we tried to revive, could not do that. Now, as per the last year financial, it has incurred around INR 8 crore EBITDA level loss last year. So that much benefit we will be getting in the forthcoming years.

Neha Manpuria: Understood. And my last question is on Agilus. I understand you mentioned that the cost related

to rebranding will probably taper off by the end of this fiscal year. But if I were to think about the growth momentum and our target of helping this business grow in line with the industry, do you think that is possible in fiscal '26? Or would it take us much longer for us to normalize growth in this business? And what according to you, Anand, is required for growth to start improving in Agilus?

Anand Kuppuswamy: Neha, this is Anand here. So, I think we will be able to come back to the growth momentum in terms of industry by the second or third quarter of next year itself. Because as of now, this year, we have seen quarter -by-quarter, we are seeing growth in terms of how our core business grows because if you look at the things like public -private partnership and other things, we will see that our core revenue has grown quite well.

And it is growing. What I'm suggesting is, it's growing. Every quarter, we are seeing improvement in the growth. And we are hopeful that by the second quarter of next year, we would be able to reach industry levels of growth.

Neha Manpuria: That would be closer to 10%? Or should I take more like...

Anand Kuppuswamy: Correct.

Moderator: The next question is from the line of Shyam Srinivasan from Goldman Sachs.

Shyam Srinivasan: Just the first one on the ARPOB dynamics. We have for the 9 months, I think, grown 10%. Even for the quarter, we have grown close to 10%. So just want to understand what are some of the drivers of it? I think Dr. Raghuvanshi talked about some of the specialties. But just want to understand, is it just mix? Or is there some pricing element that is also there on a Y -o-Y basis that is helping us keep the ARPOB buoyancy where it is?

If I were to look at some of your peers, I don't see like a double -digit, if I can make it double -digit kind of growth for others. So are we starting off a different base? Or is there something micro to Fortis that's leading to this buoyancy?

Ashutosh Raghuvanshi: So, Shyam, we have said in our commentary for last many quarters that we expect that the ARPOB growth should taper off a little bit, and it should probably be somewhere close to 6%, 7%. But it has been better than our expectation. And the reasons for that for us is

primarily case

mix. Out of this 9%, just maybe about 1.5% or so will be because of the price revisions. We

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have taken very minor price increases earlier in the year. And other than that, there is essentially the case mix, which has been happening.

As I was pointing out that some of the specialties like bone marrow transplants, et cetera, the growth has been as high as above 40%. And those are typically high ticket items. So that has led to this kind of growth. But I think we would expect that this kind of trend will persist for some more time, and we should see similar ARPOB going into the next couple of quarters as well.

Vivek Goyal: Yes. Just to add, Shyam here, apart from what Dr. Raghuvanshi said about specialty things, there is a lot of robotic work the company has done where the ticket size and ARPOB is slightly on higher side. Plus day care work has also gone up for us.

Shyam Srinivasan: Understood. That's helpful. Second question is just on the hospital margin metrics. In the 5 hospitals in the 9 -month data that are below 10%, you have -- I'm excluding Ludhiana 2 and Manesar. What are the other 3? And is this a possibility that we could look at divestments, some of these other 3 as well?

Vivek Goyal: So other 3, Shyam here is FEHI, Jaipur and Vashi. FEHI is a bottom line case. There was some exceptional expense of the previous year has been booked there. That's why it is coming below 10% type of margin. And Vashi and Jaipur, we have discussed in the past also, they are having their own problem. Jaipur particularly has not been able to come up. Vashi has come up to some extent, but Jaipur is still struggling.

Ashutosh Raghuvanshi: Yes. We are making some changes in these hospitals in terms of leadership, et cetera. And we expect that, that should yield some good results. We have seen a positive momentum in FEHI in the last quarter, which continues during the running quarter as well, where we are seeing the revenues improving. And as a result of that, a lot of that will flow to the EBITDA also. So certainly, we would be going above 10% here and slowly inching towards the 15% level. Jaipur, we have different plans. And certainly, we believe that, that hospital and the city has a potential, and we can revive it, but we will have to make some structural changes in terms of both manpower and the services offered.

Shyam Srinivasan: Got it. And my last question is just on the competitive intensity. A lot of beds being added by yourself or many of your peers. If you could kind of give us a little bit of how the industry landscape is looking like, especially on hiring and retaining talent? Are you seeing some kind of a war for clinicians, medical talent perhaps? And if you could share some of the attrition numbers, that will be helpful.

Ashutosh Raghuvanshi: Yes. Shyam, I think the talent acquisition and talent retention is an ongoing. I would say it is not

a war, but a battle, which keeps happening here and there. But I think overall, there is no reason for concern. One of the things in our case, the majority of the hospitals which are coming online are brownfield in nature. So we do not need that much of additional clinical talent, except for junior doctors and nursing staff and the support staff. So that kind of pressure is not there at the moment. But you are right that this is an ongoing thing.

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But what we have also noticed is that, in the last 10, 12 years, the number of specialists has started increasing because of all the reforms which have happened in the medical education for the last 15 years. And as a result of that, now you're seeing better supply. And the larger chains are not finding it so difficult. But I think as you go towards peripheral centers and smaller towns and cities, this problem becomes acute. But in places

like Delhi, Bombay, et cetera, it's not so

severe. There is some degree of competition, but not anything to be worried about.

Moderator: The next question is from the line of Bino Pathiparampil from Elara Capital.

Bino Pathiparampil: This is Bino from Elara. Yes, a couple of questions from my side. Dr. Raghuvanshi, we are seeing a lot of your competitors being very aggressive in expansion. They are buying a lot of land for new setups, greenfield setups in many of the big cities, plus they are aggressive on the M&A side as well. But we don't hear what is being so aggressive in expansion in the space. Is it that you don't intend to be so aggressive? Or are these sort of deals you also look at and you decide not to do it?

Ashutosh Raghuvanshi: No. The answer is very clear that we are here for good quality assets, which fit into our regional cluster strategy, as well as it is a value acquisition. So, we are going on that path, and we certainly are very active in the market evaluating some of these opportunities, and we would certainly be looking at creating some of these things pretty soon. We do have some things in the pipeline, which we are evaluating, and we would come back at the right time to inform you about them.

Bino Pathiparampil: Understood. Is that more on the M&A side or you are looking at land acquisition in large cities as well?

Ashutosh Raghuvanshi: We are looking at land as well because there are certain areas where we are seeing a lot of development and we feel that there is a future potential, whereas no assets would be available in those geographies. So in such areas, we are scouting for land as well.

Bino Pathiparampil: Got it. Second question from Diagnostics. These one-off expenses of rebranding, which are continuing now for quite a few quarters. How long do we expect this to continue?

Anand Kuppaswamy: We had planned to spend about INR 50 crores on rebranding this year. So I think over the last quarter and maybe a little bit flow through into next quarter may happen. So it will be in Q4 is what the major balance will happen in the Q4 respect.

Bino Pathiparampil: And next year, it won't be there or...

Anand Kuppaswamy: We have as such not planned anything as of now. So we will -- as of now, there's no one-off expenses planned. It will be part of the regular budget. But in case if we are having any carryover from this, we'll be doing it in the first quarter of next year.

Bino Pathiparampil: Understood. Okay. Just a couple of bookkeeping questions. This quarter, the consolidated tax rate was very low at 9% or so. Why was it low? And does it change the annual consolidated tax rate?

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Vivek Goyal: Yes. So we have created deferred tax asset on certain unabsorbed losses accumulated for one of our subsidiary. Actually, if you might be knowing 3 years back, we have stopped recognizing deferred tax asset till we see the visibility of profitability. Now it is clearly visible. So this quarter, we have booked a deferred tax asset of around INR 27 crores. So that is the only anomaly this quarter.

Bino Pathiparampil: Okay. So next quarter, the tax rate should go back to around 25%?

Vivek Goyal: Yes, you're right.

Bino Pathiparampil: Okay. And going forward, next year also likely around 25%?

Vivek Goyal: Yes, yes.

Moderator: The next question is from the line of Atul, an Individual Investor.

Atul: Yes. My question is on the Fortis brand itself, if we are able to acquire that brand?

Ashutosh Raghuvanshi: Yes. So that brand was auctioned quite recently through a court process. So that process is on. We did -- we were declared a successful bidder. However, the court needs to confirm that sale, and that process is currently on. There are some objections by the original brand holder. So we are expecting that this matter will get resolved in the forthcoming

couple of months or so.

Atul: Okay. And update on the legal side of the open offer, which is stalled?

Ashutosh Raghuwanshi: So on the legal side, the high court hearings have been sort of almost concluded. And I think there would be a couple of hearings more. Post that, we should expect some kind of a decision from the high court about the forensic audit. Other than that, there are no major cases per se.

Atul: Okay. So are we expecting the legal cost to kind of taper down from the quarter starting next year, right?

Ashutosh Raghuwanshi: So I think the early part of next year also, there will be some significantly higher legal costs. But post that, we should see these costs coming down.

Vivek Goyal: It will also depend upon the outcome of the court cases. So if the outcome is favourable, that will come down. And it all depends on how many hearings are happening.

Atul: Okay. So in terms of like your long-term vision of being a 10,000 operating bed organization, right, what will be the run rate of acquisition of these other opportunities which are there in the market, like, for example, in years like you might be targeting 2 or 3 acquisitions? So any road map on that front?

Ashutosh Raghuwanshi: As I said earlier, we are always actively evaluating the opportunities. We cannot give a kind of a particular time frame in which we will achieve this. But definitely, there are some individual assets which are coming at good value. So we will evaluate those. There may be some other opportunities which might come our way. So since this is more of an opportunistic kind of an approach, it will be difficult to spell out the exact yearly kind of number.

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But we are pretty confident that overall, over the period of next 5 years, we should be able to get to our target.

Moderator: The next question is from the line of Tausif from BNP Paribas.

Tausif: Congrats on good set of numbers. Sir, we have seen 17% growth this quarter international patient. Can you throw some color? And also, if you can share with us what is the contribution from patient from Bangladesh for Fortis?

Ashutosh Raghuwanshi: Yes. So Bangladesh numbers are not very significant for us. We do get some patients from Bangladesh in Calcutta. But overall, our number from Bangladesh is small. However, this growth is primarily from Central Asia and as well as from the Middle East and partly from Africa. But if you notice that overall revenue contribution from last year to this year hasn't grown. So, though, in absolute terms, this business has grown, but as a proportion to the overall business, it has remained almost at the same level.

Tausif: What's the current status between India and Bangladesh medical tourism? Has it been completely stopped?

Ashutosh Raghuwanshi: No, no. It doesn't get completely stopped, but because of the difficulties in travel, et cetera, the numbers have significantly come down in other places also. That's what we have heard from the industry.

Moderator: Does that answer your question, Mr. Tausif?

Tausif: Yes, that answers my question.

Moderator: The next question is from the line of Sneha Jain from SKS Capital.

Sneha Jain: Sorry if I missed it, what would be your margin guidance for businesses basically Hospital and Diagnostics?

Vivek Goyal: Yes. So we are maintaining our margin guidelines where we have said for the current financial year, we should be for Hospital business around 20.5% and for the Diagnostic business around 21%, 22%.

Sneha Jain: Okay. And sir, do you have any capex guidance?

Vivek Goyal: So, capex for all the brownfield projects that we are moving forward and there is a capex requirement for normal maintenance also. So, total capex need for a year is around INR 900 crore, which includes expansion-related need of around INR 600 crore and INR 300 crore

is basically maintenance capex.

Moderator: The next question is from the line of Nitin Agarwal from DAM Capital.

Nitin Agarwal: Congrats on a pretty good set of numbers. Sir, on the hospitals, we've outlined in the presentation of about 400 beds getting added on a brownfield basis in FY'26. Now, sir, if we take a slightly

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long view, will we have a similar sort of opportunities to add around 400 beds per year even for the next couple of years? Or this is -- how should we think about the period beyond FY'26?

Vivek Goyal: Yes. So we are estimating around this number, 350 to 400 type of number year-on-year. It all

depend on how quickly we can operationalize our brownfield expansion.

Nitin Agarwal: But, sir, there are enough opportunities for you to add incrementally another 1,000 -odd beds on a brownfield basis even after FY'26?

Vivek Goyal: Yes. So around 350 beds.

Nitin Agarwal: Okay, sir. And sir, secondly, on the debt number that you've given out for December, does it take into account the full payout for PE buyout?

Vivek Goyal: Not really. Up to December, we have only acquired IFC stake. Other 2 private equity stake we acquired in the month of January. So after acquiring private equity stake, our gross debt is around INR 2,300 crores at consol level and net debt of around INR 2,000 crores.

Nitin Agarwal: That's our peak debt right now.

Vivek Goyal: That is the debt at present, yes.

Moderator: The next question is from the line of Ankush Mahajan from Axis Securities.

Ankush Mahajan: Congrats for the good set of numbers. Sir, just try to understand what is the revised guidance for the hospital margins?

Vivek Goyal: Yes. I just mentioned 20.5% we should be closing for the full year.

Moderator: The next question is from the line of Shyam Srinivasan from Goldman Sachs.

Shyam Srinivasan: Just on overall growth guidance, top line, like I think Dr. Raghuvanshi said at the start, we're doing a little better in terms of at least ARPOB. So how should we look at, say, medium-term growth for us, like '25, '26, '27? I'm just asking a range. And can we still pencil in like 8% to 10% ARPOB in there and the rest coming from, say, volume growth, both bed expansion as well as same store?

Vivek Goyal: I will say ARPOB growth will be in the range of 5% to 6% going forward. And I'm first talking of the Hospital business, and our revenue growth should be somewhere around 14% year-on-year. So balance is coming from the bed expansion and utilizing the existing capacity better.

Shyam Srinivasan: Understood. So 14% to 15% kind of medium-term growth is what you're guiding to, right, Vivek?

Vivek Goyal: Yes.

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Shyam Srinivasan: Understood. Okay. And if I were to take your -- the question further on the margin guidance, 20.5% for this year, do you think there is scope for further margin expansion in the upcoming years?

Vivek Goyal: Yes, we are targeting margin expansion. And hopefully, we'll be seeing margin improvement year-on-year. So our ultimate target is to reach to 25% sooner than later.

Shyam Srinivasan: So, Vivek, we have kind of seen a plateau of like between 19% and 20% for a few quarters now. So what can do the step change, right? You have also divested, let's assume, the low-hanging fruit. So what can materially take us from, say, 20.5% or 21%, let's assume, to 25%? What are the key things that need to work? Or what are the key things you're pushing at?

Vivek Goyal: So I think one is, this brownfield bed expansion and availability of the bed because as I mentioned earlier also, these capacities are coming in the units which are already operating at 75%, 80% occupancy. So we see less challenge in filling those beds. That is number one.

Number two, the existing hospital - some of our existing hospitals are still

I operating at a low occupancy level.

These are big hospital, very good location, very good clinical talent we have in those and good equipment. So I think when these hospitals will also grow in terms of occupancy, that should add to the EBITDA margin. And so, these are the 2 basic levers which I am banking upon on the EBITDA margin improvement, which will provide us overall efficiency in the cost and other things.

Shyam Srinivasan: Got it. And my last question is on the top line growth prospects for diagnostic services. So I think, Anand, you mentioned that we can come back to growth in a couple of quarters and maybe target like a double-digit growth, like 9%, 10%, let's assume. So how can we gain -- if you can kind of split that into -- are you contemplating any pricing action? I don't know whether Agilus has taken any price increases, most of our peers have taken over the last 2 to 3 years.

And in terms of volume growth, we have lagged. So what are some of the efforts that we will likely see on the volume growth post the rebranding exercise that can push up our volume growth higher?

Anand Kuppaswamy: Thanks, Shyam. So on the growth side, we are seeing consistent changes and improvements on a quarter-to-quarter basis. Every quarter, we are seeing that compared to the previous quarter. So, as I told earlier, so the growth, what we are expecting somewhere around second quarter of next year is about 8% to 10% kind of overall growth, out of which it will be largely driven by volume and to some extent, by value, which will be due to the intensity of tests in the sense that the mix of tests that will have an influence on that.

As of now, price increase, we have taken a price increase in February of '24. We have taken a price increase. So I think we are not contemplating any price increase as of now. So you can imagine that the large part of this growth is going to be volume growth.

Moderator: The next question is from the line of Madhav from Fidelity.

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Madhav: My question was on the Hospital business. Manesar, given this is the first full quarter of operations, is it fair to assume that we would have booked our peak opex losses in this facility this quarter and this should keep coming down? And if you could quantify the opex loss in Q3 at Manesar?

Vivek Goyal: Yes. So Manesar, we have operationalized major operation in this quarter itself. And as a result, there is an operating loss we have booked INR 12 crores to INR 13 crores during this quarter.

Madhav: Got it. And you said that this facility should breakeven at EBITDA level Q1 of next year. So that's June quarter, basically, we expect to breakeven at this facility?

Vivek Goyal: Yes, June, September quarter, it should be breakeven.

Madhav: Got it. That's encouraging. And I think we just wanted to clarify that this is the only greenfield expansion in the next 2, 3 year pipeline, right? The remaining -- all the beds which we are adding are all brownfield in nature for Fortis, at least what has been announced so far?

Vivek Goyal: Yes, absolutely. This is the only greenfield in nature.

Moderator: As there are no further questions from the participants, I now hand the conference over to the management for closing comments.

Anurag Kalra: Thank you, ladies and gentlemen, for joining us on our quarter 3 FY '25 earnings call. In case there are any follow-up queries, Amit and myself are available to speak to you over phone or over e-mail. Please do feel free to reach out to us. Thank you very much, and have a good day.

Moderator: Thank you. On behalf of Fortis Healthcare Limited, that concludes this conference. Thank you for joining us, and you may now disconnect your lines.

Call: May 2025

FORTIS HEALTHCARE LIMITED

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FHL /SEC/202 5-26 May 26, 202 5

The National Stock Exchange of India Ltd.

Scrip Symbol: FORTIS BSE Limited

Scrip Code:532843

Sub: Transcript of Investors / Analyst's meet under Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015

Dear Madam/Sir,

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed transcript of Investors / Analysts' meet held on May 21, 2025 to discuss the Company's Audited Financial Results for the financial year ended on March 31, 2025 and same is available on the Website of the Company at below hyperlink: -

<https://www.fortishealthcare.com/investor/investor%20presentations%20&%20transcripts/earnings%20call%20transcript%20for%20the%20quarter%20and%20year%20ended%20march%202025>

The date and time of occurrence of event is May 21, 2025 at 12.00 P.M .

This is for your information and record.

Thanking You,

Yours Sincerely,

For Fortis Healthcare Limited

Satyendra Chauhan

Company Secretary & Compliance Officer

M. No. A14783

Encl :a/a

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"Fortis Healthcare Limited

Q4 FY '25 Earnings Conference Call "

May 21, 202 5

MANAGEMENT : DR. ASHUTOSH RAGHUVANSHI – MANAGING
DIRECTOR AND CHIEF EXECUTIVE OFFICER – FORTIS
HEALTHCARE LIMITED

MR. VIVEK GOYAL – CHIEF FINANCIAL OFFICER –
FORTIS HEALTHCARE LIMITED
MR. ANURAG KALRA – SENIOR VICE PRESIDENT ,
INVESTOR RELATIONS – FORTIS HEALTHCARE
LIMITED
MR. AKSHAY TIWARI – CHIEF FINANCIAL OFFICER –
AGILUS DIAGNOSTICS LIMITED

Fortis Healthcare Limited
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Moderator: Ladies and gentlemen, good day, and welcome to the Q4 FY '25 Earnings Conference Call of Fortis Healthcare Limited. As a reminder, all participant lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Anurag Kalra, Senior Vice President, Investor Relations at Fortis Healthcare Limited. Thank you, and over to you, sir.

Anurag Kalra: Thank you very much. Very good afternoon, ladies and gentlemen, and thank you for taking the time to join us on our quarter 4 FY '25 and FY '25 earnings call. The call is being chaired by our MD and CEO, Dr. Ashutosh Raghuvanshi. With him is Mr. Vivek Goyal, our Chief Financial Officer. From Agilus, we have Mr. Akshay Tiwari, the CFO of Agilus. Unfortunately, Mr. Anand could not join us today because he is currently in the midst of some travel overseas, but we'll be happy to take any diagnostic questions as well.

We will start off with some opening comments by Dr. Raghuvaran, post which we will open the floor for question and answers. Over to you, sir.

Ashutosh Raghuvanshi: Thank you, Anurag. Good afternoon, everyone, and thank you for taking the time to join this quarter 4 '25 and financial year '25 earning calls today. To begin with, I'm pleased to inform that the Board has recommended a dividend of INR 1 per share, which is equivalent to 10% of the face value for the third consecutive year, subject to the approval of shareholders. This highlights the strengthening fundamentals of company and its sustained earning growth momentum that we are witnessing.

Coming to the financial performance of the company, I shall comment on the year as a whole and then move on to quarter 4. We have witnessed another year of healthy growth and improved margins. The hospital business has been a primary driver of company's performance, consistently demonstrating year-over-year improvements in margins. For the financial year '25, consolidated revenues of the company stood at INR 7,783 crores, a growth of 12.9% over financial year '24.

Our hospital business revenues have grown 14.8% to INR 6,528 crores, while the financial year '25 diagnostic business gross revenues were at INR 1,407 crores versus INR 1,372 crores in financial year '24.

Our consolidated operating EBITDA increased 25.3% to INR 1,588 crores, which translates into a margin of 20.4% in financial year '25 versus 18.4% in financial year '24. The hospital business operating EBITDA margins have improved from 18.6% to 20.5% in financial year '25 with an EBITDA of INR 1,339 crores. The hospital business contributed approximately 84% to both our consolidated revenue and our consolidated EBITDA.

Consolidated reported profit after tax before exceptional items for the year increased 42.8% to INR 899 crores, while the reported PAT after exceptional items stands at INR 809 crores.

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On the quarterly performance, we reported a consolidated top line of INR 2,007 crores in quarter 4 of financial year '25, a growth of 12.4% over quarter 4 of financial year '24. The hospital business revenues have increased by 14.2% to INR 1,701 crores while the diagnostic business gross revenue stood at INR 348 crores in quarter 4 of financial year '25, compared to INR 338 crores in financial year '24.

The consolidated operating EBITDA in quarter 4 of financial year '25 increased 14.3% to INR 435 crores, delivering a margin of 21.7% versus 21.3% in quarter 4 of financial year '24. Operating EBITDA for the hospital business in quarter 4 of financial

year '25 grew by 11.7% to

INR 372 crores with a margin of 21.9% compared to operating EBITDA of INR 333 crores in

quarter 4 of financial year '24.

The figures for quarter 4 financial year '25 and '24 include certain year -end adjustments related to write -back of excessive provisions, unclaimed balances, expected credit loss and other adjustments, which are accounted for in the quarter, but pertain to the full year. Our consolidated reported profit after tax before exceptional items for the quarter increased 20.8% to INR 242 crores.

Coming to the balance sheet side , the company's net debt on 31st March , 2025 stood at INR 1,694 crores, while the net debt -to-EBITDA of 0.93x as on March 31, 2025, as against 0.17x on 31st March 2024.

In December 2024, we raised INR 1,550 crores through the issuance of non -convertible debentures. Leveraging these funds, along with internal accruals, we have consolidated our stake in Agilus by acquiring 31.52% stake from our private equity investors. As a result, FHL now holds 89.2% equity stake in Agilus.

Our hospital business recorded a 9% increase in ARPOB for the year, reaching to INR 2.42 crores per annum. This growth was primarily driven by the revenue gains in our key specialty areas such as oncology, neurosciences, cardiac sciences, gastroenterology, orthopedics and renal sciences, which together achieved a 16% year -on-year growth and contributed 62% to the overall hospital business revenues. Noticeably, the oncology specialty registered a growth of 25% and neurosciences reported a growth of 19% year -on-year.

Hospital occupancy improved to 69% in financial year '25 compared to 65% in financial year '24. This translates into occupied bed increasing by 5% to 2,838 compared to 2,700 beds in financial year '24. We also witnessed strong volume growth across key procedures in financial year '25 such as 72% growth in robotics surgeries and 17% in neuro and spine procedures. Most of our key facilities delivered strong performance this year with revenue of large facilities such as Shalimar Bagh and FMRI registering a growth in excess of 20 % compared to financial year '24.

In 10 of our facilities, we have reported operating EBITDA above 20%, both during the quarter 4 of financial year '25 and financial year '25 as well. These facilities, together, contributed 73% to the Hospital revenues during the quarter and during the year. In comparison, in financial year Fortis Healthcare Limited
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'24, we had 8 of our facilities operating within EBITDA margin of about 20%, contributing to 62% of hospital revenues.

Revenues from the International Patients for the quarter grew 17% to INR 145 crores, contributing 8.1% to overall hospital business revenues versus 7.9% in quarter 4 of '25. For the year '25, International Patients revenue grew 13% to INR 539 crores.

I'm pleased to share that financial year '25 has been marked by a significant development. Recently, the company successfully acquired the 'Fortis' brand and trademarks, for a consideration of INR 200 crores. We have also made significant progress in advancing our strategic growth levers, including inorganic growth, portfolio rationalization and brownfield bed expansion.

As part of company's inorganic growth strategy, Fortis signed definitive agreement in February 2025 to acquire Shrimann Superspecialty Hospital in Jalandhar, Punjab along with adjoining land parcel for INR 462 crores. This acquisition will add 228 beds to our network and offers the potential to increase the facility's total capacity to over 450 beds.

This transaction will allow us to further strengthen our presence in Punjab region from approximately 800 beds to over 1,000 beds . Then on the brownfield expansion, which is going to come in next 2 to 3 years in Amritsar and Mohali , this w

ould take the bed count to approximately 1,600 beds in Punjab. This transaction is expected to be consummated very shortly.

Continuing with the portfolio rationalization strategy, we divested business operation of Richmond Road Hospital in Bangalore in December of 2024. This is the third facility divested by the company after the divestment of Malar facility and Vadapalani facility in Chennai.

Our focus on bed expansion continues this year with Fortis Manesar, a 350 -bedded hospital facility commencing operations in September 2024, offering an entire spectrum of clinical services including all key specialties and latest state-of-art medical equipment. Including this facility, we added approximately 200 beds across our network with Shalimar Bagh and Anandpur being the other key facilities where the beds were added.

Our expansion strategy continues to focus on deepening our cluster presence. We plan to ramp up bed capacity by approximately 2,000 beds over the next couple of years. When completed, we can also expect to see some of our key facilities such as Shalimar Bagh, FMRI, Mohali and BG Road to become more than 450 beds each.

Focus on digital continues to remain core to our strategy, especially with the view to enhance patient care and operational efficiency. In financial year '25, we rolled out electronic medical record outpatient module in 12 additional facilities, bringing it to a total of 15.

Additionally, we began implementing the inpatient module of the EMR with the first implementation done at Fortis Manesar to start with and during the year we will proceed with other units as well. This enhances the real-time access to patient data for clinicians.

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Revenue from digital channels via website, mobile applications and digital campaigns, et cetera, delivered a strong growth of 35% year-on-year. Digital revenues contributed to approximately 29.6% to overall hospital revenue.

The company further augmented its medical infrastructure by commissioning several high-end equipment such as Gamma Knife, MR Linac and surgical robots in some of the key facilities.

This reaffirms our commitment to offering the most advanced treatment options and delivering precision-based treatment. Just to highlight, our capital expenditure in financial year '25 stood at approximately INR 700 crores, reflecting our confidence to further scale up operations both in terms of capacity expansion and augmenting medical infrastructure.

We expanded our clinical offerings bolstered by high-quality talent. The year saw the addition of reputed clinicians in various specialties, including neurology, cardiac sciences, oncology, gastroenterology, orthopedics and gynecology.

Turning to our diagnostic business. Agilus continues to recover and witness improvement in operating margins. Operating EBITDA margin basis gross revenue stood at 17.7% in financial year '25 versus 15.3% in financial year '24. Excluding one-offs, primarily related to rebranding expenses, the operating EBITDA margin stood at 22% in financial year '25 versus 19.6% in financial year '24.

For quarter 4, 2025 operating EBITDA margins basis gross revenue stood at 18% versus 14% in the corresponding previous period. Excluding the one-offs, the operating EBITDA margin stood at 23.4% in quarter 4 of '25 versus 16.3% in the corresponding previous period.

As part of our ongoing network expansion strategy, the total number of customer touch points reached 4,171 as on March 31, 2025. The preventive portfolio revenue in Agilus' overall revenues grew 13% in financial year '25 and contributed 11% to overall diagnostic business revenues compared to 10% in financial year '24.

We believe that Agilus has the potential to scale significantly from the current level, and efforts are in the way to strengthen its growth imperatives to drive revenues and optimize cost line

s. I

also believe that the Agilus brand is gaining strong acceptance and recognition in the market, positioning the company well to further scale its performance.

With that, I'll conclude my remarks. We are making strong progress across our strategic growth levers. I believe these initiatives will drive sustainable growth potential and strengthen our position in the health care sector.

With that, thank you, and I hand over to Anurag, please.

Anurag Kalra: Thank you, sir. Ladies and gentlemen, we will now open the floor for questions and answers.

May I request the moderator to take the lead, please.

Moderator: Thank you very much. We'll take our first question from the line of Neha Manpuria from Bank of America. Please go ahead.

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Neha Manpuria: On the hospital business, now that we have started seeing a fair bit of traction on the margin, how should we look at the margin expansion from 20.5%, 21% that we have reported in the current year? Should the step change in margins continue to get to that mid-teens where our peers are at?

And what -- given -- I think a large part of this was driven by brownfield, but just trying to understand where Manesar is in terms of ramp-up in bed capacity and profitability? And when do we think that gets to breakeven?

Vivek Goyal: Yes. This is Vivek. So, on margin expansion, we are sticking to our guidance earlier where we said that we want to achieve a margin expansion nearer to some of the best competitors. And we expect the margin to grow from the current level. You can expect like 2% growth in the forthcoming years also, similar to what we have seen in the current financial year. So similar margin expansion growth we are expecting next financial year.

Ashutosh Raghuvanshi: Regarding Manesar, we are currently operating, Neha, at about 40% occupancy, but we have

commissioned only 90 beds. Another 120 beds we will commission as the occupancy levels go up. We expect that on the entire bed capacity, which is 120 plus 90, we will have about 50% occupancy by the end of this year. The exit should be at least 50% -plus occupancy.

The uptake of this hospital has been very good. So some of the programs, which we had anticipated that we will start a little later, we are preponing them and we are going to put up the oncology, radiation oncology suite, which was planned earlier for 2 years from now within this year.

Neha Manpuria: So would it be fair to assume that at 50% occupancy, we achieve breakeven on this facility?

Ashutosh Raghuvanshi: I think so. Even before that, we should expect a breakeven.

Neha Manpuria: Understood. And my second question is on the diagnostics business. The revenue momentum remains in the low single -digit range, but margins have moved up. Is there any provision write -back, et cetera, which is sitting in the margin number? And how confident are we of growing in line with the industry or growing high single digits in the diagnostics business, given that now the Agilus brand changes nearly getting to 2 years old?

Vivek Goyal: Yes, Neha. So in diagnostic business also, we have seen now the increase in the revenue as well as the margin, which is reflecting in the financial. So, the brand change effect is now behind us, and we are seeing a double -digit type of growth number in the diagnostic business henceforth. And as regard to your other question regarding the one -off type of things. So this year, of course, there is a one -off of relating to the brand transition and some one -offs relating to the legal fees and some contingent consideration we have to pay for the past acquisition. Apart from that, I don't see much this thing. And this will discontinue from this year onwards, and '25 -'26 there will be normal type of EBITDA margin we are expecting.

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Neha Manpuria:

So Vivek, just to clarify here. So I think initially, our view was that this probably grows more high single digits. So you believe based on the Agilus momentum currently, we're confident of double -digit revenue growth. Is that correct?

Vivek Goyal: Yes. This is what we are targeting.

Neha Manpuria: Got it. And margins would be in the mid -20s as we scale up revenue?

Vivek Goyal: It should be around 23% ultimately and then moving towards 25% in a couple of years' time.

Moderator: We'll take our next question from the line of Shyam Srinivasan from Goldman Sachs.

Shyam Srinivasan: Just first one on the hospital business is how should we look at the revenue guidance for fiscal '26? And if you could break it down into volume and maybe ARPOB?

Vivek Goyal: Vivek this side. So, revenue -wise, we expect to grow around 14% -15%, similar number. And this time last year, the year we have completed, we have seen 9% type of ARPOB growth and balance growth is coming from the volume. So I'm expecting it will be the reverse this time, around 5%, 6% in the ARPOB growth and the balance is from the volume side.

And volume growth is mainly coming from some of the brownfield expansion, which has been completed and we will be operationalizing, which mainly include Noida and Faridabad and also from the capacity ramp -up in the form of better occupancy than the last year.

Shyam Srinivasan: Vivek, helpful. So we had about 5% volume growth. So you're saying this volume growth goes to like almost double, like 10%. So what are we basing in terms of for the occupancy? Should I model it on like all the expanded beds or we did end at 69% so just trying to see how we should model that?

Vivek Goyal: Yes, we are aiming around 70% -71% occupancy level at the overall level because this brownfield expansion is on the existing facility and these hospitals anyway operating at 50% type of occupancy level. So I think we will not be facing any challenge in occupancy side. And plus the Manesar facility, as Dr. Raghuvanshi alluded, it is ramping quite well.

Shyam Srinivasan: And just from a data point, how many operating beds we ended fiscal '25 and what is the expected addition this year?

Vivek Goyal: Yes. So, we ended fiscal '25 with a bed capacity of around 4,024 because we have taken out certain beds also in the Richmond Road. And we will be adding around, how much, almost 1,000 beds in the current time.

Shyam Srinivasan: So bed addition is like 25%?

Vivek Goyal: Yes. Because most of these capacities are coming. Like Noida, we have got full OC and we are operationalizing it. 150 beds we'll be getting. Then there will be Shalimar, sorry, Faridabad, which has completed and will be operationalized in the first quarter itself.

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Then Manesar, we will be opening further beds. We are expecting around 200 beds to be opened in Manesar.

FMRI, we will be completing it. Maybe the benefit we'll be getting only in the last quarter, FMRI, the 220 beds capacity expansion. And I will say BG Road is another one, which is we are expecting OC actually. And there is another capacity, I have added also the Jalandhar facility, which will be in our fold maybe by this month end also.

Shyam Srinivasan: And what's the margin profile there currently?

Vivek Goyal: They are on the similar margin profile.

Anurag Kalra: Shyam, their occupancy currently at about 60%, 62% and the margins are about 22 -odd percent.

Shyam Srinivasan: Understood. So just, Vivek, it's a lot of bed additions, right, brownfield, greenfield or acquisition. So we're confident of this 150, 200 bps of margin expansion for the hospital business this year?

Vivek Goyal: Yes, yes. So we are quite confident. And last year also, we have demonstrated 2% margin improvement and similar thing we are expecting this financial year. So we are quite confident on that.

Shyam Sr

inivasan: Got it. Last question, sorry, I've taken many questions. Just what is the quantification for this write-off and all the changes that you have mentioned? It's in the hospital revenues. Sorry, I didn't understand, I don't know whether I missed the opening comments on this one. What are the one-offs?

Vivek Goyal: Yes. One-offs is basically there is some impairment charge we have to take and that is mainly coming because of this Ludhiana 2 facility where the performance level is not up to mark, I will say. And as a result of that, based on the future profitability and cash flow, we have to adjust the carrying value of the assets.

So that, along with there is some write-off impairment we have to take for our investment in Sri Lanka assets where, as you know, the Sri Lanka stock price movement and the currency movement both affect the carrying value of the investment. So these are the 2 big items.

Apart from that, there is a positive item in this number also, where we have taken a positive -- sort of write-back of impairment, which we have done earlier for our Faridabad unit because the performance of Faridabad unit has improved tremendously. So, the total net impact is around INR 89 crores for this financial year. And for this quarter, it is INR 54 crores.

Shyam Srinivasan: So it was in the revenue line. And sorry, can you say where it's been accounted for? Sorry, I didn't understand that.

Vivek Goyal: So, it is shown separately below EBITDA, INR 54 crores as an exceptional item. Revenue and other thing is not much affected. It is, what is impacting is as an annual cleanup exercise. We do write off / write back certain unidentified receipt, which are more than 3 years old. And we also assess the provisions, which we have created in the books of fund, whether it is required, it's

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quite old and payable similarly. So those types of things, which are quite normal now, so which we are doing for last 3 years, and it is not having a significant impact also.

Moderator: We'll take our next question from the line of Deepthi Rajulapati from Axis AMC.

Deepthi Rajulapati: My question is on the diagnostic business. So last quarter, you have mentioned that we are done with rebranding exercises and these expenses will taper off in the end of FY '25. So we will not see any rebranding costs in the coming quarter or is there any carryover?

And one more question is on the variance of the margins in the diagnostic business. Revenue Q-o-Q, if we see, it's flat, but there is 2.1% variance in the margin. So, what is driving this? Just one more question is on the BG Road expansion on hospitals, how many beds are we expecting in FY '26?

Ashutosh Raghuvanshi: Yes. So regarding the first part of your question about the brand-related expenses, the one-off kind of expenses, which were there in rebranding, those are done already. So in this year, you will not find those as one-off items.

The second is about the margin expansion in spite of the revenue being flattish is that we have built in a lot of efficiencies in the lab network, the CTPs versus the lab network we have rationalized. There are several other initiatives on the cost side have been taken and that has resulted into this.

We have also upgraded our infrastructure on the diagnostics side quite a bit. We have opened a new lab of genomics in Gurgaon. At the same time, we have opened a new lab for the transplant immunology in Bangalore as well.

So with all these initiatives, we believe that high-end test volume with a higher ARPOB will also drive growth in the coming year. Regarding the number of beds in BG Road is 140 beds.

Moderator: We'll take our next question from the line of Bino Pathiparampil from Elara Capital.

Bino Pathiparampil: Most of my questions got answered, just one strategic question on the kind of expansion we are

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ing. If I look at most of your competitors, they are targeting Tier 1, Tier 2 cities with large 400-500 bed facilities. Whereas if I look at your acquisitions, be it Manesar, Jalandhar, et cetera, you are more going Tier 3, 4 and slightly smaller facilities. Is that the way forward for us?

Ashutosh Raghuvanshi: No. You see, we have already stated that our growth strategy is a cluster-based strategy. So as I was mentioning in my opening remarks also that in Punjab we have approximately 800 beds spread over 4 facilities, 2 in Ludhiana, 1 in Amritsar and 1 in Mohali.

Now all these hospitals, along with that, this is a strategic fit in Jalandhar and that's why we have chosen. But out of choice, we are not going into any new geographies, Tier 2 or Tier 3 kind of geographies. But Punjab as a state is important to us.

And then with the addition of these beds and the expansion, which we have planned in Amritsar and Mohali, and Jalandhar has a potential of 450 beds, so we will go to 450 beds there. Mohali, Fortis Healthcare Limited

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we are going to be adding another approximately 350 to 400 beds. Jalandhar, sorry, Amritsar, we are going to be adding another 150 beds.

So Punjab as a cluster, we are the dominant player already, but we will become even stronger once this happens. So that is our cluster strategy. The Manesar facility, I mean, just I would like to correct you is almost part of the growing Gurgaon. So this is not really a Tier 2 or Tier 3 kind of situation. This is a kind of a very upcoming area within the Greater Gurgaon area. And Manesar also has additional FSI, which will take this hospital size to 450 beds.

The other comment, which I made earlier is that many of our hospitals were smaller in size, but the larger hospitals, which we have, which are already doing EBITDAs of above 20%, most of them will also become 450-bed-plus after the brownfield expansion. So this will give us a profile of a hospital. I completely agree with you that the 450-bed hospital has completely different economics than a 200-bedded hospital.

So we believe in that. So we are not changing our strategy within these clusters where we are present, which is Bengaluru, Kolkata, Punjab, Delhi NCR and Mumbai -- Greater Mumbai area. In these clusters, we will be seeking more opportunities, and we would be looking at more hospitals and try to do hospitals, which are at least in the range of about 350-bed-plus.

Bino Pathiparampil: Understood. Just a related question or digging a little deeper. Mumbai, Bangalore, Kolkata are cities where we are already present. And in these cities, your competitors both listed as well as private probably in the last 2 years have -- 2.5 years have announced at least 4, 5 big projects reach, most of them greenfield, which means there is potential and there is land or whatever infra available to develop. But you haven't been so aggressive in those particular geographies in that sense. I mean, is there any reason? Or would you become more aggressive now?

Ashutosh Raghuvanshi: Yes. No, we were more focused on our brownfield expansion and execution thereof. And we had other distractions as well and brand was one of them, which we have behind us now. So we have been actively looking at opportunities, especially in Mumbai. But as you said correctly that most of these projects, which are announced are all greenfield. So we have also been looking for greenfields only because there are no acquisition targets per se available in these markets. And land costs being what they are in -- especially in Greater Mumbai area, which is of great interest to us and we believe has a lot of potential and a lot of demand supply gap, so there, that comes hard.

So we have been actively looking at it. Unfortunately, nothing has materialized so far. But we are pretty hopeful that at least few of those discussions we will be able to culminate i

nto actual

projects now. And those will be purpose-built hospitals.

Moderator: We'll take our next question from the line of Prashant Nair from AMBIT.

Prashant Nair: The first question is more of a clarification. When you mentioned that you expect diagnostics margins to get to 23% and then beyond that to 25%, you are -- this is basis gross revenues or net?

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Vivek Goyal: Net revenue only. Net revenue.

Ashutosh Raghuvanshi: Net.

Prashant Nair: All right. Understood. Secondly, in your network, now you have around 5 hospitals, which are in the sub-10% margin bracket. And then you have a few, which are the 10% to 15% bracket.

Do you have any other hospitals, which are not core to the way you look at the business now?

So you sold 3 assets over the last year or 2. So are there any more assets like this, which you would look to rationalize? Or do you think you're done with that exercise for now?

Ashutosh Raghuvanshi: So I think we do have work to do in a couple of our hospitals still in terms of improving, but we

believe that those are strategically important for us and exit is not an option. So I think, more or less, we are done as far as these rationalization is concerned.

But on the performance side, 2 hospitals, especially I will mention -- rather 3, where we are still working and we are seeing some good results. One is Escorts hospital in Delhi where our margin profile was less, but we have already consistently been achieving EBITDA of about 10% -14% and we are going above 15% over there, we have a clear visibility on that. So we are confident that this hospital will also come in to kind of a normal profitability profile.

And the other 2 units in this category are Jaipur and Vashi. Jaipur has had some issues, but now it is again recovering. The revenues trends look very healthy. We have made certain changes there, both in terms of the leadership and some of the infrastructure changes as well, and we expect very good results to come out of that. So, we expect that in 6 months to by the end of the year, we should have a healthy margin profile in Jaipur as well.

Vashi has some specific challenges because this is part of a government hospital. And that's why we have also got to serve certain free patients referred from the municipal corporation. So because of that, the profitability profile of this hospital has been low. We also had some attrition of clinician here. Because of that, the occupancy numbers were a little low.

But this is, again, an important market, important micro market. And we are working on strengthening the clinical talent over there to make sure that this also comes in the healthy zone. So that is regarding the 3 projects, which we are keeping under watch.

Prashant Nair: Just one follow-up question on Escorts Delhi. So would 15% -plus be close to what this hospital has achieved in the past when it has been part of Fortis? Or is the ceiling higher in your view?

Ashutosh Raghuvanshi: No, this is actually better than what it has ever achieved.

Vivek Goyal: And mind you, this is after absorbing the EWS bed cost because we have to provide 25 beds free of cost. So it is after observing that cost. And as Dr. Raghuvanshi mentioned, it is showing upward improvement in the EBITDA margin profile and we expect it should be settling somewhere around 15%, 16%.

Moderator: We'll take our next question from the line of Atul, an individual investor.

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Atul: My question is on any update on the Delhi High Court case further like as per last con call, it was discussed that from last conversations are pending. So is there any update on that side? And I will add another question, how is the occupancy trend going in the current quarter?

Ashutosh Raghuvanshi: Yes. So regarding the High Court, there is no fresh development. That is still a matter of sub-

judice, but that doesn't have much impact on us. Regarding the occupancy, the trend is healthy and we are doing occupancy levels of similar to what we did in last quarter.

Moderator: We'll take our next question from the line of Harsh Bhatia from Bandhan Mutual Fund.

Harsh Bhatia: Just on the hospital EBITDA part, you alluded to some one-off expenses, which are below the EBITDA line item and I'm probably referring to the hospital EBITDA. Just to be clear in terms of when we look at the fourth quarter performance on a Y-on-Y basis, the drag at the margin level is largely to do with the Manesar performance? Or is there any other one-offs that are there at the EBITDA line item? That was the first one.

Vivek Goyal: Yes. So one-off is not like one-off, one-off. It is exceptional gain and loss, which as per accounting standard only we have to show separately. So that is that one-off, I have explained what is those one-off exceptional gain or losses. So there are losses for Ludhiana 2 impairment and the impairment of investment in Lanka. And there is some impairment gain or impairment loss write-back actually for the Faridabad unit. So that is shown like an exceptional gain or loss. Regarding your other questions, the EBITDA margin side, there is no one-off, one-off. But provision for doubtful debt has slightly on the higher side if we compare with the quarter 4 of the last year. There was a positive write-back of provision in the last quarter previous year. And this quarter, it was not there.

Last quarter, we got very healthy collection in the last quarter itself, and as a result there, was write-back of provision, which we have provided based on the aging of the debt. So that has resulted into some improvement in the margins. So apart from that, there is nothing abnormal and unusual.

Harsh Bhatia: Sure. Would you be able to call out the number in terms of the recovery for the past quarter, which is not there for this quarter as well as the Manesar drag, if any?

Vivek Goyal: Yes. So Manesar - so first, I tell you about the Manesar drag. So Manesar, we have budgeted also the EBITDA loss of around INR20 crores for this, for the half year, it will be less.

Ashutosh Raghuvanshi: Correct.

Vivek Goyal: Yes. So, it will be around INR 12 crores type of EBITDA loss, which we have budgeted also, and it is there in the financial. So, INR 12 crores loss is built into this financial, which hopefully, in the next coming year, it should be some positive EBITDA number should be there.

And as regard to your quantification thing is concerned, the provision for doubtful debt for the quarter is INR 22 crores as against there was a negative provision for doubtful debt, that means the income side, INR 7.5 crores, corresponding quarter previous year.

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Harsh Bhatia: Sure. Just one clarification on Manesar. INR 12 crores was the drag from the third quarter as well and there's an INR 12 crores drag in the fourth quarter as well or...

Vivek Goyal: Just give us a minute.

Harsh Bhatia: Sure. Just one -- another point, when you look at the margin metrics and then we're guiding for the 200 bps margin expansion at the hospital level. In terms of your internal assumptions, how important are these hospitals such as Vashi, Jaipur? Basically, which hospitals are there below the 10% in your margin metrics important for you to achieve the 200 bps broader direction margin expansion?

This is something that has been -- not being considered to a large extent to provide you that, let's say, margin of safety to that extent. So basically, how important are these less than the 10% margins hospitals turnaround important to your guidance for the 200 bps margin expansion going forward?

Ashutosh Raghuvanshi: For these hospitals to really come to the category of 20 -plus is not something we have

considered. We believe that the turnaround of these 3 hospitals is going to take 6 months or 1 year or maybe longer. So we have not considered that when we say that we are expecting about 2% of increase in our profitability profile. So these hospitals are important strategically for the long term. But in short term, whatever guidance has been given is not considering that these hospitals have come to a 20 -plus category.

Harsh Bhatia: That's very helpful, sir. One last quick follow-up in terms of the bed addition for FY '26. A large part of that would be brownfield in nature or new towers as such. What should be the general break-even time line that we should work with? Should it be somewhere around 6 - to 9-month period or somewhere closer to 1 -year period? Again, these are towers at the existing site, so maybe could help us understand a bit.

Ashutosh Raghuvanshi: Yes. So since these are brownfield, absorption is pretty fast. Location to location, it will differ. However, we expect that we will open beds as the occupancy levels go up. Currently, these hospitals are operating about 75% to 80% occupancy levels. So we expect that this should happen in 6 months' time.

Moderator: We'll take the next question from the line of Amey Chalke from JM Financial.

Amey Chalke: Most of the questions are answered. Just one follow-up on the 900 -bed addition next year. So this bed addition, should we factor in back ended in the second half of this year? Or do you think that some of the hospital will get commissioned -- even some of the beds will get commissioned in the beginning of the year as well?

Vivek Goyal: Yes. We can -- we should assume 50 -50 because, as I mentioned, FMRI bed expansion is happening at the last quarter. And similarly, the BG Road also will be in the second half, we are expecting. And rest of the beds will be commissioned in the first quarter itself.

Amey Chalke: Okay. And particularly on the BG Road, the occupancy still looks around 60%. So is it ideal to open one more floor there? Or do you think we need to wait for that occupancy to go up?

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Ashutosh Raghuvanshi: Yes. So we are adding more clinical talent and modalities as well. But obviously, the beds will be ready for being commissioned. So capacity will be available to us. But we will keep on commissioning as and when the occupancy there will go up. But with the addition of clinical beds and modalities, we believe that the occupancy numbers will go up.

Moderator: We'll take our next question from the line of Abhishek Jain from Invest Well Agent. Please go ahead.

Abhishek Jain: Sir, the question is we have purchased Fortis brand under the auction. Can you tell me how much was the royalty you were paying earlier, both in absolute numbers and in percentage level? And the margin expansion that we are expecting for the current year of 150 basis points to 200 basis points, does this include the savings from this royalty payment? Or it is over and above the improvement that we are expecting?

Vivek Goyal: Yes. So, the royalty, as per the old agreement we are providing in the books till last year is 0.25% plus GST, which comes to around 0.3% impact on the EBITDA margin, positive impact on the EBITDA margin post acquisition of this brand. So that will be the impact of the brand acquisition, positive impact, 0.3% roughly on the net revenue of the hospital business. And that

has been factored in while I guided the margin expense of 2%.

Abhishek Jain: Okay. Fine, sir. And can you also give the same figures for this SRL brand and -- that we re converted into Agilus?

Vivek Goyal: Yes. So SRL also the same, 0.25% plus GST was the brand royalty applicable until we were using SRL brand. And now because we have moved to our own brand, Agilus brand, so there is no brand royalty right now.

Abhishek Jain: And the figure that you a

re saying is 0.3%, correct?

Vivek Goyal: Yes. 0.3% including GST because GST is applicable.

Moderator: We'll take our next question from the line of Abhishek Wani from Dalal Street Investment Journal.

Abhishek Wani: Yes. Sir, I want to know about the medical tourism prospective business for Fortis. So what has been the growth rate across the industry? And how has been Fortis working on it?

Ashutosh Raghuvanshi: Yes. So, we have seen about 17% growth in Quarter 4 on year -on-year. However, in the current geopolitical situation, we expect that there may not be a similar growth this year. But overall, to our context, about 8% of our revenue comes from international patients. We expect that to remain stable. However, we are not seeing very huge growth in this.

Abhishek Wani: And sir, with respect to the brownfield acquisition, so what impact it will have on our debt levels? And do we expect the margin to recover quickly as opposed to our competitors who have been working with greenfield acquisitions?

Vivek Goyal: Yes. So there will not be any incremental debt for brownfield expansion. It will be funded through internal accruals. So there will be no incremental impact. And as I mentioned in the Fortis Healthcare Limited

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earlier comment, this expansion is happening in the unit, which are already operating at 75%, 80% type of occupancy level. So we are -- we should not be facing much challenge in ramping up these beds and it should start contributing immediately.

Abhishek Wani: And sir, with respect to the legal costs, so can we expect that the legal costs will fall down in coming quarters as we already paid for the brand deal? So what is your outlook on legal costs?

Vivek Goyal: Yes. So the legal and other legacy cost is taking away almost 1% of our EBITDA margin. And I will say that will continue till we're able to resolve these court cases because there is still 1 court case pending in Delhi High Court where the regular hearing is happening.

And plus, the entity structure also, the Project Crystal, we call it, where we are trying to simplify the organizational structure. Although the Delhi NCLT has given the favorable order, we are also expecting order from the Chandigarh NCLT and then it will be simplified. So this year, at least it will continue. I think from next year onwards, we should see some reduction in this cost.

Moderator: We'll take our next question from the line of Nirali Shah from Ashika Institutional Equities.

Nirali Shah: Yes. I just had 2 very quick questions. The filing that we see today, I just wanted your view on that. IHH is increasing damages from Daiichi. So just wanted some color, some view on that.

Ashutosh Raghuvanshi: Yes. That's a litigation, which is happening in Japan between IHH and Daiichi. Being a subjective matter, we can't really comment on that.

Nirali Shah: Okay, okay. And about Fortis winning auctions for all the Fortis trademarks. So would that be any kind of value unlocking?

Ashutosh Raghuvanshi: Yes. So we -- as the previous caller was asking, we are going to save some money, which we were providing for the royalty for the brand because now we own the brand and that is a positive advantage for us.

Moderator: Ladies and gentlemen, we'll take that as the last question for today. I now hand the conference over to management for closing comments. Over to you.

Anurag Kalra: Thank you, ladies and gentlemen, for your time. If there are any follow-up questions, we are always available either through e-mail or on phone. Please do reach out to us. Thank you, and have a good day.

Moderator: Thank you. On behalf of Fortis Healthcare Limited, that concludes this conference. Thank you for joining us, and you may now disconnect your lines.

Call: Aug 2025

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August 11, 2025

FHL/SEC/202 5-26

The National Stock Exchange of India Ltd.

Scrip Symbol: FORTIS BSE Limited

Scrip Code:532843

Sub: Transcript of Investors / Analysts' meet under Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015.

Dear Madam / Sir,

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed transcript of Investors / Analysts' meet held on August 7 , 202 5 to discuss the Company's Un -Audited Financial Results for the quarter ended on June 30, 202 5 and same is available on the Website of the Company at below link:

<https://www.fortishealthcare.com/investor/investor%20presentations%20&%20transcripts/earnings%20call%20transcript%20for%20the%20quarter%20ended%20june%202025>

The date and time of occurrence of event is August 07, 2025 , at 1100 hours.

This is for your kind information and records.

Thanking you,

Yours Faithfully

For Fortis Healthcare Limited

Satyendra Chauhan

Company Secretary & Compliance Officer

ICSI Membership: A14783

Encl.: a/a

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"Fortis Healthcare Limited

Q1 FY26 Earnings Conference Call "

August 07 , 202 5

MANAGEMENT : DR. ASHUTOSH RAGHUVANSHI – MANAGING
DIRECTOR AND CHIEF EXECUTIVE OFFICER – FORTIS
HEALTHCARE LIMITED

MR. VIVEK GOYAL – CHIEF FINANCIAL OFFICER –
FORTIS HEALTHCARE LIMITED

MR. ANURAG KALRA – SENIOR VICE PRESIDENT ,
INVESTOR RELATIONS – FORTIS HEALTHCARE
LIMITED

MR. AKSHAY TIWARI – CHIEF FINANCIAL OFFICER –
AGILUS DIAGNOSTICS LIMITED

MR. ANAND K – CHIEF EXECUTIVE OFFICER – AGILUS
DIAGNOSTICS LIMITED

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Moderator: Ladies and gentlemen, good day and welcome to Fortis Healthcare Limited Q1 FY '26 Earnings Conference Call. As a reminder, all participant lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during this conference call, please signal an operator by pressing star then zero on your touchtone phone. Please note that this conference is being recorded. I now hand over the conference to Mr. Anurag Kalra, Senior Vice President, Investor Relations at Fortis Healthcare Limited. Thank you and over to you, sir.

Anurag Kalra: Thank you, Pari. A very good morning and good afternoon, ladies and gentlemen and thank you for taking the time to join us on our quarter 1 FY '26 Earnings Call. The call is being led by our CEO and Managing Director, Dr. Ashutosh Raghuvanshi. We have Mr. Vivek Goyal, our Chief Financial Officer. From Agilus, Mr. Anand joins us as the CEO and Mr. Akshay, who's the CFO of Agilus, is also here with us.

We will begin with some opening comments on the quarter gone by Dr. Raghuvanshi, post which Anand will take you through his highlights of the Diagnostics business, and then we shall open the floor for question-and-answers. Over to Dr. Raghuvanshi.

Ashutosh Raghuvanshi: Thank you, Anurag. Good morning, everyone, and thank you for taking the time to join us on our Q1 financial year '26 earnings call today. Before I take you through the financials, let me share some key business highlights and demonstrate the strength of our business going forward.

As part of our inorganic growth strategy, the company recently, through its wholly-owned subsidiary consummated the acquisition of Shrimann Superspecialty Hospital in Jalandhar, Punjab, which added 228 beds to its network. This transaction further strengthens our presence in Punjab from approximately 800 beds to over 1,000 beds. The acquisition also provides us with the opportunity to add another 225 bed by expanding the existing building and utilizing the adjacent land parcel taking the total to over 450 beds in the future.

In July 2025, the company entered into an operation and maintenance services agreement with Gleneagles India. Under the agreement, Fortis will manage the operations of approximately 700 beds across 5 hospitals and a clinic within the Gleneagles India network. Fortis is entitled to receive a monthly service fee at the rate of 3% of the net revenue.

This development marks a significant expansion of Fortis Healthcare's operational footprint and the expanded scale enhances our ability to deliver integrated high-quality health care services across more geographies. The combined strength of both the networks will help us leverage synergies and embrace efficiencies. With these additions, the company now operates 33 health care facilities, comprising over 5,700 beds across 11 states.

Coming to the financial performance, I would like to highlight that we have witnessed a healthy start to the financial year 2026. Our hospital business continues to perform well both in terms of revenue and margins. On the diagnostics front, we have seen an improvement in revenue growth and margins continue to trend upwards.

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We reported a consolidated top line figure of INR2,167 crores, a growth of 16.6% over the quarter 1 of financial year '25. Noticeably, our Hospital business revenues have grown 18.6% to INR1,838, while Q1 financial year '25 Diagnostics business net revenue have grown by 6.3% to INR329 crores versus INR309 crores in Q1 financial year '25.

The Hospital business revenue accounts for 85% of our consolidated revenue. Our consolidated operating EBITDA increased 43.2% to INR491 crores, delivering a margin of 22.6% versus 18.4% in Q1 of financial year

'25. The Hospital business reported an operating EBITDA of INR406 crores, which translates into a margin of 22.1% compared to 18.5% in Q1 of financial year '25.

Our consolidated reported profit after tax before exceptional items for the quarter increased 46.2% to INR254 crores. On the balance sheet front, the company's net debt stands at INR1,869 crores with a net debt-to-EBITDA of 0.92x as on June 30, 2025, as against 0.22x on June 30, 2024. The increase in debt was primarily due to the fund raised to partly finance the acquisition of 31.5% PE stake in Agilus Diagnostics by the company and acquisition of Fortis

brand and trademarks.

On the Hospital business, our AR POB increased by 10.2%. The increase reaching to INR2.65 crores per annum compared to INR2.41 crores per annum in Q1 financial year '25. The growth in ARPOB was driven by an improved specialty mix with oncology growing 28% year -on-year and contributing 16.4% to the revenues, up from 15.1% in Q1 financial year '25.

The other factors contributing to ARPOB growth included increasing share of complex cases as reflected by 75% year -on-year increase in robotic surgeries and also an improved payer mix with the share of institutional business at 20.3% compared to 20.9% in the same period last year.

Our occupancy improved to 69% compared to 67% in Q1 of '25. This translated into occupied beds increased by 7.8% to 2,928 beds compared to 2,715 beds in Q1 of '25. We are on track to add capacity of approximately 900 beds in the current financial year, including those at our recently acquired hospital in Jalandhar. We expect to operationalize approximately 50% of these beds in the current financial year.

In 11 of our facilities, we have reported operating EBITDA above 20% during the first quarter of financial year '26. These 11 facilities together contributed 75% of the Hospital revenues. In comparison to financial year '25, we had 10 of our facilities with operating EBITDA margin above 20%, contributing 73% to the Hospital revenue.

Several of our key hospitals such as Mohali, Noida, Anandpur, FEHI and Faridabad witnessed margin expansion compared to both corresponding and trailing quarters. In addition, many of our key facilities such as Shalimar Bagh, FMRI, FEHI, Jaipur and Faridabad registered revenue growth in excess of 20% compared to the corresponding previous period.

Revenue from international business grew 21% compared to quarter 1 of '25 and reached INR154 crores. The contribution of international business revenue stood at approximately 8%.

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in quarter 1 of financial year '26 on similar line as quarter 1 of financial year '25. Focus on digital continues to remain core to our strategy.

We have successfully implemented the inpatient modules of EMR at Fortis FEHI, the second such implementation after Fortis Manesar. This enhances the real-time access to patient data for clinicians. Revenue from digital channel via website, mobile application and digital campaigns witnessed a 16.8% year -on-year growth in Q1. Digital revenues contributed to 29.5% of the overall Hospital revenues versus 29.9% in quarter 1 of financial year '25.

The company further strengthens its medical talent with onboarding of specialists in the area of oncology, cardiac sciences, obstetrics and gynaecology, and renal sciences. We also augmented our medical infrastructure by installing second Da Vinci robots at our hospitals in Mohali and BG Road.

In the Diagnostics business, gross revenue grew 7.4% to INR369 crores compared to INR344 crores in Q1 of financial year '25. Operating EBITDA margin basis gross revenue stood at 23% versus 16.1% in Q1 of '25. Excluding one-offs, the operating EBITDA margin stood at 18.7% in Q1 of financial year '25.

As a part of our ongoing network expansion strategy, the total number of new customer touch points reached 4,261 as

of June 30, 2025. The preventive portfolio revenues in Agilus revenue grew 8.4% in Q1 of financial year '26 and contributed 12% to the operating revenues. We have witnessed a steady recovery in both revenues and operating EBITDA margins.

This is reflective of the brand building initiatives undertaken over the last few quarters. We expect this growth momentum to continue going forward. With a considerable network presence, balance B2C, B2B mix and an increased focus on product mix, I'm confident about Agilus' potential to scale up further both in terms of revenue and margins.

With this, I will conclude my comments. I believe our Hospital and Diagnostics businesses are well positioned to maintain their positive trajectory. Leveraging the strength of our balance sheet, we will continue to explore and evaluate growth opportunities that align with our cluster strategy and offer promising synergies. Thank you and I will hand over to Mr. Anand for his comments now.

Anand K : Thank you, Dr. Raghuvanshi. Good morning, everyone, and thank you for joining us today. On behalf of Agilus Diagnostics, I welcome you to our Q1 FY '26 results conference call. Agilus Diagnostics reported a gross revenue of INR368.8 crores in the Q1 of FY '26, reflecting a 7.4% growth compared to INR343.5 crores in Q1 of FY '25 and INR348.5 crores in Q4 of FY '25, a growth of 5.5% -- 5.8% compared to the trailing quarter.

Operating EBITDA for the quarter stood at INR84.7 crores, up from INR55.4 crores in Q1 of FY '25, with margins improving to 23% from 16.1% in the last year. In Q4 of FY '25, operating EBITDA was INR62.6 crores with a margin of 18%. During Q1 of FY '26, Agilus

conducted 10.1 million tests compared to 9.57 million tests in Q1 of FY '25 and 9.59 million tests in Q4 of FY '25.

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We added 10 labs and 160 -plus new customer touch points during this quarter, reflecting our continued focus on strengthening presence and accessibility across the geographies. The B2C to B2B mix stood at 51:49 in Q1 of FY '26 compared to 52:48 in Q1 of FY '25. From a product standpoint, revenue contributions for Q1 stood at 54% from routine tests, 34% from specialized tests and 12% from our Wellness portfolio.

On the geography front, revenues were driven by 31% from North, 31% from South, 20% from the West, 13% from the East and 4% from our international markets. We have also made significant enhancements to our digital platforms this quarter to elevate the customer experience including improvised digital processes that streamline internal operations and the launch of an in-house feedback management system to capture NPS feedback.

We have been focusing on next-generation diagnostics, and it has helped our genomics portfolio to grow by about 17% compared to Q1 of FY '25. We have further enriched our test portfolio with advanced offerings to support personalized diagnostics and patient care. We have launched around 30 tests in this quarter, including test in oncology and prenatal care. Some of the important tests that we have launched are Digitrack Bundle for monitoring npml, idh1 mutations, a critical tool in hematological malignancy management. The comprehensive drug assay panels which is designed for allergy testing, which can provide detailed insights for precise allergy profiling.

These new additions showcase our commitment to expanding cutting-edge diagnostic capabilities, driving precision medicine and improving patient outcomes. Thank you very much, and over to you, Anurag.

Anurag Kalra: Thank you, Anand. Ladies and gentlemen, we shall now open the floor for question-and-answers. May I please request the moderator for this.

Moderator: Thank you very much, sir. We will now begin the question and answer session. The first question is from the line of Amey Chalke from JM Financial.

Amey Chalke: Congrats to the management

on good numbers. So the first question I have on the Hospital performance, particularly the 5 hospitals: FMRI, Mohali, BG Road, Mulund and Jaipur, have seen a sharp uptick on the sequential numbers. Is it possible to highlight the reason for the same? And particularly in Jaipur and BG Road, what is the occupancy right now?

Ashutosh Raghuvanshi: Vivek?

Vivek Goyal: Yes, so this is Vivek. So you ask about Jaipur, FEHI and BG Road?

Amey Chalke: FMRI, Mohali, BG Road have seen a good sequential uptick in the revenues and -- as well as Jaipur and Mulund as well, if you can provide some reason for the same? And also, if you can provide the occupancy for Jaipur and BG Road?

Vivek Goyal: Yes, so Jaipur is operating around 65% occupancy now, and it has revived. Last year, it was having some challenges. We have discussed in the earlier call. So it has come out from that

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and it is now on the path of recovery. It has already achieved around double-digit EBITDA margin also. So that is on Jaipur.

As regard FMRI, it is doing quite well. FMRI EBITDA -- sorry, I'm talking EBITDA, the 20 beds we have added, and we're able to fill them quickly. And the occupancy level of FMRI is 80% level. BG Road, although the occupancy side, it is slightly struggling. It is at around 56%, 57%, but they're able to do some quality work. And as a result of that, the EBITDA margin are quite healthy. So -- any other unit you want to understand?

Amey Chalke: And Mulund?

Vivek Goyal: Mulund, again, the occupancy for the first quarter was not that good. It is below 60%; however, EBITDA margin is above 20%. So Mulund -- but it is recovering quite well, and we are quite hopeful. Both BG Road and Mulund will start showing good occupancy number going forward.

Amey Chalke: Okay. And on your profitability metrics, you have shown 1 unit has moved up in the 25% bucket. Is it possible to highlight, which is that unit?

Vivek Goyal: 25% bucket you are saying? Yes. So Ludhiana has moved up in -- it is now above 20%. And there are two units who have moved above 25% also, FMRI and Anandpur.

Amey Chalke: Sure. And the second question I have on the diagnostics side. The full margins -- this quarter,

we have reported very healthy margins, so you expect the -- your full year guidance of around 22% margins for the diagnostic could be easily achieved?

Anand K : Yes. Our margins, as we have seen, it will be in the range of about 22% to 23% is what we are expecting for the whole year as well.

Amey Chalke: Sure. So going ahead, you might expect some normalization of the margins in the upcoming quarters?

Anand K: So usually, the second quarter is a good quarter for us. And as you know, there will be some slight dip in the third quarter. And then the fourth quarter will again normalize. So on an average, we expect that the overall margins to be around in the range of 22% to 23%.

Amey Chalke: Sure. And on the consolidated margins, considering the hospital s are also at around 22%, you expect the consolidated margins to improve significantly over the last 1 year and if you have any guidance for the same?

Vivek Goyal: So we are sticking to our guidance, which we have provided in the beginning of the year, 2% margin improvement. We are excited with the first quarter number. And we'll see how the rest of the year will follow.

Moderator: The next question is from the line of Neha Manpuria from Bank of America.

Neha Manpuria: First on the Glenmark O&M contract that you announced. What is the game plan here because a few of the Glenmark -- sorry, a few of the Gleneagles facilities are not in our core cluster, Fortis Healthcare Limited
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like Hyderabad, Mumbai, Chennai. And this does not seem to include the Mumbai facility. So is that also at some point going to get included to the O&M contract? An

y color here?

Ashutosh Raghuvanshi : Yes. So Neha, for the starters, we have excluded the Bombay facility because that is a separate entity amongst the Gleneagles network and that would be considered separately. We would certainly explore the possibility of including that in the current arrangement.

Neha Manpuria: Understood. And what about the other facilities, sir? Because I think Gleneagles also has facilities in Hyderabad and Chennai, which technically aren't Fortis' core markets, given their exit in Chennai now. Seeing from an O&M perspective, how does Gleneagles, the O&M contract that we have signed, fit into the Fortis network?

Ashutosh Raghuvanshi: Yes. So it forms a new cluster for us. We definitely have no presence currently in the Fortis network in these markets. But we are going to double down on these markets and create further opportunities that we will explore. We do have a Gleneagles facility in Chennai, which does very high -end clinical work, has got fabulous clinical talent.

So we are going to build on the existing base of good clinicians we have available in this network, and these hospitals have been there around for a long time. So we will build further on that. And with the combined strength of Fortis and Gleneagles, we will be able to support them to perform better and, at the same time, we will get a lot of synergies, both on the clinical front and supply chain and other areas as well.

Neha Manpuria: Understood. And at the moment, given the service fees as a percentage of revenue, so we're not capturing any part of improvement in performance that you will see from an EBITDA perspective? Because what I understand is Gleneagles margins are significantly lower versus where Fortis Hospital is.

So do you see that changing as you improve profitability or is there any way we are capturing the upside that Fortis will be able to bring about in the Gleneagles from the improvement in profitability?

Ashutosh Raghuvanshi: Yes, so some of the facilities have underperformed, but the objective of this whole exercise is to get the economies of scale and get the synergies around the operations as well as supply chain, etcetera, and improve those profitability margins. However, for current, the arrangement is based only on a top line fee for Fortis. So that is the current arrangement.

Neha Manpuria: Understood. My second question is on the hospital business. I know you're sticking to your guidance of the 200 basis points margin expansion. But given how strong first quarter has been, second quarter usually tends to be stronger and that we are adding a lot of our brownfield capacity now. Could the margin surprise positively or what's keeping us at that 200 basis point margin expansion given the brownfield technically should have higher EBITDA?

Ashutosh Raghuvanshi: Yes. So you are right that the first quarter, which typically is subdued has been better. This is, of course, a result of our case mix change over a period of time and the new facilities, which we had added and the new modalities, which we had added over the last couple of years. So I
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think that trend will continue, so though we are maintaining our margin, but definitely, the momentum is strong and is expected to remain that way.

Neha Manpuria: Understood. And my last question on diagnostics. Given our presence in West and East is lower when it's as a percentage of the revenue mix that you give in the presentation, is there scope for Agilus to look at inorganic opportunities to improve performance or our focus is largely to grow the diagnostic business organically by adding more labs and touch points?

Anand K : Currently, we are looking at opportunities on a case -to-case basis based on the strategic fit and what kind of value we can get from that kind of an opportunity. But we are open to all geographies. It's not

that we're only specifically looking at certain geographies, especially in our focused geographies, we are looking at acquisitions that can help us build scale in that region. But we -- it's not that we are not open to any opportunity in a particular location. So it's not -- we're not going after any specific geographies.

Moderator: The next question is from the line of Shyam Srinivasan from Goldman Sachs.

Shyam Srinivasan: Just back again on the Gleneagles O&M. And if you look at the public disclosure from IHH, in terms of their overall India business margins and if I were to back out your Gleneagles, that's whatever -- I know I'm not talking only the 5 hospitals, but the overall, including the Bombay - Mumbai one, very low margins, Dr. Ashutosh, right?

So what are we trying to bring now in terms of operation and maintenance that is going to help improve this? Are these -- you mentioned about high -end specialty and all, but at 3.5% or 4% margins, something is missing, right? So what are the things that you need to bring on the table? And what is the ulterior motive, right?

Is it going to be eventually merged with us over time, right? In that case, the assets, at least at the starting point of margins seem to be pretty weak. So I just want to understand what the end game is on the Gleneagles facilities?

Ashutosh Raghuvanshi: Yes. So Shyam, these facilities have a good potential. They are located well in the micro markets they are in. And we believe that the full potential of these hospitals has not yet been realized. And that is a mix of a lot of things. One of the things which -- advantage, which we get in this kind of alliance is to get synergies in terms of supply chain and other operational metrics. So that clearly will be beneficial to improve the profitability profile of these hospitals. And that is the idea for giving the responsibility to manage this to Fortis. And at the same time, in the future, we will take things as they come. However, having said that, IHH has publicly stated multiple times that Fortis their main vehicle of growth in India, and there is a focus market for them. So definitely, all possible options will be on table at the right time.

Shyam Srinivasan: Just expressing a concern here, Dr. Ashutosh. If you have given bad assets and we overpay for that, I'm just saying from a valuation perspective, is something that I hope from a Fortis perspective, those things are taken care. So it's just wanted to mention that upfront.

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Ashutosh Raghuvanshi: Yes. No, absolutely. That is -- Fortis has maintained a very disciplined approach when it comes to acquisitions, and we will continue to be cautious on that. So you can be rest assured that whatever in the future happens will be done in a very transparent manner. Arm's length as well, of course.

Shyam Srinivasan: Yes. Just going on to my second question. We're getting 3 percentage net revenue starting in July, which is like INR20 crores, INR30 crores, I think. I'm just doing for full year. And is that already in our guidance? So did we -- when we guided for fiscal '26, is that -- I know you didn't announce it at the quarter end. But how should we look at the margin guidance? It should go higher now, right, versus what it was originally?

Vivek Goyal: Yes. So in that guidance, we have not considered this Gleneagles, of course. And whatever the earning will be because it will be part of the year, so that much it will be added up. Because as Dr. Raghuvanshi had mentioned in the -- for the earlier question, we will be accounting only that 3% of the net revenue and there will be some little bit cost...

Shyam Srinivasan: That goes -- yes, that goes directly to EBITDA, right, Vivek?

Vivek Goyal: Yes. yes. To that extent, the EBITDA margin will go.

Shyam Srinivasan: So -- okay. Sorry, I'm asking, so what is our current guidance? And then plus if I add 100 bps

is what it will end up, right? I'm just -- what is our 22 to 23 for our hospitals?

Vivek Goyal: It'll not be 100 bps, Shyam, because if you see the revenue you might be having some numbers...

Shyam Srinivasan: INR700 crores and 3% is INR20 crores to INR25 crores. I'm just making...

Vivek Goyal: Yes, so for the full year. And if you do percentage, it will be something around 0.2% to 0.3%.

Shyam Srinivasan: Understood. Okay. Great. Last question is on the diagnostic business. We've seen a turnaround

at least from a margin standpoint. But on growth again, there is a difference between gross revenue growth and net revenue. So maybe the gross to net ratio has changed slightly. And the other observation I had was when I look at volume growth and ASP increase, like test realization or even patient realization, it is higher than our revenue growth. So is there something I'm missing?

Anand K : The volume growth is roughly around 5%. And the average revenue per patient, that growth is about 4%. So totally, that's about 9% is what will come. But because of...

Anurag Kalra: So Shyam, the way we look at it, while the revenue growth is 9%, the way we calculate margins is that there's an element of other income also, so to make a like-for-like basis, EBITDA margins include revenue and other income, and that is the base.

Shyam Srinivasan: Anurag, I'm just talking revenue growth, I'm not looking at EBITDA yet. Revenue growth is 6%, net revenue growth ?

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Anand K : No, no. The operating revenue growth is 9%, which is 5% volume and 4% value. Whereas, what you see as gross revenue here, it includes other income as well.

Vivek Goyal: So Shyam, if I can clarify this 9% versus earlier 7.4%. In the last year financial, there is certain one-off income, which was booked. So if we take impact of that out, then the revenue growth what Anand is now mentioning is 9.3%. So actually, operationally, the revenue has grown by 9.3%, if we've taken out the impact of that one-off expenses, which we -- income which we have booked in the last quarter.

Shyam Srinivasan: No, no, this is helpful. This explains. Lastly, any one-offs in any of your margins, either in Hospital or Diagnostics for the quarter?

Vivek Goyal: No. Nothing. Nothing yet.

Moderator: The next question is from the line of Amit Goela from Rare Enterprises.

Amit Goela: Fantastic results, congratulations. Sir, one question I wanted to ask you. Sir, you said that you are expecting addition of 900 beds this year. So can we assume that you'll at least have 500 beds on your current occupancy of 65% to 70%, you will be having at least 500 to 550 paying beds next year, which can add a revenue of about INR1,500 -odd crores based on your current ARPOB?

Vivek Goyal: Yes. Mr. Goela, if I can answer this question, yes, out of 900 this 250 beds is for FMRI unit, which we will be completing by year-end only, December, January, sometime around that time. So no major revenue we are expecting from that. However, for Noida facility, 150; Faridabad, 50; and a little bit of capacity we are adding at other locations.

Those will be operating at a decent occupancy level. And there is another, say, 200 beds, we are expecting to open for Manesar facility, which, as you know, is a new facility. So there will be -- ramp-up as per new facility.

Amit Goela: Fair enough. So -- no, Vivek I'm saying for next year. So I'm saying for next full year, definitely, we can get 600 beds additional revenue?

Vivek Goyal: Yes, yes, 100%. Because all this expansion is coming at brownfield, so I think ramp-up will be quite fast.

Amit Goela: Yes. So then at least we can get -- based on your current ARPOB, we can see a revenue addition of at least INR1,500 crores here? I'm talking next year, '26-'27?

Vivek Goyal: Yes, I think so. We will be getting

around that.

Moderator: The next question is from the line of Nitin Agarwal from DAM Capital.

Nitin Agarwal: Sir, congratulations for a pretty strong performance. Dr. Raghuvanshi, just if you were to look at the last few quarters, you've had a pretty remarkable improvement in both the revenue and the operating profitability for the Hospital business. While you've been talking about sort of various measures you've been taking.

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If you can just probably take up -- just sort of summarize, if you were the three or four main things which have worked for -- which have begun to fall in place for the business over the last 4 or 5 quarters, and where do you still see opportunities for, and which are the major levers for growth barring the bed additions from here on, when you look at the business over the next, say, 2 years -- 2, 3 year perspective?

Ashutosh Raghuvanshi: Yes. I think there have been multiple factors which have led to this consistently good performance. One of the factors is that the investment, which was made in clinical manpower as well as in the infrastructure in last 3, 4 years, that has started yielding results. That is one of

the major drivers. And that also has resulted in the case mix change.

So we, across our network today, have more than 14 robots. And these -- all these robots have doing very large -- the growth has been 75% from last year to this year. So that kind of high - end work is growing. The second is that oncology, which we started investing about 6 years back, is yielding results and is growing at almost 27%, 28% CAGR.

So these kind of case mix change, which is happening is resulting in the increased ARPOB levels. And at the same time, we are working parallelly on the operational efficiencies and that is also helping to some extent. But I think we still have some more ground to cover, and there are certain areas, which we are working on at the moment.

Nitin Agarwal: And doctor, on the current network, is there an opportunity for us to -- I mean, a, I don't know if we -- I don't recall checking this number. Has there been a meaningful improvement in our ALOS and do you see opportunities to further shrink the ALOS in some of our busy hospitals?

Ashutosh Raghuvanshi: Yes. So the busy hospitals, our ALOS, we have definitely had some improvement, but it is not dramatic, but we have seen some minor improvement in some of these hospitals where the occupancy levels are above 75%, 80%. But overall, I think it has been pretty stable. Going forward, we definitely remain focused on that.

And as I was saying earlier that robotics and these kind of procedures are becoming higher in number. At the same time, the day care segment is growing very fast, so that will all help us to reduce the ALOS further.

Nitin Agarwal: And second one, on the Jalandhar acquisition. Post this acquisition, we have now a largest presence in Punjab. So I mean, how do you look at this region now from what kind of opportunities for growth do you see, if you set Punjab as a cluster now, for example?

Ashutosh Raghuvanshi: So Punjab, we have a very dominant presence. We are way far ahead of any other competition. Together all these beds make about 1,000 beds. And many of the units are performing.

Amritsar and Mohali specifically are doing very well. We have further expansion planned in Mohali as you might be aware. And at the same time, we have planning expansion in Amritsar as well.

Ludhiana is also doing all right. And Jalandhar, this facility which we have acquired is -- has got good performance in last few quarters, so we expect that to improve further. And this

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cluster, we look at with great interest because this has been the origin of the group and we have such an edge in terms of branding over there, and that is what we want

to build on further.

But currently, we are not planning anything further than the current hospitals and the brownfield expansions, which we will have in them. The brownfield expansion, we'll have about 450 beds in Mohali. We'll have about 250 beds in Jalandhar further added, and about 180 beds in Amritsar, we are going to add further. So this is the plan for the Punjab region at the moment.

Nitin Agarwal: And last one, sir, on the payer mix, any -- on the scheme patients -- I mean, how do you -- do you see the payer mix changing in any meaningful way as we go along?

Vivek Goyal: So not really. We have seen some improvement in the first quarter, whereby our PPA and the international business has really gone up. And the government business has also gone up, but it is not -- it has not gone up to that extent. So that way, payer mix has improved slightly. But with our expansion program and the geographical presence in some of the regions, our ability to reduce this is very low, but the improvement will be gradual, it will not be very dramatic.

Moderator: The next question is from the line of Bino Pathiparampil from Elara Capital.

Bino Pathiparampil: Again, congrats on a great set of numbers. Most questions have been answered. Just on your capacity mix. So your presentation talked about more than 5,700 operational beds, but what would be the total capacity beds?

Vivek Goyal: Your voice was breaking. If your question is 5,700 operational beds?

Bino Pathiparampil: Yes.

Anurag Kalra: So Bino, out of these 5,700, including the Gleneagles O&M that we've just executed, we have about 1,300 O&M beds. The rest would be those P&L beds that you're talking about.

Moderator: Sir, I think the participant has got disconnected. Can we move to the next question?

Anurag Kalra: Yes, please.

Moderator: The next question is from the line of Saion Mukherjee from Nomura.

Saion Mukherjee: Yes, this is Saion here from Nomura. Congratulations on a great set of numbers you delivered.

Sir, in response to an earlier question, you mentioned about one of the things, which you have worked on in terms of investments and case mix. You mentioned about oncology, which I think is somewhere around mid-teen contribution.

Can you also share on robotic surgery, what percentage, either in terms of revenues or number of surgeries that you do? And what's the scope for these two ratios to increase over the next,

say, 3, 4 years, if you have something in mind in the medium term?

Ashutosh Raghuvanshi: Yes. So oncology contribution is approximately 17%, 18% at the moment, but that is pure oncology. And there is some oncology, which gets identified as other specialties because
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cancers can be anywhere. So we -- our estimate is that approximately about 19% to 20% revenue is coming from oncology.

As far as robotic surgeries are concerned, we don't track that revenue separately at the moment. But we have seen a 75% year-on-year change in terms of the number of cases, procedures, we have performed. That is the kind of growth we are expecting, and we expect that growth to continue because we are adding more robots in our network.

Some of the hospitals, we have already got the second system in place and some of the hospitals, which did not have a system earlier, we are in the process of installing robotic machines as well. So I expect that growth will happen in that, maybe not 75% next year, but at least 50% next year as well.

Saion Mukherjee: Okay. So your ARPOB growth can sort of be in high single digit, at least for some time, would that be something to look forward to?

Vivek Goyal: Yes. So Saion, the ARPOB growth is -- as I was telling in the earlier calls also, this growth is mainly coming from the day care, OPD and those type of things and robotic surgeries because there consumer is high. The price increase

in it is only 1.3%. So it is very difficult to predict how much more we can do or what type of day care business we will be getting or OPD business we will be getting, but we maintain our guidance that ARPOB growth should be in the -- in the normal course, it should be around 5%, 6%.

Saion Mukherjee: Okay. Understood. My second question is on the Manesar facility. So if you can share some light in terms of how much is the -- how has been the ramp-up and whether it's EBITDA breakeven, it's making some profit or any indication you can provide?

Vivek Goyal: Yes. So ramp-up is quite good, and it is better than our expectations, Saion. And it is picking up quite well. In terms of revenues, it has started generating revenue of INR11 crores-plus per month, okay? And the EBITDA side, it is still on the negative side because there a lot of hiring and clinical talent we are adding.

And that -- the actual benefit of that may be coming in the forthcoming quarters. So, I am expecting if we're able to achieve the revenue of INR2 crores more per month, which we are expecting in the next couple of months, this unit should be breakeven on EBITDA level.

Saion Mukherjee: Okay. Understood. That's helpful. Sir, can I ask one more question on Diagnostics?

Ashutosh Raghuvanshi: Sure.

Saion Mukherjee: Okay. So sir, one question on Diagnostics. We are seeing some stability in the business now. In terms of your mix, whether we look at Wellness contribution, B2C, B2B, we are at a particular level. I mean, should we see any meaningful change in these ratios, something which we are pursuing?

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And also on the -- if you can throw some light on the network itself? Firstly, the expansion plan that you have? And is there any franchisee model involved when it comes to these collection centers, how you manage it?

Anand K: Thanks, Saion. So the product revenue mix, as you know, wellness currently contributes about 12% of our revenue, and it is one of the quite fast growing segments for us now. And we have been seeing good traction on health check up packages, which has been taken up by general population as well across all our units. So that we see that it will definitely keep growing.

When it comes to B2C and B2B mix, yes, at this point of time, yes, our mix is at about 51:49, so we expect it to be around the same over the period of next 1 year or so, then probably there will be further improvement in B2C. It all depends on which segment grows faster. So as we expand further, as we improve our network and our network productivity also improves.

The B2C component will keep going. So another important aspect in the system is, we are also going to be adding SRL brand back to us in the sense that now we have ownership of the SRL brand, and that will help us to further strengthen our presence that earlier, we were not able to provide that opportunity of having SRL as a gilus has been the new name for SRL. So we have not been able to communicate effectively. So we will be doing that process in the coming months, by which also we expect that the B2C component will further strengthen.

Saion Mukherjee: Sorry, sir, I didn't get this. So this -- can you just explain the SRL thing? So you've made the

brand change, so now you are going to use the SRL brand back. If you can just explain what it means?

Anand K : No. In fact, we had SRL brand before and we converted it to Agilus. But we were not using SRL or this process earlier due to certain legal requirements. Currently, we have got the ownership of the brand, and that is the reason we are going ahead.

Ashutosh Raghuvanshi: So Saion, if I can explain this a little further, the change which we had to do to -- from SRL to Agilus was a little abrupt. And at that time, the Board -- the courts directed us not to use SRL in any form, even to identify that this business was previously

called SRL Limited. So we were constrained in communicating effectively to people that this is the legacy of this business, which continues as a new brand.

So it was not kind of a very ideal kind of a brand change situation, and that impacted our business negatively. However, now we have acquired this brand, and now we have the ability to communicate effectively that SRL is now Agilus. So we expect that, that communication will also further help to strengthen our brand.

Moderator: The next question is from Lavanya Tottala from UBS.

Lavanya Tottala: So one question. So how the business has been trending in July, like in terms of occupancy, any sense how the momentum has been in July?

Vivek Goyal: So we are trending well. We can't obviously spell out the numbers because it is price sensitive. So we are trending quite well as per our plan.

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Lavanya Tottala: Okay. Got that. So just on this diagnostic, one clarification. So SRL will be used only for this communication that it is rebranded to Agilus, but we are not planning for dual branding or something? It's going to be Agilus, but just communicating that it was before SRL, is that right understanding?

Anand K: Correct.

Moderator: The next question is from the line of Pranav Chawla from Ambit Capital.

Prashant : Yes. This is Prashant here. A couple of questions. One, on the diagnostics side, more of a clarification. So when you talk about margins of 22% to 23%, that's on the gross revenues basis, right?

Anand K: Diagnostics, that's on the -- yes, it's on the net revenue basis.

Prashant : Okay. So you -- for this year, your margin should be in that 20% to 23% range on a net revenue basis?

Anand K: Yes.

Prashant : And second, one question on the hospital side. The Escorts Delhi Hospital, which margin bracket would it now feature in?

Vivek Goyal: It has moved up to about 15% margin, so it is around 15%, 17%. Yes, in that range. And it's consistently performing at that level. So -- and we identified certain more lever where we can improve it further.

Prashant : Great. And one last question from my side. Again, back on the Diagnostics side. So your revenue growth seems to have stabilized at a slightly higher level than you've been growing in the last few years. So where should we see this finally on a normalized basis stabilizing?

Would it be high single digit or would you go into lower double -digit range. How should we think about this now?

Anand K: So I think in the next few quarters, we'll be in the high single digit to about 10% kind of growth in the next few quarters. But as we move forward, in the next 6 to 8 quarters, we will be moving into the early double -digit numbers.

Moderator: The next question is from the line of Nancy Yadav from Allegro Advisors.

Nancy Yadav: Most of my questions have been answered already. I just wanted to ask the Ind AS adjustment number; and if possible, a breakup of the number for Hospitals versus Diagnostics?

Vivek Goyal: Yes. So there is not much impact, and we are not tracking that way because now it is all -in as per IFRS and Ind AS. So there is not much impact, that much I can tell you, at consolidated level.

Hospital business, it is very negligible. Diagnostics, still there will be a little bit, but it is not much. We -- Anurag may provide you separately.

Nancy Yadav: Sorry?

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Vivek Goyal: Anurag will be able to provide you separately if you want that number, absolute number.

Nancy Yadav: Okay. Sure. Thank you.

Moderator: The next question is from the line of Ankush Mahajan from Sanctum Wealth.

Ankush Mahajan : Sir, this time, the international customers, that growth is quite good. Would you throw some more light that how could we trend for the next three quarters for the international customers?

And what are

the main geographies that these customers are coming?

That's one part. Second, sir, this is related to robotic machines. So how many robotic machines we have? And this year, how many we are expected to bring new robotic machines? And what kind of capex we are doing on the robotic machines?

Ashutosh Raghuvanshi: So we have approximately 15 robotic machines across our network right now, and we are in the process of getting another 4 this year. So this is regarding the robotic procedures. We have seen a growth of about 75%, as I said earlier. And as far as the international patients is concerned, the overall contribution is about 8% for overall revenue.

And it's likely to increase a little bit. But in absolute terms, it is likely to increase. But as a contribution, it is likely to stay in that level. So we do get patients for oncology and other areas like that. Cardiac and oncology is the main and neuro as well. These 3 departments get a lot of patients.

Ankush Mahajan : Sir, what kind of capex we are doing for these four robotic machines?

Vivek Goyal: Yes. So the robotic machines generally cost us around INR12 crore s per machine. I'm talking da Vinci robot. And for ortho robot it costs us around INR5 crores.

Ankush Mahajan : Sir, last one from my side, that is oncology is doing very well, as mentioned in the earlier comments, growing with the 27%, 28% of growth, how could be the trend over the next 3 years in the oncology segment?

Ashutosh Raghuvanshi: Yes. So we expect that kind of growth will continue for another few years because we are adding oncology setups to some other hospitals where it is not there at the moment. So obviously, that growth is -- momentum is likely to continue.

Moderator: The next question is from the line of Harsh Bhatia from Bandhan Mutual Funds.

Harsh Bhatia : Yes. Just two quick clarifications. One, could you help us with the Noida and Faridabad occupancy as of the first quarter, FY '26?

Ashutosh Raghuvanshi: Yes.

Vivek Goyal: Yes. So Noida is 76% occupancy and Faridabad is above 80% occupancy.

Harsh Bhatia : And Noida 76% is after the 60 -bed addition?

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Vivek Goyal: Yes.

Harsh Bhatia : Okay. And just one clarification. I'm just trying to bridge the gap between this almost 300 basis points of Hospital margin improvement on a year -on-year basis. So if I look at the last year first quarter of FY '25, probably Hospitals had certain 40 to 50 bps of, one could say, one-offs in terms of some provisions, probably surgical mix was higher, so your margins were impacted to that extent.

So excluding all of that, if I just work with a 300 bps of margin improvement, and you mentioned that there are no one-offs in the current hospital margin. So I understand that Ludhiana is something that has moved up in the margin metrics. FMRI and Anandpur has also moved up in the margin metrics.

Is there anything else that is sort of helping you move up the curve in terms of the margins or broadly, these are the three or four drivers that are driving majority of that margin improvement?

Vivek Goyal: So at network level, almost all the hospitals are doing quite well. Some of the underperforming units has also started performing, like I mentioned about Jaipur. There is also a very good improvement in the field in terms of margins and overall performance . And then whatever capacity we're able to add, we're able to ramp up as per our expectation, which I've told that it is brownfield.

So that is -- so if you see the occupancy has gone up. It is stretching around 70% at the network level. So that helps a lot in the margin improvement apart from what I've told earlier.

Harsh Bhatia : Sir, would it be fair to say at least for FY '26 perspective, if I look at Faridabad, FMRI addition, Noida addition, whatever incremental beds you are adding and I'm also counting the minimal add

ition we did at FMRI in first quarter itself, that incremental beds by itself is attracting a very high specialty mixed patient pool, one would say from the beginning itself, again because this is being a brownfield that is why I'm asking this question?

Vivek Goyal: Yes. So one is the, as I mentioned these are Brownfield. So the ramp up is not issue. And plus we are adding specialties and clinical talent at all our facilities. And plus, we have invested in the technology also and that is also helping us in doing some quality work and which -- the

overall impact is in the margin improvement.

Harsh Bhatia: Sure, sir. Thank you.

Moderator: Thank you. Ladies and gentlemen, that was the last question for today. I now hand over the conference to management for closing comments.

Anurag Kalra: Ladies and gentlemen, thank you very much for taking the time on the call. Please do feel free to reach out to us if you have any further queries or clarifications. Thank you and have a good day.

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Moderator: Thank you. On behalf of Fortis Healthcare Limited, concludes this conference. Thank you for joining us and you may now disconnect your lines.

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Sub: Transcript of Investors / Analysts' meet under Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015

Dear Madam/Sir,

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed transcript of Investors / Analysts' meet held on November 12, 2025 to discuss the Company's Un -Audited Financial Results for the quarter and half year ended on September 30, 2025 and same is available on the Website of the Company at below link:

<https://www.fortishealthcare.com/investor/investor%20presentations%20&%20transcripts/earnings%20call%20transcript%20for%20the%20quarter%20ended%20september%202025>

The date and time of occurrence of event is November 12, 2025 at 11.00 A.M. (IST)

This is for your information and records.

Thanking you,

Yours Sincerely,

For Fortis Healthcare Limited

Satyendra Chauhan

Company Secretary & Compliance Officer

M. No. – A14783

Encl: a/a

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"Fortis Healthcare Limited "

Q2 FY26 Earnings Conference Call "

November 12 , 2025

MANAGEMENT : DR ASHUTOSH RAGHUVANSHI – MANAGING
DIRECTOR AND CHIEF EXECUTIVE OFFICER – FORTIS
HEALTHCARE LIMITED
MR. VIVEK GOYAL – CHIEF FINANCIAL OFFICER –
FORTIS HEALTHCARE LIMITED
MR. ANAND K – CHIEF EXECUTIVE OFFICER – AGILUS
DIAGNOSTICS
MR. AKSHAY TIWARI – CHIEF FINANCIAL OFFICER –
AGILUS DIAGNOSTICS
MR. ANURAG KALRA – HEAD OF INVESTOR
RELATIONS – FORTIS HEALTHCARE LIMITED

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Moderator: Ladies and gentlemen, good morning, and welcome to the Fortis Healthcare Limited Q2 FY'26 Earnings Conference Call. As a reminder, all participant lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Anurag Kalra, Head of Investor Relations at Fortis Healthcare Limited. Thank you, and over to you, sir.

Anurag Kalra: Thank you so much. Very good morning and good afternoon, ladies and gentlemen, and welcome to Fortis Healthcare's Quarter 2 FY26 Earnings Call. I have the pleasure of introducing you to Dr. Ashutosh Raghuvanshi, our Managing Director and CEO, who's going to chair the call. With him, we have Mr. Vivek Goyal, our Chief Financial Officer; from Agilus diagnostics, Anand, the CEO is joining us. And with him, we have Mr. Akshay Tiwari, the CFO of Agilus. We will start with some opening remarks on the quarter gone by and its performance by Dr. Raghuvanshi, post which Anand will take you through the key highlights of the diagnostics business. And then we should open the floor for questions and answers. Over to Dr. Raghuvanshi.

Ashutosh Raghuvanshi: Thank you, Anurag. Good morning, everyone, and thank you for taking the time to join us on our Q2 financial year '26 earnings call today. Before moving to the financial performance, I would like to highlight that we continue to witness a healthy growth momentum in Q2 financial year '26 and H1 of financial year '26. Both our business segments, Hospitals and Diagnostics continue to perform well both in terms of revenue and margins.

Coming to the quarterly results, we reported a consolidated top line of INR2,331 crores, a growth of 17.3% over the quarter 2 of financial year '25. Our hospital business revenues have grown 19.3% to INR1,974 crores while Q2 financial year '26 diagnostic business net revenues have grown by 7.1% to INR357 crores versus INR334 crores in Q2 financial year '25. The hospital business revenue accounts for 85% of our consolidated revenue.

Our consolidated operating EBITDA increased 28% to INR556 crores, delivering a margin of 23.9% versus 21.9% in Q2 of financial year '25. The hospital business reported an operating EBITDA of INR452 crores, driving a 150 basis points improvement in margin from 21.4% in Q2 of financial year '25 to 22.9% in Q2 of financial year '26. Our consolidated profit after tax before exceptional items for the quarter increased 20.7% to INR305 crores.

Let me also briefly touch on the H1 financial year '26 result numbers of the company. Our consolidated revenues for H1 financial year '26 stood at INR4,498 crores, up 16.9% versus H1 of financial year '25. Operating EBITDA for H1 financial year '26 increased to INR1,047 crores, reflecting a robust 310 -point improvement to 23.3% compared to 20.2% in corresponding period last year. H1 financial year '26 hospital business revenue increased by 19% to INR3,812 crores. Operating margin for the hospital business improved by 250 basis points to 22.5% for the period versus 20% in the corresponding previous period. On the balance sheet front, the company's net debt stands at INR2,219 crores with a net debt -to-EBITDA of 0.96x as on September 30, 2025, Fortis Healthcare Limited
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as against 0.16x on September 30, 2024. The increase in debt was primarily due to funds raised to part-finance the acquisition of the 31.5% PE stake in Agilus, and the acquisition of Fortis brand and trademarks and acquisition of Shrimann Hospital in Jalandhar. Our growth and expansion initiatives are gathering pace. I'm pleased to inform tha

t during the

quarter, the company entered into a 15 -year lease agreement for a 200 -bedded multi -specialty hospital in Greater Noida, a facility that we have previously been managing under an O&M agreement. The company also signed an O&M agreement for a 550 -bedded greenfield super specialty hospital to be constructed in Lucknow by Ekana Group.

In July 2025, the company consummated the acquisition of Shrimann Super speciality Hospital in Jalandhar, Punjab, taking the total beds in Punjab cluster to 965. We also entered into an operation and management services agreement with Gleneagles India to manage five hospitals and a clinic within their network.

The integration of Gleneagles units under the agreement with Fortis is progressing well. Our brownfield bed expansion plans are also progressing well with the expansion in our existing facilities, the Jalandhar acquisition and the new arrangement in Greater Noida, we have added 550 operational beds in H1 of financial year '26.

Continuing from Q1, our hospital occupancy further improved to 71% compared to 69% in Q1 of financial year '26. This translates into occupied beds increased by about 13% to 3,318 beds in Q2 of financial year '26 compared to 2,928 beds in Q1 of financial year '26. Our hospital business recorded a 5.8% increase in ARPOB, reaching INR2.51 crores per annum. Higher ARPOB was driven by an improved specialty mix led by strong growth in oncology.

The oncology segment grew 29% year -on-year increasing its revenue contribution to 16.2% from 15% in Q2 of financial year '25. The growth in ARPOB was further supported by increasing share of complex cases reflected in 66% year -on-year increase in robotic surgeries. Our revenue from medical travel grew 26% compared to Q2 of financial year '25 to reach INR169 crores.

The revenue contribution of international business stood at 8.1% in Q2 compared to 7.7% in the corresponding previous year. In 13 of our facilities, we have reported operating EBITDA above 20% during the Q2 of financial year '26. These 13 facilities together contributed 77% to the hospital revenues in comparison. In financial year '25, we had 10 of our facilities with operating EBITDA margin of 20% and contributing 73% to the hospital revenues.

Our key hospitals such as Shalimar Bagh, Faridabad and FEHI witnessed margin expansion compared to both corresponding and trailing quarters. In addition, our key facilities such as Noida, Faridabad and FEHI registered revenue growth in excess of 20% compared to the corresponding previous period. Revenue from digital channels viz website, mobile application as well as digital campaigns witnessed a 20.4% year -on-year growth. Digital revenues contributed 29.5% to overall hospital revenues versus 29.3% in Q2 of financial year '25.

The company further strengthened its medical talent with onboarding of specialists in the area of oncology, cardiac sciences, obstetrics and gynecology, neurosciences and gastroenterology.

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I'm also pleased to share that our Diagnostics business continues to witness traction both in top line and margins.

Gross revenue grew 7.3% to INR400 crores from INR372 crores in Q2 of financial year '25.

Operating EBITDA margin stood at 26.1% versus 21.5% in Q2 of financial year '25. Excluding one-offs and operating EBITDA margin stood at 24% in Q2 of financial year '25. For H1 financial year '26, gross revenues grew 7.3% to INR768 crores compared to INR716 crores in H1 of financial year '25.

Operating EBITDA margins stood at 24.6% versus 18.9% in H1 of financial year '25. Excluding one-offs, the operating EBITDA margin stood at 21.4% in H1 of financial year '25. As part of our ongoing network expansion strategy, the total number of new customer touchpoints reached 4,330 as of September 30, 2025.

The preventive portfolio revenues in Agilus revenue contributed

13% to the operating revenue.

We continue to witness a steady recovery in both revenue and operating EBITDA margins. We expect the positive growth momentum to continue and believe the company is well positioned to further scale up its revenue performance.

With this, I will conclude my comments. I believe our businesses are well positioned to sustain their growth trajectory. Leveraging the strength of our balance sheet, we will continue to explore and evaluate growth opportunities within our focused geographies. Thank you. With this, I hand over to Mr. Anand for his comments now.

Anand K : Thank you, Dr. Raghuvanshi. Good morning, everyone, and thank you for joining us today. On behalf of Agilus Diagnostics, I welcome you to our Q2 FY '26 results conference call. Agilus Diagnostics reported a revenue of INR399.6 crores in Q2 of FY '26, reflecting a growth of 7.3% compared to INR372.5 crores in Q2 of FY '25, and INR368.8 crores in Q1 of FY '26, a growth of 8.4%.

Operating EBITDA for the quarter stood at INR104 crores up from INR80 crores in Q2 of FY '25 with margins improving to 26.1% from the 21.5% in the corresponding quarter of previous

year. In Q1 FY '26, operating EBITDA was INR85 crores with a margin of 23%. In the case of H1 FY '26, we have a top line of INR768 crores with a growth of 7.3% compared to the previous year's same H1, which was at INR716 crores.

The EBITDA for the same period of H1 of FY '26 was at INR189 crores, which is at 24.6%, which is compared to the previous corresponding quarter of the previous year at INR135 crores of EBITDA with a 18.9% margin. During Q2 of FY '26, Agilus conducted 10.6 million tests compared to 10.4 million tests in Q2 of FY '25, and 10.1 million tests in Q1 of FY '26.

We recorded a gross addition of 7 labs -- 7 new labs and 200-plus new customer touchpoints during this quarter, reflecting our continued focus on strengthening presence and accessibility across geographies. The B2C to B2B revenue mix stood at 52:48 in Q2 of FY '26 compared to 53:47 in Q2 of FY '25.

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From a product standpoint, revenue contributions for Q2 FY26 stood at 53% from routine tests, 34% from specialized tests and 13% from our wellness portfolio. On the geography front, revenues were driven by 30% from the North, 33% from the South, 20% from the West, 12% from the East and 4% from our international markets. We have continued to enhance our digital platform this quarter as well to elevate the customer experience.

Our automation-led workflow improvements and real-time tracking tools have helped streamline operations and improve service delivery. We also expanded our AI-led reporting capabilities to support faster and more accurate diagnostics. Our focus for next-generation diagnostics has further strengthened our genomics portfolio, which grew by 20% compared to Q2 of FY '25.

We launched 19 new tests this quarter, expanding our capabilities in oncology, infectious diseases and autoimmune diagnostics. Three additions include the multiple myeloma screening and monitoring panels, Agilus EDGE CGP rapid was fast genomic profiling and the TB resist Plus test for drug-resistant TB detection. Other highlights include Clone Detect and Scan for bone marrow analysis and HER2-DISH for HER2 status in cancer along with anti-MCV marker for early rheumatoid arthritis diagnosis. These new additions reinforce our commitment to expanding cutting-edge diagnostic capabilities, driving precision medicine and improving patient outcomes. Thank you, and over to you, Anurag.

Anurag Kalra: Ladies and gentlemen, we will now open the floor for questions and answers. May I request the moderator to comment, please.

Moderator: Thank you very much. We will now begin the question and answer session. The first question is from the line of Tausif from BNP Paribas Exane Research.

Tausif :

Thanks for the opportunity and congrats on a good set of numbers. Sir, a few questions on the hospital business. During the quarter, we had some extreme weather conditions in some part of the Northern India. I just want to understand whether Fortis -- any of the hospital was impacted during the quarter?

Ashutosh Raghuvanshi: Yes. So in Punjab, there was a lot of flooding, as you are aware. So that did cause some disruption in our Ludhiana as well as in Mohali and Amritsar facility. However, there was no immediate problem in the surrounding areas. So the hospital kept on functioning normally.

However, the patient flow was impacted for a week or so. But overall, as you can see, the results are quite satisfactory.

Tausif : That's helpful. Second question on the recent long-term lease agreement, which you have signed for Greater Noida facility and earlier it was O&M. Can you help us understand the impact on P&L?

Vivek Goyal: So there is a P&L charge of rental, which will be below EBITDA actually. It will be around INR23.5 crores -- sorry, INR2.3 crores per month. So it will be like yearly charge will be around that number, INR2.3 crores per month, multiply by 12 will be the annual charge. As regard the profitability, this unit is doing around revenue of around INR10 crores per month now. And this has a 200-bedded capacity and which can further be extended to 250 bedded.

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It is earlier managed by -- it is a sort of plug and play for us. It is all the IT systems, our H1 system is working there, Oracle Fusion, the ERP system is working there. So that way, it will be a sort of plug-and-play model. And we hope that we will be able to ramp up it quite fast. Right now, this unit is generating EBITDA of around 2%, 3%. Okay. Yes, it is a relatively new unit, as you might be knowing, it has opened around 1.5 years back.

Tausif : That's helpful. Just last question on the Diagnostics piece. I think this quarter, we have seen a

significant ramp -up in the margin profile. What kind of trajectory one should see for -- going ahead for Diagnostics business?

Anand K : As we have guided earlier also, it was -- the margins for -- if you see the half year, we are around 24%. So I think we'll be somewhere around the 23%, 24% for the whole year is what we are expecting.

Moderator: The next question is from the line of Neha Manpuria from Bank of America. .

Neha Manpuria: Sir, my first question is on the Gleneagles O&M. Currently, does this include Mumbai also? Or is it just for the facilities except Mumbai?

Ashutosh Raghuvanshi: Yes. So it is -- Mumbai is not included in the current arrangement. However, we are looking forward to have that play in the future.

Neha Manpuria: Understood. And given the update that we saw from the approval for SEBI for open offer, how should we think about Fortis eventually at some point of time looking to take over, converting this O&M into, let's say, acquiring these assets. I mean, does these assets interest Fortis, particularly given they have assets in Hyderabad and Chennai, which Chennai, we divested, Hyderabad, we don't have a presence. So just trying to get a sense of if you think about the Gleneagles portfolio, how does that fit into Fortis, if at all, we were to consider acquiring this asset?

Ashutosh Raghuvanshi: Yes. So we are sort of integrating it into our operation. But we expect that in future, we would be evaluating and seeing how we can integrate this and merge this Fortis operation. IHH has stated that they want to operate as a still platform, which is Fortis.

Neha Manpuria: Understood. And at the moment, the O&M, the -- I mean, we have -- just have a share in revenue, right? So there's nothing that we capture from an improvement in profitability, if at all that were to be evident in the next, let's say, a few quarters?

Ashutosh Raghuvanshi:

anshi: At the moment, that is the arrangement, but we expect that to change in the future. .

Neha Manpuria: Okay. Understood. My second question is on the hospital profitability. I think the 2 hospitals that have moved to the over 20% margins. So this would be Shalimar Bagh and which other hospital would it be?

Ashutosh Raghuvanshi: I think Jalandhar is...

Vivek Goyal: One is Mulund and another is Jalandhar. Jalandhar of course. Jalandhar is a new unit.

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Neha Manpuria: Understood. For the other hospitals, particularly Noida, when should we expect Noida to get to, let's say, the mid-teen profitability? Does it require some investment for us to scale up that unit?

Is it -- would it take us 2, 3 years to probably improve profitability? Or do you see possibility of that being shortened given we've already been running this unit for 18 months now?

Vivek Goyal: Yes. So, Noida, as I mentioned, there is a lot of potential in that. That's why we have gotten to this arrangement. In our view, it will take maybe 6 months -- or more months' time when it starts generating around 15% EBITDA and then it can be ramped up further in line with our other big units in the NCR. So I will say 6 months' time definitely will be required to put things in line.

There is some investment required, which we'll be doing in the medical equipment and enhancing the clinical tenet and all those work will be requiring 3 to 6 months' time and then we can see some improvement. Already, we are seeing after Fortis taking over, the revenue is already showing some sign of improvement as compared to the earlier period.

Neha Manpuria: Understood. And sorry, one last question. If I were to strip out the Noida and Jalandhar acquisitions or O&M get converting to lease, the organic bed addition has been about 100, 150 beds. Would that be a right number?

Anurag Kalra: For quarter 2, yes, approximately about 150. That's right.

Neha Manpuria: No, for the first half, no, I was looking at the first half because there was about 200 bed addition that we saw in first quarter, and then I see another 400. So if I were to strip out first half, both of them, then it's about 100 beds organic addition in first half?

Anurag Kalra: No, Neha. In the first half, when we mentioned that we've added an operational bed capacity of close to 550 beds, Jalandhar the current operational status is about 190 beds. Greater Noida is about 170. So that means we've added close to about 200 beds from an operational perspective in H1 of FY '26.

Neha Manpuria: Okay. And second -- and so we still maintain the full year addition of about 400, 500 beds?

Ashutosh Raghuvanshi: That's correct. . Yes, that's right.

Moderator: The next question is from the line of Shyam Srinivasan from Goldman Sachs.

Shyam Srinivasan: Just the first one on hospitals top line growth. So we have done like 18%, 19% for the first half. So how should we look at second half? Maybe the base is a little less favorable, but just want to understand, can we maintain some of this momentum into second half as well?

Ashutosh Raghuvanshi: Yes. Typically, Shyam, we have seen that H1, H2 remains almost at similar level. So there is no reason for us to believe that it is going to be inferior in any way. However, having said that,

quarter 3, you could expect a little bit of dip because of the seasonal impact of festivals and holidays. But the fourth quarter is typically better. So though the base is enhanced, but we still expect that we could see a similar trend going in H2 as well.

Shyam Srinivasan: Understood. And Dr. Raghuvanshi, if you could break it down into volume growth, like maybe even occupied beds growth, large part of our growth is coming from volumes, right? So given

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that we have higher beds versus last year, that is

what is giving us the comfort that this run rate can sustain?

Ashutosh Raghuvanshi: Vivek?

Vivek Goyal: Yes. So Shyam, we are expecting around 5%, 6% ARPOB growth even in the second half and balance is all the volume growth. It is primarily coming from the bed addition as we have witnessed. And apart from bed addition, there is a lot of day care procedures and these robotic surgeries and those things are also factored into while calculating the ARPOB.

ARPOB increase is not like price increase, which I'm saying in all the calls. Price increase will be around 1%, 1.5%. Balance is all because of the mix change, the robotic surgeries because of the day care and those type of things are contributing to the ARPOB increase.

Shyam Srinivasan: Got it. That's helpful, Vivek. Second question is on the margin profile. So we had our margin guidance going from 20.5% to 22.5%. Any change to that given that in the first half, we have probably done better on hospital margins. Could you elaborate on that, please?

Vivek Goyal: Yes. So in first half, definitely, we have beaten our own estimates on the margin improvement. And we expect we will continue to do better on the margin expansion side as our units are becoming mature, there will be further improvement in the margin like Manesar has already become EBITDA positive.

Then we are having a lot of upside in the Greater Noida facility. And plus the ramp-up of the capacities where we have just installed like Noida, Faridabad there are some capacity which we will be adding in the second half. So all those things will -- should add to the margin improvement thing. So I think there is a possibility we can see higher margin improvement than what we have guided at the beginning of the year. To quantify it will be a bit higher.

Moderator: The next question is from the line of Aman Goyal from Axis Securities.

Aman Goyal: Great set of numbers. Sir, my first question is related to Manesar facility. So what is the current revenue for this quarter for Manesar and ARPOB and occupancy trend? And coming to the patient inflow, are we seeing the patient inflow from the catchment area or it's just Fortis network referral?

Vivek Goyal: So it is a mix of all. I think Manesar is doing quite okay in terms of revenue ramp-up and the profitability side also. So -- as I mentioned earlier, this unit has achieved the EBITDA positive thing, and it is less than 1 year, we're able to achieve it. So that is a good part about this. There is a lot of empanelment which was pending earlier, we could have completed.

So we expect the ramp-up should be quite smooth, and we expect to open more beds in the second half and ramp up quite fast. So I think Manesar, we should see some further improvement in the revenue, and that will lead to higher EBITDA margin contribution to the overall company.

Aman Goyal: What is the quarter run rate for Manesar this quarter?

Vivek Goyal: It is around INR40 crores per quarter, and we expect it to grow around 20%.

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Aman Goyal: Sir, my second question is related to CGHS revised rate. So I mean, what is -- are you seeing any traction from the CGHS patient or any competitive from the other private hospital? Are they trying to panel with the CGHS?

Vivek Goyal: Yes. So our contribution in CGHS is there, and there is a positive -- I will say not positive mix impact because of this rate change because on one side, there are many procedures which has been -- where the price has gone up. But at the same time, there is a lot of clarity required in the way the circular has come. So we are still evaluating that. However, looking like overall, there will be a positive impact.

There is drug price impact on the onco drugs, then there is very little clarity on the super specialty things, how it will be working and things like that. So all those clarity we are seeking.

And once

we have a full clarity, we will be able to give you more precise thing. But at present, it is looking like positive. As regarding -- we are almost operating at 70% occupancy, and there is very little

room for accommodate further patient actually. So we are actually focusing more on building new capacities and accommodate more patient in our network.

Moderator: Thank you. The next question is from the line of Abdulkader Puranwala from ICICI Securities.

Abdulkader Puranwala: Sir, with our occupancy now inching up consistently about 70%. So where should we see this occupancy on the existing setup in the next couple of years? And second one on the debt. So I mean, what are our plans to retire this debt? And by when should we see FY '25 levels coming back?

Vivek Goyal: Yes. So occupancy, first on the occupancy side. So we are at 70%, 71%. And most of the hospitals, we are operating at a very optimal occupancy level. And there are a couple of hospitals where there is scope for improvement in the occupancy and there we are concentrating more. So I will say there is a little bit improvement. But having said that, there is more capacity coming in the second half. So I will say occupancy will remain in this range, above 70% but below 75% type of number.

As regard to your other question on debt, we are very comfortable at the current debt level. It is less than 1x, and company is generating healthy cash flow with the current operations. And we are plugging that cash flow back into the system for the capex, for the growth and for the quality equipment. So I think our -- we are not very much worried currently on the debt level. But if we are unable to do certain growth thing through acquisition, I think this debt level will be -- come down in 2 years' time to zero level, which is not a desirable for me actually. We want to have more growth with this type of leverage.

Abdulkader Puranwala: Understood. And sir, I had one more question on the Diagnostics side. Sir, I mean, if you look at this quarter per se, then our number of tests has grown only 2%. Sir, any broad color if you can provide on how should we see the Diagnostics business revenue growth moving into second half and next year?

Anand K: So this quarter, there has been a slight dip in the volume mainly because of -- we had the Aam Aadmi Mohalla Clinics business, which was there until June 30. So this is not there in this quarter. So that is why there is a dip in the volumes because, as you know, that business was Fortis Healthcare Limited
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very high on volumes. So that's the major change that you see. Also, in this quarter, the usual surge in vector-borne diseases has been not very significant, considering the elongated monsoon season. So that also has some impact on the overall volumes.

Abdulkader Puranwala: Got it. And sir, any color as to how we move into next year, what is the targeted volume growth we may have for the business?

Anand K: We'll not be able to comment on this at this point of time, but I feel that we are going in a positive direction in terms of both margins as well as growth compared to where we were earlier.

Moderator: The next question is from the line of Raman KV from Sequent Investments.

Raman K V: I just have 2 questions. One is what's your current stake in Agilus post the recent acquisition? Vivek Goyal: 89%, Raman.

Raman K V: So are you planning to increase it to 100%? Or are you okay with this 89%?

Vivek Goyal: So we are comfortable with this stake. But if the opportunity come -- as I said, we have a priority for investment in the hospital business. But if opportunity comes, this stake can also be absorbed with the current balance sheet and maybe our fundraising plan in future.

Raman K V: Understood, sir. And my second question is, with respect to your Diagnostic business, your average realization per patient and

average realization per test has been -- has increased year-on-year and quarter-on-quarter despite quarter-on-quarter very low volume growth. So can you comment on that? Is it like did you take any price hike or something?

Anand K: No, we have not taken any price hike during this year. So what has actually happened is, one is our increased focus on wellness portfolio, where there are a lot of packages. So because of the higher ticket size, you're seeing a higher average revenue per acquisition or average revenue per patient. And also you -- as I was discussing earlier, we had the Aam Aadmi Mohalla Clinics business in earlier quarters as well as the previous year, which was high on volume but lower in ticket size. So that has an impact overall on both the volumes as well as the average revenue per patient. That's the only difference.

Raman K V: So this Aam Aadmi Mohalla Clinic will be the re in the subsequent quarter? Or have you discontinued the business?

Anand K: We have discontinued, discontinued from 30th of June.

Moderator: The next question is from the line of Atul Minocha, an Individual Investor.

Atul Minocha: My question is, is there any delay on FMRI capacity addition in terms of the project execution because what I observed is, earlier, it was -- the capacity addition was coming in the last quarter of this year, but I think it has moved as per the presentation. So I just want to get some understanding around that?

Ashutosh Raghuvanshi: Yes. So we had originally planned it for the operationalization by the third quarter, end of third quarter. So there is a delay of about 3 months. Now we expect to commission it by March end.

So obviously, it will go into the next financial year.

Atul Minocha: Okay. And if I can ask another question. Am I permitted to ask another one? So my next question is with respect to this international patient flow. How is it like shaping up in terms of the capabilities which we are building? Is it -- if you can throw some light on that?

Ashutosh Raghuvanshi: So it has increased by 26% year -on-year. And it is -- overall, the international business constitutes about 8% of our business. We expect it to remain more or less at the same level. But in absolute terms, it will continue to grow in double digits. .

Moderator: The next question is from the line of Madhav from Fidelity.

Madhav : Sir, just wanted to check on our bed addition plan. Like this year, I think you said we can add about 400 -plus beds organically. Given the brownfield pipeline we have, what could that addition look like in FY '27 and which facilities will be commissioned beds in the next fiscal?

Vivek Goyal: So next fiscal, as Dr. Raghuvanshi mentioned in the earlier question, major bed addition will be coming from the FMRI bed addition, where we'll be adding 225 beds. There will be another, say, 70 beds we may add at one of our facility in Kolkata. And then there will be ramp -up where we will be opening more bed in Manesar and maybe Bangalore where some bed capacity is there. So that will be the major capacity addition in the next financial year.

Madhav: So next year as well, we could be closer to about 400 -plus beds in terms of addition, organically?

Vivek Goyal: Yes, it will be around 300, 400 range.

Madhav : Okay. Got it. And the FMRI facility for us, it's currently running at what occupancy?

Vivek Goyal: Around 85% occupancy.

Madhav: Okay. So this should be absorbed fairly quickly, right, the new beds once they commission?

Vivek Goyal: We expect so.

Madhav: And sir, the second question was on the hospital margins. I think we've kind of indicated that we wanted to get to the 25% EBITDA margin. Now that we are probably closer to, I don't know, maybe 23%, like you said, we could be higher than the guidance. Do you think there's a chance for us to kind of do that a bit fast

or do you think we could be above 25% as well in the hospital business in the next few years?

Vivek Goyal: Yes, the pace at which our facility is ramping up, I think the target is not very far away. I will put that way. I will not like to give any definitive time line. But yes, next couple of years, it is definitely looking like we are reaching there.

Madhav : Got it. And in terms of inorganic opportunities, are we looking at more such opportunities like the Jalandhar bolt -on that we did earlier this year? Can there be more such opportunities which you are evaluating?

Ashutosh Raghuvanshi: Yes, we are always keen to look for such facilities and such opportunities. As Vivek was saying earlier, that with the healthy balance sheet, we have the capacity to absorb some more. So we do keep looking at it. However, we are very disciplined about the valuation as well as the geographical synergy with our existing clusters and our focused geography.

Moderator: The next question is from the line of Kritika Damani from Prospera Financial Solutions.

Kritika Damani: Congratulations on a strong quarter. I wanted to understand the bridge between top line and profitability, like revenue grew 5% sequentially and EBITDA was nearly 50%, which implies a sharp improvement in the operational leverage. Could you elaborate on what specifically drove this? Was it a better efficiency mix or the higher ARPOB or improved utilization or simply stronger cost discipline across facilities? And do you see this operating leverage sustaining as more capacities like Manesar and Jalandhar scale up?

Vivek Goyal: Yes, it is mainly to do with the operational efficiency, which naturally come if we're able to achieve higher occupancy and we ramp up our existing hospital in a better way. So as I mentioned earlier, there are two, three hospitals where still there is a scope for better occupancy and there is a ramp -up possibility in our Greater Noida and Manesar facility. So all these are further opportunities that will be giving us better margin and overall EBITDA margin may go up.

Kritika Damani: Also, the EMR and the HOS rollouts have always been an important part of Fortis' modernization journey. So beyond the clinical documentation benefits, have the digital systems started showing measurable productivity improvements like reduction in patient turnaround time or better cost

tracking? Also, how far have we from like having a unified digital backbone across the network?

Ashutosh Raghuvanshi: Yes. So our rollout of EMR is still ongoing. There was -- there are about 4 hospitals where the IP modules have already been implemented, but it is still work in progress. I think we will take till the end of this year financial year to implement the entire EMR. Of course, we are seeing differences in patient satisfaction and service levels are definitely improving. But it's currently not really measurable.

Once we have the entire implementation, then we would look at that. We are also in the process of improving our patient app, which is -- which also has a component of a doctor's app as well, which makes the patient journey seamless. So we are coming up with a new version of that as well using the latest technology.

Moderator: The next follow-up question is from the line of Shyam Srinivasan from Goldman Sachs.

Shyam Srinivasan: Vivek, sir, just on that CGHS again. So you said there is some prices increase have gone up. Is there some quantum like an average number for us to look at? And also, there was some euphoria around CGHS after a long time, 10, 15 years maybe. And then now you seem a little more cautious.

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What are these additional requirements that is still keeping us a little on the sidelines, you don't look like you want to increase your CGHS exposure.

O I'm just trying to understand, is there

anything specific in the -- I know it's a long document and people are still trying to figure out, but what is the hesitancy?

Vivek Goyal: So there is always a sort of hesitancy, Shyam, because of the non-predictability about the payment. Okay? So it is always a question mark when the money will be coming, the deductions and all those stuff and suddenly some circulars come, which change the entire equation completely.

So obviously, that's why it makes this payer less attractive. But having said that, in almost all our hospitals, we are maintaining a decent mix of payers. And our government payer where this particular CGHS also come is generally contributing 18% to 20% of our total overall revenue. And I think with the expansion, which we are doing and with the new hospitals and things like that, I think this ratio will maintain. And this is a welcome step where CGHS has grown some -- increased after a long period of time. As I mentioned earlier, we are just evaluating because a lot of new things have come into this. And there are certain restrictions also in the form of drug use and things like that. So we are trying to absorb this and trying to understand more engaging with the CGHS official, how it will be impacting us. Looking like it will be a positive impact, that much I can tell you.

Shyam Srinivasan: Got it. And currently, it only impacts 4% of your revenues, right? So the government PCU 8%, ECHS 7.5%, that is a subsequent step. And is it usually coincidental in the sense after CGHS 3 months later, you get to negotiate the others? Or how has it worked historically?

Vivek Goyal: Generally, this government panel move in tandem. So I think we may expect some similar type of things from the ECHS and some of the PSUs, which also follow CGHS rate. And sometimes it's automatic also because the agreement is linked to CGHS rate. So that way, it will be having a positive impact. The impact will be similar for almost all the governments. ECHS yet to come out with a similar, but we are expecting that also.

Shyam Srinivasan: And there is no quantification, right? 10% higher, 12% higher, 15% higher, anything like an average number of what the rates are higher?

Vivek Goyal: Give us some time -- Shyam, give us some time. Till date, whatever calculation I have seen, there are a lot of questions more than the answers. So that's why I am not giving you.

Shyam Srinivasan: Okay. Understood. Sir, last question. Yes. Last question. I know there is an open offer. December is probably the time line. I know it's not a question to you, but more to your parent. But what are some of the other legal things that have now opened up following the SEBI approval?

Like is there anything that is stopping us now? Some of the legal expenses that was there for us in our P&L? Is that now going to 0. I noticed some legal expense in Diagnostics INR1 crores. But I'm just saying, how should we look at that piece for us? And I don't know whether you can comment on the open offer, but yes?

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Ashutosh Raghuvanshi: Yes. The open offer is closed as of yesterday. We have -- IHH has already made the announcements regarding that. So open offer is behind us now. Legal fees is definitely going to

reduce, but it will certainly not become 0 because we still have the high court case, and we also have certain proceedings which the company has initiated against the ex promoters for recovery of the which they owe to the company.

So those proceedings will continue to go on for some time. But other than that, most of other legal issues are behind us. As you are aware, that brand issue is also resolved. We have owned the brand and registered with us. We own the SRL brand as well now and the Fortis brand as well. Other than that, we also have done the scheme of sort of restructuring, which I would

d

request Vivek to share a little bit about.

Vivek Goyal: Yes, Shyam. So there is -- as you know, when we acquired this RHT asset, we have acquired the company. So the structure was still alive, like one entity paying BTC to another entity, trust fees to the other entity, the asset owning entity. And that was creating a sort of imbalance and operational inefficiencies. So that we have filed a scheme and now all those units are -- barring few, which are in the pipeline, around 3.

All the units are -- the operating entity and the asset owning entity are one. So there will not be two entities. So that will save a lot of operational challenges as well. Plus, we are also trying to now simplify the organizational structure where there are multiple entities, which we are trying to reduce. And all those things will add one on the simplicity and second to the compliance cost. And so that initiative is on. The Agilus also we are trying to do similar thing where 3 operating entities are having the diagnostic business.

Shyam Srinivasan: Sir, any quantification on any of the legal expenses? I know is not going to zero, but that will be helpful, sir?

Vivek Goyal: It will not be zero. Like earlier, we were incurring around INR30 crores, INR40 crores annually. Now I expect it will come down to 50% of that amount, maybe lower. It all depends on the number of hearing we have to do and things like that. It all will be depending on that. But it has come down substantially.

Moderator: The next question is from the line of Ritika Khandelwal from Perpetuity Ventures.

Ritika Khandelwal: Hi, I just wanted to add on the government part. Also what will be ...

Moderator: Ladies and gentlemen, the line for the management has been disconnected, please stay connected while we join them. Ladies and gentlemen, the line for the management has been connected.

Ritika Khandelwal, please proceed with your question.

Ritika Khandelwal: So what are your government receivable days? Just wanted to continue on the question of CGHS and ECHS?

Vivek Goyal: Yes. Generally, it is around 180 days, we are getting the money.

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Moderator: Ladies and gentlemen, that was the last question for today. I would now like to hand the conference over to Mr. Anurag Kalra for closing comments. Thank you, and over to you, sir.

Anurag Kalra: I think we had one more question that came in at the last minute. Is it possible to take that gentleman now?

Moderator: Sure, sir. The next question is from the line of Justin George, an Individual Investor.

Justin George: In our portfolio, we got around 33 hospitals. In these 33 hospitals, FMRI, Mohali and Ludhiana are greenfield hospitals. All other hospitals have been acquired, and now we are doing brownfield expansion. Do we have any plan for new greenfield hospitals?

Ashutosh Raghuvanshi: Yes. So we are open to doing greenfield hospitals as well. In our portfolio, Noida, Shalimar Bagh are also greenfield hospitals actually. They were not acquired. But we are open to doing Greenfields. And as and when some appropriate opportunity comes, we will certainly look at that.

Moderator: Ladies and gentlemen, that was the last question for today. I would now like to hand the conference over to Mr. Anurag Kalra for closing comments. Thank you, and over to you, sir.

Anurag Kalra: Thanks so much, ladies and gentlemen, for joining us on the call today. If there are any follow-up questions or information required, please reach out to my colleague, Amit or myself. We'll help you as best possible. Thank you very much again for your time, and have a good day.

Moderator: On behalf of Fortis Healthcare Limited, that concludes this conference. Thank you for joining us today, and you may now disconnect your lines.

Call: Feb 2026

FORTIS HEALTHCARE LIMITED

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February 20, 2026

FHL/SEC/202 5-26

The National Stock Exchange of India Ltd.

Scrip Symbol: FORTIS BSE Limited

Scrip Code:532843

Sub: Transcript of Investors / Analysts' meet under Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015

Dear Madam/Sir,

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed transcript of Investors / Analysts' meet held on February 16, 2026 to discuss the Company's Un -Audited Financial Results for the quarter and period ended on December 31 , 2025 and same is available on the Website of the Company at below hyperlink: -

[https://www.fortishealthcare.com/investor/investor%20presentations% 20&%20transcripts/earnings%20call%20transcript%20for%20the%20quarter%20ended%20december%202025](https://www.fortishealthcare.com/investor/investor%20presentations%20&%20transcripts/earnings%20call%20transcript%20for%20the%20quarter%20ended%20december%202025)

The date and time of occurrence of event is February 16, 2026 at 11:00 A.M.

This is for your information and records.

Thanking you,

Yours Sincerely ,

For Fortis Healthcare Limited

Satyendra Chauhan

Company Secretary & Compliance Officer

M. No. – A14783

Encl: as above

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"Fortis Healthcare Limited

Q3 FY '26 Earnings Conference Call "

February 16, 2026

MANAGEMENT : DR. ASHUTOSH RAGHUVANSHI – MANAGING
DIRECTOR AND CHIEF EXECUTIVE OFFICER – FORTIS

HEALTHCARE LIMITED
MR. VIVEK GOYAL – CHIEF FINANCIAL OFFICER –
FORTIS HEALTHCARE LIMITED
MR. ANURAG KALRA – HEAD OF INVESTOR
RELATIONS – FORTIS HEALTHCARE LIMITED
MR. AKSHAY TIWARI – CHIEF FINANCIAL OFFICER –
AGILUS DIAGNOSTICS LIMITED
MR. ANAND K – CHIEF EXECUTIVE OFFICER – AGILUS
DIAGNOSTICS LIMITED

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Moderator: Ladies and gentlemen, good day, and welcome to the Fortis Healthcare Limited Q3 FY '26 Earnings Conference Call. As a reminder, all participant lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during this conference call, please signal an operator by pressing star then zero on your touchtone phone. Please note that this conference is being recorded.

I would now like to hand the conference over to Mr. Anurag Kalra, the Head of Investor Relations for the opening remarks. Thank you, and over to you, sir.

Anurag Kalra: Thank you, Alaric. A very good morning, good afternoon, ladies and gentlemen. Welcome to Fortis Healthcare's Quarter 3 FY '26 Earnings Call. The call is being led by our MD and CEO, Dr. Ashutosh Raghuvanshi. With him, we have our Chief Financial Officer, Mr. Vivek Goyal. From Agilus Diagnostics, Mr. Anand, the CEO, joins us. Along with him, we have Mr. Akshay Tiwari, the CFO.

We will start with some opening comments by Dr. Raghuvanshi, post which Anand will take you through the highlights of the diagnostics business, and then we can open the floor for question and answers. Over to Dr. Raghuvanshi. Sir.

Ashutosh Raghuvanshi: Thank you, Anurag. Good morning, everyone, and thank you for taking the time to join us on our Q3 financial year '26 earnings call today. I wish you all a Happy New Year, and hope all of you are doing well. I shall dive right into the quarterly results and share my thoughts on the business performance and way forward.

Our business performance in Q3 has been good, considering the seasonal impact of festivals in some of our key geographies. Both our hospitals and diagnostic businesses witnessed a sustained growth momentum and delivered margin expansion compared to the corresponding quarter last year.

For the quarter, our consolidated revenues were at INR2,265 crores, registering a growth of 17.5% over the corresponding previous period. Our hospital business revenues grew 19.4% to INR1,938 crores while the diagnostics business net revenue reported a growth of 7.3% to INR327 crores.

Our consolidated operating EBITDA increased 34.8% to INR505 crores, which translates into a margin of 22.3% in quarter 3 financial year '26 versus 19.4% in quarter 3 of financial year '25. The hospital business operating EBITDA margins have improved from 20% in quarter 3 of '25 to 21.7% in quarter 3 of financial year '26 with an EBITDA of INR420 crores.

Our consolidated profit before tax before exceptional item for the quarter increased 21.9% to INR312 crores. Reported PAT stood at INR197 crores versus INR250 crores in quarter 3 of '25. The decline in PAT was primarily due to the one-off expense for the quarter INR55 crores pertaining to New Laboratory Codes, offset by reversal of impairment in an associated company amounting to INR9 crores. This resulted in net exceptional loss of INR46 crores.

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Coming to the 9-month performance of financial year '26, our consolidated revenue stood at INR6,763 crores, registering a growth of 17.1% versus 9 months of financial year '25. Operating EBITDA increased to INR1,553 crores, reflecting a robust 300 basis point improvement to 23% compared to 20% in the corresponding previous period.

Hospital business revenue for 9 months grew 19.1% to INR5,749 crores, while the operating margin improved from 20% to 22.2% in the corresponding previous period. On the balance sheet front, the company's net debt stands at INR2,547 crores with a net debt to EBITDA of 1.24x as on December 31, 2025.

The increase in debt was primarily due to the fund raised or to partly finance the acquisition of PE stake in Agilus Diagnostics by the company, acquisition of Fortis brand a

nd trademark and

acquisition of Shrimann Hospital Jal andhar.

I'm pleased to announce that we continue to progress well on cluster based inorganic growth strategy. To that effect, in January 2026, we acquired the 125 -bedded People Tree Hospital in Yeshwanthpur, Bengaluru for INR430 crores. This was done through a 100% acquisition of TMI Healthcare Limited Private Limited, along with the underlying land, building and an adjacent land parcel that enables future expansion to over 300 beds within the same location.

With growing awareness on mental health, in November 2025, the company launched Adayu, a 36-bedded specialized mental health care facility located in Gurugram. This facility offers evidence-based treatment through a multidisciplinary approach to deliver comprehensive, compassionate and world-class care.

Our bed expansion plans are progressing well. During the year so far, we have added approximately 750 operational beds, including the Bengaluru acquisition and the Adayu facility. The bed addition also accounts for our Jalandhar acquisition, Greater Noida lease facility and expansion in our existing facilities, including Manesar, Noida and Faridabad.

A bit of flavor on the hospital business. Our hospital occupancy in Q3 financial year '26 remains steady at 67% compared to the same period last year. However, the number of occupied beds increased 14% to 3,189 beds in Q3 from 2,790 beds in Q3 of financial year '25. Our hospital business recorded a 4.5% increase in ARPOB, reaching INR2.56 crores per annum. The growth in ARPOB was supported by increase in share of complex cases reflected in, for instance, a 52% year-on-year increase in robotic surgeries.

Just to provide all of you a perspective of the hospital business for 9 months of financial year '26, in 13 of our facilities, we have reported operating EBITDA above 20% during this period, and these 13 facilities together contribute 77% of the hospital revenues. In comparison, in '25, we had 10 of our facilities with operating EBITDA margin above 20% and contributing 73% of the hospital revenue.

Revenues from digital channel, the website, mobile application and digital campaigns witnessed a 19% year-on-year growth. Digital revenues contributed approximately 30% to overall hospital revenues in Q3 of financial year '26.

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Coming to the diagnostic business front, we continue to witness year-on-year improvement in top line and margins. Gross revenue grew 8.3% to INR371 crores from INR342 crores in Q3 of financial year '25. Operating EBITDA margin gross revenue stood at 23.1% versus 14.4% in Q3 of financial year '25.

On a like-for-like basis, excluding one-offs, the operating EBITDA margin stood at 21.3% in Q3 of financial year '25 as against 23.1% in Q3 of financial year '26. As part of our ongoing network expansion strategy, the total number of new customer touch points has reached 4,370 as of December 31, 2025.

The revenue contribution from preventive portfolio has increased to 12% in Q3 from 10% in Q3 of '25, while the contribution from specialized portfolio has increased to 35% from 33% in the corresponding previous quarter. We believe our diagnostic business is further positioned to scale its revenue while sustaining healthy margins.

With this, I conclude my remarks. Our business remains on track to maintain this growth trajectory. We continue to make strong progress across our strategic growth levers, which we expect will drive sustainable growth and further strengthen our position in the health care sector. With that, I hand over to Anand for his comments.

Anand K: Thank you, Dr. Raghuvanshi. Good morning, everyone, and thank you for joining us today. On behalf of Agilus Diagnostics, I'm pleased to welcome you all to our Q3 '26 results conference call.

During the quarter, Agilus reported gross revenues of INR371 crore

es compared to INR342 crores

in the Q3 of FY '25, reflecting an 8.3% year-on-year growth. Operating EBITDA stood at INR86 crores, a substantial improvement from INR49 crores last year, with margins at 23.1% compared to 14.4% in Q3 of '25.

We conducted 9.9 million tests during the quarter, up from 9.6 million in the same quarter last year, reflecting 3.6% volume growth. Our B2C-B2B mix stood at 52-48 indicating balanced traction across both the channels. We also strengthened our network through gross additions of over 175 customer touch points, further improving accessibility across markets.

For the 9 months period ending December '25, revenue stood at INR1,139 crores, up by 7.7% from INR1,058 crores last year, while operating EBITDA rose to INR275 crores from INR185 crores. EBITDA margins improved to 24.1% from 17.5% in the same period last year.

During the 9-month period, we conducted 30.7 million tests against 29.6 million tests last year, supported by gross additions of 550-plus customer touch points across our network. We also

expanded our network footprint meaningfully adding 8 stat labs and 9 HLMS, strengthening accessibility and coverage across both focus cities and emerging micro markets. Growth across geographies and product lines remains well distributed with routine specialized and wellness portfolios all contributing to the momentum. Our preventive health and wellness portfolio continues to perform well, contributing around 12% to the overall mix, supported by Fortis Healthcare Limited
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growing customer adoption of preventive panels and corporate wellness offerings. The segment remains an important long-term growth driver aided by the rising health awareness across the markets.

This quarter, we strengthened our test portfolio with the introduction of CNS biopsy with Reflex to IHC, enhancing our capability and capacity in neuro-oncology diagnostics. The RA extended panel offering a more comprehensive assessment for autoimmune disorders and the acute leukemia panel, which supports faster and more targeted classification of hematological malignancies.

Our continued investments in automation, digital tracking and workflow optimization have strengthened efficiency, turnaround times and customer experience alongside steady expansion in our specialized and genomics offerings.

Additionally, we also installed the Illumina NovaSeq X at our global reference laboratory in Mumbai, significantly scaling our NGS capabilities in genomics and proteomics. This enables high-throughput sequencing across oncology, infectious diseases, reproductive health and rare disease diagnostics.

Overall, the Q3 reflects a notable year-on-year improvement in both growth and profitability, supported by disciplined execution, network expansion and operational enhancements. We remain positive about the opportunities ahead and remain committed to delivering consistent high-quality growth.

Thank you, and over to Anurag.

Anurag Kalra: Thank you, Anand. Ladies and gentlemen, we shall now open the floor for questions. May I ask please request the moderator to begin.

Moderator: Thank you, sir. We will now begin with the question and answer session. Our first question comes from the line of Neha Manpuria from Bank of America.

Neha Manpuria: The first question is on the People Tree acquisition in Bangalore. How should we think about this facility because it's just a 125-bed facility? I think it's not fully ramped up now. So how should we think about 3 years in this facility and it moving to, let's say, mid -20s margin? And would it require additional investment to get to that mid -20% margin?

Ashutosh Raghuvanshi: Neha, so we have this facility, as Vivek had said earlier, is running sub-optimally at the moment. So it needs to be brought to Fortis standards. It would require some investment in that regard.

The investment

in the first phase is not very huge.

But at the same time, we are also going to expand and start the expansion work for creating further beds so that the capacity can be taken to 300 beds, and that would entail some capex, both in medical equipment as well as civil infrastructure. So over the next 3 to 4 years, we should see this as a high-end 300-bedded super speciality hospital with all the modalities of treatment available.

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Neha Manpuria: Dr. Raghuvanshi, the 300 beds that you're talking about, so currently, it's at 125 beds and then you have the adjacent land, which allows you to add more capacity. So essentially, we'll build out the new capacity in the adjacent land. Is that how we get to this 300 number?

Ashutosh Raghuvanshi: That's right, that's right. We will be starting the construction pretty soon.

Neha Manpuria: Okay. And in terms of doctors, et cetera, hiring doctors because Bangalore is another market, which we are seeing a fair bit of capacity coming on board, so how do you think we're positioned to get, let's say, a full-fledged doctor team for a multi-specialty hospital that we plan?

Ashutosh Raghuvanshi: I think we are very positive about it. I think Fortis carries a good brand, and it attracts physicians in all different regions, and I don't see any challenge in that regard. Our experience of another hospital, which we had opened in Nagarbhavi and ramping it up though that was also about 100-bedded hospital. And that is, by the way, currently doing EBITDA close to upwards of 25%. So we are pretty confident that we can attract good clinical talent in this micro market.

Neha Manpuria: Understood. My second question is on the expansion. If I were to strip out all the acquisitions we have done, including Noida, from the 750, I think the organic bed addition this year has been

closer to about 250 -odd beds, 250 beds, 300 beds. What would this number be next year? And other than the 200 beds in FMRI, what would be the other large expansions that we should look at in fiscal '27 -'28?

Vivek Goyal: Neha, Vivek this side. So, you are right. First 9 months, we could add 250 brownfield beds. There are some beds in pipeline for Noida and that will further add 50 beds, maybe probably, in this current financial year. And next year, we can target around 400 -plus bed in the brownfield, which majorly contributed by FMRI because expansion is almost steady. We are planning to commission by April.

Neha Manpuria: Understood. And again, sorry to harp on this doctor point, given that we have 2 other large hospitals that will be coming very close to where FMRI is, are we -- do we -- I mean is the doctor team fully equipped for this additional 200 beds that we will add? Or is that something we need to spruce up some specialty and you could see more doctor hiring for that?

Ashutosh Raghuvanshi: No, no. Doctor hiring is something which is an ongoing process and that keeps happening. But the current physicians, only we'll be able to utilize this capacity very effectively because occupancy rates of this hospital have been typically high around it has been operating at 80% occupancy level. So we are pretty confident that we have the clinical whereabout, and we have a very stable clinical team.

Moderator: The next question comes from the line of Tausif from BNP Paribas Exane Research.

Tausif Shaikh: First few questions on the O&M agreement with Gleneagles. Can you call out the O&M fees for this quarter? And how is the performance of Gleneagles for last 9 months? And also, where do you stand currently on the inclusion of Mumbai Hospital in the O&M agreement?

Vivek Goyal: So we are in the integration phase for O&M. As you know, this has been started this quarter only. So we have earned INR5 crores as O&M fee in the current quarter. And the revenue for Fortis Healthcare Limited

ended

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this quarter, excluding Bombay, is INR172 crores and EBITDA is almost 3% after absorbing 3% fees to us. So that means this will be around 6%. And that was in the mutual stage.

Tausif Shaikh: And what would have been the growth in 9 months?

Vivek Goyal: Sorry.

Tausif Shaikh: And what would have been the growth for 9 months of Gleneagles, revenue growth?

Vivek Goyal: Nine months growth is actually negative for the unit we are looking at. It is almost 4% negative. And there are a lot of disturbance. There is clinician attrition. Management team has also been changed. So all those actions has been taken. I think we will start seeing the result probably from the next financial year onward.

Tausif Shaikh: Okay. That's helpful. Second follow-up question on the recent acquisition in Bangalore. Can you tell us when do you plan to commercialize the adjacent land additional beds? Is it something one can look at the next 18 months? What's the plan for beyond '28, FY '28?

Vivek Goyal: So it will be like a sort of brownfield expansion like we do in our facility, which typically take 24 to 30 months' time. And Bangalore is a slightly difficult location, that's why I'm saying 30 months' time because approval generally takes more time. But we will start the work immediately on that.

Tausif Shaikh: So it's something beyond FY '28 one should look at it?

Vivek Goyal: Yes. But in the meantime, we will operationalize and bring operational efficiency in the existing beds.

Moderator: The next question comes from the line of Aman Goyal from Axis Securities.

Aman Goyal: Sir, my question is related to, since we are experiencing aggressive expansion in the NCR region, so to maintain our clinical talent in the region, are we seeing any higher MGs for these marquee hospitals or doctors? Or are there any revised revenue share over the last 12 months?

Vivek Goyal: So I will say this is an ongoing process that Dr. Raghuvanshi mentioned. This pull and push always remain, and we need to correct sometime the sales -- the remuneration and take out of the doctors. And that will be more than compensated by the revenue growth. As we know, the company is in the growth phase, so a little bit increase may not be affecting our margin that much.

Aman Goyal: Sir, my next question is related to the Manesar facility. If you could throw some color on the revenue occupancy trends and all.

Vivek Goyal: Yes. So Manesar is targeting INR15 crores per month revenue. And the EBITDA side, it has already started contributing positive EBITDA. So that way, Manesar is good story for us. And I think, from here onward, it will start contributing more. We are starting the -- we have already started the work for onco block, and that will further add to the profitability of this unit.

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Aman Goyal: Sir, the EBITDA margin is in single digit? Or if you could throw some on the EBITDA contribution in terms of percentage-wise, where is the facility for...

Vivek Goyal: Yes, it is very minimal. As I said, it is just breakeven. It has just started one year back.

Aman Goyal: My last question is related to the -- are you operating any facility above 80%, 85%, and we are still evaluating the opportunity for the brownfield expansion?

Vivek Goyal: Yes, sometimes our units hit 80%, like Shalimar Bagh is one example, which very often hits 80% occupancy. And we have plans for expansion. We have got all the approval now. And we will start work for the Shalimar Bagh immediately.

Moderator: The next question comes from the line of Sanjay Shah from KSA Securities.

Sanjay Shah: Congrats on great numbers. So my question was regarding, we see many new players are entering via asset-light or PE-backed model. So how we are prepared? How does Fortis differentiate itself? Is it in clinical depth or doctor's e

cosystem? How Fortis differentiates itself?

Ashutosh Raghuvanshi: So I think one of the major differentiation is in terms of the kind of infrastructure we're able to provide and there's a legacy of the institution, and there is an environment, which is very conducive for clinicians to be able to practice effectively.

And that's what probably makes us attractive and we have a lot of involvement of physicians in clinical governance and broad policymaking as well. So that makes them feel included, and that's what makes it attractive for them to be a part of us.

Sanjay Shah: So my second question was regarding our international patient revenue, which is growing steadily. And how scalable is this segment? And what investments is required to meaningfully grow in double digits from here?

Ashutosh Raghuvanshi: So I mean the percentage of revenue, this has been stable at about 8%, 9% and somewhere as total revenue percentage. It has remained in that level, and it is likely to remain in that level.

Because the international traffic is subject to so many other geopolitical situations, which are continuously evolving right now, so I think one cannot really bank on that. But what we are doing is that we are focusing on newer markets, especially the West and the East Africa and also a little bit, the Middle East and Central Asia. So those are the geographies which we are focusing on. We are doing direct marketing there and opening some information centers in these geographies to increase this business.

Moderator: The next question comes from the line of Karan Vora from Goldman Sachs.

Karan Vora: My first question is with respect to Gleneagles. So I think we highlighted the INR5 crore number we booked. But is that the full quarter's number? Or we were only able to book a part of it? So if you can clarify on that one?

Vivek Goyal: Yes, it is almost full quarter, Karan, because we have started this in July somewhere, mid-July, so it is almost full.

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Karan Vora: Got it. And it will be included in our ARPOB number as in this will help improve our ARPOB numbers as well, right, just from a calculation standpoint?

Vivek Goyal: Yes, yes, you're right.

Karan Vora: Okay. Got it. And for Gleneagles, like I think we've highlighted some of the challenges, but can you elaborate more on the challenges we are facing at Gleneagles at individual hospitals? And what are we trying to tackle those challenges going forward?

Vivek Goyal: So I will say there it's basically challenge for each facilities, and it is not common for all. So there are challenges but all hospitals looking like have good potential. For example, Chennai facility, there's a lot of potential, there is expansion plan, but I think execution is the problem. And last year was totally disturbed year.

And I think, as I said, next year onwards, it will start showing results. Similarly, Hyderabad, there are 2 facilities, one is Lakdikapul and the other is LB Nagar. And LB Nagar also, there is a lot of potential.

We are putting cancer facility there. Lakdikapul is having a space constraint, and we are trying to tackle that problem separately. And as regard to Bangalore, there are 2 facilities, Kengeri, and one small facility. And I think Kengeri is doing quite okay, but that other small hospital, a lot of work needs to be done.

Karan Vora: Got it. And like the same management team, which operates as a broader Fortis hospitals or which looks after the broader hospitals will be taking care of this or we have hired any regional or some second layer of management, which will look after this, to fix all the issues at Gleneagles?

Ashutosh Raghuvanshi: Yes. So this essentially falls into our larger structure. So of course, we will have to strengthen it at a regional level, as you said. So we have made some changes in some of the leadership at

hospit

al levels. At the same time, we have appointed 1 or 2 people on the regional level as well. But the Bangalore cluster, for example, reports to the Bangalore cluster head, who looks after the other Fortis hospitals. So similarly, we have positioned everything into our common structure of Fortis.

Karan Vora: Got it. And last question is with respect to FY '27 guidance. Now that we are almost close to end of FY '26 and we have pretty much meet or beat our guidance, so any qualitative color? Do you think a 20% kind of a top line growth is possible for FY '27? And can margins improve, say, 150 basis points more? Or any color around that will be helpful?

Vivek Goyal: So I will say, you have seen that the company is in the growth trajectory, and it is also showing improvement in the margin. We feel there is still scope for margin improvement, especially with the brownfield expansion that we have discussed in one of the questions on the next year. Brownfield Expansion and that brownfield is coming in one of our premier facility, FMRI. So we expect the -- you will continue to see the growth trajectory what we are seeing in the current financial year at least for 2 years.

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Karan Vora: Any number or range you can throw, if possible?

Vivek Goyal: It will be very difficult to give a specific number, you can understand.

Moderator: The next question comes from the line Vivek Agrawal from Citigroup Global Services.

Vivek Agrawal: So just wanted to understand, as far as the hospital business is concerned, if you can break down what is the growth in existing units and how much has been contributed from the new units?

Vivek Goyal: So the acquisition which we have done, the Jalandhar and the Greater Noida, that has contributed around 4% of the revenue growth. Balance is all from the existing units.

Vivek Agrawal: Understood. And how the margin trajectory looks like for the existing units? So overall margins have been expanded, let's say, around 170 basis points. But in the existing units, how the margins have panned out?

Vivek Goyal: So existing unit also, major benefit is coming from the existing unit for the margin expansion.

And so it is majorly from the existing unit because Greater Noida is the new unit, so it is hardly contributing to EBITDA. Actually, it is dragging the margin a little bit. And Jalandhar, of course, is 25% plus EBITDA margin.

Vivek Agrawal: So is it fair to assume that overall margin expansion in the existing units is much better than 170 bps?

Vivek Goyal: Yes. I've already told about Jalandhar. Jalandhar starts contributing as per our flagship hospital, and Greater Noida is on the improvement trajectory.

Vivek Agrawal: Understood. Sir, one question on institutional patients. So have you started seeing impact of better pricing in ECHS, CGHS, etc.? And how you see that trend in coming quarters?

Vivek Goyal: So we have started seeing the positive result from the CGHS particularly. ECHS, the circular is new, and there is still some doubts which need to be cleared, and team is working with the authorities to get it cleared. But until now, the number is quite positive.

Vivek Agrawal: Understood. Sir, one question on debt, right? Now after The People Tree acquisition, the debt is close to around INR3,000 crores, close to INR1.5 crores EBITDA. So now how to look at the company's ability to grow further inorganically? Are you flexible enough to take more debt, let's say, you can go to 2.5x, 3x of EBITDA? Or now onwards, you need to mainly depend on, whatever, for example, greenfield or brownfield plans that are in place?

Vivek Goyal: First of all, the debt EBITDA number is not that alarming. We have still room to take some more debt for our growth aspiration. Having said that, you might be aware that open offer is now settled. And IHH in the public forum has expressed their w

illingness to increase their stake in

the company. So in all probability, there will be equity infusion by IHH, and that will be used for either debt reduction or for growth opportunity.

Moderator: The next question comes from the line of Kartik Aggarwal, an Individual Investor.

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Kartik Aggarwal: You have reported a 14.3% increase in OBDs. I want to know what is the source of this growth? Is it coming from existing hospitals or new bed addition?

Vivek Goyal: So this is partly -- this is a mixture of both because when we say occupied bed increase, it is increase of bed in Jalandhar as well as Greater Noida because earlier, these hospitals were not there and a little bit increase in the Adayu also. And rest of the thing is in our existing facilities.

Kartik Aggarwal : So which of the existing facilities did you report this increase in?

Vivek Goyal: So almost all the facilities have shown good ramp -up, Shalimar Bagh, FMRI, all our flagship hospitals have shown good attraction. Except I will say BG Road, which is still struggling. And we are working on how to improve the occupancy level of BG Road. I will particularly like to mention about Mulund because earlier, we were having Mulund and BG Road in the same category, but Mulund had started operating at 65% plus occupancy level, which is quite increasing. So I think next we have to improve the occupancy level for the BG Road.

Moderator: The next question comes from the line of Madhav Marda from Fidelity Investments. Since there is no response on the participant, we will move to the next participant, that is Nikhil Goyal , an Individual Investor .

Nikhil Goyal : First of all, congratulations for a good set of numbers. My questions are two. One is on ARPOB. So ARPOB is currently INR2.65 crores. I want to understand what is our target ARPOB maybe 1, 2 years down the line? Are we expecting any increase in it? Or are we expecting it to remain flat going forward?

The second question is on the distribution of our revenue across the different EBITDA margin buckets that we have. So I have observed over a period of time that some hospitals are consistently under the less than 10% margin range. So are there any actions that we are planning there, either in terms of improvement or in terms of having them out of the system?

Vivek Goyal: So ARPOB increase, we are expecting around 4% to 5% ARPOB increase going forward also. This year, we are trending around this level. And I want to clarify that the ARPOB increase should not be confused with price increase.

So, 4.5% ARPOB increase, 2%, 2.5% is only price increase and balance is because of the case mix and majorly coming from the high end work we are doing, especially in the onco business, where apart from the IP business, a lot of daycare business comes.

And as a result of which, the revenue goes up, at the same time, beds remain constant and this results in the ARPOB increase. So we are expecting around 4% to 5% ARPOB increase going forward for the next 2 years.

As regard to distribution of EBITDA for the low EBITDA margin unit, I will say all these units are not very big units except Jaipur and Manesar. Manesar, we have discussed, is a new unit. I expect this unit to move up in the EBITDA margin later.

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As well as in Jaipur also, we are considering other plans for expansion. And with that, we feel that these units should also move up in the EBITDA margin trajectory. Other units are very small and these units are having their own challenges and very difficult to correct those challenges.

Nikhil Goyal : Thank you for the details. But just a follow -up question. So as you said, Jaipur is facing challenges and it is very difficult to correct those challenges. I understand Manesar has the potential to move up the ladder, but Jaipur

has been in that bucket for a very long time.

And why are we planning an expansion in a hospital where there are challenges, too? And historically, we haven't been able to move it also? Is it because you believe that once scale comes, the margins would improve? Or is there something else that we're trying to do there?

Vivek Goyal: So I will say it is a question of strengthening the clinical program, okay? So Jaipur is missing onco business very much. And that has resulted into the margin dip. Along with the onco business, there, a lot of business comes from the other specialty also. And it has space and it has all the things, which can absorb one onco block there in Jaipur.

So we were having the plan for earlier also. But we have put on hold because of the challenge Jaipur has faced last year and so. And now Jaipur has come back to the original plan, and we will evaluate again that plan.

Nikhil Goyal : And by when we will have onco also installed in Jaipur, if you have any time lines in mind?

Vivek Goyal: So these type of specialty takes some time. And generally, it is 18 to 24 months' time. So in the next year budget, we were planning to put something for Jaipur.

Nikhil Goyal : Understood, sir. Just one last question. On the ARPOB, you mentioned that half of it comes from price and half of it comes from product mix. Going forward also, we are expecting a similar trend, like half of it comes from price and half of it comes from product mix?

Vivek Goyal: Absolutely.

Moderator: The next question comes from the line of Shaleen from UBS Global.

Shaleen: Sir, I just want to check about you that capital is very much needed for our growth. So do we have any sense on the time line what could be the potential investment from IHH can come in?

Vivek Goyal: So as you know, they have just completed the open offer. There is a cooling period. Until then, they cannot go ahead with the further equity infusion. I think that cooling period end by May.

And this is their Board level discussion, then our Board level discussion on the equity need. As I said, currently, we are in a very comfortable position.

But from the management side, we have expressed that we want to create sufficient room in our balance sheet for future expansion. We feel we are in an idle position looking at current market dynamic to reap maximum benefit from this current market, so we will like to take that advantage. I think in 3 to 6 months' time, we will be having more clarity about the exact timing.

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Shaleen: Got it, sir. And I assume it will be the preference or warrant kind of a thing, right, given IHH wants to increase the stake and it's not going to be any other further dilution?

Vivek Goyal: Because earlier, there was a restriction for IHH -- it's a restriction, not a restriction in real sense for increasing their stake. With the open offer completed, that restriction has gone, is not more there. So as I said, it is a Board level decision. We have expressed our desire to have more equity in the balance sheet to create room for further growth. And I think in 3 to 6 months' time this will be done. You are right, in our estimate also, it will be preferential allotment.

Shaleen: Okay, sure. Lastly, sir, I know it's again maybe a little early, but any sense that what kind of a fund you would like to have it or be comfortable with, when you're looking for such kind of a raise?

Vivek Goyal: Very difficult to give a number. We would like to have at least 5% type of base for preferential and all, which is allowed also as per SEBI regulation. Again, this is our desire. How IHH will take, it is their decision.

Shaleen: But can I actually also think about doing creeping, creeping acquisition?

Vivek Goyal: At least, we have not heard from them during our discussion with them that they are looking for that option. They also want to grow Indian business. Fortis is doing quite well for them, and they want to grow the business. They have openly mentioned in many investor conference that Fortis is their growth engine for India. And I think they will like to increase price equity.

Shaleen: Congratulations on a good set of numbers.

Vivek Goyal: Thank you.

Moderator: The next question comes from the line of Nikhil Mathur from HDFC Mutual Fund.

Nikhil Mathur: I want to visit some 9-month numbers. So can you highlight the bed count addition exact in this 9 months? And where all have you added these beds?

Vivek Goyal: Yes. So bed count addition is 750 total. Out of that, as Neha earlier has asked, 500-plus bed is through acquisition. And 250 is through the brownfield expansion majorly coming from the Noida, Faridabad and Manesar facility. There is some more beds we are planning to open in Noida. The final finishing work is going on. I think that will be completed by this quarter.

And then we have, as I mentioned earlier, FMRI, which is our flagship hospital. We were planning to open that block in January. It has delayed a little bit because of all these environmental and other issues, but we are targeting to open those beds by April this year.

Nikhil Mathur: So can you split the occupancy between these new additions, including the acquisition and base business because occupancy has largely started 9 months at around 69% in 9 months versus a similar number last year? So I guess there's some drag on overall occupancy from these new additions. So what's that number be like?

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Vivek Goyal: So the occupancy drag is mainly coming from the Noida, Greater Noida facility as well as little bit from the Adayu because Adayu we have opened only in December. Jalandhar start contributing in the occupancy metrics quite well. But Greater Noida and Adayu are very small unit, it will not be having much impact. 50 basis point occupancy may have gone up if we exclude this new unit.

Nikhil Mathur: So of the total addition, 50 basis points, is the drag, right?

Vivek Goyal: Yes. And I've told you which 2 units are dragging it.

Nikhil Mathur: Okay. Got it. So if I refer to your Jan presentation of the bed count addition from FY '25 to FY '30, which gives a CAGR of around 10.7%, now FY '26 obviously, is a big addition year, which kind of brings you to somewhere around 6,800 beds, so the bed count addition from FY '26 to FY '30 goes down to almost 6%, 6.5% as per the current plan.

So my question is, are we at a risk of kind of growth slowing down because the bed count addition itself will be some 8%, 9% post FY '26 and your ARPOB expectation is 4%, 5%. So that gives me to 11%, 12%. Maybe the base occupancy moves up maybe a few percentage points.

So is there a growth slowdown you might have to revisit your M&A strategy a bit more aggressively?

Vivek Goyal: Yes, that is one thing which we are now targeting very aggressively. And we have done a couple

of acquisitions in the last quarter. We have a couple of others which are in pipeline. We can't disclose the details as of today. But yes, you are right, so there will be -- the growth will be coming from 3 buckets. One is the brownfield expansion.

Apart from what we have disclosed earlier, we have identified certain more, which will be coming in the next year investor presentation. Then we will also be having this acquisition thing.

And then there is some greenfield project also we are looking at.

Nikhil Mathur: But sir, sorry to be a bit cynical about this, there are so much expansion -- I mean every top corporate chain in this country is looking for bed expansion. So does that leave the current valuation environment conducive enough for you to keep doing M&A? Because the Bangalore one seems slightly on the higher side, if I look at on an EV per bed basis and with capex als

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needed for the further expansion there. So just some broad thoughts, are the valuations conducive enough?

Vivek Goyal: Yes, this is a good acquisition. I give you math. First of all, the location is fantastic, okay? This is Bangalore one. And there is a potential to reach up to 300 beds. And if you do 300 -bed greenfield expansion in that locality, it will minimum cost you INR1,000 crores, okay, with all the facility which we are planning.

And this acquisition will be costing us INR800 crores. So in the metro, in the heart of the city, with 300 -bed capacity, I don't think so it is expensive acquisition. And I think a little bit risk we have to take. And having said that, Fortis has now established itself in the cluster where they are present, and we are quite confident we can handle any competition.

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Moderator: The next question comes from the line of Neha Manpuria from Bank of America.

Neha Manpuria: You mentioned there are a few M&A in the pipeline. From a priority perspective, what markets would you be interested in for potential M&A, given a cluster -based approach? That's the first one. And second one, for the 400 -bed addition that we have outlined for the next 2 years, what would be the rough capex ?

Vivek Goyal: So the brownfield expansion for the next year mainly is coming from FMRI. Major portion is coming from FMRI and a little bit increase in the Manesar thing for which almost entire capex has already been incurred. A little bit medical equipment we have to acquire, so there is no much capex for the brownfield -- meaningful capex for the brownfield expansion for the beds which are coming up.

Of course, the new project which we are starting like Mohali, Shalimar Bagh, these 2 units which we have taken, those expansion we will be starting, there will be additional capex . That we will be able to tell you once the plan is finalized. What is the next one? What was the next question we have from you?

Neha Manpuria: I was just asking, you mentioned there's a few M&A in the pipeline that you're considering. Given we have this cluster -based approach, what are the markets that Fortis is looking at from a priority perspective to add assets?

Vivek Goyal: We are doing -- we are looking at the acquisition opportunity in the existing cluster itself. So we want to strengthen and deepen our presence in existing cluster. We feel we have a very strong brand equity in whichever cluster we are present -- we are there, and we want to further strengthen our position in those clusters.

Neha Manpuria: And this would include Hyderabad as well now with the Gleneagles O&M that we have?

Vivek Goyal: Yes. Hyderabad is now a sort of new cluster for us. We are slightly mindful for Hyderabad because of the intense competition there, but it is not ruled out. From the cluster approach, Hyderabad is definitely there.

Moderator: The next question comes from the line of Madhav Marda from Fidelity Group.

Madhav Marda: Sir, my question was on the CGHS and ECHS rate hikes, which have come in. Have you been able to quantify what kind of benefit we can get on an annualized basis, say, in FY '27?

Vivek Goyal: Yes, it is positive impact, Madhav, and it is not that meaningful in this quarter . As I said, there are -- if you have gone through that circular for CGHS, there is a super specialty hospital category for which the registration yet not have been opened. So all those things will be finalized maybe by next year.

Similarly, ECHS also, there is a lot of confusion about the drug pricing and things like that. And there, a lot of clarity team is asking. So I think let us wait for some time. This much I can tell you that there will be positive impact.

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Madhav Marda: Okay. Got it. And sir, my second question, I think you me

ntioned we've identified some more

brownfield expansion potential for Fortis. Could you give some more clarity in terms of which units are these where we've identified more brownfield expansion? And over what period of time this could come in for us?

Vivek Goyal: So we continuously look for brownfield expansion opportunity if there is any. Like last quarter, we have taken approval from the Board for opening an onco block in Faridabad unit. So similarly, these two units, which we have taken Jalandhar as well as TMI, there is a scope for further expansion. There is adjacent land parcel we have taken for expansion only.

And thirdly, the Greater Noida also, there is a potential to go up to 250 to 300 beds. Currently, it is operating at 150 beds. So we have just taken over in a quarter. So we are stabilizing the things. So all those things will be coming. Plus there is some opportunity in the existing units. We are finalizing the things. I don't want to name it because there is some approval and related also may be coming.

Moderator: The next question comes from the line of Atul Minocha, an Individual Investor .

Atul Minocha: My question is, if we are leveraging AI for improving operational efficiency? That is my first question. And second question is, with respect to recent budget announcement, right, government has mentioned to create regional medical hubs. So is there any work which you are doing as part of mental health or any other body to work with the government, and you can share some details around it. Those are my questions.

Vivek Goyal: So AI is a new theme, and health care, there are a lot of use cases for AI, and we are also working on some of the use cases. And we are taking help of our parent company, IHH. They are slightly more advanced in AI. So we are taking help from them, and I think you will see much more replication of AI, at least in our health care network. And as regard to engagement with the government, it is an ongoing process, and we keep engaged with the government and continue to work with them to provide quality health care to the masses.

Atul Minocha: But my question was more specific with respect to medical tourism coming from Europe and U.S., you know medical visa and things. So the government has mentioned that they will be creating 5 hubs during this project. So if you can share more details around that.

Vivek Goyal: I will say currently the country is not equipped to handle patients from those geographies because they have other options also.

Moderator: The next question comes from the line of Amar Ahir from Raedan Capital .

Amar Ahir: Sir, my question was that on a consolidated level, how many -- what's the number of beds you are going to add in FY '27?

Vivek Goyal: We mentioned about 430 -odd beds, excluding any existing we will be able to complete it.

Amar Ahir: Okay. 430 beds we will be adding, and how many will be operational?

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Vivek Goyal: I think almost all will be operational. FMRI, we are planning to operationalize in two phases. FMRI is a 200 -plus bed expansion. So we'll first open 100 beds. We are quite hopeful we will be able to fill it quite fast, and then we'll open the next batch of beds also. So FMRI is the only question mark that 100 beds may be operationalized this year or next year depending upon the ramp up. Other will be operationalized fully.

Moderator: Thank you. Ladies and gentlemen, that was the last question for today. I would now like to hand the conference over to Mr. Anurag Kalra for the closing remarks.

Anurag Kalra: Thank you, Alaric . Ladies and gentlemen, thank you for your time today. If there are further follow -up questions, data clarifications, please feel free to reach out to me or my colleague,

Amit, we'll be glad to help you. Thank you, and have a good day.

Moderator: Thank you, sir. Ladies and gentlemen, on

behalf of Fortis Health Limited, that concludes this

conference call. Thank you for joining us, and you may now disconnect your lines.

Call: May 2026

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May 29, 2026

FHL/SEC/202 6-27

The National Stock Exchange of India Ltd.

Scrip Symbol: FORTIS BSE Limited

Scrip Code:532843

Sub: Transcript of Investors / Analyst's meet under Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015

Dear Madam/Sir

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed transcript of Investors / Analysts' meet held on May 25, 2026 to discuss the Company's Audited Financial Results for the quarter and financial year ended on March 31, 2026 and same is available on the Website of the Company at below hyperlink: -

<https://www.fortishealthcare.com/investor/investor%20presentations%20&%20transcripts/earnings%20call%20transcript%20for%20the%20quarter%20and%20year%20ended%20march%202026>

The date and time of occurrence of event is May 25, 2026 at 11.00 A.M

This is for your information and record.

Thanking you,

Yours Sincerely,

For Fortis Healthcare Limited

Satyendra Chauhan

Company Secretary & Compliance Officer

ICSI Membership: A14783

Encl: a/a

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"Fortis Healthcare Limited "

Q4 FY26 Earnings Conference Call "

May 25 , 2026

MANAGEMENT : DR ASHUTOSH RAGHUVANSHI – MANAGING
DIRECTOR AND CHIEF EXECUTIVE OFFICER – FORTIS
HEALTHCARE LIMITED

MR. VIVEK GOYAL – CHIEF FINANCIAL OFFICER –
FORTIS HEALTHCARE LIMITED
MR. ANURAG KALRA – HEAD, INVESTOR RELATIONS –
FORTIS HEALTHCARE LIMITED
MR. ANAND K – MANAGING DIRECTOR AND CHIEF
EXECUTIVE OFFICER – AGILUS DIAGNOSTICS
MR. AKSHAY TIWARI – CHIEF FINANCIAL OFFICER –
AGILUS DIAGNOSTICS

Fortis Healthcare Limited
May 25 , 202 6

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Moderator: Ladies and gentlemen, good day, and welcome to Fortis Healthcare Limited Q4 FY26 Earnings Conference Call. As a reminder, all participant lines will be in the listen -only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touch -tone phone. Please note that this conference i s being recorded.

I now hand the conference over to Mr. Anurag Kalra, Head of Investor Relations. Thank you, and over to you, Mr. Kalra.

Anurag Kalra: Thank you, Renju. A very good morning and good afternoon, ladies and gentlemen, and thank you indeed for taking the time to join us on our quarter 4 FY26 and FY26 earnings call. The call is being chaired by our MD and CEO, Dr. Ashutosh Raghuvanshi. With him, we have our Chief Financial Officer, Mr. Vivek Goyal.

From Agilus, we have the CEO, Mr. Anand; and t he CFO, Mr. Akshay Tiwari. We will start the call with some opening remarks on the performance by Dr. Raghuvanshi, post which Anand will take you through certain key highlights of the Diagnostics business, and then we can open the floor for question and an swers. Over to Dr. Raghuvanshi.

Ashutosh Raghuvanshi: Thank you, Anurag. Good morning and good afternoon, everyone. Thank you for taking the time to join us on our Q4 financial year '26 and financial year '26 earnings call today. To begin with, I'm pleased to inform you that our Board has recommended a divide nd of INR1 per share, which is equivalent to 10% of the face value for the fourth consecutive year, subject to the approval of shareholders. This underscores the continued strength of the company's fundamentals and earnings trajectory.

We witnessed a stead y performance across both the Hospitals and the Diagnostic business, enabling us to conclude the financial year on a healthy note. Coming to the financial performance of the company, I shall comment on the year as a whole and then move on to quarter 4.

For the financial year 2026, consolidated revenues for the company stood at INR9,128 crores, a growth of 17.3% over the financial year '25. Our hospital business revenue have grown 19.1% to INR7,773 crores in financial year '26, while diagnostic business net revenues were at INR1,355 crores in financial year '26, a growth of 8%. Our consolidated operating EBITDA increased 31.3% to INR2,085 crores, which translates into a margin of 22.8% in financial year '26 versus 20.4% in financial year '25.

For the hospital business, operating EBITDA margins have improved from 20.5% in financial year '25 to 22.2% in financial year '26. The hospital business now contributes approximately 85% to our consolidated revenues and our consolidated EBITDA . Our consolidated profit aft er tax for the year increased 31.5% to INR1,064 crores in financial year '26 compared to INR809 crores in financial year '25.

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Now on the performance for the quarter. We reported a consolidated top line of INR2,365 crores, a growth of 17.8% over quarter 4 o f financial year '25. The Hospital business grew 19% to INR2,023 crores, while the Diagnostic business net revenue stood at INR341 crores in quarter 4 of financial year '26 compared to INR306 crores in quarter 4 of financial year '25. The consolidated operating EBITDA margins were at 22.5% versus 21.7% in Q4 of financial year '25.

Operating EBITDA for the Hospital business in Q4 financial year '26 grew to 19.9% INR446 crores with a margin of 22.1% compared to operating EBITDA of INR372 crores in quarter 4 of financial year '25. Our consolidated profit after tax for the quarter increased 44.2% to INR271 crores.

Coming to the balance sheet side, the company's net debt stands at INR2,334 crores with a net

debt-to-EBITDA of 1.09x

as on March 31, 2026, as against 0.93 on March 31, 2025. The increase in debt was primarily due to acquisition undertaken during the year, amongst other investments, which we made.

Let me now briefly touch upon the Hospital business. Our hospital occupancy in financial year '26 was 68% compared to 69% in financial year '25. However, the number of occupied beds increased by 15% from 3,270 beds in financial year '26, up to 2,838 beds in financial year '25. Our hospital business recorded a 3.4% increase in ARPOB, reaching INR2.51 crores per annum in financial year '26.

Key procedure volumes across focus specialties such as Radiation Therapy, Robotic Surgeries witnessed steady growth during the year, increasing by 19% and 66%, respectively. Revenue from focus specialties comprising Oncology, Neurosciences, Cardiac Sciences, Gastroenterology, Orthopedics and Renal Sciences grew 18.9% and contributed 62% to overall hospital business revenue.

Revenue from international patients grew by 18.5% during the year to reach INR639 crores, contributing 7.8% to overall hospital business revenue. In financial year '26, 13 of our facilities reported operating EBITDA margin above 20%, collectively contributing 76% of the hospital revenue.

In comparison, in financial year '25, we had 10 of our facilities with operating EBITDA margin above 20%, contributing 73% of the hospital revenues. This clearly reflects the growing strength of the business and the result of various initiatives and investments we have undertaken over the past few years.

The year gone by has also been marked by significant progress and strategic development across the company, including the addition of approximately 800 beds to our network through a combination of brownfield expansion and inorganic acquisitions. In January 2026, the company acquired a 125-bedded People Tree Hospital in Yeshwanthpur, Bengaluru, along with an adjacent land parcel, enabling future expansion to over 300 beds.

In September '25, the company strengthened its NCR presence with a long-term lease arrangement for a 200-bedded multi-specialty hospital in Greater Noida. In July 2025, the

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company consummated the acquisition of 228-bed at Shrimann Super specialty Hospital in Jalandhar, along with an adjacent land parcel, enabling future expansion to over 450 beds. As you can see, all these acquisition or arrangements are focused on our key growth clusters and will further enable us to leverage our existing strength and position in these geographies. Let me also briefly touch upon our focus emerging care segment. In November 2025, the company launched Adayu, a 36-bedded specialized mental health care facility in Gurugram.

We believe that mental health will increasingly demand more care and dedicated infrastructure distinct from the multi-specialty hospital space and hence, our foray through Adayu. During the year, the company also expanded capacity across its existing network by adding approximately 250 beds through brownfield expansion, primarily in Manesar, Noida and Faridabad.

Over the next 4 years, we plan to ramp up bed capacity further through brownfield expansion by adding around 1,800 beds. Of these in financial year '27, we expect to add capacity of more than 400 beds with a new tower at FMRI expected to be operationalized within weeks and balanced bed addition at Noida, Manesar and Amritsar as well as FHKI Kolkata coming during the year.

In financial year '26, the company further augmented its medical infrastructure by commissioning several high-end equipment, which included, amongst others, soft tissue surgical robot, MRI machines, cath labs and PET CT.

This reaffirms our commitment to offering advanced treatment options and delivering physician-based care across our network. To highlight our capital expenditure in financial year '26 stood at approximately INR70

0 crores, reflecting our confidence to further scale up operations, both in terms of capacity expansion and enhancement of medical infrastructure.

Turning to our Diagnostic business. We witnessed a steady improvement in top line and margins for financial year '26. Gross revenues stood at INR1,527 crores in financial year '26 compared to INR1,407 crores in financial year '25. Operating EBITDA margins, excluding one-offs, stood at 23.2% in financial year '26 as compared to 22% in financial year '25. For the quarter, gross revenue stood at INR387 crores compared to INR348 crores in quarter 4 of financial year '25. The operating EBITDA margin, excluding the one-offs, stood at 20.1% in quarter 4 of financial year '26 as against 23.4% in quarter 4 of financial year '25. I will let Anand take you through further details on the Diagnostic business. With this, I conclude my remarks.

Our business remains well positioned to sustain this momentum, supported by strategic

initiatives and investments that we believe will drive long-term growth and further strengthen our position in the health care sector. Thank you, and I will hand over to Mr. Anand for his comments now.

Anand K: Thank you, Dr. Raghuvanshi. Good morning, everyone, and thank you for joining us today. On behalf of Agilus Diagnostics, I'm pleased to welcome you all for our Q4 '26 and FY26 results conference call. During the quarter, Agilus reported gross revenues of INR387 crores compared to INR348 crores in Q4 of '25, reflecting 11.1% year -on-year growth.

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Operating EBITDA stood at INR85 crores in Q4 of FY26 compared to INR63 crores in the corresponding quarter last year, resulting in operating EBITDA margin of 22% in Q4 of FY26 versus 18% in Q4 of FY25. We processed around 10 million tests in Q4 of FY26 versus 9.6 million tests in Q4 of FY25, reflecting a 5% volume growth, supported by the gross additions of our 125 Customer touchpoints with the B2C to B2B ratio fixed at 53:47.

For the full year ending March '26, revenue stood at INR1,527 crores, up by 8.5% from INR1,407 crores last year, while operating EBITDA rose to INR360 crores in FY26 from INR249 crores in FY25. The operating EBITDA margins stood at 23.6% in FY26 compared to 17.7% in the FY25. During the year, we conducted 40.8 million tests versus 39.2 million tests last year.

The B2C to B2B revenue ratio remained stable at 52 to 48 in the FY26, reflecting balanced traction across both the channels. We expanded our network with gross additions of over 675 Customer touchpoints and gross addition of 20 -plus labor atories, including 10 hospital lab management laboratories, strengthening Agilus footprint and improving accessibility across focus cities as well as emerging micro markets.

Growth remained well spread across segments. We continue to see healthy growth in our preventive health and wellness portfolio, supported by increasing awareness and adoption of preventive panels and corporate wellness programs. This preventive health revenues contributed 13% to the overall mix in FY26 versus 11% during the last year, reinforcing its position as a key long-term growth driver.

During the year, we expanded our portfolio with 50 -plus new tests across oncology, molecular diagnostics, prenatal care, immunology, infectious diseases, gastroenterology and neuro -oncology, strengthening our advanced diagnostic capabilities. We also advanced our genomic capabilities with the operationalization of the Illumina NovaSeq X at our global reference laboratory in Mumbai.

During the quarter, we enabled routine clinical use of the platform. We also completed validations across key areas, including hereditary cancer panels, whole exome sequencing, carrier screening and genetic disorder panels. Our continued investments in automation, digital tracking and workflow optimization have enhanced efficiency, improved

turnaround times and

elevated the overall customer experience.

We rolled out geospatial phlebotomist assignment and strengthened nearby labs visibility to drive better organic traffic. We further enhanced the website with disease -wise test categories and a real -time loyalty points ledger, which while enabling automated follow -ups to improve card conversion.

Overall, the FY26 revenues and performance reflects a steady year -on-year improvement in both revenue and profitability, supported by disciplined execution, network expansion and operational enhancements. We remain committed to delivering consistent and high -quality growth. Thank you, and over to you. Anurag .

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Anurag Kalra: Thank you, Anand. Ladies and gentlemen, we shall now open the floor for question and answers, and I please request the moderator to begin.

Moderator: Thank you. We will now begin the question -and-answer session. The first question comes from the line of Neha Manpuria with Bank of America.

Neha Manpuria: Based on the annual data that we have given...

Anurag Kalra: Ms. Manpuria, sorry for interrupting. We cannot hear you. Can you speak a little louder?

Neha Manpuria: Yes, sorry about that. Is this better now?

Anurag Kalra: Yes, please go ahead.

Neha Manpuria: Okay. Sorry about that. So based on the hospital -wise information that we have given for the larger hospitals, it seems like we've seen occupancy dropping in a lot of our larger hospitals like

FMRI, BG Road, Faridabad, etcetera. I think, Faridabad, you had indicated that it's because of the expansion, but how should we think of occupancy improvement, particularly in the larger hospitals as we go ahead? That's my first question.

Vivek Goyal: Neha, Vivek this side. So occupancy drop in some of the hospital attributed towards the drop in the international business to some extent. If you see our international business growth in this quarter is around 11%, while for a year, it has grown around 18% to 20%. So that is the main reason for occupancy drop. Another reason for the occupancy drop in some of our hospital in North side, Punjab side is because of the medical onco drug capping that has also led to a drop in the revenue and occupancy to some extent.

Neha Manpuria: Vivek, how much impact would this discontinuation of onco treatment have on our revenue? Is there a number that we have? And in your view, do you think there is a possibility that this gets resolved and therefore, that business comes back?

Vivek Goyal: So Neha, most of the drop is coming where we have larger share of ECHS and CGHS business, mostly in Punjab region, okay, where the share of ECHS and CGHS business is higher. And as you know, the drug prices are capped at 30% to the MRP. So that has led to some of the drugs where we are not earning that type of margin, we have stopped taking those patients. And we have asked patient to take drugs from either CGHS stores or to procure from the market. And that has led to drop in some business. I don't have immediate number of the drop, but most of the drop is coming in that region.

Neha Manpuria: Understood. Okay. And my second question is, given that we are seeing new capacity come in, both in Gurgaon and in Noida also you've seen, any comment that you can make in terms of doctor attrition or doctor cost in these markets, how we should think about that, given these are both crucial markets for Fortis?

Ashutosh Raghuvanshi: Yes. So as far as, Neha, doctor costs are concerned, there has been a slight increase, as you would notice when you go through the details. But we do not expect anything further to happen
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because I think we have already taken the necessary steps in this direction, minimum number of movements we can expect in terms of senior clinicians, we are not expecting any major

changes.

As far as occupancy with the enhanced bed capacity in the region is concerned, I don't think it is a concern because currently, all these hospitals are operating at very high occupancy figures. FMRI, for instance, occupancy is almost close to 90% on some of the days, it is up to 95%, which is almost unmanageable. So luckily, we are having the new beds getting commissioned over the next week or so. So we don't expect that there will be any lag in absorption of this new capacity which we are creating. And we are fairly comfortable in terms of our clinician pool as well.

Neha Manpuria: Okay. And sorry, one last question, if I may, Dr. Raghuvanshi. What is your view on margins for next year in the hospital business? I know we've guided for 25% being the target by FY28, but how should we think about margin expansion given the fact that there is new capacity while brownfield that's coming in as well as this doctor dynamic that you've mentioned?

Ashutosh Raghuvanshi: Yes. No, I think we have been very much tracking on our plans and that guidance remains absolutely intact. If at all, we are more confident now that we can continue to deliver this progress every year, at least to 1.5% to 2% year-on-year.

Moderator: Next question comes from the line Shyam Srinivasan with Goldman Sachs.

Shyam Srinivasan: Just Vivek, just on the technical point on this discontinuation of the -- some of the CGHS business, right? Why were they getting counted in occupancies in the first place because they could be daycare, right, chemo business, I understand, but not necessarily occupancy, right?

Vivek Goyal: Right. No, sometimes what happened, some of the relative business also we lose, isn't it? We are not able to -- so that is having an impact, and that has resulted into some sort of occupancy pressure in some of the units. But most of the occupancy drop is attributed in the larger unit, which Neha has pointed out, like FMRI and all is because of the international business.

Shyam Srinivasan: Understood. And when do you think this gets resolved, this CGHS issue? Or how does this pan out now? What happens next?

Vivek Goyal: Very difficult, Shyam, to give any time line for this because government CGHS and ECHS has already issued a circular towards that. So industry has given their representation. I don't know how it will go forward. But having said that, we are moving on alternate business opportunities.

And as Dr. Raghuvanshi mentioned, most of our hospitals are operating at a very high occupancy level. And this drug price is having a negative impact on the revenue, but not on the margin to some extent because margin-wise, we are quite okay.

Shyam Srinivasan: Understood. Sir, that brings to my second question, just on overall guidance, Dr. Raghuvanshi, Vivek on fiscal '27 for hospitals and maybe for Anand on just on Diagnostics business as well.

Vivek Goyal: Yes. So like Dr. Raghuvanshi mentioned for hospital business, we can -- you are talking revenue -

Shyam Srinivasan: Yes, sir, both. All what yo u can give us?

Vivek Goyal: So revenue -wise, we are well poised to see on hospital side, 15% plus revenue growth. And EBITDA side, we expect another 150 basis point margin improvement for the current financial year. And for diagnostic, Anand, if you want t o...

Anand K: Shyam, for Diagnostics, as we had guided earlier, this year, we said that we'll be around 22% to 23% in terms of EBITDA. So we are there. So I think for the coming year as well, we'll be around 23% to 24% kind of EBITDA margins. We are making further investments into new centers and all this year. In terms of revenue, I think from where we were, we have improved quite well, and we have come to about 8.5% now. And we'll definitely improve and touch about double digits this year is what we are h oping.

Shyam Srinivasan: Sorry, if I c

an squeeze in last question, Anand. So many of your peers have talked about better volume development. Some of them have raised their volume guidance as well. So anything to kind of double -click on your revenue? How do es that double -digit split out into, say, mix and volume?

Anand K: So this year, we are seeing roughly about 5 -plus fee kind of volume -to-value relationship, so 5% growth in volume. So we expect that next year, it would be similar about 70% coming from volume and 30% coming from value.

Moderator: Next question comes from the line of Amey Chalke with JM Financial.

Amey Chalke: Congrats to the management on good numbers. So first question was on top 10 hospitals. First half of the question already answered on the larger hospital. But if you look at the smaller hospitals, which have done well, particularly FEHI and Jaipur, they have been doing well for last 2, 3 quarters. So what is going right here? And also, is this growth a profitable one? Or is it the margi ns remain in the low teens only for these hospitals?

Vivek Goyal: Yes. So FEHI has improved a lot. If you see, the EBITDA margin has improved, and we expect there is a further scope of improvement in the EBITDA margin in FEHI. Because of the operational ef ficiency and new facilities, which we are putting in FEHI and the new clinical talent team, which we have put there. As regard to Jaipur is concerned, Jaipur has also shown last year a very good progress. It is -- we expect the Jaipur to grow around 15% pl us next financial year with EBITDA margin in the range of 13%, 14%.

Amey Chalke: And the second question I have on the FMRI tower expansion. You mentioned in the opening remarks that the tower is likely to get opened in coming weeks. So do you expect the e ntire capacity to start in one go? Or -- and also, what is the current capacity utilization of FMRI?

Vivek Goyal: So FMRI, the capacity utilization is quite healthy. We are operating at around 85%, 90% occupancy level. We are actually facing bed shortage. We expect to operationalize this new tower in a week time. Our initial plan is to operationalize in a phased manner. We don't want to open all the beds immediately because it is having impact on the cost also. And as we move on the occupancy ramp -up curve, we will open the further beds. In the current financial year, our

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target -- our plan is to open around 100 beds. But if the occupancy ramps up fast, we can open all the beds also.

Amey Chalke: Considering once this entire tower is fully operationalized, F MRI is already operating at healthy margins. So is there a scope for further margin improvement on account of this brownfield expansion?

Vivek Goyal: Yes, yes, of course. As I mentioned in my earlier calls, this brownfield expansion, our experience is quite good. We're able to ramp up the capacity quite fast. And with the economy of scale, we're able to ramp up the margin also, which this we have seen in our Faridabad unit where we added 50 beds last year. We have also seen the same phenomena in our Noi da hospital, where we have added around 100 beds last year. So we expect a similar and rather better thing in FMRI.

Amey Chalke: Sure. Just last question, if I can squeeze in. On the Diagnostic business, the business has started doing well in terms of grow th as well as margins. Is there any thought on the demerger front on the diagnostic side and just keeping the Hospital business as a stand -alone business going ahead?

Vivek Goyal: Yes. Currently, our focus is more on improving the profitability margin. The business has started showing good result and focus is on getting full potential out of the business. So we'll continue to strengthen the business performance. And I think this is not the right time to do any sort of value locking at this point. And once t he business mature and start performing as per the

market

and as per competition, then I think we can look for some alternatives.

Moderator: Next question comes from the line of Tausif with BNP Paribas Exane Research.

Tausif Shaikh: On your BG Road this year, we have seen the occupancy levels dipping to 55% while earlier you highlighted that some of the occupancy dip is mainly because of international patients and onco capping. But I guess Bangalore market does not have these dynamics. Any color on the same would be helpful on the Bangalore market and BG Road?

Vivek Goyal: So BG Road is one of the unit on our target where the occupancy level is low. And there, we have expansion in pipeline, brownfield expansion in pipeline. So, this unit, the occupancy level is low because, one, we are not taking a lot of government business here. So it is more of the TPA and cash business.

Secondly, there is an intense competition around that region. And in that region, generally, the occupancy level remained low, around 63%, 64%, but we are at 55%. So our immediate endeavor is to bring it above 60% level and operationalize on the enhanced base and then look forward to increasing the occupancy level there. So I think currently, on occupancy front, this unit is performing suboptimal, and this is on our radar for the improvement.

Tausif Shaikh: Okay. That's helpful. Can you provide us the revenue contribution from 800 new beds we have added this year, a breakup between acquired asset and brownfield expansion, that would be helpful?

Vivek Goyal: Yes. So, if we talk about out for the current quarter, if the revenue -- reported revenue for the hospital business is INR2,000 crores and for the net revenue if we take out the new units, which

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is the TMI People Tree basically, Adayu and Jalandhar, then it will be INR1,928 crores. So there is very little revenue contribution actually.

Tausif Shaikh: Also, last one, if I can ask, what is the revenue and EBITDA of Gleneagles for the full year?

Vivek Goyal: Gleneagles. So Gleneagles, we don't have full number right now. So they are trending around INR70 per month. Yes, we don't have number for Gleneagles right now because it is not getting consolidated with us. So we don't track that way.

Moderator: Next question comes from the line of Damayanti Kerai with HSBC.

Damayanti Kerai: My first question is on your CGHS and International Patient business, where some of the issues have created some turbulence. So you mentioned about working on some risk mitigation strategy. And in the CGHS part, I just wanted to understand while we don't know whether the onco price cap will be removed or not. But considering the price increase happening in that part of the portfolio, how do you see this business moving in, say, '27-'28 in terms of growth?

Vivek Goyal: Yes. So first on the onco side, as I mentioned, there is no clarity, first of all, with us. However, CGHS and ECHS has gone ahead in increasing the prices for almost all the specialties. The full impact of that has still not come because the super specialty category, which they have created, the registration is in progress, and there is a lot of clarity which is needed on the billing side. Having said so, this is an important payer for us, and we are in the growth phase.

So we want to explore maximum out of this business. As regard international business is concerned, we are tracking well. We are broadening our market base. We are concentrating more on Africa and other countries to mitigate this international business dip, which we have witnessed in the last quarter. Having said that, the domestic business growth has more or less taken care of what we have lost in the international business.

Damayanti Kerai: Okay. So in international business, still say travel situation, etcetera, normalize, maybe we will see similar trends. And at the same time, the domestic growths

should be taken care of any loss happening on the international business part?

Vivek Goyal: Yes, yes. This is what we are seeing currently.

Damayanti Kerai: Okay. My second question is, earlier you discussed about your parent, IHH increasing stake in the company, etcetera, or investing more. So any update or any time line to look there?

Vivek Goyal: Yes. So recently, the group CEO of IHH has given an interview where he has mentioned that there is a plan to increase their stake to 50%, and they have also mentioned some amount around INR10,000 crores fresh equity infusion in the company for meeting the growth aspirations. So whatever we have -- we are hearing from IHH, I think India is a focused market for them, and they want to increase their footprint in India through Fortis only. And that should -- that is the plan. And I think as and when we need the capital, they are with us, and that will be happening as we need the growth capital.

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Damayanti Ke rai: Okay. Good to hear. But we don't have any specific timeline, right, when these investments will come in?

Vivek Goyal: No. As of today, there is no specific time line I can give you, but it will happen over a period of time as and when the companies need the capital.

Moderator: Next question comes from the line of Tushar Manudhane with Motilal Oswal Financial Services Limited.

Tushar Manudhane: Sir, firstly, on the number of doctors that got added in FY26 on the organic basis, if I exclude the acquisitions on-led addition of doctors, if you could share the number?

Ashutosh Raghuvanshi: I don't think, Tushar, we have that number offhand. This is an ongoing thing. We don't really track it that way. But I don't have that number.

Tushar Manudhane: No issue. We can take it offline.

Ashutosh Raghuvanshi: Yes, we can take it offline...

Tushar Manudhane: Sure. Sir, secondly, for your guidance for the revenue growth in FY27, you've guided for -- the growth is all organic basis? Or are you including certain acquisitions which you think are sort of close to completion?

Vivek Goyal: No, that guidance is for ongoing basis. So we have not considered any acquisition there, and the acquisition will be over and above that.

Tushar Manudhane: Understood. And just to a little dig deeper into EBITDA margin guidance. As I see the hospitals which are 20% plus sort of a margin are already running at 70 -plus sort of an occupancy. And probably addition of beds will definitely drive the EBITDA given the strong scale -up of the additional beds. But purely from a margin point of view, just wanted to understand what will drive the EBITDA margin -- in while we have very less number of hospitals into the category of, say, less than 15% EBITDA margin.

Vivek Goyal: Sure. So I think it is a very important question to deliberate on. You rightly said that some of the hospitals are already operating at a decent occupancy level, but there are hospitals, large hospitals, which are not operating at that level. Like we have discussed about BG Road, Mulund is operating at around 65% plus and that can also be ramped up to 70% type of occupancy. So there is a scope in the existing big hospitals where we can increase the occupancy. Second lever is some of the new units, in where we were incurring losses in the last financial year, like Manesar, like Greater Noida and Ludhiana too. The units have started contributing positively. And looking at the current trend, we feel that these units will start handsomely in the EBITDA margin side. So these units which are on the lower part of the pyramid, these units will move up further. So that is another lever which we are looking for. And thirdly, as you rightly mentioned about the brownfield expansion, which we have done, that will lead to margin improvement. And last but not the least, the acquisitions which we have done last year.

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That process of integration was on. And as we complete that process, we hope those units will start -- also start contributing in the overall pie of the revenue as well as EBITDA. So all put together, along with our focus on optimizing the cost and optimize our operations, I think we are quite confident the guidance which Dr.Raghuvanshi just has given will be met.

Tushar Manudhane: Got it. And just lastly to understand or maybe I missed in the initial remarks or the earlier conversation in terms of time line for this Gleneagles transaction?

Vivek Goyal: Yes. So we -- actually, we can't give time lines for this. This is under deliberation. We are -- we will come back to you people because if this happens, it will be required majority of minority approval. So we will come back to you as and when we are ready with the proposal. It is under deliberation and evaluation stage right now.

Moderator: The next question comes from the line of Aman Goyal with IIFL Capital.

Aman Goyal: Congratulations on a great set of numbers. Sir, my first question is related to the Manesar facility. Can you please throw some color on what is the revenue run rate and the KPIs for Manesar? And is there any effect on the FMRI units from Manesar facility?

Vivek Goyal: Not really. Manesar is doing quite well. In terms of revenue, it is exceeding our expectation also, whatever number we have taken over this unit. So revenue -wise, it is doing quite well. And it is having a rather positive impact on the FMRI unit because doctor has got a larger base to operate with.

And it is having perfect synergy with our FMRI unit. And as I mentioned earlier, last year, there was an EBITDA loss over this unit because it is a new unit for us. It has already breakeven. It has started contributing to the EBITDA, and we hope that this unit should do well in the current financial year for us.

Aman Goyal: So how much revenue did Manesar reported in FY26?

Vivek Goyal: Yes, INR140 crores is the revenue number for last financial year for Manesar.

Aman Goyal: And the occupancy, occupancy -wise?

Vivek Goyal: It is around 48%, but it is quite deceptive because during the year, we have opened up new beds also. So we are opening beds as we are moving on the occupancy return.

Aman Goyal: And sir, my question is on the flagship unit. Since we have witnessed some occupancy dip due to the international business and all, but there is a huge ARPOB growth for FMRI, FEHI. What is the driver for this ARPOB growth? Year -on-year basis, I'm saying, let's say, in FMRI, we have reported INR4.7 crores ARPOB versus INR4.2, and it's a significant jump. Similarly in FEHI, almost INR3 crores from INR2.4 crores...

Vivek Goyal: Yes. One reason for FMRI ARPOB increase is the international business itself because if you see in a year, we have grown by 20%. And international business, this ARPOB is on the gross basis. So international business generally includes markup and that lead to the higher ARPOB when we see at the gross level. So that is one reason for the ARPOB increase in this business.

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Plus we have added a lot of new technology in this unit like MR Linac was started in this unit. Gamma Knife was started. This unit is having very good onco business. And with the onco business, there is a lot of day care business that comes. And all these factors lead to ARPOB increase in this hospital. The last quarter only, there was dip in the international business but when you see year -on-year number, it is looking all right.

Aman Goyal: And sir, my next question is on the Bangalore cluster . Since you mentioned in this call, the Bangalore has a very high competition market. And what is the driver to introduce new hospital like we recently added People Tree. There -- I think margin is under 10% for that

hospital. So

what is the -- I mean, the main motivation behind adding new hospital in a market where the competition is already at an intense level?

Ashutosh Raghuvanshi: Yes. So there are micro markets within any large city, and this is a new micro market. And also at the same time, this micro market is not as highly competitive as the one in BG Road. BG Road, our boundary wall is shared by another large major hospital of the city. So whereas here, there is a large drain age area, which is grossly underserved at the moment.

So we believe that there is a huge opportunity here. This area is also seeing a lot of large residential developments of IIM coming as well. So this is a more a futuristic opportunity, we believe. And in a city like Bangalore, multiple positioning is very important because if you don't have a cluster and you don't have that kind of presence and then you are not in the competition. So, you have to have a network in order to be a significant player over there.

Aman Goyal: Okay. Understood. Sir, my last question is on the bed expansion plan. So we have moved some beds from FY28 to '29 now. So any particular reason? And what is the capex guidance for FY27 to '29... ?

Vivek Goyal: Yes. So only FMRI unit has been moved to the '27 financial year. It was supposed to be starting in the last quarter of the last financial year. So we are moving quite well in our brownfield expansion program. Next year, we are targeting around 500 -plus bed expansion, commissioning of 500-plus beds on the brownfield side. As regard the capex guidance, we expect to incur around INR900 crores annually on the capex , which may include 60% towards maintenance capex and balance is the growth capex .

Aman Goyal: Okay. Sir, my question was basically in the January PPT, we have dedicated 465 beds for FY28. Now in the latest PPT, we are guiding 173 beds. So -- so that 300 bed deviation is largely now in FY29. So which facilities are the -- I mean, the defer of 300 beds?

Vivek Goyal: It's mainly Shalimar Bagh, if I can mention. One is the Shalimar Bagh, which is the big extension, where we are expecting some approval related issues to be sorted out. And once it is sorted out, we will move fast on this expansion. So Shalimar Bagh, definitely, there is a delay in the expansion.

Moderator: Next question comes from the line of Sanjay Shah with KSA Shares and Securities Private Limited.

Sanjay Shah: My question was regarding Indian...

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Moderator: Sorry for interrupting, can you speak a little louder?

Sanjay Shah: Sure. Sir, Indian hospital sector is seeing aggressive expansion by large players. How do management view increasing PE -backed competition and asset -light model? And how it differentiates Fortis most today among others?

Ashutosh Raghuvanshi: Yes. I think, Sanjay, the main differentiator is going to be the clinical quality outcome, patient satisfaction. And we continue to remain focused on it. As far as the PE -backed platforms are

concerned, I think that's a future opportunity for the legacy players who are the kind of strategic in the market. And this would give future opportunities for us. There are concerns about the valuations, et cetera, being driven by these transactions. But that, I think, will be something which market will take care of over a period of time.

Sanjay Shah: That's great. Sir, what is the normal maturation cycle for a hospital to move from loss-making to EBITDA to mature 20% plus EBITDA...

Ashutosh Raghuvanshi: Yes. So typically, the cash breakeven happens at 18 to 24 months. And then after that, it takes about maybe a year to get to about 18%, 20% EBITDA.

Sanjay Shah: So that is for brownfield or greenfield?

Ashutosh Raghuvanshi: No, that's for greenfield.

Sanjay Shah: Yes. And about brownfield, sir?

Ashutosh Raghuvanshi: Brownfield will depend on the already

existing operations. So for example, in most of our brownfield, the capacity absorption was immediate. So because that is just a hospital becoming larger. So it drives in economy of scale, then the profitability of the hospital becomes better immediately.

Moderator: Next question comes from the line of Jestin George, an Individual Investor.

Jestin George: Our major brownfield expansion like Towers, FMRI, Noida and Shalimar Bagh. How many beds adding in BG Road and in Mohali? Or is there any other new to -- new brownfield expansion coming in any other hospitals?

Ashutosh Raghuvanshi: Yes. So Mr. George, we are having new towers coming up in Mohali and Shalimar Bagh, which will take about 2.5, 3 years near about. These are large expansions and also in Amritsar. So these 3 large ones are going to come.

There is some brownfield capacity in Bangalore, which we are yet to commission. But as discussed earlier, the occupancy levels currently are lower. So we are waiting for that to improve before we add more capacity. We have about 100 beds more over there. Mulund, we have already built a new capacity a few years back, and that is well absorbed now.

We are currently at about 65% occupancy levels over there. So there is a possibility of adding another tower in Mulund facility, which we are in the process of planning. So these are some of the areas where we are looking for to expand further.

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Jestin George: The BG Road brownfield expansion, new tower or the same building?

Ashutosh Raghuvanshi: It's a brownfield in the same building, yes. The building is already constructed. Once the occupancy numbers improve, we will commission...

Moderator: Next question comes from the line of Atul Minocha, an Individual Investor.

Atul Minocha: And my question is more towards Dr. Raghuvanshi, the strategy like we will be moving towards the preventative health care in the future. And what I have in my individual capacity observed is like there are a lot of online players who are going in that category and kind of if I say it, they're kind of eating their lunch. So like there are many players who are doing online consultation and everything.

So what is going to be your strategy to be available at the places where your customer is while I do understand you have -- we have our own digital platform, -- my app, but the kind of presence those other platforms have and the way we are capturing, I see the patient base is quite getting captured. So thought on that side?

Ashutosh Raghuvanshi: Yes. So you are right that there are a lot of platforms which are providing this primary consultations. But those primary consultations don't really always lead necessarily into hospitalization and the channels are not well formed. We do understand the importance of that space. But our whole operation is primarily focused on tertiary quaternary and critical care. So with those facilities will always be provided in hospital. So this doesn't form a very large segment of patients, but it is very important for the patient. We understand that.

So we will gradually develop our digital presence with our app, which you're already aware of. We will enhance those offerings and increase our presence in the primary space. But that is not a focus area right now because it is a business where essentially initially, there is a lot of cash burn if you focus that on -- and if you compare to whatever the examples you are giving. So we will be very mindful of this and gradually keep on enhancing our digital presence through our app and other means.

Atul Minocha: My question is, if I add to it, like are there opportunities to kind of collaborate with these platforms on the preventive side and also to kind of explore the opportunity coming out of these engagements because ultimately, when you are available at a customer touch point, based on those inte

reactions, there might be conversions which might ultimately drive some patient revenue towards the tertiary health care which might be required.

Because if you're not present on the customer's side itself, these routes where they're happening, the customer routing that are happening, you might not be aware in those situations. And what my experience has been, my personal experience is we are kind of little bit lagging on the IT side of the things, platform, app usage and other aspects also in patient journey. So that whole digital experience and capturing converting from preventive to tertiary and those dimensions, I think are important.

Ashutosh Raghuvanshi: Yes. I already said that we do realize the importance of that space, but collaborating with these platforms is not an option we will consider. We have had experiences earlier on. We believe

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that, that is dilutive to our presence. And so we are not at the moment looking to do -- go on that direction. But we do understand the importance of this, and we will continue to develop our platform, which requires a lot of improvement.

Moderator: Thank you. Ladies and gentlemen, due to time constraints, we have reached the end of question-and-answer session. I now hand the conference over to Anurag Kalra for closing comments.

Anurag Kalra: Thank you, Renju. Ladies and gentlemen, thank you for your time today. If there are any follow-up queries or clarifications, me and my colleague, Amit are available. Please do feel free to reach out to us over phone or email. Thank you very much, again, and have a good day.

Moderator: Thank you. On behalf of Fortis Healthcare Limited, that concludes this conference. Thank you for joining us. You may now disconnect your lines.